VHM- Valley Hospital Medical Center
620 Shadow Lane
Las Vegas, NV 89106-4194
(702) 388-4000

Patient Information

Patient Name: VU, NANCY
Home Address: 6750 DEL REY AVE UNIT 124
LAS VEGAS, NV 891461058
Mobile: (510) 449-6994
Employer Name: Retired
Employer Phone:
Retire Date: 07/01/2000

Sex: Female **Language:** Vietnamese
DOB: 07/12/1935 **Race:** Asian
Age: 90 Years **SSN:**
Religion: None
Marital Status: Widowed
Contact: Phone Calls and Visitor
EMail: No Email Address

Guarantor Information

Guarantor Name: VU, NANCY
Patient's Reltn: Self
Billing Address: 6750 DEL REY AVE UNIT 124
LAS VEGAS, NV 891461058
Billing Phone: (510) 449-6994

Sex: Female **DOB:** 07/12/1935
Employer Name: Retired
Employer Phone:

Contact Information

Emergency Contact
Contact Name: KHANH DANG
Patient's Reltn: Mother
Sex:
Preferred Phone: (669) 312-8595

Next of Kin
Contact Name:
Patient's Reltn:
Sex:
Preferred Phone:

Primary Insurance

Subscriber Name: VU, NANCY
Patient's Reltn: Self
Sex: Female
DOB: 07/11/1935

Employer Name: Retired
Employer Phone:
Financial Class: Medicare
Group Name:

Insurance Name: Medicare Part A Only - VHS
Claim Address: MEDICARE PART A
MECHANICSBURG, PA 17055
Insurance Phone: (877) 235-8073
Policy Number: 9U55KH1VF40
Group Number:
Authorization Number:
Authorization Phone:
Auth Req:
Authorization Contact:

Secondary Insurance

Subscriber Name: VU, NANCY
Patient's Reltn: Self
Sex: Female
DOB: 07/11/1935

Employer Name: Retired
Employer Phone:
Financial Class: Medicaid
Group Name:

Insurance Name: Medicaid Secondary to Medicare -
Claim Address: MEDICAID 2ND MCARE
Reno, NV 89520
Insurance Phone: (877) 638-3472
Policy Number: 00002923815
Group Number:
Authorization Number:
Authorization Phone:
Auth Req:
Authorization Contact:

Encounter Information

Reg Dt/Tm: 03/30/2026 07:06 PDT
Est Dt of Arrival:
Inpt Adm Dt/Tm: 03/30/2026 09:37 PDT
Disch Dt/Tm:
Observ Dt/Tm:
VIP Indicator:
Admit Reason: Hypoxemia
Accident:
Accident Dt/Tm:

Patient Type: Inpatient
Medical Service: Intermediate
Location: VHM MICU
Room/Bed: 016 / 01
Est Responsibility: 0.00

Admit Type: Emergency
Admit Source: Emergency Department
Reg Clerk: Stasiewski RN, Sabryna
Admitting: Delmundo DO, Lloyd
Attending Techathaveewat DO, Worawith
PCP: Wu MD, Julie U
Referring: No Referring Physician
Last Visit Date: 03/30/2026

VU, NANCY
Female / 90 Years

MEDICAL RECORD

EL0012

Patient Identification

1101330304-63892637

VU, NANCY
07/12/1935 90 Ye F
ADM/REG DT: 03/30/2026 09:37

CCPS_REG_XR_FACESHEET Printed By: USER, XR DEFAULT on 04/01/2026 16:04to printer cr_facesheet_12002
Registration last updated by: Contreras, Theresa on 03/31/2026 15:10

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female

Admit: 3/30/2026
Disch: Disch Time:
FIN: VHM1101330304
Readmit Risk: 39 - LOW 03/31/26 05:45
Attending: Techathaveewat DO,Worawith

History and Physical

DOCUMENT NAME: History & Physical
SERVICE DATE/TIME: 3/30/2026 13:20 PDT
RESULT STATUS: Auth (Verified)
PERFORM INFORMATION: Hirabayashi DO,Ellyn (3/30/2026 13:20 PDT)
SIGN INFORMATION: Techathaveewat DO,Worawith (3/30/2026 13:28 PDT); Hirabayashi DO,Ellyn (3/30/2026 13:20 PDT)

Chief Complaint/Reason for Consultation

Patient brought in by EMS from home with complaints of SOB. Per the family, patient started to complain of SOB this morning. Patient found with a RA sat of <50%. Patient provided with BVM ventilation and then placed on NRB. Patient A/Ox0. EMS unable to obtain demographics on scene. BGL 119

History of Present Illness

90 years old minimally speaking female with PMH of ESRD on HD MWF, on home oxygen 2 to 5 L, hypertension, dementia, bedbound was brought by EMS from home with complaints of shortness of breath started overnight. History was limited patient was on continuous BiPAP and addition to language barrier with hard of hearing therefore history was gathered through family members including patient's daughter and brother who lives with patient. Patient complained of shortness of breath started overnight with mucus production, cough, generalized weakness and family called EMS due to worsening symptom. Reports compliance with dialysis session with last dialysis session on last Friday and that supposed to get dialysis today. Patient was placed on nonrebreather and transition to continuous BiPAP in ED. Labs shows hyper-K potassium 5.9 and 6.9 on ABG therefore receive hyper-K protocol including calcium, albuterol, insulin. ABG showed 7.5/39/153/34. Patient was able to follow simple commands with family and denies chest pain.

Past Medical History: ESRD on HD MWF, oxygen dependent home oxygen 2 to 5 L, hypertension, dementia
Past Surgical History: Breast implant
Social History: Denies tobacco, alcohol, and drug use. Lives with brother, mostly bedbound unable to ambulate by herself
Family History: Noncontributory
Allergies: NKDA
Medications: Hydralazine 50 mg 3 times daily, losartan 100 mg daily, metoprolol 50 mg ER, sevelamer 800 mg 3 times daily, amlodipine 10 mg, there is prednisone 10 mg last filled March 5 however her family denies taking prednisone

Objective

Measurements (most recent)

Height (cm)	157.48 cm	Stasiewski RN, Sabryna	03/30 07:08
Height (inches)	62.00 inch	SYSTEM	03/30 07:08
Weight	40.82 kg	Stasiewski RN, Sabryna	03/30 07:08
Weight (lbs)	89.99 lb	SYSTEM	03/30 07:08
Body Mass Index	16.46 kg/m2	Stasiewski RN, Sabryna	03/30 07:08

Histories

Allergies

Allergies (Active and Proposed Allergies Only)
No Known Allergies (Severity: Unknown severity, Onset: Unknown)

Past Medical History

Active Problems(3)
Hemodialysis
Hepatitis A virus
High cholesterol

Past Surgical History

No surgery history documented.

Social History

No social history documented.

Medications

Home Medications

No medications documented.

Inpatient Medications

Medications (21) Active
Scheduled: (10)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 10 mg Tab (amLODIPine) 10 mg 1 Tabs, Oral, Daily
cefTRIAXone/D5W (Rocephin) 50 mL, IV Piggyback, q24H Interval
doxycycline/D5W (doxycycline) 100 mg 100

History and Physical

Vital Signs (24 hrs)	Last Charted		Minimum		Maximum	
Temp	36.9	03/30/2026 07:11	36.9	03/30/2026 07:11	36.9	03/30/2026 07:11
Heart Rate Monitored	91	03/30/2026 12:20	84	03/30/2026 09:00	**H** 104	03/30/2026 07:30
Resp Rate	18	03/30/2026 12:20	12	03/30/2026 07:08	**H** 26	03/30/2026 07:38
SBP	**H** 179	03/30/2026 12:20	130	03/30/2026 07:08	**H** 179	03/30/2026 10:00
DBP	**H** 83	03/30/2026 12:20	65	03/30/2026 08:00	**C** 91	03/30/2026 07:08
MAP	119	03/30/2026 12:20	94	03/30/2026 08:00	119	03/30/2026 09:30
SpO2	95	03/30/2026 12:20	95	03/30/2026 08:00	99	03/30/2026 08:15
O2 Flow Rate	on 6 l/min		on 50 %		on 50 %	
O2 Therapy	Nasal cannula		BiPAP		BiPAP	

Ventilator Settings (Last Within 24hrs)

Ventilator Mode: PC-SIMV (07:37) FIO2: 50 % (11:30)
Ventilator Frequency, Mandatory: 14 br/min (07:37)
Pressure Support: 4 cm H2O (07:37)
Positive End Expiratory Pressure: 4 cm H2O (07:37)

Intake and Output (current encounter)

No Data Available

Physical Exam

General: Thin appearing minimi speaking elderly female on continuous BiPAP
Mental Status: Alert and oriented to person, place, time. Normal affect.
Head: Normocephalic, atraumatic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact. Anicteric.
Ear, Nose and Throat: Oropharynx clear, moist mucous membranes. Trachea midline.
Neck: No obvious neck masses
Respiratory: Clear to auscultation bilaterally. No wheezing, crackles or rhonchi. Normal respiratory effort.
Cardiovascular: Regular rate and rhythm, no murmurs, gallops or rubs.
Gastrointestinal: Non-tender, non-distended. Normal bowel sounds.
Genitourinary: No suprapubic tenderness.
Neurologic: Cranial nerves II-XII grossly intact. Moves all extremities.
Light touch sensation is grossly intact bilaterally.

Assessment/Plan

Assessment:
Assessment:
Acute hypoxic respiratory failure 2/2 pneumonia versus fluid overload
Dyspnea

mL, IV Piggyback, q12H
heparin 5000 units/mL injection 1 mL (heparin) 5,000 units 1 mL, SubCutaneous, q12H
hydrALAZINE 50 mg Tab (hydrALAZINE) 50 mg 1 Tabs, Oral, TID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
metoprolol succinate 50 mg XL Tab (Metoprolol Succinate ER) 50 mg 1 Tabs, Oral, qHS
sevelamer carbonate 800 mg Tab (sevelamer) 800 mg 1 Tabs, Oral, TID w/Meals
sodium citrate 4% soln 5 mL (sodium citrate) 200 mg 5 mL, IntraCatheter, On Call
Continuous: (0)
PRN: (11)
acetaminophen 325 mg Tab (Tylenol) 650 mg 2 Tabs, Oral, q4H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q3H
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID
Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 25 gm 50 mL, IV Push, q1H Interval
docusate sodium 100 mg Cap (Colace) 100 mg 1 Caps, Oral, BID
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H
glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q4H
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q4H
melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H

72 hour Antibiotic History

Completed/Discontinued Antibiotics	Stop Date/Time	Last Administered	First Administered
azithromycin 500 mg, 250 mL, 250 mL/hr, IV Piggyback, Once	03/30/2026 10:10	03/30/2026 10:09	03/30/2026 10:09

History and Physical

Sepsis with RBC, HR 2/2 respiratory source
Troponinemia 2/2 demand ischemia
ESRD on HD MWF
Hyperkalemia 2/2 above
Oxygen dependent reportedly on 2 to 5 L at home
Hypertension
Dementia

cefTRIAXone 1 gm, 50 mL, 100 mL/hr, IV Piggyback, Once	03/30/2026 09:18	03/30/2026 09:10	03/30/2026 09:10

Plan:
ICU for continuous BiPAP requirement
CT chest/abdomen/pelvis pending
Acute on chronic hypoxic respite failure secondary to pneumonia in addition to fluid overload
– Continue antibiotics with Rocephin
– Breathing treatment
– Goal SpO2 92%, will titrate down to high flow if tolerable
– Infectious workup pending
Nephrology consulted appreciate recommendation
– S/p hyper-K protocol, stat HD
Troponinemia secondary to demand ischemia, patient denies chest pain
– TTE pending
PRN antihypertensives and antiemetics
Replete electrolytes as needed, routine labs, and supportive care.

Patient has 5 children but main caretaker is brother who lives with her. ICU team discussed with extensively regarding CODE STATUS with patient's daughter and brother at the bedside who had different opinion regarding CODE STATUS. They wish to proceed with full code no shock for now but has plan to discuss among family members.

Consults: ICU, nephrology Dr. Vornicu
Diet: N.p.o., nursing swallow when able
Activity: Early and progressive mobilization
GI prophylaxis: None
DVT prophylaxis: Subcu heparin, SCD
CODE STATUS: Full code, no shock per family

Will discuss plan with attending physician. Awaiting further recommendations.

Ellyn Hirabayashi, DO
Internal Medicine - PGY3
Valley Hospital Medical Center

Discharge Planning:

Attending addendum: I was physically present and involved in the discussion and management of the patient by the resident, and during critical portions of assessment and services furnished.

Worawith Techathaveewat, DO
Internal Medicine

Results
Recent Labs
Blood Gases

Art Na	137 mmol/L (Normal)	03/30/2026 08:43
Art K	6.3 mmol/L (Critical)	03/30/2026 08:43
Art iCa	0.96 mmol/L (Low)	03/30/2026 08:43
pH Arterial	7.54 (High)	03/30/2026 08:43
Art pCO2	39.4 mmHg (Normal)	03/30/2026 08:43
Art pO2	153.0 mmHg (High)	03/30/2026 08:43
HCO3 Art	34.4 mmol/L (High)	03/30/2026 08:43
Arterial pCO2(T)	39.4 mmHg (N/A)	03/30/2026 08:43
Arterial pH(T)	7.54 (N/A)	03/30/2026 08:43
Arterial pO2(T)	153.0 mmHg (N/A)	03/30/2026 08:43
Art Base Excess	10.6 mmol/L (High)	03/30/2026 08:43
Art sO2	99 % (High)	03/30/2026 08:43
Art tHB	10.3 gm/dL (Low)	03/30/2026 08:43
Art Hct	32 % (Low)	03/30/2026 08:43
POC Pt Temp	37.0 DegC (N/A)	03/30/2026 08:43
POC FIO2	50 % (N/A)	03/30/2026 08:43
POC Sample	Arterial (N/A)	03/30/2026 08:43
POC Site	Radial Right (N/A)	03/30/2026 08:43
POC Allen Test	POS (N/A)	03/30/2026 08:43
POC Del Sys	BIPAP (N/A)	03/30/2026 08:43
POC Vent Mode	NO VENT (N/A)	03/30/2026 08:43
POC Set RR	14 br/min (N/A)	03/30/2026 08:43

History and Physical

POC IPAP	8 cm H2O (N/A)	03/30/2026 08:43
POC EPAP	4 cm H2O (N/A)	03/30/2026 08:43
Notified To and RBV	DR DAVIDSON (N/A)	03/30/2026 08:43
Notified By	vhsmurrat (N/A)	03/30/2026 08:43
Notified Note	ALL CV WITH RB (N/A)	03/30/2026 08:43

Cardiac

BNP	2004 pg/mL (High)	03/30/2026 07:31
Troponin TNIH	238.44 ng/L (Critical)	03/30/2026 10:37

General Chemistry

Glucose Level	189 mg/dL (High)	03/30/2026 07:31
POC Glucose	102 mg/dL (Normal)	03/30/2026 09:34
Sodium	139 mmol/L (Normal)	03/30/2026 07:31
Potassium	5.9 mmol/L (High)	03/30/2026 07:31
Chloride	94 mmol/L (Low)	03/30/2026 07:31
CO2	27 mmol/L (Normal)	03/30/2026 07:31
Anion Gap	18 mmol/L (High)	03/30/2026 07:31
BUN	44 mg/dL (High)	03/30/2026 07:31
Creatinine	4.80 mg/dL (High)	03/30/2026 07:31
BUN/Creat Ratio	9 (Normal)	03/30/2026 07:31
Calcium	8.6 mg/dL (Low)	03/30/2026 07:31
Albumin. Level	3.4 gm/dL (Normal)	03/30/2026 11:15
TP	6.3 gm/dL (Normal)	03/30/2026 11:15
A/G Ratio	1.2 (Normal)	03/30/2026 11:15
T Bili	0.4 mg/dL (Normal)	03/30/2026 11:15
D Bili	0.2 mg/dL (Normal)	03/30/2026 11:15
Alk Phos	150 units/L (High)	03/30/2026 11:15
AST	57 units/L (High)	03/30/2026 11:15
ALT	51 units/L (Normal)	03/30/2026 11:15

History and Physical

Estimated Creatinine Clearance	5.02 mL/min ()	03/30/2026 08:46
eGFR Cr	8 mL/min/1.73m2 (N/A)	03/30/2026 07:31
eGFR Pediatric	Not Reported (N/A)	03/30/2026 07:31
Lactic Acid Lvl	1.8 mmol/L (Normal)	03/30/2026 08:48
MAR Glucose Result	102 ()	03/30/2026 09:49

General Hematology

WBC	12.25 x10e3/mcL (High)	03/30/2026 07:31
RBC	4.86 x10e6/mcL (Normal)	03/30/2026 07:31
Hgb	11.2 gm/dL (Normal)	03/30/2026 07:31
Hct	33.7 % (Normal)	03/30/2026 07:31
MCV	69.3 Femtoliters (Low)	03/30/2026 07:31
MCH	23.0 pg (Low)	03/30/2026 07:31
MCHC	33.2 gm/dL (Normal)	03/30/2026 07:31
RDW-SD	44.7 Femtoliters (Normal)	03/30/2026 07:31
RDW-CV	18.6 % (High)	03/30/2026 07:31
Plt	230 x10e3/mcL (Normal)	03/30/2026 07:31
MPV	See comment Femtoliters (N/A)	03/30/2026 07:31
Imm Plt Fract	4.0 % (Normal)	03/30/2026 07:31
NRBC %	0.2 /100 WBC (Normal)	03/30/2026 07:31
NRBC #	0.02 x10e3/mcL (High)	03/30/2026 07:31
Neut % Auto	84.3 % (N/A)	03/30/2026 07:31
Lymph % Auto	10.9 % (N/A)	03/30/2026 07:31
Mono % Auto	3.8 % (N/A)	03/30/2026 07:31
Eos % Auto	0.2 % (N/A)	03/30/2026 07:31
Baso % Auto	0.2 % (N/A)	03/30/2026 07:31
Immature Grans %	0.6 % (Normal)	03/30/2026 07:31
Neut # Auto	10.33 x10e3/mcL (High)	03/30/2026 07:31
Lymph # Auto	1.33 x10e3/mcL (Normal)	03/30/2026 07:31

History and Physical

Mono # Auto	0.47 x10e3/mcL (Normal)	03/30/2026 07:31
Eos # Auto	0.02 x10e3/mcL (Normal)	03/30/2026 07:31
Baso # Auto	0.03 x10e3/mcL (Normal)	03/30/2026 07:31
Immature Grans # Auto	0.07 x10e3/mcL (Normal)	03/30/2026 07:31
RBC Morph	Abnormal (Abnormal)	03/30/2026 07:31
Plt Morphology	Normal (Normal)	03/30/2026 07:31
Polychrom	1+ (Abnormal)	03/30/2026 07:31
Micro	2+ (Abnormal)	03/30/2026 07:31
Plt Est	Adequate (Normal)	03/30/2026 07:31
Schistocytes	1+ (Abnormal)	03/30/2026 07:31
Target Cells (Codocytes)	3+ (Abnormal)	03/30/2026 07:31
Teardrop Cells (Dacrocytes)	1+ (Abnormal)	03/30/2026 07:31

Hepatitis

Hep A IgM	Reactive (Abnormal)	03/30/2026 10:54
Hep B Core IgM	Non-Reactive (Normal)	03/30/2026 11:04
Hepatitis B Surface Antigen	Non-Reactive (Normal)	03/30/2026 11:04
Hep Bs Ab Qn	31.9 milli intl units/mL (High)	03/30/2026 11:04
Hep Bs Ab Qn Interp	Reactive (Abnormal)	03/30/2026 11:04
Hep C Ab	Non-Reactive (Normal)	03/30/2026 10:54

Other Chemistry

Procalcitonin Lvl	1.56 ng/mL (High)	03/30/2026 11:15

Thyroid

T3 Free	2.87 pg/mL (Normal)	03/30/2026 11:15
T4 Free	1.74 ng/dL (High)	03/30/2026 11:15
TSH Ultrasensitive	3.046 mc Intl units/mL (Normal)	03/30/2026 11:15

Abnormal Labs
Blood Gases

History and Physical

Art K	6.3 mmol/L (Critical)	03/30/2026 08:43
Art iCa	0.96 mmol/L (Low)	03/30/2026 08:43
pH Arterial	7.54 (High)	03/30/2026 08:43
Art pO2	153.0 mmHg (High)	03/30/2026 08:43
HCO3 Art	34.4 mmol/L (High)	03/30/2026 08:43
Art Base Excess	10.6 mmol/L (High)	03/30/2026 08:43
Art sO2	99 % (High)	03/30/2026 08:43
Art tHB	10.3 gm/dL (Low)	03/30/2026 08:43
Art Hct	32 % (Low)	03/30/2026 08:43

Cardiac

BNP	2004 pg/mL (High)	03/30/2026 07:31
Troponin TNIH	238.44 ng/L (Critical)	03/30/2026 10:37

General Chemistry

Glucose Level	189 mg/dL (High)	03/30/2026 07:31
Potassium	5.9 mmol/L (High)	03/30/2026 07:31
Chloride	94 mmol/L (Low)	03/30/2026 07:31
Anion Gap	18 mmol/L (High)	03/30/2026 07:31
BUN	44 mg/dL (High)	03/30/2026 07:31
Creatinine	4.80 mg/dL (High)	03/30/2026 07:31
Calcium	8.6 mg/dL (Low)	03/30/2026 07:31
Alk Phos	150 units/L (High)	03/30/2026 11:15
AST	57 units/L (High)	03/30/2026 11:15

General Hematology

WBC	12.25 x10e3/mcL (High)	03/30/2026 07:31
MCV	69.3 Femtoliters (Low)	03/30/2026 07:31
MCH	23.0 pg (Low)	03/30/2026 07:31
RDW-CV	18.6 % (High)	03/30/2026 07:31
MPV	See comment Femtoliters (N/A)	03/30/2026 07:31

History and Physical

NRBC #	0.02 x10e3/mcL (High)	03/30/2026 07:31
Neut # Auto	10.33 x10e3/mcL (High)	03/30/2026 07:31
RBC Morph	Abnormal (Abnormal)	03/30/2026 07:31
Polychrom	1+ (Abnormal)	03/30/2026 07:31
Micro	2+ (Abnormal)	03/30/2026 07:31
Schistocytes	1+ (Abnormal)	03/30/2026 07:31
Target Cells (Codocytes)	3+ (Abnormal)	03/30/2026 07:31
Teardrop Cells (Dacrocytes)	1+ (Abnormal)	03/30/2026 07:31

Hepatitis

Hep A IgM	Reactive (Abnormal)	03/30/2026 10:54
Hep Bs Ab Qn	31.9 milli intl units/mL (High)	03/30/2026 11:04
Hep Bs Ab Qn Interp	Reactive (Abnormal)	03/30/2026 11:04

Other Chemistry

Procalcitonin Lvl	1.56 ng/mL (High)	03/30/2026 11:15

Thyroid

T4 Free	1.74 ng/dL (High)	03/30/2026 11:15

Microbiology

Culture Blood ARD **In-Lab**

Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	03/30/2026 08:41

Culture Blood ARD **In-Lab**

Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	03/30/2026 08:41

Blood Gases (Last Within 24hrs)

History and Physical

pH Arterial: 7.54 High (08:43)
Art pO2: 153 mmHg High (08:43)
Art pCO2: 39.4 mmHg (08:43)
HCO3 Art: 34.4 mmol/L High (08:43)
Art sO2: 99 % High (08:43)

Electronically Signed By: Hirabayashi, Ellyn DO
On: 03.30.2026 13:20 PDT

Electronically Signed On: 03.30.2026 13:28 PDT
Techathaveewat, Worawith DO

DOCUMENT NAME:	History & Physical
SERVICE DATE/TIME:	3/30/2026 11:56 PDT
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Cho DO,Claire Sungkyung (3/30/2026 11:57 PDT)
SIGN INFORMATION:	Delmundo DO,Lloyd (3/30/2026 16:20 PDT); Cho DO,Claire Sungkyung (3/30/2026 11:57 PDT)

Chief Complaint/Reason for Consultation
Patient brought in by EMS from home with complaints of SOB. Per the family, patient started to complain of SOB this morning. Patient found with a RA sat of <50%. Patient provided with BVM ventilation and then placed on NRB. Patient A/Ox0. EMS unable to obtain demographics on scene. BGL 119

History of Present Illness
HPI: 90 years old minimally speaking female with PMH of ESRD on HD MWF, on home oxygen 2 to 5 L, hypertension, dementia, bedbound was brought by EMS from home with complaints of shortness of breath started overnight. History was limited patient was on continuous BiPAP and addition to language barrier with hard of hearing therefore history was gathered through family members including patient's daughter and brother who lives with patient. Patient complained of shortness of breath started overnight with mucus production, cough, generalized weakness and family called EMS due to worsening symptom. Reports compliance with dialysis session with last dialysis session on last Friday and that supposed to get dialysis today. Patient was placed on nonrebreather and transition to continuous BiPAP in ED. Labs shows hyper-K potassium 5.9 and 6.9 on ABG therefore receive hyper-K protocol including calcium, albuterol, insulin. ABG showed 7.5/39/153/34. Patient was able to follow simple commands with family and denies chest pain.

Past Medical History: ESRD on HD MWF, oxygen dependent home oxygen 2 to 5 L, hypertension, dementia
Past Surgical History: Breast implant
Social History: Denies tobacco, alcohol, and drug use. Lives with brother, mostly bedbound unable to ambulate by herself
Family History: Noncontributory
Allergies: NKDA
Medications: Hydralazine 50 mg 3 times daily, losartan 100 mg daily, metoprolol 50 mg

Medications
Home Medications
No medications documented.

Inpatient Medications
Medications (21) Active
Scheduled: (10)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 10 mg Tab (amLODIPine) 10 mg 1 Tabs, Oral, Daily
cefTRIAXone/D5W (Rocephin) 50 mL, IV Piggyback, q24H Interval
doxycycline/D5W (doxycycline) 100 mg 100 mL, IV Piggyback, q12H
heparin 5000 units/mL injection 1 mL (heparin) 5,000 units 1 mL, SubCutaneous, q12H
hydrALAZINE 50 mg Tab (hydrALAZINE) 50 mg 1 Tabs, Oral, TID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
metoprolol succinate 50 mg XL Tab (Metoprolol Succinate ER) 50 mg 1 Tabs, Oral, qHS
sevelamer carbonate 800 mg Tab (sevelamer) 800 mg 1 Tabs, Oral, TID w/Meals
sodium citrate 4% soln 5 mL (sodium citrate)

History and Physical

ER, sevelamer 800 mg 3 times daily, amlodipine 10 mg, there is prednisone 10 mg last filled March 5 however her family denies taking prednisone

Review of Systems

Review of systems negative except as otherwise described in HPI

Objective

Measurements (most recent)

Height (cm)	157.48 cm	Stasiewski RN, Sabryna	03/30 07:08
Height (inches)	62.00 inch	SYSTEM	03/30 07:08
Weight	40.82 kg	Stasiewski RN, Sabryna	03/30 07:08
Weight (lbs)	89.99 lb	SYSTEM	03/30 07:08
Body Mass Index	16.46 kg/m2	Stasiewski RN, Sabryna	03/30 07:08

Vital Signs (24 hrs)	Last Charted		Minimum		Maximum	
Temp	36.9	03/30/2026 07:11	36.9	03/30/2026 07:11	36.9	03/30/2026 07:11
Heart Rate Monitored	90	03/30/2026 11:30	84	03/30/2026 09:00	**H** 104	03/30/2026 07:30
Resp Rate	18	03/30/2026 11:30	12	03/30/2026 07:08	**H** 26	03/30/2026 07:38
SBP	**H** 148	03/30/2026 11:30	130	03/30/2026 07:08	**H** 179	03/30/2026 10:00
DBP	70	03/30/2026 11:30	65	03/30/2026 08:00	**C** 91	03/30/2026 07:08
MAP	100	03/30/2026 11:30	94	03/30/2026 08:00	119	03/30/2026 09:30
SpO2	99	03/30/2026 11:30	95	03/30/2026 08:00	99	03/30/2026 08:15
O2 Flow Rate	on 50 %		on 50 %		on 50 %	
O2 Therapy	BiPAP		BiPAP		BiPAP	

Intake and Output (current encounter)

No Data Available

Physical Exam

General: Thin appearing minimi speaking elderly female on continuous BiPAP
Mental Status: Alert and oriented to person, place, time. Normal affect.
Head: Normocephalic, atraumatic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact. Anicteric.
Ear, Nose and Throat: Oropharynx clear, moist mucous membranes. Trachea midline.
Neck: No obvious neck masses
Respiratory: Clear to auscultation bilaterally. No wheezing, crackles or rhonchi. Normal respiratory effort.
Cardiovascular: Regular rate and rhythm, no murmurs, gallops or rubs.
Gastrointestinal: Non-tender, non-distended. Normal bowel sounds.
Genitourinary: No suprapubic tenderness.
Neurologic: Cranial nerves II-XII grossly intact. Moves all extremities.
Light touch sensation is grossly intact bilaterally.

200 mg 5 mL, IntraCatheter, On Call
Continuous: (0)
PRN: (11)
acetaminophen 325 mg Tab (Tylenol) 650 mg 2 Tabs, Oral, q4H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q3H
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID
Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 25 gm 50 mL, IV Push, q1H Interval
docusate sodium 100 mg Cap (Colace) 100 mg 1 Caps, Oral, BID
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H
glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q4H
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q4H
melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H

72 hour Antibiotic History

Completed/Discontinued Antibiotics	Stop Date/Time	Last Administered	First Administered
azithromycin 500 mg, 250 mL, 250 mL/hr, IV Piggyback, Once	03/30/2026 10:10	03/30/2026 10:09	03/30/2026 10:09
cefTRIAXone 1 gm, 50 mL, 100 mL/hr, IV Piggyback, Once	03/30/2026 09:18	03/30/2026 09:10	03/30/2026 09:10

Results

Recent Labs

Blood Gases

Art Na	137 mmol/L (Normal)	03/30/2026 08:43
Art K	6.3 mmol/L (Critical)	03/30/2026 08:43

History and Physical

Assessment/Plan

Assessment:
Acute hypoxic respiratory failure 2/2 pneumonia versus fluid overload
Dyspnea
Sepsis with RBC, HR 2/2 respiratory source
Troponinemia 2/2 demand ischemia
Hyper-K
ESRD on HD MWF
Oxygen dependent reportedly on 2 to 5 L at home
Hypertension
Dementia

Plan:
ICU for continuous BiPAP requirement
Acute on chronic hypoxic respite failure secondary to pneumonia in addition to fluid overload
– Continue antibiotics with Rocephin
–Breathing treatment
– Goal SpO2 92%, will titrate down to high flow if tolerable
–CT thorax/abdomen/pelvis for sepsis workup, infectious workup pending
Nephrology consulted appreciate recommendation
– S/p hyper-K protocol, stat HD
Troponinemia secondary to demand ischemia, patient denies chest pain
– Echocardiogram
PRN antihypertensives and antiemetics
Replete electrolytes as needed, routine labs, and supportive care.

Consults: Nephrology Dr. Vornicu
Diet: N.p.o., nursing swallow when able
Activity: Early and progressive mobilization
GI prophylaxis: None
DVT prophylaxis: Subcu heparin, SCD
CODE STATUS: Full code, no shock per family

Patient has 5 children but main caretaker is brother who lives with her. Discussed with extensively regarding CODE STATUS with patient's daughter and brotherat the bedside who had different opinion regarding CODE STATUS. They wish to proceed with full code no shock for now but has plan to discuss among family members.

Will discuss case with attending physician. Further recommendations to follow.

Claire Cho, DO
Internal Medicine, PGY-3
Please contact "internal medicine, resident" on YouCallMD.

I discussed with resident/fellow at length. I was present for critical and key services and am participating in patient management, in combination with the resident or fellow. Agree with assessment and plan

on low flow o2
down to 6 liters
off bipap
control BP

Art iCa	0.96 mmol/L (Low)	03/30/2026 08:43
pH Arterial	7.54 (High)	03/30/2026 08:43
Art pCO2	39.4 mmHg (Normal)	03/30/2026 08:43
Art pO2	153.0 mmHg (High)	03/30/2026 08:43
HCO3 Art	34.4 mmol/L (High)	03/30/2026 08:43
Arterial pCO2(T)	39.4 mmHg (N/A)	03/30/2026 08:43
Arterial pH(T)	7.54 (N/A)	03/30/2026 08:43
Arterial pO2(T)	153.0 mmHg (N/A)	03/30/2026 08:43
Art Base Excess	10.6 mmol/L (High)	03/30/2026 08:43
Art sO2	99 % (High)	03/30/2026 08:43
Art tHB	10.3 gm/dL (Low)	03/30/2026 08:43
Art Hct	32 % (Low)	03/30/2026 08:43
POC Pt Temp	37.0 DegC (N/A)	03/30/2026 08:43
POC FIO2	50 % (N/A)	03/30/2026 08:43
POC Sample	Arterial (N/A)	03/30/2026 08:43
POC Site	Radial Right (N/A)	03/30/2026 08:43
POC Allen Test	POS (N/A)	03/30/2026 08:43
POC Del Sys	BIPAP (N/A)	03/30/2026 08:43
POC Vent Mode	NO VENT (N/A)	03/30/2026 08:43
POC Set RR	14 br/min (N/A)	03/30/2026 08:43
POC IPAP	8 cm H2O (N/A)	03/30/2026 08:43
POC EPAP	4 cm H2O (N/A)	03/30/2026 08:43
Notified To and RBV	DR DAVIDSON (N/A)	03/30/2026 08:43
Notified By	vhsmurrat (N/A)	03/30/2026 08:43
Notified Note	ALL CV WITH RB (N/A)	03/30/2026 08:43

Cardiac

BNP	2004 pg/mL (High)	03/30/2026 07:31
Troponin TNIH	238.44 ng/L (Critical)	03/30/2026 10:37

History and Physical

is about to get HD
needs HD
empiric abx
If stable after HD, we will downgrade, as this looks like volume with effusions
Follow up imaging
code status is full
Please see above
ICU status for now while getting HD

DW staff

L. Del Mundo, DO
Pulmonary/Critical Care

General Chemistry

Glucose Level	189 mg/dL (High)	03/30/2026 07:31
POC Glucose	102 mg/dL (Normal)	03/30/2026 09:34
Sodium	139 mmol/L (Normal)	03/30/2026 07:31
Potassium	5.9 mmol/L (High)	03/30/2026 07:31
Chloride	94 mmol/L (Low)	03/30/2026 07:31
CO2	27 mmol/L (Normal)	03/30/2026 07:31
Anion Gap	18 mmol/L (High)	03/30/2026 07:31
BUN	44 mg/dL (High)	03/30/2026 07:31
Creatinine	4.80 mg/dL (High)	03/30/2026 07:31
BUN/Creat Ratio	9 (Normal)	03/30/2026 07:31
Calcium	8.6 mg/dL (Low)	03/30/2026 07:31
Albumin. Level	3.4 gm/dL (Normal)	03/30/2026 11:15
TP	6.3 gm/dL (Normal)	03/30/2026 11:15
A/G Ratio	1.2 (Normal)	03/30/2026 11:15
T Bili	0.4 mg/dL (Normal)	03/30/2026 11:15
D Bili	0.2 mg/dL (Normal)	03/30/2026 11:15
Alk Phos	150 units/L (High)	03/30/2026 11:15
AST	57 units/L (High)	03/30/2026 11:15
ALT	51 units/L (Normal)	03/30/2026 11:15
Estimated Creatinine Clearance	5.02 mL/min ()	03/30/2026 08:46
eGFR Cr	8 mL/min/1.73m2 (N/A)	03/30/2026 07:31
eGFR Pediatric	Not Reported (N/A)	03/30/2026 07:31
Lactic Acid Lvl	1.8 mmol/L (Normal)	03/30/2026 08:48
MAR Glucose Result	102 ()	03/30/2026 09:49

General Hematology

WBC	12.25 x10e3/mcL (High)	03/30/2026 07:31
RBC	4.86 x10e6/mcL (Normal)	03/30/2026 07:31

History and Physical

Hgb	11.2 gm/dL (Normal)	03/30/2026 07:31
Hct	33.7 % (Normal)	03/30/2026 07:31
MCV	69.3 Femtoliters (Low)	03/30/2026 07:31
MCH	23.0 pg (Low)	03/30/2026 07:31
MCHC	33.2 gm/dL (Normal)	03/30/2026 07:31
RDW-SD	44.7 Femtoliters (Normal)	03/30/2026 07:31
RDW-CV	18.6 % (High)	03/30/2026 07:31
Plt	230 x10e3/mcL (Normal)	03/30/2026 07:31
MPV	See comment Femtoliters (N/A)	03/30/2026 07:31
Imm Plt Fract	4.0 % (Normal)	03/30/2026 07:31
NRBC %	0.2 /100 WBC (Normal)	03/30/2026 07:31
NRBC #	0.02 x10e3/mcL (High)	03/30/2026 07:31
Neut % Auto	84.3 % (N/A)	03/30/2026 07:31
Lymph % Auto	10.9 % (N/A)	03/30/2026 07:31
Mono % Auto	3.8 % (N/A)	03/30/2026 07:31
Eos % Auto	0.2 % (N/A)	03/30/2026 07:31
Baso % Auto	0.2 % (N/A)	03/30/2026 07:31
Immature Grans %	0.6 % (Normal)	03/30/2026 07:31
Neut # Auto	10.33 x10e3/mcL (High)	03/30/2026 07:31
Lymph # Auto	1.33 x10e3/mcL (Normal)	03/30/2026 07:31
Mono # Auto	0.47 x10e3/mcL (Normal)	03/30/2026 07:31
Eos # Auto	0.02 x10e3/mcL (Normal)	03/30/2026 07:31
Baso # Auto	0.03 x10e3/mcL (Normal)	03/30/2026 07:31
Immature Grans # Auto	0.07 x10e3/mcL (Normal)	03/30/2026 07:31

Thyroid

T3 Free	2.87 pg/mL (Normal)	03/30/2026 11:15

Abnormal Labs

History and Physical

Blood Gases

Art K	6.3 mmol/L (Critical)	03/30/2026 08:43
Art iCa	0.96 mmol/L (Low)	03/30/2026 08:43
pH Arterial	7.54 (High)	03/30/2026 08:43
Art pO2	153.0 mmHg (High)	03/30/2026 08:43
HCO3 Art	34.4 mmol/L (High)	03/30/2026 08:43
Art Base Excess	10.6 mmol/L (High)	03/30/2026 08:43
Art sO2	99 % (High)	03/30/2026 08:43
Art tHB	10.3 gm/dL (Low)	03/30/2026 08:43
Art Hct	32 % (Low)	03/30/2026 08:43

Cardiac

BNP	2004 pg/mL (High)	03/30/2026 07:31
Troponin TNIH	238.44 ng/L (Critical)	03/30/2026 10:37

General Chemistry

Glucose Level	189 mg/dL (High)	03/30/2026 07:31
Potassium	5.9 mmol/L (High)	03/30/2026 07:31
Chloride	94 mmol/L (Low)	03/30/2026 07:31
Anion Gap	18 mmol/L (High)	03/30/2026 07:31
BUN	44 mg/dL (High)	03/30/2026 07:31
Creatinine	4.80 mg/dL (High)	03/30/2026 07:31
Calcium	8.6 mg/dL (Low)	03/30/2026 07:31
Alk Phos	150 units/L (High)	03/30/2026 11:15
AST	57 units/L (High)	03/30/2026 11:15

General Hematology

WBC	12.25 x10e3/mcL (High)	03/30/2026 07:31
MCV	69.3 Femtoliters (Low)	03/30/2026 07:31
MCH	23.0 pg (Low)	03/30/2026 07:31
RDW-CV	18.6 % (High)	03/30/2026 07:31

History and Physical

MPV	See comment Femtoliters (N/A)	03/30/2026 07:31
NRBC #	0.02 x10e3/mcL (High)	03/30/2026 07:31
Neut # Auto	10.33 x10e3/mcL (High)	03/30/2026 07:31

CBC, BMP, Coagulation Trend (last 4 resulted)

WBC	**12.25 H** 03/30/2026 07:31			
Hgb	**11.2** 03/30/2026 07:31			
Hct	**33.7** 03/30/2026 07:31			
Baso # Auto	**0.03** 03/30/2026 07:31			
Baso % Auto	**0.2** 03/30/2026 07:31			
Eos # Auto	**0.02** 03/30/2026 07:31			
Eos % Auto	**0.2** 03/30/2026 07:31			
Immature Grans %	**0.6** 03/30/2026 07:31			
Immature Grans # Auto	**0.07** 03/30/2026 07:31			
Lymph # Auto	**1.33** 03/30/2026 07:31			
Lymph % Auto	**10.9** 03/30/2026 07:31			
Mono # Auto	**0.47** 03/30/2026 07:31			

History and Physical

Mono % Auto	**3.8** 03/30/2026 07:31			
Neut # Auto	**10.33 H** 03/30/2026 07:31			
Neut % Auto	**84.3** 03/30/2026 07:31			
Plt	**230** 03/30/2026 07:31			
Glucose Level	**189 H** 03/30/2026 07:31			
Sodium	**139** 03/30/2026 07:31			
Potassium	**5.9 H** 03/30/2026 07:31			
Chloride	**94 L** 03/30/2026 07:31			
CO2	**27** 03/30/2026 07:31			
Creatinine	**4.80 H** 03/30/2026 07:31			
BUN	**44 H** 03/30/2026 07:31			
BUN/Creat Ratio	**9** 03/30/2026 07:31			
Calcium	**8.6 L** 03/30/2026 07:31			
Albumin. Level	**3.4** 03/30/2026 11:15			
T Bili	**0.4** 03/30/2026 11:15			

History and Physical

D Bili	**0.2** 03/30/2026 11:15			
Alk Phos	**150 H** 03/30/2026 11:15			
AST	**57 H** 03/30/2026 11:15			
ALT	**51** 03/30/2026 11:15			

Blood Glucose Trend

Glucose Level: 189 mg/dL High (03/30/26 07:31:00)

POC Glucose: 102 mg/dL (03/30/26 09:34:00)

Electronically Signed By: Cho, Claire DO
On: 03.30.2026 11:57 PDT

Electronically Signed On: 03.30.2026 16:20 PDT
Delmundo, Lloyd DO

Orders

Order: **Referral to Case Management**			
Order Date/Time: 4/1/2026 16:02 PDT			
Signed Date/Time: 4/1/2026 16:02 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Consults	Activity Type: Case Management Consults
End-state Date/Time: 4/1/2026 16:02 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 4/1/2026 16:02 PDT			
Order Details: 4/1/26 4:02:47 PM PDT, Routine			
Order Comment: Ordered by Discern Expert secondary to: Discharge To, Anticipated - Home with home health			

Orders

Order: **Diet**			
Order Date/Time: 3/30/2026 10:38 PDT			
Signed Date/Time: 3/30/2026 10:38 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Nutrition Services	Activity Type: Diets TransMedRec
Ordering Physician: Cho DO,Claire Sungkyung		Consulting Physician:	
Entered By: Cho DO,Claire Sungkyung on 3/30/2026 10:38 PDT			
Order Details: 3/30/26 10:38:00 AM PDT, Type: Renal Dialysis, Consistency: 6 - Soft & Bite Sized (Dysphagia), Consistency 0 - Thin, One on One, Special Instructions: medication 1 at a time			
Order Comment:			

Order: **Consult to Case Management (Case Management Consult)**			
Order Date/Time: 4/1/2026 10:27 PDT			
Signed Date/Time: 4/1/2026 10:27 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Consults	Activity Type: Case Management Consults
End-state Date/Time: 4/1/2026 10:27 PDT		End-state Reason:	
Ordering Physician: Hirabayashi DO,Ellyn		Consulting Physician:	
Entered By: Hirabayashi DO,Ellyn on 4/1/2026 10:27 PDT			
Order Details: 4/1/26 10:27:00 AM PDT, Routine, Reason for Consult: D/C to Home w Home Health, No Isolation, anticipate HHC PT/OT for strength/conditioning CNA for ADLs and medication management Already use A+, please verify			
Order Comment:			

Order: **Activity**			
Order Date/Time: 3/30/2026 10:38 PDT			
Signed Date/Time: 3/30/2026 10:38 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Patient Care	Activity Type: Patient Activity TransMedRec
Ordering Physician: Cho DO,Claire Sungkyung		Consulting Physician:	
Entered By: Cho DO,Claire Sungkyung on 3/30/2026 10:38 PDT			
Order Details: 3/30/26 10:38:00 AM PDT, Type: Early and Progressive Mobilization, Routine, Constant order			
Order Comment:			

Medications

Documented medications

Order: **minoxidil (minoxidil 2.5 mg oral tablet)**
Order Date/Time: 3/30/2026 23:50 PDT
Order Details: 0 Refill(s), Maintenance, 37.5

Order: **ondansetron (ondansetron 4 mg oral tablet)**
Order Date/Time: 3/30/2026 23:50 PDT
Order Details: 0 Refill(s), Maintenance, 37.5

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Medications

Documented medications

Order: **amLODIPine (amLODIPine 10 mg oral tablet)**
Order Date/Time: 3/30/2026 23:49 PDT
Order Details: 0 Refill(s), Maintenance, 37.5

Order: **hydrALAZINE (hydrALAZINE 50 mg oral tablet)**
Order Date/Time: 3/30/2026 23:49 PDT
Order Details: 0 Refill(s), Maintenance, 37.5

Order: **losartan (losartan 100 mg oral tablet)**
Order Date/Time: 3/30/2026 23:49 PDT
Order Details: 0 Refill(s), Maintenance, 37.5

Order: **multivitamin (Rena-Vite oral tablet)**
Order Date/Time: 3/30/2026 23:49 PDT
Order Details: 0 Refill(s), Maintenance, 37.5

Order: **sevelamer (sevelamer 800 mg oral tablet)**
Order Date/Time: 3/30/2026 23:49 PDT
Order Details: 0 Refill(s), Maintenance, 37.5

Order: **zolpidem (zolpidem 5 mg oral tablet)**
Order Date/Time: 3/30/2026 23:49 PDT
Order Details: 0 Refill(s), Maintenance, 37.5

Order: **metoprolol (metoprolol succinate 50 mg oral tablet,extended release)**
Order Date/Time: 3/30/2026 23:48 PDT
Order Details: 0 Refill(s), Maintenance, 37.5

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Orders - Admission/Discharge (Curaspan)

Order: **Level of Care Change**			
Order Date/Time: 3/31/2026 06:54 PDT			
Signed Date/Time: 3/31/2026 06:54 PDT			
Order Status: Completed	Department Status: Completed	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/31/2026 07:42 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/31/2026 06:54 PDT			
Order Details: 3/31/26 6:54:50 AM PDT, Admitting: Delmundo DO, Lloyd, Attending: Techathaveewat DO, Worawith, Admit as Inpatient, Acute Intermediate Care			
Order Comment: Ordered secondary to modification of Level Of Care on the Admission/Observation/Transfer Order.			

Action Type: Complete	Action Date/Time: 3/31/2026 07:42 PDT	Action Personnel: Phelps,Shanae
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type:
Order Details: 03/31/26 6:54:50 PDT, Admitting: Delmundo DO, Lloyd, Attending: Techathaveewat DO, Worawith, Admit as Inpatient, Acute Intermediate Care		
Review Information: Doctor Cosign: Not Required		
Order Comment:		

Action Type: Order	Action Date/Time: 3/31/2026 06:54 PDT	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 03/31/26 6:54:50 PDT, Admitting: Delmundo DO, Lloyd, Attending: Techathaveewat DO, Worawith, Admit as Inpatient, Acute Intermediate Care		
Review Information: Nurse Review: Electronically Signed, Parala RN,Marielle Vianne on 3/31/2026 07:17 PDT Doctor Cosign: Not Required		
Order Comment: Ordered secondary to modification of Level Of Care on the Admission/Observation/Transfer Order.		

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Orders - Admission/Discharge (Curaspan)

Order: **Change Attending Physician**			
Order Date/Time: 3/31/2026 06:54 PDT			
Signed Date/Time: 3/31/2026 06:54 PDT			
Order Status: Completed	Department Status: Completed	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/31/2026 07:42 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/31/2026 06:54 PDT			
Order Details: 3/31/26 6:54:50 AM PDT, Admitting: Delmundo DO, Lloyd, Attending: Techathaveewat DO, Worawith			
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00			

Action Type: Complete	Action Date/Time: 3/31/2026 07:42 PDT	Action Personnel: Phelps,Shanae
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type:
Order Details: 03/31/26 6:54:50 PDT, Admitting: Delmundo DO, Lloyd, Attending: Techathaveewat DO, Worawith		
Review Information: Doctor Cosign: Not Required		
Order Comment:		

Action Type: Order	Action Date/Time: 3/31/2026 06:54 PDT	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 03/31/26 6:54:50 PDT, Admitting: Delmundo DO, Lloyd, Attending: Techathaveewat DO, Worawith		
Review Information: Nurse Review: Electronically Signed, Parala RN,Marielle Vianne on 3/31/2026 07:17 PDT Doctor Cosign: Not Required		
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00		

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Orders - Admission/Discharge (Curaspan)

Order: **Admission/Observation/Transfer**			
Order Date/Time: 3/30/2026 09:37 PDT			
Signed Date/Time: 3/30/2026 09:37 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Admit/Transfer/Observation	Activity Type: Admit/Transfer/Observation
End-state Date/Time: 3/30/2026 09:37 PDT		End-state Reason:	
Ordering Physician: Techathaveewat DO,Worawith		Consulting Physician:	
Entered By: Davidson MD,Jeffrey A on 3/30/2026 09:37 PDT			
Order Details: 3/30/26 9:37:00 AM PDT, Estimated LOS 2 to 3 Nights, Admit as Inpatient, Level of Care Acute Intermediate Care, Admitting: Delmundo DO, Lloyd Attending: Techathaveewat DO, Worawith, Hypoxemia			
Order Comment:			

Action Type: Modify	Action Date/Time: 3/31/2026 06:54 PDT	Action Personnel: Cho DO,Claire Sungkyung
Responsible Provider/Electronically Signed by: Techathaveewat DO, Worawith	Supervising Provider:	Communication Type: Phone w/ Readback (Cosign)
Order Details: 03/30/26 9:37:00 PDT, Estimated LOS 2 to 3 Nights, Admit as Inpatient, Level of Care Acute Intermediate Care, Admitting: Delmundo DO, Lloyd Attending: Techathaveewat DO, Worawith, Hypoxemia		
Review Information: Nurse Review: Electronically Signed, Parala RN,Marielle Vianne on 3/31/2026 07:17 PDT Doctor Cosign: Electronically Signed, Techathaveewat DO,Worawith on 3/31/2026 11:19 PDT		
Order Comment:		

Action Type: Order	Action Date/Time: 3/30/2026 09:37 PDT	Action Personnel: Davidson MD,Jeffrey A
Responsible Provider/Electronically Signed by: Milne,C Dean DO	Supervising Provider:	Communication Type: Phone w/ Readback (Cosign)
Order Details: 03/30/26 9:37:00 PDT, Estimated LOS 2 to 3 Nights, Admit as Inpatient, Level of Care Critical Care, Admitting: Delmundo DO, Lloyd Attending: Milne, C Dean DO, Hypoxemia		
Review Information: Nurse Review: Electronically Signed, Alex RN,Christina on 3/30/2026 09:53 PDT Doctor Cosign: Electronically Signed, Techathaveewat DO,Worawith on 3/30/2026 10:22 PDT		
Order Comment:		

Medication Administration Summary

Medications

Administration Date/Time: 4/1/2026 15:18 PDT
Medication Name: **labetalol**
Ingredients: labetalol 20 mg/4 mL inj 10 mg 2 mL
Admin Details: (Auth) IV Push, Right Hand Apical Heart Rate: 85 bpm

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Medication Administration Summary

Medications

Administration Date/Time: 4/1/2026 13:15 PDT
Medication Name: **sodium citrate**
Ingredients: sodcit4%5mLHEMO 200 mg 5 mL
Admin Details: (Auth) IntraCatheter

Administration Date/Time: 4/1/2026 13:15 PDT
Medication Name: **hydrALAZINE**
Ingredients: hydr50Tab 50 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 4/1/2026 13:15 PDT
Medication Name: **sevelamer**
Ingredients: sevelamer carbonate 800 mg Tab 800 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 4/1/2026 13:15 PDT
Medication Name: **spironolactone**
Ingredients: spir25Tab 25 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 4/1/2026 13:15 PDT
Medication Name: **heparin**
Ingredients: heparin 5000 units/mL 1 mL Inj 5000 units 1 mL
Admin Details: (Auth) SubCutaneous, Left Lowr Abdom Quad

Administration Date/Time: 4/1/2026 13:14 PDT
Medication Name: **cefTRIAXone (Rocephin)**
Ingredients: ceft2/D5W50 2 gm 50 mL
Admin Details: (Auth) IV Piggyback, Right Arm

Administration Date/Time: 4/1/2026 12:13 PDT
Medication Name: **ipratropium-albuterol (DuoNeb)**
Ingredients: albuterol-ipratropium Inh Sol 3 mL 3 mL
Admin Details: (Auth) NEB
Peripheral Pulse Rate: 84 bpm; Respiratory Rate: 18 br/min

Administration Date/Time: 4/1/2026 11:48 PDT
Medication Name: **losartan**
Ingredients: losa100Tab 100 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 4/1/2026 11:48 PDT
Medication Name: **amLODIPine**
Ingredients: amlo10Tab 10 mg 1 Tabs
Admin Details: (Auth) Oral

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Medication Administration Summary

Medications

Administration Date/Time: 4/1/2026 07:58 PDT
Medication Name: **ethanol topical (Nozin POPswab 62% topical swab)**
Ingredients: Nozin POPswab 62% topical swab 1 amp
Admin Details: (Auth) Topical

Administration Date/Time: 4/1/2026 07:58 PDT
Medication Name: **doxycycline**
Ingredients: doxy100/D5W100 100 mg 100 mL
Admin Details: (Auth) IV Piggyback, Right Arm

Administration Date/Time: 4/1/2026 07:58 PDT
Medication Name: **sevelamer**
Ingredients: sevelamer carbonate 800 mg Tab 800 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 4/1/2026 07:44 PDT
Medication Name: **hydrALAZINE**
Ingredients: hydrALAZINE 20 mg/mL vial 10 mg 0.5 mL
Admin Details: (Auth) IV Push, Right Arm

Administration Date/Time: 4/1/2026 06:51 PDT
Medication Name: **ipratropium-albuterol (DuoNeb)**
Ingredients: albuterol-ipratropium Inh Sol 3 mL 3 mL
Admin Details: (Auth) NEB
Peripheral Pulse Rate: 83 bpm; Respiratory Rate: 18 br/min

Administration Date/Time: 4/1/2026 05:51 PDT
Medication Name: **labetalol**
Admin Details: Auth (Verified)
Medication Administration Reason: Hypertension; Medication Effective: No

Administration Date/Time: 4/1/2026 05:36 PDT
Medication Name: **labetalol**
Ingredients: labetalol 20 mg/4 mL inj 10 mg 2 mL
Admin Details: (Auth) IV Push, Right Lower Forearm
Apical Heart Rate: 84 bpm

Administration Date/Time: 4/1/2026 05:36 PDT
Medication Name: **hydrALAZINE**
Ingredients: hydr50Tab 50 mg
Admin Details: (Auth) Oral

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Medication Administration Summary

Medications

Administration Date/Time: 4/1/2026 01:36 PDT
Medication Name: **ipratropium-albuterol (DuoNeb)**
Ingredients: albuterol-ipratropium Inh Sol 3 mL 3 mL
Admin Details: (Auth) NEB
Respiratory Rate: 18 br/min

Administration Date/Time: 4/1/2026 00:45 PDT
Medication Name: **labetalol**
Admin Details: Auth (Verified)
Medication Administration Reason: Hypertension; Medication Effective: No

Administration Date/Time: 4/1/2026 00:30 PDT
Medication Name: **labetalol**
Ingredients: labetalol 20 mg/4 mL inj 10 mg 2 mL
Admin Details: (Auth) IV Push, Right Lower Forearm

Administration Date/Time: 3/31/2026 21:46 PDT
Medication Name: **heparin**
Ingredients: heparin 5000 units/mL 1 mL Inj 5000 units 1 mL
Admin Details: (Auth) SubCutaneous, Left Lowr Abdom Quad

Administration Date/Time: 3/31/2026 21:46 PDT
Medication Name: **ethanol topical (Nozin POPswab 62% topical swab)**
Ingredients: Nozin POPswab 62% topical swab 1 amp
Admin Details: (Auth) Topical

Administration Date/Time: 3/31/2026 21:45 PDT
Medication Name: **doxycycline**
Ingredients: doxy100/D5W100 100 mg 100 mL
Admin Details: (Auth) IV Piggyback, Right Lower Forearm

Administration Date/Time: 3/31/2026 21:41 PDT
Medication Name: **metoprolol (Metoprolol Succinate ER)**
Ingredients: meto50XLTab 50 mg
Admin Details: (Auth) Oral
Apical Heart Rate: 102 bpm

Administration Date/Time: 3/31/2026 21:41 PDT
Medication Name: **hydrALAZINE**
Ingredients: hydr50Tab 50 mg
Admin Details: (Auth) Oral

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Medication Administration Summary

Medications

Administration Date/Time: 3/31/2026 19:12 PDT
Medication Name: **sevelamer**
Ingredients: sevelamer carbonate 800 mg Tab 800 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 3/31/2026 18:29 PDT
Medication Name: **ipratropium-albuterol (DuoNeb)**
Ingredients: albuterol-ipratropium Inh Sol 3 mL 3 mL
Admin Details: (Auth) NEB
Peripheral Pulse Rate: 102 bpm; Respiratory Rate: 18 br/min

Administration Date/Time: 3/31/2026 14:54 PDT
Medication Name: **diphenhydrAMINE (Benadryl)**
Ingredients: diph25Cap 25 mg 1 Caps
Admin Details: (Auth) Oral

Administration Date/Time: 3/31/2026 13:40 PDT
Medication Name: **ipratropium-albuterol (DuoNeb)**
Ingredients: albuterol-ipratropium Inh Sol 3 mL 3 mL
Admin Details: (Auth) NEB
Peripheral Pulse Rate: 93 bpm; Respiratory Rate: 16 br/min

Administration Date/Time: 3/31/2026 12:01 PDT
Medication Name: **hydrALAZINE**
Ingredients: hydr50Tab 50 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 3/31/2026 12:01 PDT
Medication Name: **sevelamer**
Ingredients: sevelamer carbonate 800 mg Tab 800 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 3/31/2026 10:13 PDT
Medication Name: **cefTRIAXone (Rocephin)**
Ingredients: ceft2/D5W50 2 gm 50 mL
Admin Details: (Auth) IV Piggyback, Right Arm

Administration Date/Time: 3/31/2026 08:57 PDT
Medication Name: **losartan**
Ingredients: losa100Tab 100 mg 1 Tabs
Admin Details: (Auth) Oral

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Medication Administration Summary

Medications

Administration Date/Time: 3/31/2026 08:57 PDT
Medication Name: **heparin**
Ingredients: heparin 5000 units/mL 1 mL Inj 5000 units 1 mL
Admin Details: (Auth) SubCutaneous, Left Lowr Abdom Quad

Administration Date/Time: 3/31/2026 08:57 PDT
Medication Name: **ethanol topical (Nozin POPswab 62% topical swab)**
Ingredients: Nozin POPswab 62% topical swab 1 amp
Admin Details: (Auth) Topical

Administration Date/Time: 3/31/2026 08:57 PDT
Medication Name: **amLODIPine**
Ingredients: amlo10Tab 10 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 3/31/2026 08:57 PDT
Medication Name: **sevelamer**
Ingredients: sevelamer carbonate 800 mg Tab 800 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 3/31/2026 08:50 PDT
Medication Name: **doxycycline**
Ingredients: doxy100/D5W100 100 mg 100 mL
Admin Details: (Auth) IV Piggyback, Right Arm

Administration Date/Time: 3/31/2026 07:38 PDT
Medication Name: **ipratropium-albuterol (DuoNeb)**
Admin Details: (Not Done) Patient Refused
Peripheral Pulse Rate, albuterol-ipratropium, Respiratory Rate

Administration Date/Time: 3/31/2026 07:27 PDT
Medication Name: **labetalol**
Admin Details: Auth (Verified)
Medication Effective: Yes; Medication Administration Reason: Hypertension

Administration Date/Time: 3/31/2026 07:12 PDT
Medication Name: **labetalol**
Ingredients: labetalol 20 mg/4 mL inj 10 mg 2 mL
Admin Details: (Auth) IV Push, Right Lower Forearm
Apical Heart Rate: 87 bpm

Administration Date/Time: 3/31/2026 04:52 PDT
Medication Name: **hydrALAZINE**
Admin Details: Auth (Verified)
Medication Administration Reason: Hypertension; Medication Effective: Yes

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Medication Administration Summary

Medications

Administration Date/Time: 3/31/2026 04:24 PDT
Medication Name: **hydrALAZINE**
Admin Details: (Not Given) NPO

Administration Date/Time: 3/31/2026 04:22 PDT
Medication Name: **hydrALAZINE**
Ingredients: hydrALAZINE 20 mg/mL vial 10 mg 0.5 mL
Admin Details: (Auth) IV Push, Right Lower Forearm

Administration Date/Time: 3/31/2026 01:35 PDT
Medication Name: **labetalol**
Admin Details: Auth (Verified)
Medication Effective: Yes; Medication Administration Reason: Hypertension

Administration Date/Time: 3/31/2026 01:31 PDT
Medication Name: **ipratropium-albuterol (DuoNeb)**
Ingredients: albuterol-ipratropium Inh Sol 3 mL 3 mL
Admin Details: (Auth) NEB
Respiratory Rate: 20 br/min

Administration Date/Time: 3/31/2026 01:20 PDT
Medication Name: **labetalol**
Ingredients: labetalol 20 mg/4 mL inj 10 mg 2 mL
Admin Details: (Auth) IV Push, Right Upper Forearm
Apical Heart Rate: 91 bpm

Administration Date/Time: 3/31/2026 01:00 PDT
Medication Name: **ethanol topical (Nozin POPswab 62% topical swab)**
Ingredients: Nozin POPswab 62% topical swab 1 amp
Admin Details: (Modified) Topical

Administration Date/Time: 3/30/2026 21:13 PDT
Medication Name: **doxycycline**
Ingredients: doxy100/D5W100 100 mg 100 mL
Admin Details: (Auth) IV Piggyback, Right Upper Forearm

Administration Date/Time: 3/30/2026 21:00 PDT
Medication Name: **metoprolol (Metoprolol Succinate ER)**
Admin Details: (Not Done) Failed swallow screen
Apical Heart Rate, metoprolol

Administration Date/Time: 3/30/2026 21:00 PDT
Medication Name: **hydrALAZINE**
Admin Details: (Not Done) Failed swallow screen
hydrALAZINE

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Medication Administration Summary

Medications

Administration Date/Time: 3/30/2026 20:11 PDT
Medication Name: **heparin**
Ingredients: heparin 5000 units/mL 1 mL Inj 5000 units 1 mL
Admin Details: (Auth) SubCutaneous, Right Lwr Abdom Quad

Administration Date/Time: 3/30/2026 19:52 PDT
Medication Name: **ipratropium-albuterol (DuoNeb)**
Ingredients: albuterol-ipratropium Inh Sol 3 mL 3 mL
Admin Details: (Auth) NEB

Administration Date/Time: 3/30/2026 18:54 PDT
Medication Name: **labetalol**
Ingredients: labetalol 20 mg/4 mL inj 10 mg 2 mL
Admin Details: (Auth) IV Push, Left Arm
Apical Heart Rate: 98 bpm

Administration Date/Time: 3/30/2026 18:30 PDT
Medication Name: **sodium citrate**
Ingredients: sodcit4%5mL 200 mg 5 mL
Admin Details: (Auth) IntraCatheter

Administration Date/Time: 3/30/2026 17:17 PDT
Medication Name: **sevelamer**
Admin Details: (Not Given) NPO

Administration Date/Time: 3/30/2026 15:45 PDT
Medication Name: **hydrALAZINE**
Admin Details: (Not Given) NPO

Administration Date/Time: 3/30/2026 14:07 PDT
Medication Name: **ipratropium-albuterol (DuoNeb)**
Ingredients: albuterol-ipratropium Inh Sol 3 mL 3 mL
Admin Details: (Auth) NEB
Peripheral Pulse Rate: 88 bpm; Respiratory Rate: 25 br/min

Administration Date/Time: 3/30/2026 12:00 PDT
Medication Name: **sevelamer**
Admin Details: (Not Done) Not Appropriate at this Time
sevelamer

Administration Date/Time: 3/30/2026 11:43 PDT
Medication Name: **iopamidol**
Ingredients: Isovue 370 100ml Per 1ml 85 mL 85 mL
Admin Details: (Auth) IV

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Medication Administration Summary

Medications

Administration Date/Time: 3/30/2026 10:44 PDT
Medication Name: **heparin**
Admin Details: Auth (Verified)
Pharmacist Intervention Time: 1-5 Minutes; Pharmacy Intervention Reduce Cost: Yes; Intervention Type Pharmacy: Anticoagulation Therapy Review; Prescriber Response Pharmacy: Accepted

Administration Date/Time: 3/30/2026 10:42 PDT
Medication Name: **ipratropium-albuterol (DuoNeb)**
Admin Details: Auth (Verified)
Intervention Type Pharmacy: Optimize formulation; Prescriber Response Pharmacy: Accepted; Pharmacist Intervention Time: 1-5 Minutes; Pharmacy Intervention Reduce Cost: Yes

Administration Date/Time: 3/30/2026 10:20 PDT
Medication Name: **Dextrose 50% in Water intravenous solution (Dextrose 50% in Water Injection)**
Admin Details: Auth (Verified)
Medication Administration Reason: Other: Insulin given for hyperkalemia.

Administration Date/Time: 3/30/2026 10:09 PDT
Medication Name: **azithromycin**
Ingredients: azit500REC 500 mg 250 mL
Admin Details: (Auth) IV Piggyback, Left Arm

Administration Date/Time: 3/30/2026 09:49 PDT
Medication Name: **insulin regular (insulin regular 100 units/mL)**
Ingredients: Sodium Chloride 0.9% 10 mL; insureg10ml vial 6 units 0.06 mL
Admin Details: (Auth) IV Push, Left Arm
MAR Glucose Result: 102

Administration Date/Time: 3/30/2026 09:44 PDT
Medication Name: **Dextrose 50% in Water intravenous solution (Dextrose 50% in Water Injection)**
Ingredients: Dextrose 50%/Water 50 mL inj 12.5 gm 25 mL
Admin Details: (Auth) IV Push, Left Arm
MAR Glucose Result: 102

Administration Date/Time: 3/30/2026 09:38 PDT
Medication Name: **calcium gluconate + Sodium Chloride 0.9% intravenous solution 100 mL**
Ingredients: calc1000mgvial 2 gm 20 mL; NS100 100 mL
Admin Details: (Auth) IV Piggyback, Left Arm

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Progress Note - Therapy

DOCUMENT NAME:	Progress Note - Therapy
SERVICE DATE/TIME:	3/30/2026 18:13 PDT
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Joshua,Bethina (3/30/2026 18:15 PDT)
SIGN INFORMATION:	Joshua,Bethina (3/30/2026 18:15 PDT)

Treatment completed, 1.5L removed, pt BP elevated throughout treatment, cath packed with citrate both ports.

Electronically Signed By: Joshua, Bethina
On: 03.30.2026 18:15 PDT

Physical Therapy Forms

DOCUMENT TYPE:	PT Examination - Text
SERVICE DATE/TIME:	4/1/2026 09:32 PDT
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Dampog PT,Lynne Rose (4/1/2026 09:27 PDT)
SIGNED INFORMATION:	Dampog PT,Lynne Rose (4/1/2026 09:27 PDT)

PT Evaluation/Reevaluation Entered On: 4/1/2026 9:37 PDT
Performed On: 4/1/2026 9:32 PDT by Dampog PT, Lynne Rose

General Info
PT Assessment Type : Initial Evaluation
Physical Therapy Orders : PT Evaluation and Treatment - 03/31/26 12:22:00 PDT, Routine, Stop date 03/31/26 12:22:00 PDT
Precautions RTF : Active Orders:
» Precautions, 03/30/26 10:38:00 PDT, Routine, Precautions: Aspiration | Fall, Ordered
» Oxygen Therapy, 03/30/26 9:37:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 between 90 - 95%, Ordered

Discontinued Orders:
» Oxygen Therapy, 03/30/26 7:14:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 92%, Discontinued

Affect/Behavior Rehab : Appropriate, Cooperative
Basic Command Following : Intact
Pain Symptoms : No
Suspected Stroke Patient : No

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

Home Environment
Living Environment : Home Environment
Living Situation: Home with family care Performed By: Caravella SLP, Crystal M 03/31/2026

Physical Therapy Forms

Sensory Deficits: Visual impairment, left eye, Visual impairment, right eye Performed By: Parala RN, Marielle Vianne 03/30/2026
Living Situation : Home with family care
Lives With : Other: brothers, sister

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

Location Bed
Primary Bedroom : 1st floor
Primary Bathroom : 1st floor
Kitchen : 1st floor
Laundry : 1st floor

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

Home Environment II
Living Environment : Home Environment
Living Situation: Home with family care Performed By: Caravella SLP, Crystal M 03/31/2026
Sensory Deficits: Visual impairment, left eye, Visual impairment, right eye Performed By: Parala RN, Marielle Vianne 03/30/2026
Home Equipment Rehab : Hospital bed, Manual wheelchair, Mechanical lift

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

Prior Functional Status Grid
ADL : Dependent
Mobility : Dependent
Instrumental ADL : Dependent
Cognitive-Communication Skills : Independent

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

LE Range/Strength
LE Overall Range of Motion Grid
Left Lower Extremity Passive Range : Within functional limits
Left Lower Extremity Active Range : Within functional limits
Right Lower Extremity Passive Range : Within functional limits
Right Lower Extremity Active Range : Within functional limits

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

Lt Lower Extremity Strength : Other: grossly 3 to 3+/5
Rt Lower Extremity Strength : Other: grossly 3 to 3+/5

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

Balance
Kansas University Balance Scale : Yes
Sitting Balance Score : 1+

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

Mobility
Mobility Grid
Roll Left : Rehab Total assistance
Roll Right : Rehab Total assistance
Supine to Sit : Rehab Total assistance
Sit to Supine : Rehab Total assistance

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

Long Term Goals
PT Patient,Caregiver Goal : -

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

Plan

Physical Therapy Forms

PT Anticipated Treatments, Needs : Discharge/Discontinue PT Treatment
Reason Plan not Established w/ Family : eval only
PT Plan/Goals Established w Pt/Caregiver : No

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

Additional Information
Additional Information PT : THERAPY SESSION TIME: 0854-0932
RN CLEARED FOR THERAPY: Aleja
PAIN LOCATION AND LEVEL: 0/10
ORIENTATION: x 3
PLOF: Lives with brothers and sister, uses hoyer lift from transfers when going to dialysis. DEpendent with ADL's. Per brother pt had HHPT previously and pt was able to take a few steps.

PT orders received, chart reviewed, pt is a90 yo female admitted with c/o SOB. PMHx: ESRD on HD, home 02 2-5L, htn, dementia. Upon arrival, pt was semi-supine in bed. Pt's brother at bedside and assisted with translation. Pt agreeable to PT eval and eval completed. Performed B/L LE AAROM-AROM. Total assist with supine to sit EOB.
At end of session, pt was in semi-fowlers position in bed. All needs met. All safety prec. in place. Pt is at her baseline. No further skilled PT services warranted in acute setting. Pt's brother reports that HHPT stopped seeing pt approx. 6 wks ago and requesting for HHPT to resume again, states he called them but has not heard back.

Environmental Safety Implemented : Bed in low position, Call device within reach, Familiarize patient with environment, Lock wheeled equipment, Personal items within reach, Encourage personal mobility support item use/within reach

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

PT Non Timed Modalities 2026
PT Eval High Complexity Total Number Min : 15
PT Total Non Timed : 15

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

PT Timed Modalities 2026
PT Timed Min Total : 23
PT Timed Unit Total : 2
PT THERAPEUTIC ACT 15MIN : 15
Activity Therapeutic Num of Units : 1
PT THERAPEUTIC EXER 15MIN : 8
Exercise Therapeutic Num of Units : 1

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

Occupational Therapy Forms

DOCUMENT TYPE:	OT Evaluation - Text
SERVICE DATE/TIME:	4/1/2026 09:28 PDT
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Bolor OT,Shelley A (4/1/2026 09:28 PDT)
SIGNED INFORMATION:	Bolor OT,Shelley A (4/1/2026 09:28 PDT)

OT Evaluation/Reevaluation Entered On: 4/1/2026 9:34 PDT

Occupational Therapy Forms

Performed On: 4/1/2026 9:28 PDT by Bolor OT, Shelley A

General Information
OT Assessment Type : Initial Evaluation
Occupational Therapy Orders : OT Evaluation and Treatment - 03/31/26 12:22:00 PDT, Routine, Stop date 03/31/26 12:22:00 PDT
Precautions RTF : Active Orders:
» Precautions, 03/30/26 10:38:00 PDT, Routine, Precautions: Aspiration | Fall, Ordered
» Oxygen Therapy, 03/30/26 9:37:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 between 90 - 95%, Ordered

Discontinued Orders:
» Oxygen Therapy, 03/30/26 7:14:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 92%, Discontinued

Orientation Assessment Rehab : Identifies self
Affect/Behavior Rehab : Appropriate, Calm, Cooperative
Basic Command Following : Intact
Safety/Judgment : Impaired
Pain Symptoms : No
Suspected Stroke Patient : No
Culture/Spiritual Beliefs to Incorporate : Yes

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Home Environment
Living Environment : Home Environment
Living Situation: Home with family care Performed By: Caravella SLP, Crystal M 03/31/2026
Sensory Deficits: Visual impairment, left eye, Visual impairment, right eye Performed By: Parala RN, Marielle Vianne 03/30/2026
Living Situation : Home with family care
Lives With : Other: siblings/family
Lives In : Apartment

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Rehabilitation Stairs Grid
Primary Bedroom : 1st floor
Primary Bathroom : 1st floor
Kitchen : 1st floor
Laundry : 1st floor

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Home Environment II
Living Environment : Home Environment
Living Situation: Home with family care Performed By: Parala RN, Marielle Vianne 03/30/2026
Sensory Deficits: Visual impairment, left eye, Visual impairment, right eye Performed By: Parala RN, Marielle Vianne 03/30/2026
Home Equipment Rehab : Manual wheelchair, Other: hoyer lift

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

ADL : Dependent

Occupational Therapy Forms

Mobility : Dependent
Instrumental ADL : Dependent
Cognitive-Communication Skills : Independent

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Basic ADL
Activities of Daily Living Grid
Grooming : Min A
UE Dressing : Maximal Assistance
LE Dressing : Total Assistance
Toileting : Total Assistance

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Limiting Factors : Weakness

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Balance
General Balance : Sitting
General Balance Assessment : Poor

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

UE Strength/ROM
Upper Extremity Overall ROM Grid
Left Upper Extremity Passive Range : Within functional limits
Left Upper Extremity Active Range : Within functional limits
Right Upper Extremity Passive Range : Within functional limits
Right Upper Extremity Active Range : Within functional limits

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Left Upper Extremity Strength Grid
Scapular Elevation : 3
Shoulder Flexion : 3
Shoulder Extension : 3
Shoulder Abduction : 3
Shoulder Adduction : 3
Shoulder External Rotation : 3
Shoulder Internal Rotation : 3
Elbow Flexion : 3
Elbow Extension : 3
Forearm Pronation : 3
Forearm Supination : 3
Wrist Flexion : 3
Wrist Extension : 3
Hand Grip : 3
Finger Flexion : 3
Finger Extension : 3

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Right Upper Extremity Strength Grid
Scapular Elevation : 3
Shoulder Flexion : 3
Shoulder Extension : 3
Shoulder Abduction : 3
Shoulder Adduction : 3
Shoulder External Rotation : 3

Occupational Therapy Forms

Shoulder Internal Rotation : 3
Elbow Flexion : 3
Elbow Extension : 3
Forearm Pronation : 3
Forearm Supination : 3
Wrist Flexion : 3
Wrist Extension : 3
Hand Grip : 3
Finger Flexion : 3
Finger Extension : 3

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Mobility
Mobility Grid
Roll Left : Rehab Total assistance
Roll Right : Rehab Total assistance
Supine to Sit : Rehab Total assistance
Sit to Supine : Rehab Total assistance

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

OT Assessment
Impairments/Limitations : Basic activity of daily living deficits, Mobility deficits, Strength deficits

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Education
Teaching Method : Demonstration, Explanation
Learner(s) Present : Self

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Occupational Therapy Education Grid
Activity of Daily Living Training : Verbalizes understanding, Demonstrates
Bed Positioning : Verbalizes understanding, Demonstrates

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Long Term Goals
OT Patient/Caregiver Goal : n/a

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Long Term Goal 5

	Long Term Goal 1
Goal :	OT IE only
	Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Plan
OT Anticipated Treatments, Needs : Discharge/Discontinue OT treatment
Post-Acute Discharge Therapy Recommendation OT : No further skilled therapy services warranted
IDR OT Recommend/Goal : no further OT services rec post d/c
OT Evaluation Charge : Yes
OT Tx Plan/Goals Established w Patient : Yes

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Additional Information

Occupational Therapy Forms

Additional Information OT : RN CLEARED FOR THERAPY: Aleja
PAIN LOCATION AND LEVEL: none
ORIENTATION: AxOx3
PLOF: Dependent

OT orders received, chart reviewed, pt is admitted for hypoxemia. Upon arrival, pt was in semi supine. Pt. brother, David, translates for pt (Vietnamese).Pt agreeable to OT eval and eval completed. See ADL/mobility grid for details.
At end of session, pt was returned to supine c all needs within reach and RN aware.

BARRIERS TO SAFE DC/IMPAIRMENTS AND LIMITATIONS: n/a
THERAPY RECOMMENDATION: no further OT services rec-- family care at home to resume

Patient Position : Semi-Fowler's
Environmental Safety Implemented : Adequate room lighting, Bed in low position, Call device within reach, Familiarize patient with environment, Lock wheeled equipment, Personal items within reach, Encourage personal mobility support item use/within reach, Encourage sensory support item use/within reach

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

OT Non Timed Modalities 2026
OT Treatment Start Time : 4/1/2026 8:53 PDT
OT Treatment End Time : 4/1/2026 9:31 PDT
Eval High Complexity Total Number Min : 15
OT Total NonTimed Mod : 15

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

OT Timed Modalities 2026
OT Time Treatment Started : 4/1/2026 8:53 PDT
OT Time Treatment Ended : 4/1/2026 9:31 PDT
Rehab Patient : No
OT Timed Min Total : 23
OT Timed Units Total : 2
OT SELF CARE MGT TRAIN 15M : 23
OT SELF CARE MGT TRAIN Num of Units : 2

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Speech Therapy Forms

DOCUMENT TYPE:	Clinical Swallowing - Text
SERVICE DATE/TIME:	3/31/2026 08:15 PDT
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Caravella SLP,Crystal M (3/31/2026 08:58 PDT)
SIGNED INFORMATION:	Caravella SLP,Crystal M (3/31/2026 08:58 PDT)

ST Bedside Swallow Eval Entered On: 3/31/2026 9:09 PDT
Performed On: 3/31/2026 8:15 PDT by Caravella SLP, Crystal M

Speech Therapy Forms

General Information
Speech Orders : ST Bedside Swallow Exam - 03/31/26 7:11:00 PDT, Bedside Swallow Exam, Routine, Stop date 03/31/26 7:11:00 PDT, 20179621233.0
Speech Therapy Eval and Treat - 03/31/26 7:11:00 PDT, Bedside Swallow Exam, Routine, Stop date 03/31/26 7:11:00 PDT
ST Bedside Swallow Exam - 03/30/26 15:26:00 PDT, Bedside Swallow Exam, Routine, Stop date 03/30/26 15:26:00 PDT, 20176418327.0
Speech Therapy Eval and Treat - 03/30/26 15:26:00 PDT, Bedside Swallow Exam, Routine, Stop date 03/30/26 15:26:00 PDT
Precautions RTF : Active Orders:
» Precautions, 03/30/26 10:38:00 PDT, Routine, Precautions: Aspiration | Fall, Ordered
» Oxygen Therapy, 03/30/26 9:37:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 between 90 - 95%, Ordered

Discontinued Orders:
» Oxygen Therapy, 03/30/26 7:14:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 92%, Discontinued

History of Swallow Studies : No qualifying data available
Level of Consciousness Rehab : Alert
Affect/Behavior Rehab : Flat
Basic Command Following : Impaired
Orientation Assessment Rehab : Identifies self, Not oriented to situation, Not oriented to time
Pain Interfering With SLP Treatment : No

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Home Environment
Living Environment : Home Environment
Living Situation: Home with family care Performed By: Parala RN, Marielle Vianne 03/30/2026
Sensory Deficits: Visual impairment, left eye, Visual impairment, right eye Performed By: Parala RN, Marielle Vianne 03/30/2026
Living Situation : Home with family care

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Pre-Assessment
Oxygen Therapy : Nasal cannula
SpO2 : 97 %
Respiratory Comments : 3LNC
Tracheostomy Information : Oxygen Flow Rate: 2 L/min Performed by: Rapelje RRT, Glenn 03/31/26 07:41:00
FIO2: 30 (Modified) Performed by: Parala RN, Marielle Vianne 03/31/26 00:00:00

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Oral-Facial Exam
Facial Movement : Within functional limits
Dentition : Own teeth, Missing teeth

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Oral Structure Grid
Labial Structure : Within functional limits
Mandible : Within functional limits
Lingual Structure : Within functional limits

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Oral Function Grid

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Speech Therapy Forms

Labial : Within functional limits
Mandible : Within functional limits
Lingual : Within functional limits
Velopharyngeal : Within functional limits

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Secretion Management : Adequate

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Swallowing Exam
Patient Position : Upright 90 degrees
Reflexive Cough : Absent

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Liquids Swallowing Exam Adult Grid

	Ice Chips	Thin Liquid by Half Teaspoon	Thin Liquid by Bottle	Thin Liquid by Cup
Presentation :	Speech-Language Pathologist	Speech-Language Pathologist	Self, Speech-Language Pathologist	Speech-Language Pathologist
Oral Symptoms :	None observed	None observed	None observed	None observed
Pharyngeal Symptoms :	None observed	None observed	None observed	None observed
	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Consistency Swallowing Exam Adult Grid

	Puree Consistency	Soft Consistency	Regular Consistency
Presentation :	Speech-Language Pathologist	Speech-Language Pathologist	Self
Oral Symptoms :	None observed	None observed	Oral residue, Slowed oral transit time
Pharyngeal Symptoms :	None observed	None observed	None observed
	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Assessment/Recommendations
Clinical Swallowing Impression Grid
Within Functional Limits : Liquids, Solids

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Diet Consistency : IDDSI 6 Soft and Bite Sized
Liquid Viscosity : Thin
Medications : Whole with drink

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Speech Therapy Forms

Supervision : 1:1
Positioning : Upright 90 degrees
Safe Feeding/Swallow Precautions : Remain upright after meals 30 minutes, Small bites of food
Clinical Swallowing Recommendations : Dysphagia treatment

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Education
Learner(s) Present for Session : Explanation
Learner(s) Present : Self
Barriers to Learning : Language barrier, Acuity of Illness, Cognitive deficits, Emotional state, Vision impairment

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Document learning evaluation for Responsible Learner(s)
Diet Restrictions/Recommendations : Needs practice/supervision
Feeding Strategies : Needs practice/supervision
Speech Language/Pathology Tx Plan : Needs practice/supervision
Swallowing Strategies : Needs practice/supervision

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Long Term Goals
SLP Patient, Caregiver Goals : "I want more cookies"

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Cognitive-Communication LTG Grid

	SLP Long Term Goal #1
Goal :	Pt will consume thin liquids via cup/straw and hard solid foods without evidence of pulmonary compromise
Status :	Initial
	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Short Term Goals
Clinical Swallowing STG Grid

	Goal #1	Goal #2
Type :	Thin liquid by straw	Soft solid consistency
Presentation :	Therapist	Therapist
Total Amount :	Whole meal	Whole meal
Descriptors :	No clinical signs of aspiration	No clinical signs of aspiration
Status :	Initial	Initial
	Caravella SLP, Crystal M -	Caravella SLP, Crystal M -

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Speech Therapy Forms

	3/31/2026 8:58 PDT	3/31/2026 8:58 PDT

Plan
Planned Treatments : Dysphagia therapy
Frequency : 1-2 times per week
Duration : 1-2 weeks
SLP Intensity Per Day : 30-60 minutes
Post-Acute Discharge Therapy Recommendation ST : Continued intermittent therapy to address functional deficits
Rehabilitation Potential : Good
Plan/Goals Established With Patient/Caregiver : Yes

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Additional Information
Additional Information SLP : Bedside swallow eval completed. Pt found in bed awake and alert. Pt asking to eat/drink and states that she is hungry. Pt admitted d/t SOB from home with family. Pt bedbound at home, hx of Dementia. Pt HOH and with visual deficits. Pt speaks both Vietnamese and English. Pt able to follow approx 50% simple directives during exam today. Pt found to be oriented x 2 and to year.

Pt presented with ice chips and thin liquid via tsp/cup/straw, puree, softened and hard solids. Pt assisted with feeding d/t visual deficits. Good anterior hyolaryngeal elevation appreicated during swallows with timely swallow initiations. No overt s/o aspiration observed with trials. Pt unable to follow directives for 3 oz water challenge. Pt with slow mastication of hard solid and trace to mild oral residue d/t missing dentition. No difficulty observed with softened solids.

Pt was able to take whole pills one at a time with RN after she had been cleared for a diet. No pocketing of pills noted post swallows.

Recommend:
Soft and bite sized diet with Thin liquids, 1:1 assist, pills whole 1 at a time
ST to f/u in tx

D/w nurse Aleja

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

ST Non Timed Modalities 2026
ST Treatment Start Time : 3/31/2026 8:15 PDT
ST Treatment End Time : 3/31/2026 9:09 PDT
ST Eval Oral/Swallow Func Minutes : 15
ST Total Non Timed : 15

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

ST Timed Modalities 2026
ST Timed Min Total : 39
ST Timed Units Total : 3
ST THERAP EXERCISE-15M : 39
ST THERAP EXERCISE Units : 3

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Speech Therapy

DOCUMENT TYPE:	Clinical Swallowing - Text
SERVICE DATE/TIME:	3/31/2026 08:15 PDT
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Caravella SLP,Crystal M (3/31/2026 08:58 PDT)
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Speech Therapy Eval and Treat - 03/30/26 15:26:00 PDT, Bedside Swallow Exam, Routine, Stop date 03/30/26 15:26:00 PDT
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» Precautions, 03/30/26 10:38:00 PDT, Routine, Precautions: Aspiration | Fall, Ordered
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Discontinued Orders:
» Oxygen Therapy, 03/30/26 7:14:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 92%, Discontinued

History of Swallow Studies : No qualifying data available
Level of Consciousness Rehab : Alert
Affect/Behavior Rehab : Flat
Basic Command Following : Impaired
Orientation Assessment Rehab : Identifies self, Not oriented to situation, Not oriented to time
Pain Interfering With SLP Treatment : No

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Home Environment
Living Environment : Home Environment
Living Situation: Home with family care Performed By: Parala RN, Marielle Vianne 03/30/2026
Sensory Deficits: Visual impairment, left eye, Visual impairment, right eye Performed By: Parala RN, Marielle Vianne 03/30/2026
Living Situation : Home with family care

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Pre-Assessment
Oxygen Therapy : Nasal cannula
SpO2 : 97 %
Respiratory Comments : 3LNC

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Speech Therapy

Tracheostomy Information : Oxygen Flow Rate: 2 L/min Performed by: Rapelje RRT, Glenn 03/31/26 07:41:00
FIO2: 30 (Modified) Performed by: Parala RN, Marielle Vianne 03/31/26 00:00:00

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Oral-Facial Exam
Facial Movement : Within functional limits
Dentition : Own teeth, Missing teeth

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Oral Structure Grid
Labial Structure : Within functional limits
Mandible : Within functional limits
Lingual Structure : Within functional limits

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Oral Function Grid
Labial : Within functional limits
Mandible : Within functional limits
Lingual : Within functional limits
Velopharyngeal : Within functional limits

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Secretion Management : Adequate

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Swallowing Exam
Patient Position : Upright 90 degrees
Reflexive Cough : Absent

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Liquids Swallowing Exam Adult Grid

	Ice Chips	Thin Liquid by Half Teaspoon	Thin Liquid by Bottle	Thin Liquid by Cup
Presentation :	Speech-Language Pathologist	Speech-Language Pathologist	Self, Speech-Language Pathologist	Speech-Language Pathologist
Oral Symptoms :	None observed	None observed	None observed	None observed
Pharyngeal Symptoms :	None observed	None observed	None observed	None observed
	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Consistency Swallowing Exam Adult Grid

	Puree Consistency	Soft Consistency	Regular Consistency
Presentation :	Speech-Language Pathologist	Speech-Language Pathologist	Self
Oral Symptoms :	None observed	None observed	Oral residue, Slowed oral transit time

Speech Therapy

Pharyngeal Symptoms :	None observed	None observed	None observed
	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Assessment/Recommendations
Clinical Swallowing Impression Grid
Within Functional Limits : Liquids, Solids
Caravella SLP, Crystal M - 3/31/2026 8:58 PDT
Diet Consistency : IDDSI 6 Soft and Bite Sized
Liquid Viscosity : Thin
Medications : Whole with drink
Supervision : 1:1
Positioning : Upright 90 degrees
Safe Feeding/Swallow Precautions : Remain upright after meals 30 minutes, Small bites of food
Clinical Swallowing Recommendations : Dysphagia treatment
Caravella SLP, Crystal M - 3/31/2026 8:58 PDT
Education
Learner(s) Present for Session : Explanation
Learner(s) Present : Self
Barriers to Learning : Language barrier, Acuity of Illness, Cognitive deficits, Emotional state, Vision impairment
Caravella SLP, Crystal M - 3/31/2026 8:58 PDT
Document learning evaluation for Responsible Learner(s)
Diet Restrictions/Recommendations : Needs practice/supervision
Feeding Strategies : Needs practice/supervision
Speech Language/Pathology Tx Plan : Needs practice/supervision
Swallowing Strategies : Needs practice/supervision
Caravella SLP, Crystal M - 3/31/2026 8:58 PDT
Long Term Goals
SLP Patient, Caregiver Goals : "I want more cookies"
Caravella SLP, Crystal M - 3/31/2026 8:58 PDT
Cognitive-Communication LTG Grid

	SLP Long Term Goal #1
Goal :	Pt will consume thin liquids via cup/straw and hard solid foods without evidence of pulmonary compromise
Status :	Initial
	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Speech Therapy

Short Term Goals
Clinical Swallowing STG Grid

	Goal #1	Goal #2
Type :	Thin liquid by straw	Soft solid consistency
Presentation :	Therapist	Therapist
Total Amount :	Whole meal	Whole meal
Descriptors :	No clinical signs of aspiration	No clinical signs of aspiration
Status :	Initial	Initial
	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Plan
Planned Treatments : Dysphagia therapy
Frequency : 1-2 times per week
Duration : 1-2 weeks
SLP Intensity Per Day : 30-60 minutes
Post-Acute Discharge Therapy Recommendation ST : Continued intermittent therapy to address functional deficits
Rehabilitation Potential : Good
Plan/Goals Established With Patient/Caregiver : Yes

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Additional Information
Additional Information SLP : Bedside swallow eval completed. Pt found in bed awake and alert. Pt asking to eat/drink and states that she is hungry. Pt admitted d/t SOB from home with family. Pt bedbound at home, hx of Dementia. Pt HOH and with visual deficits. Pt speaks both Vietnamese and English. Pt able to follow approx 50% simple directives during exam today. Pt found to be oriented x 2 and to year.

Pt presented with ice chips and thin liquid via tsp/cup/straw, puree, softened and hard solids. Pt assisted with feeding d/t visual deficits. Good anterior hyolaryngeal elevation appreicated during swallows with timely swallow initiations. No overt s/o aspiration observed with trials. Pt unable to follow directives for 3 oz water challenge. Pt with slow mastication of hard solid and trace to mild oral residue d/t missing dentition. No difficulty observed with softened solids.

Pt was able to take whole pills one at a time with RN after she had been cleared for a diet. No pocketing of pills noted post swallows.

Recommend:
Soft and bite sized diet with Thin liquids, 1:1 assist, pills whole 1 at a time
ST to f/u in tx

D/w nurse Aleja

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

ST Non Timed Modalities 2026
ST Treatment Start Time : 3/31/2026 8:15 PDT
ST Treatment End Time : 3/31/2026 9:09 PDT

Speech Therapy

ST Eval Oral/Swallow Func Minutes : 15
ST Total Non Timed : 15

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

ST Timed Modalities 2026
ST Timed Min Total : 39
ST Timed Units Total : 3
ST THERAP EXERCISE-15M : 39
ST THERAP EXERCISE Units : 3

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Progress Note - Physician

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	4/1/2026 15:54 PDT
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Joshi MD,Dhiresh R (4/1/2026 15:57 PDT)
SIGN INFORMATION:	Joshi MD,Dhiresh R (4/1/2026 15:57 PDT)

Subjective
No fever, stable, on nasal cannula

Allergies
Allergies (Active and Proposed Allergies Only)
No Known Allergies (Severity: Unknown severity, Onset: Unknown)

Past Medical History
Active Problems(3)
Hemodialysis
Hepatitis A virus
High cholesterol

Objective

Vital Signs (24 hrs)	Last Charted		Minimum		Maximum	
Temp	36.7	04/01/2026 08:00	36.7	04/01/2026 08:00	37	03/31/2026 16:00
Apical Heart Rate	85	04/01/2026 15:18	84	04/01/2026 05:36	**H** 102	03/31/2026 21:41
Resp Rate	20	04/01/2026 12:20	15	04/01/2026 05:00	**C** 37	03/31/2026 18:00
SBP	**H** 174	04/01/2026 06:01	**H** 147	03/31/2026 17:52	**C** 187	04/01/2026 02:37
DBP	77	04/01/2026 06:01	67	03/31/2026 16:00	**H** 82	04/01/2026 00:01
MAP	110	04/01/2026 06:01	97	03/31/2026 16:00	118	04/01/2026 00:01
SpO2	100	04/01/2026 12:20	**C** 84	03/31/2026 16:00	100	03/31/2026 18:29
O2 Flow Rate	on 2 l/min		on 2 l/min		on 2 l/min	
O2 Therapy	Nasal cannula		Nasal cannula		Nasal cannula	

Progress Note - Physician

Ventilator Settings (Last Within 24hrs)
Ventilator Mode: SPN-BiPAP (02:02)
Inspiratory Pressure: 5 cm H2O (02:02)
Expiratory Pressure: 8 cm H2O (02:02)

Precautions
Aspiration Precautions | Fall Precautions

Physical Exam
Neck: No obvious neck masses
Respiratory: Clear to auscultation bilaterally. No wheezing, crackles or rhonchi. Normal respiratory effort.
Cardiovascular: Regular rate and rhythm, no murmurs, gallops or rubs.
Gastrointestinal: Non-tender, non-distended. Normal bowel sounds.
Genitourinary: No suprapubic tenderness.

Inpatient Medications
Medications (25) Active
Scheduled: (12)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 10 mg Tab (amLODIPine) 10 mg 1 Tabs, Oral, Daily
cefTRIAXone/D5W (Rocephin) 50 mL, IV Piggyback, q24H Interval
doxycycline/D5W (doxycycline) 100 mg 100 mL, IV Piggyback, q12H
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
heparin 5000 units/mL injection 1 mL (heparin) 5,000 units 1 mL, SubCutaneous, q12H
hydrALAZINE 50 mg Tab (hydrALAZINE) 50 mg 1 Tabs, Oral, TID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
losartan 100 mg Tab (losartan) 100 mg 1 Tabs, Oral, Daily
metoprolol succinate 50 mg XL Tab (Metoprolol Succinate ER) 50 mg 1 Tabs, Oral, qHS
sevelamer carbonate 800 mg Tab (sevelamer) 800 mg 1 Tabs, Oral, TID w/Meals
spironolactone 25 mg Tab (spironolactone) 25 mg 1 Tabs, Oral, BID
Continuous: (0)
PRN: (13)
acetaminophen 325 mg Tab (Tylenol) 650 mg 2 Tabs, Oral, q4H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q3H
benzonatate 100 mg Cap (Tessalon Perles) 100 mg 1 Caps, Oral, TID
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID
Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 25 gm 50 mL, IV Push, q1H Interval
docusate sodium 100 mg Cap (Colace) 100 mg 1 Caps, Oral, BID
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H
glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q2H Interval
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q2H Interval
melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H
sodium citrate 4% soln 5 mL (sodium citrate) 200 mg 5 mL, IntraCatheter, As Directed

Progress Note - Physician

72 hour Antibiotic History

Active Antibiotics	Day of Therapy:	Last Administered	First Administered
cefTRIAXone 2 gm, 50 mL, 100 mL/hr, IV Piggyback, q24H Interval	2	04/01/2026 13:14	03/31/2026 10:13
doxycycline 100 mg, 100 mL, 100 mL/hr, IV Piggyback, q12H	3	04/01/2026 07:58	03/30/2026 21:13

Completed/Discontinued Antibiotics	Stop Date/Time	Last Administered	First Administered
azithromycin 500 mg, 250 mL, 250 mL/hr, IV Piggyback, Once	03/30/2026 10:10	03/30/2026 10:09	03/30/2026 10:09
cefTRIAXone 1 gm, 50 mL, 100 mL/hr, IV Piggyback, Once	03/30/2026 09:18	03/30/2026 09:10	03/30/2026 09:10

Results
Recent Labs
General Chemistry

Glucose Level	98 mg/dL (Normal)	04/01/2026 10:27
POC Glucose	75 mg/dL (Normal)	03/31/2026 07:11
Sodium	141 mmol/L (Normal)	04/01/2026 10:27
Potassium	3.8 mmol/L (Normal)	04/01/2026 10:27
Chloride	101 mmol/L (Normal)	04/01/2026 10:27
CO2	30 mmol/L (Normal)	04/01/2026 10:27
Anion Gap	10 mmol/L (Normal)	04/01/2026 10:27
BUN	20 mg/dL (High)	04/01/2026 10:27
Creatinine	2.26 mg/dL (High)	04/01/2026 10:27
BUN/Creat Ratio	9 (Normal)	04/01/2026 10:27
Calcium	8.9 mg/dL (Normal)	04/01/2026 10:27
Albumin. Level	3.3 gm/dL (Normal)	04/01/2026 10:27
TP	6.1 gm/dL (Normal)	04/01/2026 10:27
A/G Ratio	1.1 (Normal)	04/01/2026 10:27
T Bili	0.3 mg/dL (Normal)	04/01/2026 10:27
D Bili	0.1 mg/dL (Normal)	04/01/2026 10:27
Alk Phos	118 units/L (High)	04/01/2026 10:27
AST	35 units/L (Normal)	04/01/2026 10:27
ALT	36 units/L (Normal)	04/01/2026 10:27
Mg Lvl	1.9 mg/dL (Normal)	04/01/2026 10:27
Phosphorus	4.4 mg/dL (Normal)	04/01/2026 10:27
Estimated Creatinine Clearance	9.79 mL/min ()	04/01/2026 11:02

Progress Note - Physician

eGFR Cr	20 mL/min/1.73m2 (N/A)	04/01/2026 10:27
eGFR Pediatric	Not Reported (N/A)	04/01/2026 10:27
Iron	77 mcg/dL (Normal)	03/31/2026 13:21
Total Iron Binding Capacity	148 mcg/dL (Low)	03/31/2026 13:21
Transferrin Saturation	52 % (High)	03/31/2026 13:21
Folate Lvl	21.85 ng/mL (High)	03/31/2026 13:21
Vitamin B12 Lvl	479 pg/mL (Normal)	03/31/2026 13:21
Ferritin Lvl	1244.0 ng/mL (High)	03/31/2026 13:21

General Hematology

WBC	5.23 x10e3/mcL (Normal)	04/01/2026 06:12
RBC	3.57 x10e6/mcL (Low)	04/01/2026 06:12
Hgb	8.3 gm/dL (Low)	04/01/2026 06:12
Hct	25.2 % (Low)	04/01/2026 06:12
MCV	70.6 Femtoliters (Low)	04/01/2026 06:12
MCH	23.2 pg (Low)	04/01/2026 06:12
MCHC	32.9 gm/dL (Normal)	04/01/2026 06:12
RDW-SD	45.5 Femtoliters (Normal)	04/01/2026 06:12
RDW-CV	18.2 % (High)	04/01/2026 06:12
Plt	164 x10e3/mcL (Normal)	04/01/2026 06:12
MPV	see comment Femtoliters (N/A)	04/01/2026 06:12
Imm Plt Fract	3.8 % (Normal)	04/01/2026 06:12
Neut % Auto	74.4 % (N/A)	04/01/2026 06:12
Lymph % Auto	14.9 % (N/A)	04/01/2026 06:12
Mono % Auto	5.9 % (N/A)	04/01/2026 06:12
Eos % Auto	3.6 % (N/A)	04/01/2026 06:12
Baso % Auto	0.8 % (N/A)	04/01/2026 06:12
Immature Grans %	0.4 % (Normal)	04/01/2026 06:12
Neut # Auto	3.89 x10e3/mcL (Normal)	04/01/2026 06:12
Lymph # Auto	0.78 x10e3/mcL (Low)	04/01/2026 06:12
Mono # Auto	0.31 x10e3/mcL (Normal)	04/01/2026 06:12
Eos # Auto	0.19 x10e3/mcL (Normal)	04/01/2026 06:12
Baso # Auto	0.04 x10e3/mcL (Normal)	04/01/2026 06:12

Immature Grans # Auto	0.02 x10e3/mcL (Normal)	04/01/2026 06:12
Retic %	0.68 % (Normal)	03/31/2026 13:21
Retic Abs	2.41 x10e4/mcL (Normal)	03/31/2026 13:21
Imm Retic Frac%	12.6 % (High)	03/31/2026 13:21

Hepatitis

Hep A Total Ab	Reactive (Abnormal)	03/31/2026 13:21

Infectious Disease

Flu A RNA	Negative (Normal)	03/30/2026 11:15
Flu B RNA	Negative (Normal)	03/30/2026 11:15
Influenza B	Not Detected (Normal)	03/31/2026 01:48
Influenza A	Not Detected (Normal)	03/31/2026 01:48
RSV	Not Detected (Normal)	03/31/2026 01:48

Other Microbiology

MRSA by PCR	Not Detected (Normal)	03/31/2026 01:48
Adenovirus	Not Detected (Normal)	03/31/2026 01:48
Coronavirus 229E	Not Detected (Normal)	03/31/2026 01:48
Coronavirus HKU1	Not Detected (Normal)	03/31/2026 01:48
Coronavirus NL63	Not Detected (Normal)	03/31/2026 01:48
Coronavirus OC43	Not Detected (Normal)	03/31/2026 01:48
Human Metapneumovirus	Not Detected (Normal)	03/31/2026 01:48
Human Rhinovirus/Enterovirus	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 1	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 2	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 3	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 4	Not Detected (Normal)	03/31/2026 01:48
Bordetella pertussis	Not Detected (Normal)	03/31/2026 01:48
Bordetella parapertussis	Not Detected (Normal)	03/31/2026 01:48
Chlamydia pneumoniae	Not Detected (Normal)	03/31/2026 01:48
Mycoplasma pneumoniae	Not Detected (Normal)	03/31/2026 01:48

SARS-CoV-2/Covid-19

SARS-CoV2 RNA	Negative (Normal)	03/30/2026 11:15
SARS-CoV-2	Not Detected (Normal)	03/31/2026 01:48

Progress Note - Physician

Differential (Last Within 24hrs)
- Automated -
Neut % Auto: 74.4 % (06:12)
Lymph % Auto: 14.9 % (06:12)
Mono % Auto: 5.9 % (06:12)
Eos % Auto: 3.6 % (06:12)
Baso % Auto: 0.8 % (06:12)
Immature Grans %: 0.4 % (06:12)
Neut # Auto: 3.89 x10e3/mcL (06:12)
Lymph # Auto: 0.78 x10e3/mcL Low (06:12)
Mono # Auto: 0.31 x10e3/mcL (06:12)
Baso # Auto: 0.04 x10e3/mcL (06:12)
Immature Grans # Auto: 0.02 x10e3/mcL (06:12)

Microbiology

Culture Blood ARD **Preliminary**
Source: Blood Body Site:
Collected Dt/Tm: 03/31/2026 02:12 Last Updated Dt/Tm: 04/01/2026 08:01

No growth at 24 hours.

Culture Blood ARD **Preliminary**
Source: Blood Body Site:
Collected Dt/Tm: 03/31/2026 02:04 Last Updated Dt/Tm: 04/01/2026 08:01

No growth at 24 hours.

Culture Sputum **Completed**
Source: Sputum Expectorated Body Site:
Collected Dt/Tm: 03/30/2026 22:34 Last Updated Dt/Tm: 03/31/2026 12:42

>10 epithelial cells /lpf
>25 White Blood Cells /lpf
Many gram positive cocci in pairs
Many Gram Negative Rods
Few Gram Negative Coccobacilli
Few Gram Positive Rods
Gram stain contains >=10 squamous epithelial cells per low power field which suggests oropharyngeal and/or nasopharyngeal contamination with no predominant bacterial morphology. Culture will not be performed. Notified: ALAJA RUIZ,RN 3/31/2026 12:27:14 PP

Progress Note - Physician

Culture Blood ARD **Preliminary**

Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	04/01/2026 09:01

No growth at 48 hours.

Culture Blood ARD **Preliminary**

Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	04/01/2026 00:22

Morganella morganii
Identification obtained by Sepsityper rapid identification method.
Susceptibility to follow.

Assessment/Plan
Diagnoses
Acute on chronic hypoxic respiratory failure (J96.21)
Acute pulmonary edema (J81.0)
Elevated troponin (R79.89)
End-stage renal disease (N18.6)
Hyperkalemia (E87.5)
Hyperkalemia (E87.5)
Pneumonia (J18.9)
Sepsis (A41.9)
1. Hypoxemia (R09.02)
Leukocytosis, resolving
Morganella bacteremia
Bilateral pleural effusion
Hepatitis A, IgM positive

Continue ceftriaxone pending sensitivity of Morganella
Adjust per creatinine clearance
Aspiration precautions
Aggressive pulmonary hygiene
Monitor LFTs
Repeat blood cultures
Echo
Patient has permacath
If repeat cultures comes back positive will need catheter change
D/W RN

Electronically Signed By: Joshi, Dhiresh MD
On: 04.01.2026 15:57 PDT

Progress Note - Physician

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	4/1/2026 13:55 PDT
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Vornicu MD,Maria D (4/1/2026 13:55 PDT)
SIGN INFORMATION:	Vornicu MD,Maria D (4/1/2026 13:55 PDT)

Subjective
End-stage renal disease on dialysis
Patient seen and examined during dialysis
Tolerating well
Discussed with dialysis staff
Dialysis parameters reviewed
Blood pressure elevated
Just received medications
Blood culture positive for morganella 1 out of 2

Chief Complaint/Reason for Consultation
Chief Complaint: Patient brought in by EMS from home with complaints of SOB. Per the family, patient started to complain of SOB this morning. Patient found with a RA sat of <50%. Patient provided with BVM ventilation and then placed on NRB. Patient A/Ox0. EMS unable to obtain demographics on scene. BGL 119

Allergies
Allergies (Active and Proposed Allergies Only)
No Known Allergies (Severity: Unknown severity, Onset: Unknown)

Objective

Measurements (most recent)

Height (cm)	157.48 cm	Erickson RD, LD, Lauren	03/31 11:07
Height (inches)	62.00 inch	SYSTEM	03/31 11:16
Weight	41.40 kg	Parala RN, Marielle Vianne	04/01 06:13
Weight (lbs)	91.27 lb	SYSTEM	04/01 06:13
Body Mass Index	15.12 kg/m2	Parala RN, Marielle Vianne	04/01 06:13
Scale Type	Bed scale	Parala RN, Marielle Vianne	04/01 06:13

Vital Signs (24 hrs)	**Last Charted**		**Minimum**		**Maximum**	
Temp	36.7	04/01/2026 08:00	36.7	04/01/2026 08:00	37	03/31/2026 16:00
Apical Heart Rate	84	04/01/2026 05:36	84	04/01/2026 05:36	**H** 102	03/31/2026 21:41
Resp Rate	20	04/01/2026 12:20	15	04/01/2026 05:00	**C** 37	03/31/2026 18:00
SBP	**H** 174	04/01/2026 06:01	131	03/31/2026 14:06	**C** 187	04/01/2026 02:37
DBP	77	04/01/2026 06:01	60	03/31/2026 14:06	**H** 82	04/01/2026 00:01
MAP	110	04/01/2026 06:01	86	03/31/2026 14:06	118	04/01/2026 00:01
SpO2	100	04/01/2026 12:20	**C** 84	03/31/2026 16:00	100	03/31/2026 18:29

Progress Note - Physician

O2 Flow Rate	on 2 l/min	on 2 l/min	on 2 l/min
O2 Therapy	Nasal cannula	Nasal cannula	Nasal cannula

Ventilator Settings (Last Within 24hrs)
Ventilator Mode: SPN-BiPAP (02:02)
Inspiratory Pressure: 5 cm H2O (02:02)
Expiratory Pressure: 8 cm H2O (02:02)

Intake and Output (current encounter)

	03/30 07:06 04/01 13:55	04/01 07:00 04/01 13:55	03/31 07:00 04/01 06:59	03/30 07:00 03/31 06:59	03/29 07:00 03/30 06:59
Intake	1713.3	300	1113.3	300	0
Output	3300	1500	0	1800	0
Net Total	-1586.7	-1200	1113.3	-1500	0
Urine Count	1	1	0	0	0
Stool Count	2	1	0	1	0

Physical Exam

Inpatient Medications
Medications (25) Active
Scheduled: (12)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 10 mg Tab (amLODIPine) 10 mg 1 Tabs, Oral, Daily
cefTRIAXone/D5W (Rocephin) 50 mL, IV Piggyback, q24H Interval
doxycycline/D5W (doxycycline) 100 mg 100 mL, IV Piggyback, q12H
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
heparin 5000 units/mL injection 1 mL (heparin) 5,000 units 1 mL, SubCutaneous, q12H
hydrALAZINE 50 mg Tab (hydrALAZINE) 50 mg 1 Tabs, Oral, TID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
losartan 100 mg Tab (losartan) 100 mg 1 Tabs, Oral, Daily
metoprolol succinate 50 mg XL Tab (Metoprolol Succinate ER) 50 mg 1 Tabs, Oral, qHS
sevelamer carbonate 800 mg Tab (sevelamer) 800 mg 1 Tabs, Oral, TID w/Meals
spironolactone 25 mg Tab (spironolactone) 25 mg 1 Tabs, Oral, BID
Continuous: (0)
PRN: (13)
acetaminophen 325 mg Tab (Tylenol) 650 mg 2 Tabs, Oral, q4H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q3H
benzonatate 100 mg Cap (Tessalon Perles) 100 mg 1 Caps, Oral, TID
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID
Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 25 gm 50 mL, IV Push, q1H Interval
docusate sodium 100 mg Cap (Colace) 100 mg 1 Caps, Oral, BID
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H

Progress Note - Physician

glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q2H Interval
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q2H Interval
melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H
sodium citrate 4% soln 5 mL (sodium citrate) 200 mg 5 mL, IntraCatheter, As Directed

72 hour Antibiotic History

Active Antibiotics	Day of Therapy:	Last Administered	First Administered
cefTRIAXone 2 gm, 50 mL, 100 mL/hr, IV Piggyback, q24H Interval	2	04/01/2026 13:14	03/31/2026 10:13
doxycycline 100 mg, 100 mL, 100 mL/hr, IV Piggyback, q12H	3	04/01/2026 07:58	03/30/2026 21:13

Completed/Discontinued Antibiotics	Stop Date/Time	Last Administered	First Administered
azithromycin 500 mg, 250 mL, 250 mL/hr, IV Piggyback, Once	03/30/2026 10:10	03/30/2026 10:09	03/30/2026 10:09
cefTRIAXone 1 gm, 50 mL, 100 mL/hr, IV Piggyback, Once	03/30/2026 09:18	03/30/2026 09:10	03/30/2026 09:10

Results
Recent Labs
General Chemistry

Glucose Level	98 mg/dL (Normal)	04/01/2026 10:27
POC Glucose	75 mg/dL (Normal)	03/31/2026 07:11
Sodium	141 mmol/L (Normal)	04/01/2026 10:27
Potassium	3.8 mmol/L (Normal)	04/01/2026 10:27
Chloride	101 mmol/L (Normal)	04/01/2026 10:27
CO2	30 mmol/L (Normal)	04/01/2026 10:27
Anion Gap	10 mmol/L (Normal)	04/01/2026 10:27
BUN	20 mg/dL (High)	04/01/2026 10:27
Creatinine	2.26 mg/dL (High)	04/01/2026 10:27
BUN/Creat Ratio	9 (Normal)	04/01/2026 10:27
Calcium	8.9 mg/dL (Normal)	04/01/2026 10:27
Albumin. Level	3.3 gm/dL (Normal)	04/01/2026 10:27
TP	6.1 gm/dL (Normal)	04/01/2026 10:27
A/G Ratio	1.1 (Normal)	04/01/2026 10:27
T Bili	0.3 mg/dL (Normal)	04/01/2026 10:27

Progress Note - Physician

D Bili	0.1 mg/dL (Normal)	04/01/2026 10:27
Alk Phos	118 units/L (High)	04/01/2026 10:27
AST	35 units/L (Normal)	04/01/2026 10:27
ALT	36 units/L (Normal)	04/01/2026 10:27
Mg Lvl	1.9 mg/dL (Normal)	04/01/2026 10:27
Phosphorus	4.4 mg/dL (Normal)	04/01/2026 10:27
Estimated Creatinine Clearance	9.79 mL/min ()	04/01/2026 11:02
eGFR Cr	20 mL/min/1.73m2 (N/A)	04/01/2026 10:27
eGFR Pediatric	Not Reported (N/A)	04/01/2026 10:27
Iron	77 mcg/dL (Normal)	03/31/2026 13:21
Total Iron Binding Capacity	148 mcg/dL (Low)	03/31/2026 13:21
Transferrin Saturation	52 % (High)	03/31/2026 13:21
Folate Lvl	21.85 ng/mL (High)	03/31/2026 13:21
Vitamin B12 Lvl	479 pg/mL (Normal)	03/31/2026 13:21
Ferritin Lvl	1244.0 ng/mL (High)	03/31/2026 13:21

General Hematology

WBC	5.23 x10e3/mcL (Normal)	04/01/2026 06:12
RBC	3.57 x10e6/mcL (Low)	04/01/2026 06:12
Hgb	8.3 gm/dL (Low)	04/01/2026 06:12
Hct	25.2 % (Low)	04/01/2026 06:12
MCV	70.6 Femtoliters (Low)	04/01/2026 06:12
MCH	23.2 pg (Low)	04/01/2026 06:12
MCHC	32.9 gm/dL (Normal)	04/01/2026 06:12
RDW-SD	45.5 Femtoliters (Normal)	04/01/2026 06:12
RDW-CV	18.2 % (High)	04/01/2026 06:12
Plt	164 x10e3/mcL (Normal)	04/01/2026 06:12
MPV	see comment Femtoliters (N/A)	04/01/2026 06:12
Imm Plt Fract	3.8 % (Normal)	04/01/2026 06:12
Neut % Auto	74.4 % (N/A)	04/01/2026 06:12
Lymph % Auto	14.9 % (N/A)	04/01/2026 06:12
Mono % Auto	5.9 % (N/A)	04/01/2026 06:12
Eos % Auto	3.6 % (N/A)	04/01/2026 06:12
Baso % Auto	0.8 % (N/A)	04/01/2026 06:12
Immature Grans %	0.4 % (Normal)	04/01/2026 06:12

Progress Note - Physician

Neut # Auto	3.89 x10e3/mcL (Normal)	04/01/2026 06:12
Lymph # Auto	0.78 x10e3/mcL (Low)	04/01/2026 06:12
Mono # Auto	0.31 x10e3/mcL (Normal)	04/01/2026 06:12
Eos # Auto	0.19 x10e3/mcL (Normal)	04/01/2026 06:12
Baso # Auto	0.04 x10e3/mcL (Normal)	04/01/2026 06:12
Immature Grans # Auto	0.02 x10e3/mcL (Normal)	04/01/2026 06:12
Retic %	0.68 % (Normal)	03/31/2026 13:21
Retic Abs	2.41 x10e4/mcL (Normal)	03/31/2026 13:21
Imm Retic Frac%	12.6 % (High)	03/31/2026 13:21

Hepatitis

Hep A Total Ab	Reactive (Abnormal)	03/31/2026 13:21

Infectious Disease

Flu A RNA	Negative (Normal)	03/30/2026 11:15
Flu B RNA	Negative (Normal)	03/30/2026 11:15
Influenza B	Not Detected (Normal)	03/31/2026 01:48
Influenza A	Not Detected (Normal)	03/31/2026 01:48
RSV	Not Detected (Normal)	03/31/2026 01:48

Other Microbiology

MRSA by PCR	Not Detected (Normal)	03/31/2026 01:48
Adenovirus	Not Detected (Normal)	03/31/2026 01:48
Coronavirus 229E	Not Detected (Normal)	03/31/2026 01:48
Coronavirus HKU1	Not Detected (Normal)	03/31/2026 01:48
Coronavirus NL63	Not Detected (Normal)	03/31/2026 01:48
Coronavirus OC43	Not Detected (Normal)	03/31/2026 01:48
Human Metapneumovirus	Not Detected (Normal)	03/31/2026 01:48
Human Rhinovirus/Enterovirus	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 1	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 2	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 3	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 4	Not Detected (Normal)	03/31/2026 01:48
Bordetella pertussis	Not Detected (Normal)	03/31/2026 01:48

Bordetella parapertussis	Not Detected (Normal)	03/31/2026 01:48
Chlamydia pneumoniae	Not Detected (Normal)	03/31/2026 01:48
Mycoplasma pneumoniae	Not Detected (Normal)	03/31/2026 01:48

SARS-CoV-2/Covid-19

SARS-CoV2 RNA	Negative (Normal)	03/30/2026 11:15
SARS-CoV-2	Not Detected (Normal)	03/31/2026 01:48

BMP, Mg, and Phos (Last Within 24hrs)
Sodium: 141 mmol/L (10:27)
Potassium: 3.8 mmol/L (10:27)
Chloride: 101 mmol/L (10:27)
CO2: 30 mmol/L (10:27)
BUN: 20 mg/dL High (10:27)
Creatinine: 2.26 mg/dL High (10:27)
Glucose Level: 98 mg/dL (10:27)
Calcium: 8.9 mg/dL (10:27)
Mg Lvl: 1.9 mg/dL (10:27)
Phosphorus: 4.4 mg/dL (10:27)

LFT (Last Within 24hrs)
AST: 35 units/L (10:27)
ALT: 36 units/L (10:27)
Alk Phos: 118 units/L High (10:27)
T Bili: 0.3 mg/dL (10:27)
D Bili: 0.1 mg/dL (10:27)
TP: 6.1 gm/dL (10:27)

Microbiology

Culture Blood ARD **Preliminary**

Source: Blood Body Site:
Collected Dt/Tm: 03/31/2026 02:12 Last Updated Dt/Tm: 04/01/2026 08:01

No growth at 24 hours.

Culture Blood ARD **Preliminary**

Source: Blood Body Site:
Collected Dt/Tm: 03/31/2026 02:04 Last Updated Dt/Tm: 04/01/2026 08:01

No growth at 24 hours.

Progress Note - Physician

Culture Sputum **Completed**

Source:	Sputum Expectorated	Body Site:	
Collected Dt/Tm:	03/30/2026 22:34	Last Updated Dt/Tm:	03/31/2026 12:42

>10 epithelial cells /lpf
>25 White Blood Cells /lpf
Many gram positive cocci in pairs
Many Gram Negative Rods
Few Gram Negative Coccobacilli
Few Gram Positive Rods
Gram stain contains >=10 squamous epithelial cells per low power field which suggests oropharyngeal and/or nasopharyngeal contamination with no predominant bacterial morphology. Culture will not be performed. Notified: ALAJA RUIZ,RN 3/31/2026 12:27:14 PP

Culture Blood ARD **Preliminary**

Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	04/01/2026 09:01

No growth at 48 hours.

Culture Blood ARD **Preliminary**

Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	04/01/2026 00:22

Morganella morganii
Identification obtained by Sepsityper rapid identification method.
Susceptibility to follow.

Assessment/Plan
Assessment:
Physical Exam
General: Thin appearing elderly female
Mental Status: Baseline dementia
Head: Normocephalic, atraumatic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact. Anicteric.
Ear, Nose and Throat: Oropharynx clear, moist mucous membranes. Trachea midline.
Neck: No obvious neck masses
Respiratory: Clear to auscultation bilaterally. No wheezing, crackles or rhonchi. Normal respiratory effort.
Cardiovascular: Regular rate and rhythm, no murmurs, gallops or rubs.
Gastrointestinal: Non-tender, non-distended. Normal bowel sounds.
Genitourinary: No suprapubic tenderness.
Neurologic: Cranial nerves II-XII grossly intact. Moves all extremities.

Progress Note - Physician

Light touch sensation is grossly intact bilaterally.

End-stage renal disease on hemodialysis Monday Wednesday Friday via IJ permacath
Hyperkalemia
Hypertension
Shortness of breath
Pneumonia
Bacteremia

Plan
Seen and examined during dialysis today
Tolerating well
Monitor daily for dialysis needs
Potassium controlled with dialysis
Continue amlodipine hydralazine metoprolol
Continue sevelamer. Monitor phosphorus level
Antibiotics. Follow-up cultures
Discussed with ID. Will hold off permacath exchange for now

Discharge Planning:
» Discharge Planning:
» Discharge To, Anticipated: Home with family care

Electronically Signed By: Vornicu, Maria MD
On: 04.01.2026 13:55 PDT

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	4/1/2026 13:27 PDT
RESULT STATUS:	Modified
PERFORM INFORMATION:	Delmundo DO,Lloyd (4/1/2026 15:43 PDT); Kim DO,Andrew H (4/1/2026 13:28 PDT)
SIGN INFORMATION:	Delmundo DO,Lloyd (4/1/2026 15:43 PDT); Kim DO,Andrew H (4/1/2026 13:28 PDT)

Addendum by Delmundo DO, Lloyd on April 01, 2026 15:42:43 PDT
I discussed with resident/fellow at length. I was present for critical and key services and am participating in patient management, in combination with the resident or fellow.
Agree with assessment and plan

on 2 liters nc
hd
echo normal efu
abx
IS
PTOT
control bp

L. Del Mundo, DO

Progress Note - Physician

Pulmonary/Critical Care

Electronically Signed By: Delmundo, Lloyd DO
On: 04.01.2026 15:43 PDT

Subjective
no acute events overnight
on 2L NC sat well
vitals otherwise stable
appears comfortable
sitting in bed

Chief Complaint/Reason for Consultation
Chief Complaint: Patient brought in by EMS from home with complaints of SOB. Per the family, patient started to complain of SOB this morning. Patient found with a RA sat of <50%. Patient provided with BVM ventilation and then placed on NRB. Patient A/Ox0. EMS unable to obtain demographics on scene. BGL 119

Past Medical History
Active Problems(3)
Hemodialysis
Hepatitis A virus
High cholesterol

Social History
No social history documented.

Family History
No Family History documented.

Objective

Measurements (most recent)

Height (cm)	157.48 cm	Erickson RD, LD, Lauren	03/31 11:07
Height (inches)	62.00 inch	SYSTEM	03/31 11:16
Weight	41.40 kg	Parala RN, Marielle Vianne	04/01 06:13
Weight (lbs)	91.27 lb	SYSTEM	04/01 06:13
Body Mass Index	15.12 kg/m2	Parala RN, Marielle Vianne	04/01 06:13
Scale Type	Bed scale	Parala RN, Marielle Vianne	04/01 06:13

Vital Signs (24 hrs)	**Last Charted**		**Minimum**		**Maximum**	
Temp	36.7	04/01/2026 08:00	36.7	04/01/2026 08:00	37	03/31/2026 16:00
Apical Heart Rate	84	04/01/2026 05:36	84	04/01/2026 05:36	**H** 102	03/31/2026 21:41
Resp Rate	20	04/01/2026 12:20	15	04/01/2026 05:00	**C** 39	03/31/2026 13:53

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Progress Note - Physician

SBP	**H** 174	04/01/2026 06:01	131	03/31/2026 14:06	**C** 187	04/01/2026 02:37
DBP	77	04/01/2026 06:01	60	03/31/2026 14:06	**H** 82	04/01/2026 00:01
MAP	110	04/01/2026 06:01	86	03/31/2026 14:06	118	04/01/2026 00:01
SpO2	100	04/01/2026 12:20	**C** 84	03/31/2026 16:00	100	03/31/2026 13:53
O2 Flow Rate	on 2 l/min		on 2 l/min			
O2 Therapy	Nasal cannula		Nasal cannula			

Perfusion Assessment

Capillary Refill Left Foot: Less than 2 seconds (04/01/26 08:00:00)

Capillary Refill Left Hand: Less than 2 seconds (04/01/26 08:00:00)

Capillary Refill Right Foot: Less than 2 seconds (04/01/26 08:00:00)

Capillary Refill Right Hand: Less than 2 seconds (04/01/26 08:00:00)

Dorsalis Pedis Pulse, Left: 1+ Barely detectable/thready (04/01/26 08:00:00)

Dorsalis Pedis Pulse, Right: 1+ Barely detectable/thready (04/01/26 08:00:00)

Posttibial Pulse, Left: 1+ Barely detectable/thready (03/31/26 20:00:00)

Posttibial Pulse, Right: 1+ Barely detectable/thready (03/31/26 20:00:00)

Radial Pulse, Left: 2+ Slightly diminished but greater than 1+ (04/01/26 08:00:00)

Radial Pulse, Right: 2+ Slightly diminished but greater than 1+ (04/01/26 08:00:00)

Pain Scores (Last Within 24hrs)
Numeric Pain Scale: Unable to obtain score; sleeping (eyes closed), no distress (10:00)

Ventilator Settings (Last Within 24hrs)

Ventilator Mode: SPN-BiPAP (02:02)

Inspiratory Pressure: 5 cm H2O (02:02)

Expiratory Pressure: 8 cm H2O (02:02)

Intake and Output (current encounter)

	03/30 07:06	04/01 07:00	03/31 07:00	03/30 07:00	03/29 07:00
	04/01 13:27	04/01 13:27	04/01 06:59	03/31 06:59	03/30 06:59
Intake	1413.3	0	1113.3	300	0

Progress Note - Physician

Output	1800	0	0	1800	0
Net Total	-386.7	0	1113.3	-1500	0
Urine Count	1	1	0	0	0
Stool Count	2	1	0	1	0

Stroke Assessments
Glasgow Coma Scale
Eye Opening Response Glasgow: Spontaneously (04/01/26 08:00:00)
Best Verbal Response Glasgow: Disoriented and converses (04/01/26 08:00:00)
Best Motor Response Glasgow: Obeys commands (04/01/26 08:00:00)
Glasgow Coma Score: 14 (04/01/26 08:00:00)

Physical Exam

Home Medications
amLODIPine (amLODIPine 10 mg oral tablet)
hydrALAZINE (hydrALAZINE 50 mg oral tablet)
losartan (losartan 100 mg oral tablet)
metoprolol (metoprolol succinate 50 mg oral tablet, extended release)
minoxidil (minoxidil 2.5 mg oral tablet)
multivitamin (Rena-Vite oral tablet)
ondansetron (ondansetron 4 mg oral tablet)
sevelamer (sevelamer 800 mg oral tablet)
zolpidem (zolpidem 5 mg oral tablet)

Inpatient Medications
Medications (25) Active
Scheduled: (12)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 10 mg Tab (amLODIPine) 10 mg 1 Tabs, Oral, Daily
cefTRIAXone/D5W (Rocephin) 50 mL, IV Piggyback, q24H Interval
doxycycline/D5W (doxycycline) 100 mg 100 mL, IV Piggyback, q12H
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
heparin 5000 units/mL injection 1 mL (heparin) 5,000 units 1 mL, SubCutaneous, q12H
hydrALAZINE 50 mg Tab (hydrALAZINE) 50 mg 1 Tabs, Oral, TID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
losartan 100 mg Tab (losartan) 100 mg 1 Tabs, Oral, Daily
metoprolol succinate 50 mg XL Tab (Metoprolol Succinate ER) 50 mg 1 Tabs, Oral, qHS
sevelamer carbonate 800 mg Tab (sevelamer) 800 mg 1 Tabs, Oral, TID w/Meals
spironolactone 25 mg Tab (spironolactone) 25 mg 1 Tabs, Oral, BID
Continuous: (0)
PRN: (13)
acetaminophen 325 mg Tab (Tylenol) 650 mg 2 Tabs, Oral, q4H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q3H
benzonatate 100 mg Cap (Tessalon Perles) 100 mg 1 Caps, Oral, TID
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID

Progress Note - Physician

Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 25 gm 50 mL, IV Push, q1H Interval
docusate sodium 100 mg Cap (Colace) 100 mg 1 Caps, Oral, BID
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H
glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q2H Interval
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q2H Interval
melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H
sodium citrate 4% soln 5 mL (sodium citrate) 200 mg 5 mL, IntraCatheter, As Directed

72 hour Antibiotic History

Active Antibiotics	Day of Therapy:	Last Administered	First Administered
cefTRIAXone 2 gm, 50 mL, 100 mL/hr, IV Piggyback, q24H Interval	2	04/01/2026 13:14	03/31/2026 10:13
doxycycline 100 mg, 100 mL, 100 mL/hr, IV Piggyback, q12H	3	04/01/2026 07:58	03/30/2026 21:13

Completed/Discontinued Antibiotics	Stop Date/Time	Last Administered	First Administered
azithromycin 500 mg, 250 mL, 250 mL/hr, IV Piggyback, Once	03/30/2026 10:10	03/30/2026 10:09	03/30/2026 10:09
cefTRIAXone 1 gm, 50 mL, 100 mL/hr, IV Piggyback, Once	03/30/2026 09:18	03/30/2026 09:10	03/30/2026 09:10

Results
Recent Labs
General Chemistry

Glucose Level	98 mg/dL (Normal)	04/01/2026 10:27
POC Glucose	75 mg/dL (Normal)	03/31/2026 07:11
Sodium	141 mmol/L (Normal)	04/01/2026 10:27
Potassium	3.8 mmol/L (Normal)	04/01/2026 10:27
Chloride	101 mmol/L (Normal)	04/01/2026 10:27
CO2	30 mmol/L (Normal)	04/01/2026 10:27
Anion Gap	10 mmol/L (Normal)	04/01/2026 10:27
BUN	20 mg/dL (High)	04/01/2026 10:27
Creatinine	2.26 mg/dL (High)	04/01/2026 10:27
BUN/Creat Ratio	9 (Normal)	04/01/2026 10:27

Progress Note - Physician

Calcium	8.9 mg/dL (Normal)	04/01/2026 10:27
Albumin. Level	3.3 gm/dL (Normal)	04/01/2026 10:27
TP	6.1 gm/dL (Normal)	04/01/2026 10:27
A/G Ratio	1.1 (Normal)	04/01/2026 10:27
T Bili	0.3 mg/dL (Normal)	04/01/2026 10:27
D Bili	0.1 mg/dL (Normal)	04/01/2026 10:27
Alk Phos	118 units/L (High)	04/01/2026 10:27
AST	35 units/L (Normal)	04/01/2026 10:27
ALT	36 units/L (Normal)	04/01/2026 10:27
Mg Lvl	1.9 mg/dL (Normal)	04/01/2026 10:27
Phosphorus	4.4 mg/dL (Normal)	04/01/2026 10:27
Estimated Creatinine Clearance	9.79 mL/min ()	04/01/2026 11:02
eGFR Cr	20 mL/min/1.73m2 (N/A)	04/01/2026 10:27
eGFR Pediatric	Not Reported (N/A)	04/01/2026 10:27
Iron	77 mcg/dL (Normal)	03/31/2026 13:21
Total Iron Binding Capacity	148 mcg/dL (Low)	03/31/2026 13:21
Transferrin Saturation	52 % (High)	03/31/2026 13:21
Folate Lvl	21.85 ng/mL (High)	03/31/2026 13:21
Vitamin B12 Lvl	479 pg/mL (Normal)	03/31/2026 13:21
Ferritin Lvl	1244.0 ng/mL (High)	03/31/2026 13:21

General Hematology

WBC	5.23 x10e3/mcL (Normal)	04/01/2026 06:12
RBC	3.57 x10e6/mcL (Low)	04/01/2026 06:12
Hgb	8.3 gm/dL (Low)	04/01/2026 06:12
Hct	25.2 % (Low)	04/01/2026 06:12
MCV	70.6 Femtoliters (Low)	04/01/2026 06:12
MCH	23.2 pg (Low)	04/01/2026 06:12
MCHC	32.9 gm/dL (Normal)	04/01/2026 06:12
RDW-SD	45.5 Femtoliters (Normal)	04/01/2026 06:12
RDW-CV	18.2 % (High)	04/01/2026 06:12
Plt	164 x10e3/mcL (Normal)	04/01/2026 06:12
MPV	see comment Femtoliters (N/A)	04/01/2026 06:12
Imm Plt Fract	3.8 % (Normal)	04/01/2026 06:12
Neut % Auto	74.4 % (N/A)	04/01/2026 06:12

Progress Note - Physician

Lymph % Auto	14.9 % (N/A)	04/01/2026 06:12
Mono % Auto	5.9 % (N/A)	04/01/2026 06:12
Eos % Auto	3.6 % (N/A)	04/01/2026 06:12
Baso % Auto	0.8 % (N/A)	04/01/2026 06:12
Immature Grans %	0.4 % (Normal)	04/01/2026 06:12
Neut # Auto	3.89 x10e3/mcL (Normal)	04/01/2026 06:12
Lymph # Auto	0.78 x10e3/mcL (Low)	04/01/2026 06:12
Mono # Auto	0.31 x10e3/mcL (Normal)	04/01/2026 06:12
Eos # Auto	0.19 x10e3/mcL (Normal)	04/01/2026 06:12
Baso # Auto	0.04 x10e3/mcL (Normal)	04/01/2026 06:12
Immature Grans # Auto	0.02 x10e3/mcL (Normal)	04/01/2026 06:12
Retic %	0.68 % (Normal)	03/31/2026 13:21
Retic Abs	2.41 x10e4/mcL (Normal)	03/31/2026 13:21
Imm Retic Frac%	12.6 % (High)	03/31/2026 13:21

Hepatitis

Hep A Total Ab	Reactive (Abnormal)	03/31/2026 13:21

Infectious Disease

Flu A RNA	Negative (Normal)	03/30/2026 11:15
Flu B RNA	Negative (Normal)	03/30/2026 11:15
Influenza B	Not Detected (Normal)	03/31/2026 01:48
Influenza A	Not Detected (Normal)	03/31/2026 01:48
RSV	Not Detected (Normal)	03/31/2026 01:48

Other Microbiology

MRSA by PCR	Not Detected (Normal)	03/31/2026 01:48
Adenovirus	Not Detected (Normal)	03/31/2026 01:48
Coronavirus 229E	Not Detected (Normal)	03/31/2026 01:48
Coronavirus HKU1	Not Detected (Normal)	03/31/2026 01:48
Coronavirus NL63	Not Detected (Normal)	03/31/2026 01:48
Coronavirus OC43	Not Detected (Normal)	03/31/2026 01:48
Human Metapneumovirus	Not Detected (Normal)	03/31/2026 01:48
Human Rhinovirus/Enterovirus	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 1	Not Detected (Normal)	03/31/2026 01:48

Parainfluenza Virus 2	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 3	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 4	Not Detected (Normal)	03/31/2026 01:48
Bordetella pertussis	Not Detected (Normal)	03/31/2026 01:48
Bordetella parapertussis	Not Detected (Normal)	03/31/2026 01:48
Chlamydia pneumoniae	Not Detected (Normal)	03/31/2026 01:48
Mycoplasma pneumoniae	Not Detected (Normal)	03/31/2026 01:48

SARS-CoV-2/Covid-19

SARS-CoV2 RNA	Negative (Normal)	03/30/2026 11:15
SARS-CoV-2	Not Detected (Normal)	03/31/2026 01:48

Abnormal Labs

General Chemistry

BUN	20 mg/dL (High)	04/01/2026 10:27
Creatinine	2.26 mg/dL (High)	04/01/2026 10:27
Alk Phos	118 units/L (High)	04/01/2026 10:27
Total Iron Binding Capacity	148 mcg/dL (Low)	03/31/2026 13:21
Transferrin Saturation	52 % (High)	03/31/2026 13:21
Folate Lvl	21.85 ng/mL (High)	03/31/2026 13:21
Ferritin Lvl	1244.0 ng/mL (High)	03/31/2026 13:21

General Hematology

RBC	3.57 x10e6/mcL (Low)	04/01/2026 06:12
Hgb	8.3 gm/dL (Low)	04/01/2026 06:12
Hct	25.2 % (Low)	04/01/2026 06:12
MCV	70.6 Femtoliters (Low)	04/01/2026 06:12
MCH	23.2 pg (Low)	04/01/2026 06:12
RDW-CV	18.2 % (High)	04/01/2026 06:12
MPV	see comment Femtoliters (N/A)	04/01/2026 06:12
Lymph # Auto	0.78 x10e3/mcL (Low)	04/01/2026 06:12
Imm Retic Frac%	12.6 % (High)	03/31/2026 13:21

Hepatitis

Hep A Total Ab	Reactive (Abnormal)	03/31/2026 13:21

Progress Note - Physician

CBC, BMP, Coagulation Trend (last 4 resulted)

WBC	**5.23** 04/01/2026 06:12	**5.13** 03/31/2026 13:21	**5.87** 03/31/2026 02:04	**12.25 H** 03/30/2026 07:31
Hgb	**8.3 L** 04/01/2026 06:12	**8.3 L** 03/31/2026 13:21	**8.4 L** 03/31/2026 02:04	**11.2** 03/30/2026 07:31
Hct	**25.2 L** 04/01/2026 06:12	**26.0 L** 03/31/2026 13:21	**25.6 L** 03/31/2026 02:04	**33.7** 03/30/2026 07:31
Baso # Auto	**0.04** 04/01/2026 06:12	**0.04** 03/31/2026 02:04	**0.03** 03/30/2026 07:31	
Baso % Auto	**0.8** 04/01/2026 06:12	**0.7** 03/31/2026 02:04	**0.2** 03/30/2026 07:31	
Eos # Auto	**0.19** 04/01/2026 06:12	**0.12** 03/31/2026 02:04	**0.02** 03/30/2026 07:31	
Eos % Auto	**3.6** 04/01/2026 06:12	**2.0** 03/31/2026 02:04	**0.2** 03/30/2026 07:31	
Immature Grans %	**0.4** 04/01/2026 06:12	**0.5** 03/31/2026 02:04	**0.6** 03/30/2026 07:31	
Immature Grans # Auto	**0.02** 04/01/2026 06:12	**0.03** 03/31/2026 02:04	**0.07** 03/30/2026 07:31	
Lymph # Auto	**0.78 L** 04/01/2026 06:12	**0.73 L** 03/31/2026 02:04	**1.33** 03/30/2026 07:31	
Lymph % Auto	**14.9** 04/01/2026 06:12	**12.4** 03/31/2026 02:04	**10.9** 03/30/2026 07:31	
Mono # Auto	**0.31** 04/01/2026 06:12	**0.22** 03/31/2026 02:04	**0.47** 03/30/2026 07:31	
Mono % Auto	**5.9** 04/01/2026 06:12	**3.7** 03/31/2026 02:04	**3.8** 03/30/2026 07:31	
Neut # Auto	**3.89** 04/01/2026 06:12	**4.73** 03/31/2026 02:04	**10.33 H** 03/30/2026 07:31	
Neut % Auto	**74.4** 04/01/2026 06:12	**80.7** 03/31/2026 02:04	**84.3** 03/30/2026 07:31	
Retic Abs	**2.41** 03/31/2026 13:21			
Retic %	**0.68** 03/31/2026 13:21			
Plt	**164** 04/01/2026 06:12	**158** 03/31/2026 13:21	**141** 03/31/2026 02:04	**230** 03/30/2026 07:31
Glucose Level	**98** 04/01/2026 10:27	**72 L** 03/31/2026 02:04	**189 H** 03/30/2026 07:31	
Sodium	**141** 04/01/2026 10:27	**140** 03/31/2026 02:04	**139** 03/30/2026 07:31	
Potassium	**3.8** 04/01/2026 10:27	**4.2** 03/31/2026 02:04	**5.9 H** 03/30/2026 07:31	
Chloride	**101** 04/01/2026 10:27	**100** 03/31/2026 02:04	**94 L** 03/30/2026 07:31	

Progress Note - Physician

CO2	**30** 04/01/2026 10:27	**31** 03/31/2026 02:04	**27** 03/30/2026 07:31	
Creatinine	**2.26 H** 04/01/2026 10:27	**2.19 H** 03/31/2026 02:04	**4.80 H** 03/30/2026 07:31	
BUN	**20 H** 04/01/2026 10:27	**17** 03/31/2026 02:04	**44 H** 03/30/2026 07:31	
BUN/Creat Ratio	**9** 04/01/2026 10:27	**8** 03/31/2026 02:04	**9** 03/30/2026 07:31	
Mg Lvl	**1.9** 04/01/2026 10:27	**1.9** 03/31/2026 02:04		
Phosphorus	**4.4** 04/01/2026 10:27	**5.6 H** 03/31/2026 02:04	**10.5 H** 03/30/2026 13:08	
Calcium	**8.9** 04/01/2026 10:27	**9.0** 03/31/2026 02:04	**8.6 L** 03/30/2026 07:31	
Albumin. Level	**3.2** 04/01/2026 10:27	**3.3** 04/01/2026 10:27	**3.4** 03/31/2026 02:04	**3.6** 03/30/2026 22:34
T Bili	**0.3** 04/01/2026 10:27	**0.8** 03/30/2026 22:34	**0.4** 03/30/2026 11:15	
D Bili	**0.1** 04/01/2026 10:27	**0.2** 03/30/2026 22:34	**0.2** 03/30/2026 11:15	
Alk Phos	**118 H** 04/01/2026 10:27	**125 H** 03/30/2026 22:34	**150 H** 03/30/2026 11:15	
AST	**35** 04/01/2026 10:27	**97 H** 03/30/2026 22:34	**57 H** 03/30/2026 11:15	
ALT	**36** 04/01/2026 10:27	**49** 03/30/2026 22:34	**51** 03/30/2026 11:15	

Blood Glucose Trend
Glucose Level: 98 mg/dL (04/01/26 10:27:00)

CBC (Last Within 24hrs)
WBC: 5.23 x10e3/mcL (06:12)
Hgb: 8.3 gm/dL Low (06:12)
Hct: 25.2 % Low (06:12)
Plt: 164 x10e3/mcL (06:12)

Differential (Last Within 24hrs)
- Automated -
Neut % Auto: 74.4 % (06:12)
Lymph % Auto: 14.9 % (06:12)
Mono % Auto: 5.9 % (06:12)
Eos % Auto: 3.6 % (06:12)
Baso % Auto: 0.8 % (06:12)
Immature Grans %: 0.4 % (06:12)

Neut # Auto: 3.89 x10e3/mcL (06:12)
Lymph # Auto: 0.78 x10e3/mcL Low (06:12)
Mono # Auto: 0.31 x10e3/mcL (06:12)
Baso # Auto: 0.04 x10e3/mcL (06:12)
Immature Grans # Auto: 0.02 x10e3/mcL (06:12)

BMP, Mg, and Phos (Last Within 24hrs)
Sodium: 141 mmol/L (10:27)
Potassium: 3.8 mmol/L (10:27)
Chloride: 101 mmol/L (10:27)
CO2: 30 mmol/L (10:27)
BUN: 20 mg/dL High (10:27)
Creatinine: 2.26 mg/dL High (10:27)
Glucose Level: 98 mg/dL (10:27)
Calcium: 8.9 mg/dL (10:27)
Mg Lvl: 1.9 mg/dL (10:27)
Phosphorus: 4.4 mg/dL (10:27)

LFT (Last Within 24hrs)
AST: 35 units/L (10:27)
ALT: 36 units/L (10:27)
Alk Phos: 118 units/L High (10:27)
T Bili: 0.3 mg/dL (10:27)
D Bili: 0.1 mg/dL (10:27)
TP: 6.1 gm/dL (10:27)

Microbiology

Culture Blood ARD **Preliminary**

Source:	Blood	Body Site:	
Collected Dt/Tm:	03/31/2026 02:12	Last Updated Dt/Tm:	04/01/2026 08:01

No growth at 24 hours.

Culture Blood ARD **Preliminary**

Source:	Blood	Body Site:	
Collected Dt/Tm:	03/31/2026 02:04	Last Updated Dt/Tm:	04/01/2026 08:01

No growth at 24 hours.

Culture Sputum **Completed**

Source:	Sputum Expectorated	Body Site:	
Collected Dt/Tm:	03/30/2026 22:34	Last Updated Dt/Tm:	03/31/2026 12:42

Progress Note - Physician

>10 epithelial cells /lpf
>25 White Blood Cells /lpf
Many gram positive cocci in pairs
Many Gram Negative Rods
Few Gram Negative Coccobacilli
Few Gram Positive Rods
Gram stain contains >=10 squamous epithelial cells per low power field which suggests oropharyngeal and/or nasopharyngeal contamination with no predominant bacterial morphology. Culture will not be performed. Notified: ALAJA RUIZ,RN 3/31/2026 12:27:14 PP

Culture Blood ARD		**Preliminary**	
Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	04/01/2026 09:01

No growth at 48 hours.

Culture Blood ARD		**Preliminary**	
Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	04/01/2026 00:22

Morganella morganii
Identification obtained by Sepsityper rapid identification method.
Susceptibility to follow.

Cardiology Labs
BNP: 2004 pg/mL High (03/30/26 07:31:00)
Troponin TNIH: 328.77 ng/L Critical (03/30/26 22:34:00)
Troponin TNIH: 379.71 ng/L Critical (03/30/26 17:36:00)
Troponin TNIH: 292.67 ng/L Critical (03/30/26 13:08:00)

Assessment/Plan
Assessment:
Physical Exam
General: Thin appearing minimi speaking elderly female
Mental Status: Alert and oriented to person, place, time. Normal affect.
Head: Normocephalic, atraumatic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact. Anicteric.
Ear, Nose and Throat: Oropharynx clear, moist mucous membranes. Trachea midline.
Neck: No obvious neck masses
Respiratory: Clear to auscultation bilaterally. No wheezing, crackles or rhonchi. Normal respiratory effort.
Cardiovascular: Regular rate and rhythm, no murmurs, gallops or rubs.
Gastrointestinal: Non-tender, non-distended. Normal bowel sounds.
Genitourinary: No suprapubic tenderness.

Progress Note - Physician

Neurologic: Cranial nerves II-XII grossly intact. Moves all extremities.
Light touch sensation is grossly intact bilaterally.

Assessment
Acute hypoxic respiratory failure 2/2 pneumonia versus fluid overload
Dyspnea
Sepsis with RBC, HR 2/2 respiratory source
Troponinemia 2/2 demand ischemia
ESRD on HD MWF
Hyperkalemia 2/2 above
Oxygen dependent reportedly on 2 to 5 L at home
Hypertension
Dementia

Recommendations
Currently on 2 L nasal cannula, Wean as tolerated
Goal SpO2 > 92%
Uses O2 at home at 2-3L, so she is at baseline
Hypoxia has improved after HD
Nephrology consulted, HD per nephrology
Monitor I's and O strictly
Currently on ceftriaxone and doxycycline
Echo done, EF 65-70% LV normal
Breathing treatments prn
Encourage IS use
PT/OT as tolerated
Rest of care per primary team

Discharge Planning:
» Discharge Planning:
» Discharge To, Anticipated: Home with family care

Electronically Signed By: Kim, Andrew DO
On: 04.01.2026 13:28 PDT

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	4/1/2026 07:11 PDT
RESULT STATUS:	Unauth
PERFORM INFORMATION:	Hirabayashi DO,Ellyn (4/1/2026 07:11 PDT)
SIGN INFORMATION:	Hirabayashi DO,Ellyn (4/1/2026 12:15 PDT)

Subjective

Chief Complaint/Reason for Consultation
Chief Complaint: Patient brought in by EMS from home with complaints of SOB. Per the family, patient started to complain of SOB this morning. Patient found with a RA sat of <50%. Patient provided with BVM ventilation and then placed on NRB. Patient A/Ox0. EMS unable to obtain demographics on scene. BGL 119

Past Medical History

Progress Note - Physician

Active Problems(3)
Hemodialysis
Hepatitis A virus
High cholesterol

Past Surgical History
No surgery history documented.

Social History
No social history documented.

Objective

Measurements (most recent)

Height (cm)	157.48 cm	Erickson RD, LD, Lauren	03/31 11:07
Height (inches)	62.00 inch	SYSTEM	03/31 11:16
Weight	41.40 kg	Parala RN, Marielle Vianne	04/01 06:13
Weight (lbs)	91.27 lb	SYSTEM	04/01 06:13
Body Mass Index	15.12 kg/m2	Parala RN, Marielle Vianne	04/01 06:13
Scale Type	Bed scale	Parala RN, Marielle Vianne	04/01 06:13

Vital Signs (24 hrs)	**Last Charted**		**Minimum**		**Maximum**	
Temp	37.1	04/01/2026 00:00	**L** 36.3	03/31/2026 08:00	37.2	03/31/2026 12:00
Apical Heart Rate	84	04/01/2026 05:36	84	04/01/2026 05:36	**H** 102	03/31/2026 21:41
Resp Rate	18	04/01/2026 06:58	14	03/31/2026 08:59	**C** 39	03/31/2026 13:53
SBP	**H** 174	04/01/2026 06:01	126	03/31/2026 12:06	**C** 187	04/01/2026 02:37
DBP	77	04/01/2026 06:01	60	03/31/2026 14:06	**C** 99	03/31/2026 12:06
MAP	110	04/01/2026 06:01	86	03/31/2026 14:06	118	04/01/2026 00:01
SpO2	99	04/01/2026 06:58	**C** 84	03/31/2026 16:00	100	03/31/2026 07:41
O2 Flow Rate	on 2 l/min		on 2 l/min		on 2 l/min	
O2 Therapy	Nasal cannula		Nasal cannula		Nasal cannula	

Ventilator Settings (Last Within 24hrs)
Ventilator Mode: SPN-BiPAP (02:02)
Inspiratory Pressure: 5 cm H2O (02:02)
Expiratory Pressure: 8 cm H2O (02:02)

Progress Note - Physician

Intake and Output (current encounter)

	03/30 07:06 04/01 07:11	04/01 07:00 04/01 07:11	03/31 07:00 04/01 06:59	03/30 07:00 03/31 06:59	03/29 07:00 03/30 06:59
Intake	1213.3	0	913.3	300	0
Output	1800	0	0	1800	0
Net Total	-586.7	0	913.3	-1500	0
Stool Count	1	0	0	1	0

Physical Exam

Home Medications
amLODIPine (amLODIPine 10 mg oral tablet)
hydrALAZINE (hydrALAZINE 50 mg oral tablet)
losartan (losartan 100 mg oral tablet)
metoprolol (metoprolol succinate 50 mg oral tablet, extended release)
minoxidil (minoxidil 2.5 mg oral tablet)
multivitamin (Rena-Vite oral tablet)
ondansetron (ondansetron 4 mg oral tablet)
sevelamer (sevelamer 800 mg oral tablet)
zolpidem (zolpidem 5 mg oral tablet)

Inpatient Medications
Medications (24) Active
Scheduled: (11)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 10 mg Tab (amLODIPine) 10 mg 1 Tabs, Oral, Daily
cefTRIAXone/D5W (Rocephin) 50 mL, IV Piggyback, q24H Interval
doxycycline/D5W (doxycycline) 100 mg 100 mL, IV Piggyback, q12H
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
heparin 5000 units/mL injection 1 mL (heparin) 5,000 units 1 mL, SubCutaneous, q12H
hydrALAZINE 50 mg Tab (hydrALAZINE) 50 mg 1 Tabs, Oral, TID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
losartan 100 mg Tab (losartan) 100 mg 1 Tabs, Oral, Daily
metoprolol succinate 50 mg XL Tab (Metoprolol Succinate ER) 50 mg 1 Tabs, Oral, qHS
sevelamer carbonate 800 mg Tab (sevelamer) 800 mg 1 Tabs, Oral, TID w/Meals
Continuous: (0)
PRN: (13)
acetaminophen 325 mg Tab (Tylenol) 650 mg 2 Tabs, Oral, q4H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q3H
benzonatate 100 mg Cap (Tessalon Perles) 100 mg 1 Caps, Oral, TID
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID
Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 25 gm 50 mL, IV Push, q1H Interval
docusate sodium 100 mg Cap (Colace) 100 mg 1 Caps, Oral, BID
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H
glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q2H Interval
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q2H Interval

Progress Note - Physician

melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H
sodium citrate 4% soln 5 mL (sodium citrate) 200 mg 5 mL, IntraCatheter, As Directed

72 hour Antibiotic History

Active Antibiotics	Day of Therapy:	Last Administered	First Administered
doxycycline 100 mg, 100 mL, 100 mL/hr, IV Piggyback, q12H	2	03/31/2026 21:45	03/30/2026 21:13
cefTRIAXone 2 gm, 50 mL, 100 mL/hr, IV Piggyback, q24H Interval	2	03/31/2026 10:13	03/31/2026 10:13

Completed/Discontinued Antibiotics	Stop Date/Time	Last Administered	First Administered
azithromycin 500 mg, 250 mL, 250 mL/hr, IV Piggyback, Once	03/30/2026 10:10	03/30/2026 10:09	03/30/2026 10:09
cefTRIAXone 1 gm, 50 mL, 100 mL/hr, IV Piggyback, Once	03/30/2026 09:18	03/30/2026 09:10	03/30/2026 09:10

Results
Recent Labs
General Chemistry

Glucose Level	72 mg/dL (Low)	03/31/2026 02:04
POC Glucose	75 mg/dL (Normal)	03/31/2026 07:11
Sodium	140 mmol/L (Normal)	03/31/2026 02:04
Potassium	4.2 mmol/L (Normal)	03/31/2026 02:04
Chloride	100 mmol/L (Normal)	03/31/2026 02:04
CO2	31 mmol/L (Normal)	03/31/2026 02:04
Anion Gap	9 mmol/L (Normal)	03/31/2026 02:04
BUN	17 mg/dL (Normal)	03/31/2026 02:04
Creatinine	2.19 mg/dL (High)	03/31/2026 02:04
BUN/Creat Ratio	8 (Normal)	03/31/2026 02:04
Calcium	9.0 mg/dL (Normal)	03/31/2026 02:04
Albumin. Level	3.4 gm/dL (Normal)	03/31/2026 02:04
Mg Lvl	1.9 mg/dL (Normal)	03/31/2026 02:04
Phosphorus	5.6 mg/dL (High)	03/31/2026 02:04
Estimated Creatinine Clearance	10.11 mL/min ()	03/31/2026 02:58
eGFR Cr	See Comment mL/min/1.73m2 (N/A)	03/31/2026 02:04

Progress Note - Physician

eGFR Pediatric	Not Reported (N/A)	03/31/2026 02:04
Iron	77 mcg/dL (Normal)	03/31/2026 13:21
Total Iron Binding Capacity	148 mcg/dL (Low)	03/31/2026 13:21
Transferrin Saturation	52 % (High)	03/31/2026 13:21
Folate Lvl	21.85 ng/mL (High)	03/31/2026 13:21
Vitamin B12 Lvl	479 pg/mL (Normal)	03/31/2026 13:21
Ferritin Lvl	1244.0 ng/mL (High)	03/31/2026 13:21

General Hematology

WBC	5.13 x10e3/mcL (Normal)	03/31/2026 13:21
RBC	3.64 x10e6/mcL (Normal)	03/31/2026 13:21
Hgb	8.3 gm/dL (Low)	03/31/2026 13:21
Hct	26.0 % (Low)	03/31/2026 13:21
MCV	71.4 Femtoliters (Low)	03/31/2026 13:21
MCH	22.8 pg (Low)	03/31/2026 13:21
MCHC	31.9 gm/dL (Normal)	03/31/2026 13:21
RDW-SD	46.7 Femtoliters (Normal)	03/31/2026 13:21
RDW-CV	18.3 % (High)	03/31/2026 13:21
Plt	158 x10e3/mcL (Normal)	03/31/2026 13:21
MPV	See comment Femtoliters (N/A)	03/31/2026 13:21
Imm Plt Fract	4.6 % (Normal)	03/31/2026 13:21
Neut % Auto	80.7 % (N/A)	03/31/2026 02:04
Lymph % Auto	12.4 % (N/A)	03/31/2026 02:04
Mono % Auto	3.7 % (N/A)	03/31/2026 02:04
Eos % Auto	2.0 % (N/A)	03/31/2026 02:04
Baso % Auto	0.7 % (N/A)	03/31/2026 02:04
Immature Grans %	0.5 % (Normal)	03/31/2026 02:04
Neut # Auto	4.73 x10e3/mcL (Normal)	03/31/2026 02:04
Lymph # Auto	0.73 x10e3/mcL (Low)	03/31/2026 02:04
Mono # Auto	0.22 x10e3/mcL (Normal)	03/31/2026 02:04
Eos # Auto	0.12 x10e3/mcL (Normal)	03/31/2026 02:04
Baso # Auto	0.04 x10e3/mcL (Normal)	03/31/2026 02:04

Immature Grans # Auto	0.03 x10e3/mcL (Normal)	03/31/2026 02:04
Retic %	0.68 % (Normal)	03/31/2026 13:21
Retic Abs	2.41 x10e4/mcL (Normal)	03/31/2026 13:21
Imm Retic Frac%	12.6 % (High)	03/31/2026 13:21

Hepatitis

Hep A Total Ab	Reactive (Abnormal)	03/31/2026 13:21

Infectious Disease

Flu A RNA	Negative (Normal)	03/30/2026 11:15
Flu B RNA	Negative (Normal)	03/30/2026 11:15
Influenza B	Not Detected (Normal)	03/31/2026 01:48
Influenza A	Not Detected (Normal)	03/31/2026 01:48
RSV	Not Detected (Normal)	03/31/2026 01:48

Other Microbiology

MRSA by PCR	Not Detected (Normal)	03/31/2026 01:48
Adenovirus	Not Detected (Normal)	03/31/2026 01:48
Coronavirus 229E	Not Detected (Normal)	03/31/2026 01:48
Coronavirus HKU1	Not Detected (Normal)	03/31/2026 01:48
Coronavirus NL63	Not Detected (Normal)	03/31/2026 01:48
Coronavirus OC43	Not Detected (Normal)	03/31/2026 01:48
Human Metapneumovirus	Not Detected (Normal)	03/31/2026 01:48
Human Rhinovirus/Enterovirus	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 1	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 2	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 3	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 4	Not Detected (Normal)	03/31/2026 01:48
Bordetella pertussis	Not Detected (Normal)	03/31/2026 01:48
Bordetella parapertussis	Not Detected (Normal)	03/31/2026 01:48
Chlamydia pneumoniae	Not Detected (Normal)	03/31/2026 01:48
Mycoplasma pneumoniae	Not Detected (Normal)	03/31/2026 01:48

SARS-CoV-2/Covid-19

SARS-CoV2 RNA	Negative (Normal)	03/30/2026 11:15
SARS-CoV-2	Not Detected (Normal)	03/31/2026 01:48

Progress Note - Physician

Abnormal Labs

General Chemistry

Glucose Level	72 mg/dL (Low)	03/31/2026 02:04
Creatinine	2.19 mg/dL (High)	03/31/2026 02:04
Phosphorus	5.6 mg/dL (High)	03/31/2026 02:04
Total Iron Binding Capacity	148 mcg/dL (Low)	03/31/2026 13:21
Transferrin Saturation	52 % (High)	03/31/2026 13:21
Folate Lvl	21.85 ng/mL (High)	03/31/2026 13:21
Ferritin Lvl	1244.0 ng/mL (High)	03/31/2026 13:21

General Hematology

Hgb	8.3 gm/dL (Low)	03/31/2026 13:21
Hct	26.0 % (Low)	03/31/2026 13:21
MCV	71.4 Femtoliters (Low)	03/31/2026 13:21
MCH	22.8 pg (Low)	03/31/2026 13:21
RDW-CV	18.3 % (High)	03/31/2026 13:21
MPV	See comment Femtoliters (N/A)	03/31/2026 13:21
Lymph # Auto	0.73 x10e3/mcL (Low)	03/31/2026 02:04
Imm Retic Frac%	12.6 % (High)	03/31/2026 13:21

Hepatitis

Hep A Total Ab	Reactive (Abnormal)	03/31/2026 13:21

CBC, BMP, Coagulation Trend (last 4 resulted)

WBC	**5.13** 03/31/2026 13:21	**5.87** 03/31/2026 02:04	**12.25 H** 03/30/2026 07:31	
Hgb	**8.3 L** 03/31/2026 13:21	**8.4 L** 03/31/2026 02:04	**11.2** 03/30/2026 07:31	
Hct	**26.0 L** 03/31/2026 13:21	**25.6 L** 03/31/2026 02:04	**33.7** 03/30/2026 07:31	
Baso # Auto	**0.04** 03/31/2026 02:04	**0.03** 03/30/2026 07:31		
Baso % Auto	**0.7** 03/31/2026 02:04	**0.2** 03/30/2026 07:31		
Eos # Auto	**0.12** 03/31/2026 02:04	**0.02** 03/30/2026 07:31		
Eos % Auto	**2.0** 03/31/2026 02:04	**0.2** 03/30/2026 07:31		

Progress Note - Physician

Immature Grans %	**0.5** 03/31/2026 02:04	**0.6** 03/30/2026 07:31		
Immature Grans # Auto	**0.03** 03/31/2026 02:04	**0.07** 03/30/2026 07:31		
Lymph # Auto	**0.73 L** 03/31/2026 02:04	**1.33** 03/30/2026 07:31		
Lymph % Auto	**12.4** 03/31/2026 02:04	**10.9** 03/30/2026 07:31		
Mono # Auto	**0.22** 03/31/2026 02:04	**0.47** 03/30/2026 07:31		
Mono % Auto	**3.7** 03/31/2026 02:04	**3.8** 03/30/2026 07:31		
Neut # Auto	**4.73** 03/31/2026 02:04	**10.33 H** 03/30/2026 07:31		
Neut % Auto	**80.7** 03/31/2026 02:04	**84.3** 03/30/2026 07:31		
Retic Abs	**2.41** 03/31/2026 13:21			
Retic %	**0.68** 03/31/2026 13:21			
Plt	**158** 03/31/2026 13:21	**141** 03/31/2026 02:04	**230** 03/30/2026 07:31	
Glucose Level	**72 L** 03/31/2026 02:04	**189 H** 03/30/2026 07:31		
Sodium	**140** 03/31/2026 02:04	**139** 03/30/2026 07:31		
Potassium	**4.2** 03/31/2026 02:04	**5.9 H** 03/30/2026 07:31		
Chloride	**100** 03/31/2026 02:04	**94 L** 03/30/2026 07:31		
CO2	**31** 03/31/2026 02:04	**27** 03/30/2026 07:31		
Creatinine	**2.19 H** 03/31/2026 02:04	**4.80 H** 03/30/2026 07:31		
BUN	**17** 03/31/2026 02:04	**44 H** 03/30/2026 07:31		
BUN/Creat Ratio	**8** 03/31/2026 02:04	**9** 03/30/2026 07:31		
Mg Lvl	**1.9** 03/31/2026 02:04			
Phosphorus	**5.6 H** 03/31/2026 02:04	**10.5 H** 03/30/2026 13:08		
Calcium	**9.0** 03/31/2026 02:04	**8.6 L** 03/30/2026 07:31		
Albumin. Level	**3.4** 03/31/2026 02:04	**3.6** 03/30/2026 22:34	**3.4** 03/30/2026 11:15	

Progress Note - Physician

T Bili	**0.8** 03/30/2026 22:34	**0.4** 03/30/2026 11:15		
D Bili	**0.2** 03/30/2026 22:34	**0.2** 03/30/2026 11:15		
Alk Phos	**125 H** 03/30/2026 22:34	**150 H** 03/30/2026 11:15		
AST	**97 H** 03/30/2026 22:34	**57 H** 03/30/2026 11:15		
ALT	**49** 03/30/2026 22:34	**51** 03/30/2026 11:15		

Assessment/Plan
Assessment:
Subjective:
Patient is a 90-year-old female with a PMHx of ESRD on HD, chronic hypoxic respiratory failure requiring 2–5 L home O2 who presented to Valley Hospital with reports of increasing shortness of breath. Patient was initially admitted to the ICU for stat dialysis and continuous BiPAP.

Patient seen examined at bedside. Today she reports that she is feeling much better. Discussed current condition
and possible discharge pending sensitivities of bacteria and whether or not she can be transition to oral antibiotic therapy. She does already have home health set up, placed case management consult to verify.
Pending sensitivities and ID recommendations.

Physical exam:
General: Thin appearing Asian woman sitting up in bed.
Mental Status: Alert and oriented to person, place, time. Normal affect.
Head: Normocephalic, atraumatic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact. Anicteric.
Ear, Nose and Throat: Oropharynx clear, moist mucous membranes. Trachea midline.
Neck: No obvious neck masses
Respiratory: Clear to auscultation bilaterally. No wheezing, crackles or rhonchi. Normal respiratory effort. On LFNC
Cardiovascular: Regular rate and rhythm, no murmurs, gallops or rubs. Right sided subclavian HD catheter noted
Gastrointestinal: Non-tender, non-distended. Normal bowel sounds.
Genitourinary: No suprapubic tenderness.
Neurologic: Cranial nerves II-XII grossly intact. Moves all extremities.
Light touch sensation is grossly intact bilaterally.

Assessment:
Acute on chronic hypoxic respiratory failure 2/2 pneumonia versus fluid overload, improved
Dyspnea
Morganella morganii bacteremia
Sepsis with RBC, HR 2/2 respiratory source
Troponinemia 2/2 demand ischemia
ESRD on HD MWF
Hyperkalemia 2/2 above
Hepatitis A IgM positive
Oxygen dependent reportedly on 2 to 5 L at home
Hypertension
Dementia

Progress Note - Physician

Plan:
Downgraded from ICU level of care
CT chest/abdomen/pelvis with interlobular septal thickening and adjacent ground-glass opacities in bilateral lungs possibly pulmonary edema versus infectious/inflammatory versus less likely hemorrhagic. Small to moderate right and small left-sided pleural effusions.
Acute on chronic hypoxic respite failure secondary to pneumonia in addition to fluid overload
– Blood cultures 1/2 positive for gram-negative rods
– Continue antibiotics with Rocephin, doxycycline. Will adjust as indicated by cultures and sensitivities
– Breathing treatments
– Goal SpO2 92%, titrated down to LFNC 2L
ID is on consult. Appreciate recommendations.
– Continue ceftriaxone, doxycycline
– Blood culture 1/2 positive for Morganella morganii. Pending susceptibilities
Nephrology consulted appreciate recommendation
– S/p stat HD
Troponinemia secondary to demand ischemia, patient denies chest pain
– TTE pending
PRN antihypertensives and antiemetics
Replete electrolytes as needed, routine labs, and supportive care.

Patient has 5 children but main caretaker is brother who lives with her. ICU team discussed with extensively regarding CODE STATUS with patient's daughter and brother at the bedside who had different opinion regarding CODE STATUS. They wish to proceed with full code no shock for now but has plan to discuss among family members.

Consults: ICU, nephrology Dr. Vornicu
Diet: N.p.o., pending speech therapy evaluation
Activity: Early and progressive mobilization
GI prophylaxis: None
DVT prophylaxis: Subcu heparin, SCD
CODE STATUS: Full code, no shock per family
Dispo: IMC

Will discuss plan with attending physician. Awaiting further recommendations.

Ellyn Hirabayashi, DO
Internal Medicine - PGY3
Valley Hospital Medical Center

Discharge Planning:
» Discharge Planning:
» Discharge To, Anticipated: Home with family care

Progress Note - Physician

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	3/31/2026 13:04 PDT
RESULT STATUS:	Modified
PERFORM INFORMATION:	Delmundo DO,Lloyd (3/31/2026 17:01 PDT); Kim DO, Andrew H (3/31/2026 13:04 PDT)
SIGN INFORMATION:	Delmundo DO,Lloyd (3/31/2026 17:01 PDT); Kim DO, Andrew H (3/31/2026 13:04 PDT)

Addendum by Delmundo DO, Lloyd on March 31, 2026 17:01:08 PDT
I discussed with resident/fellow at length. I was present for critical and key services and am participating in patient management, in combination with the resident or fellow.
Agree with assessment and plan

wob on 2 liters via NC
hd per nephrology abx on doxy
echo pending read
much better than yesterday
IS
PTOT
Will continue to follow from a pulmonary standpoint

L. Del Mundo, DO
Pulmonary/Critical Care

Electronically Signed By: Delmundo, Lloyd DO
On: 03.31.2026 17:01 PDT

Subjective
downgraded from ICU yesterday
now on 2L NC, was previously on bipap
vitals stable
doing better after HD

Chief Complaint/Reason for Consultation
Chief Complaint: Patient brought in by EMS from home with complaints of SOB. Per the family, patient started to complain of SOB this morning. Patient found with a RA sat of <50%. Patient provided with BVM ventilation and then placed on NRB. Patient A/Ox0. EMS unable to obtain demographics on scene. BGL 119

Past Medical History
Active Problems(3)
Hemodialysis
Hepatitis A virus
High cholesterol

Social History
No social history documented.

Family History
No Family History documented.

Progress Note - Physician

Objective

Measurements (most recent)

Height (cm)	157.48 cm	Erickson RD, LD, Lauren	03/31 11:07
Height (inches)	62.00 inch	SYSTEM	03/31 11:16
Weight	37.50 kg	Parala RN, Marielle Vianne	03/30 23:48
Weight (lbs)	82.67 lb	SYSTEM	03/30 23:48
Body Mass Index	15.12 kg/m2	Parala RN, Marielle Vianne	03/30 23:48
Scale Type	Bed scale	Parala RN, Marielle Vianne	03/30 23:48

Vital Signs (24 hrs)	**Last Charted**		**Minimum**		**Maximum**	
Temp	37.2	03/31/2026 12:00	36.9	03/30/2026 22:45	37.1	03/30/2026 19:57
Apical Heart Rate	87	03/31/2026 07:12	87	03/31/2026 07:12	98	03/30/2026 18:54
Resp Rate	19	03/31/2026 11:00	14	03/31/2026 08:59	**C** 52	03/31/2026 03:45
SBP	**H** 159	03/31/2026 11:00	**H** 148	03/31/2026 10:14	**C** 198	03/31/2026 07:00
DBP	67	03/31/2026 11:00	65	03/31/2026 10:14	**H** 90	03/30/2026 21:30
MAP	97	03/31/2026 11:00	94	03/31/2026 10:14	129	03/30/2026 18:00
SpO2	99	03/31/2026 11:00	**C** 86	03/31/2026 04:15	100	03/30/2026 21:29
O2 Flow Rate					on 4 l/min	
O2 Therapy					Nasal cannula	

Perfusion Assessment

Capillary Refill Left Foot: Less than 2 seconds (03/31/26 08:00:00)

Capillary Refill Left Hand: Less than 2 seconds (03/31/26 08:00:00)

Capillary Refill Right Foot: Less than 2 seconds (03/31/26 08:00:00)

Capillary Refill Right Hand: Less than 2 seconds (03/31/26 08:00:00)

Dorsalis Pedis Pulse, Left: 1+ Barely detectable/thready (03/31/26 08:00:00)

Dorsalis Pedis Pulse, Right: 1+ Barely detectable/thready (03/31/26 08:00:00)

Posttibial Pulse, Left: 1+ Barely detectable/thready (03/31/26 00:00:00)

Posttibial Pulse, Right: 1+ Barely detectable/thready (03/31/26 00:00:00)

Progress Note - Physician

Pain Scores (Last Within 24hrs)
FACES Pain Scale Score: 0 = No hurt (17:00)
Numeric Pain Scale: 0 = No pain (12:00)

Ventilator Settings (Last Within 24hrs)
Ventilator Mode: SPN-BiPAP (02:05) FIO2: 30 % (00:00)
Inspiratory Pressure: 5 cm H2O (02:05)
Expiratory Pressure: 8 cm H2O (02:05)

Intake and Output (current encounter)

	03/30 07:06 03/31 13:04	03/31 07:00 03/31 13:04	03/30 07:00 03/31 06:59	03/29 07:00 03/30 06:59	03/28 07:00 03/29 06:59
Intake	452.6	152.6	300	0	0
Output	1800	0	1800	0	0
Net Total	-1347.4	152.6	-1500	0	0

Stroke Assessments
Glasgow Coma Scale
Eye Opening Response Glasgow: Spontaneously (03/31/26 08:00:00)
Best Verbal Response Glasgow: Disoriented and converses (03/31/26 08:00:00)
Best Motor Response Glasgow: Obeys commands (03/31/26 08:00:00)
Glasgow Coma Score: 14 (03/31/26 08:00:00)

Physical Exam

Home Medications
amLODIPine (amLODIPine 10 mg oral tablet)
hydrALAZINE (hydrALAZINE 50 mg oral tablet)
losartan (losartan 100 mg oral tablet)
metoprolol (metoprolol succinate 50 mg oral tablet, extended release)
minoxidil (minoxidil 2.5 mg oral tablet)
multivitamin (Rena-Vite oral tablet)
ondansetron (ondansetron 4 mg oral tablet)
sevelamer (sevelamer 800 mg oral tablet)
zolpidem (zolpidem 5 mg oral tablet)

Inpatient Medications
Medications (22) Active
Scheduled: (11)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 10 mg Tab (amLODIPine) 10 mg 1 Tabs, Oral, Daily

Progress Note - Physician

cefTRIAXone/D5W (Rocephin) 50 mL, IV Piggyback, q24H Interval
doxycycline/D5W (doxycycline) 100 mg 100 mL, IV Piggyback, q12H
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
heparin 5000 units/mL injection 1 mL (heparin) 5,000 units 1 mL, SubCutaneous, q12H
hydrALAZINE 50 mg Tab (hydrALAZINE) 50 mg 1 Tabs, Oral, TID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
losartan 100 mg Tab (losartan) 100 mg 1 Tabs, Oral, Daily
metoprolol succinate 50 mg XL Tab (Metoprolol Succinate ER) 50 mg 1 Tabs, Oral, qHS
sevelamer carbonate 800 mg Tab (sevelamer) 800 mg 1 Tabs, Oral, TID w/Meals
Continuous: (0)
PRN: (11)
acetaminophen 325 mg Tab (Tylenol) 650 mg 2 Tabs, Oral, q4H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q3H
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID
Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 25 gm 50 mL, IV Push, q1H Interval
docusate sodium 100 mg Cap (Colace) 100 mg 1 Caps, Oral, BID
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H
glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q2H Interval
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q2H Interval
melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H

72 hour Antibiotic History

Active Antibiotics	Day of Therapy:	Last Administered	First Administered
cefTRIAXone 2 gm, 50 mL, 100 mL/hr, IV Piggyback, q24H Interval	1	03/31/2026 10:13	03/31/2026 10:13
doxycycline 100 mg, 100 mL, 100 mL/hr, IV Piggyback, q12H	2	03/31/2026 08:50	03/30/2026 21:13

Completed/Discontinued Antibiotics	Stop Date/Time	Last Administered	First Administered
azithromycin 500 mg, 250 mL, 250 mL/hr, IV Piggyback, Once	03/30/2026 10:10	03/30/2026 10:09	03/30/2026 10:09
cefTRIAXone 1 gm, 50 mL, 100 mL/hr, IV Piggyback, Once	03/30/2026 09:18	03/30/2026 09:10	03/30/2026 09:10

Results
Recent Labs
Blood Gases

Progress Note - Physician

Art Na	137 mmol/L (Normal)	03/30/2026 08:43
Art K	6.3 mmol/L (Critical)	03/30/2026 08:43
Art iCa	0.96 mmol/L (Low)	03/30/2026 08:43
pH Arterial	7.54 (High)	03/30/2026 08:43
Art pCO2	39.4 mmHg (Normal)	03/30/2026 08:43
Art pO2	153.0 mmHg (High)	03/30/2026 08:43
HCO3 Art	34.4 mmol/L (High)	03/30/2026 08:43
Arterial pCO2(T)	39.4 mmHg (N/A)	03/30/2026 08:43
Arterial pH(T)	7.54 (N/A)	03/30/2026 08:43
Arterial pO2(T)	153.0 mmHg (N/A)	03/30/2026 08:43
Art Base Excess	10.6 mmol/L (High)	03/30/2026 08:43
Art sO2	99 % (High)	03/30/2026 08:43
Art tHB	10.3 gm/dL (Low)	03/30/2026 08:43
Art Hct	32 % (Low)	03/30/2026 08:43
POC Pt Temp	37.0 DegC (N/A)	03/30/2026 08:43
POC FIO2	50 % (N/A)	03/30/2026 08:43
POC Sample	Arterial (N/A)	03/30/2026 08:43
POC Site	Radial Right (N/A)	03/30/2026 08:43
POC Allen Test	POS (N/A)	03/30/2026 08:43
POC Del Sys	BIPAP (N/A)	03/30/2026 08:43
POC Vent Mode	NO VENT (N/A)	03/30/2026 08:43
POC Set RR	14 br/min (N/A)	03/30/2026 08:43
POC IPAP	8 cm H2O (N/A)	03/30/2026 08:43
POC EPAP	4 cm H2O (N/A)	03/30/2026 08:43
Notified To and RBV	DR DAVIDSON (N/A)	03/30/2026 08:43
Notified By	vhsmurrat (N/A)	03/30/2026 08:43
Notified Note	ALL CV WITH RB (N/A)	03/30/2026 08:43

Cardiac

BNP	2004 pg/mL (High)	03/30/2026 07:31
Troponin TNIH	328.77 ng/L (Critical)	03/30/2026 22:34

General Chemistry

Glucose Level	72 mg/dL (Low)	03/31/2026 02:04
POC Glucose	75 mg/dL (Normal)	03/31/2026 07:11
Sodium	140 mmol/L (Normal)	03/31/2026 02:04
Potassium	4.2 mmol/L (Normal)	03/31/2026 02:04
Chloride	100 mmol/L (Normal)	03/31/2026 02:04
CO2	31 mmol/L (Normal)	03/31/2026 02:04
Anion Gap	9 mmol/L (Normal)	03/31/2026 02:04

Progress Note - Physician

BUN	17 mg/dL (Normal)	03/31/2026 02:04
Creatinine	2.19 mg/dL (High)	03/31/2026 02:04
BUN/Creat Ratio	8 (Normal)	03/31/2026 02:04
Calcium	9.0 mg/dL (Normal)	03/31/2026 02:04
Albumin. Level	3.4 gm/dL (Normal)	03/31/2026 02:04
TP	6.4 gm/dL (Normal)	03/30/2026 22:34
A/G Ratio	1.3 (Normal)	03/30/2026 22:34
T Bili	0.8 mg/dL (Normal)	03/30/2026 22:34
D Bili	0.2 mg/dL (Normal)	03/30/2026 22:34
Alk Phos	125 units/L (High)	03/30/2026 22:34
AST	97 units/L (High)	03/30/2026 22:34
ALT	49 units/L (Normal)	03/30/2026 22:34
Mg Lvl	1.9 mg/dL (Normal)	03/31/2026 02:04
Phosphorus	5.6 mg/dL (High)	03/31/2026 02:04
Estimated Creatinine Clearance	10.11 mL/min ()	03/31/2026 02:58
eGFR Cr	See Comment mL/min/1.73m2 (N/A)	03/31/2026 02:04
eGFR Pediatric	Not Reported (N/A)	03/31/2026 02:04
Lactic Acid Lvl	1.8 mmol/L (Normal)	03/30/2026 08:48
Hgb A1C	4.3 % (Normal)	03/30/2026 11:15
MAR Glucose Result	102 ()	03/30/2026 09:49

General Hematology

WBC	5.87 x10e3/mcL (Normal)	03/31/2026 02:04
RBC	3.65 x10e6/mcL (Normal)	03/31/2026 02:04
Hgb	8.4 gm/dL (Low)	03/31/2026 02:04
Hct	25.6 % (Low)	03/31/2026 02:04
MCV	70.1 Femtoliters (Low)	03/31/2026 02:04
MCH	23.0 pg (Low)	03/31/2026 02:04
MCHC	32.8 gm/dL (Normal)	03/31/2026 02:04
RDW-SD	45.2 Femtoliters (Normal)	03/31/2026 02:04
RDW-CV	18.6 % (High)	03/31/2026 02:04
Plt	141 x10e3/mcL (Normal)	03/31/2026 02:04
MPV	9.6 Femtoliters (Normal)	03/31/2026 02:04
Imm Plt Fract	3.7 % (Normal)	03/31/2026 02:04

Progress Note - Physician

NRBC %	0.2 /100 WBC (Normal)	03/30/2026 07:31
NRBC #	0.02 x10e3/mcL (High)	03/30/2026 07:31
Neut % Auto	80.7 % (N/A)	03/31/2026 02:04
Lymph % Auto	12.4 % (N/A)	03/31/2026 02:04
Mono % Auto	3.7 % (N/A)	03/31/2026 02:04
Eos % Auto	2.0 % (N/A)	03/31/2026 02:04
Baso % Auto	0.7 % (N/A)	03/31/2026 02:04
Immature Grans %	0.5 % (Normal)	03/31/2026 02:04
Neut # Auto	4.73 x10e3/mcL (Normal)	03/31/2026 02:04
Lymph # Auto	0.73 x10e3/mcL (Low)	03/31/2026 02:04
Mono # Auto	0.22 x10e3/mcL (Normal)	03/31/2026 02:04
Eos # Auto	0.12 x10e3/mcL (Normal)	03/31/2026 02:04
Baso # Auto	0.04 x10e3/mcL (Normal)	03/31/2026 02:04
Immature Grans # Auto	0.03 x10e3/mcL (Normal)	03/31/2026 02:04
RBC Morph	Abnormal (Abnormal)	03/30/2026 07:31
Plt Morphology	Normal (Normal)	03/30/2026 07:31
Polychrom	1+ (Abnormal)	03/30/2026 07:31
Micro	2+ (Abnormal)	03/30/2026 07:31
Plt Est	Adequate (Normal)	03/30/2026 07:31
Schistocytes	1+ (Abnormal)	03/30/2026 07:31
Target Cells (Codocytes)	3+ (Abnormal)	03/30/2026 07:31
Teardrop Cells (Dacrocytes)	1+ (Abnormal)	03/30/2026 07:31

Hepatitis

Hep A IgM	Reactive (Abnormal)	03/30/2026 10:54
Hep B Core IgM	Non-Reactive (Normal)	03/30/2026 11:04
Hepatitis B Surface Antigen	Non-Reactive (Normal)	03/30/2026 11:04
Hep Bs Ab Qn	31.9 milli intl units/mL (High)	03/30/2026 11:04
Hep Bs Ab Qn Interp	Reactive (Abnormal)	03/30/2026 11:04
Hep C Ab	Non-Reactive (Normal)	03/30/2026 10:54

Infectious Disease

Flu A RNA	Negative (Normal)	03/30/2026 11:15
Flu B RNA	Negative (Normal)	03/30/2026 11:15

Other Chemistry

Procalcitonin Lvl	1.56 ng/mL (High)	03/30/2026 11:15

Other Microbiology

MRSA by PCR	Not Detected (Normal)	03/31/2026 01:48

SARS-CoV-2/Covid-19

SARS-CoV2 RNA	Negative (Normal)	03/30/2026 11:15

Thyroid

T3 Free	2.87 pg/mL (Normal)	03/30/2026 11:15
T4 Free	1.74 ng/dL (High)	03/30/2026 11:15
TSH Ultrasensitive	3.046 mc Intl units/mL (Normal)	03/30/2026 11:15

Abnormal Labs

Blood Gases

Art K	6.3 mmol/L (Critical)	03/30/2026 08:43
Art iCa	0.96 mmol/L (Low)	03/30/2026 08:43
pH Arterial	7.54 (High)	03/30/2026 08:43
Art pO2	153.0 mmHg (High)	03/30/2026 08:43
HCO3 Art	34.4 mmol/L (High)	03/30/2026 08:43
Art Base Excess	10.6 mmol/L (High)	03/30/2026 08:43
Art sO2	99 % (High)	03/30/2026 08:43
Art tHB	10.3 gm/dL (Low)	03/30/2026 08:43
Art Hct	32 % (Low)	03/30/2026 08:43

Cardiac

BNP	2004 pg/mL (High)	03/30/2026 07:31
Troponin TNIH	328.77 ng/L (Critical)	03/30/2026 22:34

General Chemistry

Glucose Level	72 mg/dL (Low)	03/31/2026 02:04
Creatinine	2.19 mg/dL (High)	03/31/2026 02:04
Alk Phos	125 units/L (High)	03/30/2026 22:34
AST	97 units/L (High)	03/30/2026 22:34
Phosphorus	5.6 mg/dL (High)	03/31/2026 02:04

General Hematology

Hgb	8.4 gm/dL (Low)	03/31/2026 02:04

Progress Note - Physician

Hct	25.6 % (Low)	03/31/2026 02:04
MCV	70.1 Femtoliters (Low)	03/31/2026 02:04
MCH	23.0 pg (Low)	03/31/2026 02:04
RDW-CV	18.6 % (High)	03/31/2026 02:04
NRBC #	0.02 x10e3/mcL (High)	03/30/2026 07:31
Lymph # Auto	0.73 x10e3/mcL (Low)	03/31/2026 02:04
RBC Morph	Abnormal (Abnormal)	03/30/2026 07:31
Polychrom	1+ (Abnormal)	03/30/2026 07:31
Micro	2+ (Abnormal)	03/30/2026 07:31
Schistocytes	1+ (Abnormal)	03/30/2026 07:31
Target Cells (Codocytes)	3+ (Abnormal)	03/30/2026 07:31
Teardrop Cells (Dacrocytes)	1+ (Abnormal)	03/30/2026 07:31

Hepatitis

Hep A IgM	Reactive (Abnormal)	03/30/2026 10:54
Hep Bs Ab Qn	31.9 milli intl units/mL (High)	03/30/2026 11:04
Hep Bs Ab Qn Interp	Reactive (Abnormal)	03/30/2026 11:04

Other Chemistry

Procalcitonin Lvl	1.56 ng/mL (High)	03/30/2026 11:15

Thyroid

T4 Free	1.74 ng/dL (High)	03/30/2026 11:15

CBC, BMP, Coagulation Trend (last 4 resulted)

WBC	**5.87** 03/31/2026 02:04	**12.25 H** 03/30/2026 07:31		
Hgb	**8.4 L** 03/31/2026 02:04	**11.2** 03/30/2026 07:31		
Hct	**25.6 L** 03/31/2026 02:04	**33.7** 03/30/2026 07:31		
Baso # Auto	**0.04** 03/31/2026 02:04	**0.03** 03/30/2026 07:31		
Baso % Auto	**0.7** 03/31/2026 02:04	**0.2** 03/30/2026 07:31		
Eos # Auto	**0.12** 03/31/2026 02:04	**0.02** 03/30/2026 07:31		
Eos % Auto	**2.0** 03/31/2026 02:04	**0.2** 03/30/2026 07:31		

Progress Note - Physician

Immature Grans %	**0.5** 03/31/2026 02:04	**0.6** 03/30/2026 07:31		
Immature Grans # Auto	**0.03** 03/31/2026 02:04	**0.07** 03/30/2026 07:31		
Lymph # Auto	**0.73 L** 03/31/2026 02:04	**1.33** 03/30/2026 07:31		
Lymph % Auto	**12.4** 03/31/2026 02:04	**10.9** 03/30/2026 07:31		
Mono # Auto	**0.22** 03/31/2026 02:04	**0.47** 03/30/2026 07:31		
Mono % Auto	**3.7** 03/31/2026 02:04	**3.8** 03/30/2026 07:31		
Neut # Auto	**4.73** 03/31/2026 02:04	**10.33 H** 03/30/2026 07:31		
Neut % Auto	**80.7** 03/31/2026 02:04	**84.3** 03/30/2026 07:31		
Plt	**141** 03/31/2026 02:04	**230** 03/30/2026 07:31		
Glucose Level	**72 L** 03/31/2026 02:04	**189 H** 03/30/2026 07:31		
Sodium	**140** 03/31/2026 02:04	**139** 03/30/2026 07:31		
Potassium	**4.2** 03/31/2026 02:04	**5.9 H** 03/30/2026 07:31		
Chloride	**100** 03/31/2026 02:04	**94 L** 03/30/2026 07:31		
CO2	**31** 03/31/2026 02:04	**27** 03/30/2026 07:31		
Creatinine	**2.19 H** 03/31/2026 02:04	**4.80 H** 03/30/2026 07:31		
BUN	**17** 03/31/2026 02:04	**44 H** 03/30/2026 07:31		
BUN/Creat Ratio	**8** 03/31/2026 02:04	**9** 03/30/2026 07:31		
Mg Lvl	**1.9** 03/31/2026 02:04			
Phosphorus	**5.6 H** 03/31/2026 02:04	**10.5 H** 03/30/2026 13:08		
Calcium	**9.0** 03/31/2026 02:04	**8.6 L** 03/30/2026 07:31		
Albumin. Level	**3.4** 03/31/2026 02:04	**3.6** 03/30/2026 22:34	**3.4** 03/30/2026 11:15	
T Bili	**0.8** 03/30/2026 22:34	**0.4** 03/30/2026 11:15		
D Bili	**0.2** 03/30/2026 22:34	**0.2** 03/30/2026 11:15		

Progress Note - Physician

Alk Phos	**125 H** 03/30/2026 22:34	**150 H** 03/30/2026 11:15		
AST	**97 H** 03/30/2026 22:34	**57 H** 03/30/2026 11:15		
ALT	**49** 03/30/2026 22:34	**51** 03/30/2026 11:15		

Blood Glucose Trend

Glucose Level: 72 mg/dL Low (03/31/26 02:04:00)

POC Glucose: 75 mg/dL (03/31/26 07:11:00)

POC Glucose: 70 mg/dL Low (03/31/26 02:14:00)

CBC (Last Within 24hrs)
WBC: 5.87 x10e3/mcL (02:04)
Hgb: 8.4 gm/dL Low (02:04)
Hct: 25.6 % Low (02:04)
Plt: 141 x10e3/mcL (02:04)

Differential (Last Within 24hrs)

- Automated -

Neut % Auto: 80.7 % (02:04)

Lymph % Auto: 12.4 % (02:04)

Mono % Auto: 3.7 % (02:04)

Eos % Auto: 2 % (02:04)

Baso % Auto: 0.7 % (02:04)

Immature Grans %: 0.5 % (02:04)

Neut # Auto: 4.73 x10e3/mcL (02:04)

Lymph # Auto: 0.73 x10e3/mcL Low (02:04)

Mono # Auto: 0.22 x10e3/mcL (02:04)

Baso # Auto: 0.04 x10e3/mcL (02:04)

Immature Grans # Auto: 0.03 x10e3/mcL (02:04)

BMP, Mg, and Phos (Last Within 24hrs)
Sodium: 140 mmol/L (02:04)
Potassium: 4.2 mmol/L (02:04)
Chloride: 100 mmol/L (02:04)
CO2: 31 mmol/L (02:04)
BUN: 17 mg/dL (02:04)
Creatinine: 2.19 mg/dL High (02:04)
Glucose Level: 72 mg/dL Low (02:04)
Calcium: 9 mg/dL (02:04)
Mg Lvl: 1.9 mg/dL (02:04)
Phosphorus: 5.6 mg/dL High (02:04)

Progress Note - Physician

LFT (Last Within 24hrs)
AST: 97 units/L High (22:34)
ALT: 49 units/L (22:34)
Alk Phos: 125 units/L High (22:34)
T Bili: 0.8 mg/dL (22:34)
D Bili: 0.2 mg/dL (22:34)
TP: 6.4 gm/dL (22:34)

Microbiology

Culture Blood ARD **In-Lab**

Source:	Blood	Body Site:	
Collected Dt/Tm:	03/31/2026 02:12	Last Updated Dt/Tm:	03/31/2026 00:41

Culture Blood ARD **In-Lab**

Source:	Blood	Body Site:	
Collected Dt/Tm:	03/31/2026 02:04	Last Updated Dt/Tm:	03/31/2026 00:41

Culture Sputum **Completed**

Source:	Sputum Expectorated	Body Site:	
Collected Dt/Tm:	03/30/2026 22:34	Last Updated Dt/Tm:	03/31/2026 12:42

>10 epithelial cells /lpf
>25 White Blood Cells /lpf
Many gram positive cocci in pairs
Many Gram Negative Rods
Few Gram Negative Coccobacilli
Few Gram Positive Rods
Gram stain contains >=10 squamous epithelial cells per low power field which suggests oropharyngeal and/or nasopharyngeal contamination with no predominant bacterial morphology. Culture will not be performed. Notified: ALAJA RUIZ,RN 3/31/2026 12:27:14 PP

Culture Blood ARD **Preliminary**

Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	03/31/2026 09:02

No growth at 24 hours.

Culture Blood ARD **Preliminary**

Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	03/31/2026 10:02

Progress Note - Physician

Morganella morganii
Identification obtained by Sepsityper rapid identification method.
Culture confirmation to follow.
Susceptibility to follow.

Cardiology Labs
BNP: 2004 pg/mL High (03/30/26 07:31:00)
Troponin TNIH: 328.77 ng/L Critical (03/30/26 22:34:00)
Troponin TNIH: 379.71 ng/L Critical (03/30/26 17:36:00)
Troponin TNIH: 292.67 ng/L Critical (03/30/26 13:08:00)

Assessment/Plan
Assessment:
Physical Exam
General: Thin appearing minimi speaking elderly female
Mental Status: Alert and oriented to person, place, time. Normal affect.
Head: Normocephalic, atraumatic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact. Anicteric.
Ear, Nose and Throat: Oropharynx clear, moist mucous membranes. Trachea midline.
Neck: No obvious neck masses
Respiratory: Clear to auscultation bilaterally. No wheezing, crackles or rhonchi. Normal respiratory effort.
Cardiovascular: Regular rate and rhythm, no murmurs, gallops or rubs.
Gastrointestinal: Non-tender, non-distended. Normal bowel sounds.
Genitourinary: No suprapubic tenderness.
Neurologic: Cranial nerves II-XII grossly intact. Moves all extremities.
Light touch sensation is grossly intact bilaterally.

Assessment
Acute hypoxic respiratory failure 2/2 pneumonia versus fluid overload
Dyspnea
Sepsis with RBC, HR 2/2 respiratory source
Troponinemia 2/2 demand ischemia
ESRD on HD MWF
Hyperkalemia 2/2 above
Oxygen dependent reportedly on 2 to 5 L at home
Hypertension
Dementia

Recommendations
Currently on 2 L nasal cannula,Wean as tolerated
Goal SpO2 greater than 92%
O2 level appears to be at baseline
Hypoxia likely secondary to pulmonary edema/fluid overload
Nephrology consulted, HD per nephrology
Monitor I's and O strictly

Progress Note - Physician

Currently on doxycycline
Echo done, read pending
Breathing treatments as needed
Encourage IS use
PT/OT as tolerated
Rest of care per primary team

Discharge Planning:
» Discharge Planning:
» Discharge To, Anticipated: Home with family care

Electronically Signed By: Kim, Andrew DO
On: 03.31.2026 13:04 PDT

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	3/31/2026 12:23 PDT
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Vornicu MD,Maria D (3/31/2026 12:24 PDT)
SIGN INFORMATION:	Vornicu MD,Maria D (3/31/2026 12:24 PDT)

Subjective
End-stage renal disease on dialysis
Hemodialysis yesterday with 1.5 L ultrafiltration
Currently on nasal cannula
Baseline dementia

Chief Complaint/Reason for Consultation
Chief Complaint: Patient brought in by EMS from home with complaints of SOB. Per the family, patient started to complain of SOB this morning. Patient found with a RA sat of <50%. Patient provided with BVM ventilation and then placed on NRB. Patient A/Ox0. EMS unable to obtain demographics on scene. BGL 119

Allergies
Allergies (Active and Proposed Allergies Only)
No Known Allergies (Severity: Unknown severity, Onset: Unknown)

Objective

Measurements (most recent)

Height (cm)	157.48 cm	Erickson RD, LD, Lauren	03/31 11:07
Height (inches)	62.00 inch	SYSTEM	03/31 11:16
Weight	37.50 kg	Parala RN, Marielle Vianne	03/30 23:48
Weight (lbs)	82.67 lb	SYSTEM	03/30 23:48
Body Mass Index	15.12 kg/m2	Parala RN, Marielle Vianne	03/30 23:48

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Progress Note - Physician

Scale Type	Bed scale	Parala RN, Marielle Vianne	03/30 23:48

Vital Signs (24 hrs)	**Last Charted**		**Minimum**		**Maximum**	
Temp	37.2	03/31/2026 12:00	36.9	03/30/2026 22:45	37.1	03/30/2026 19:57
Apical Heart Rate	87	03/31/2026 07:12	87	03/31/2026 07:12	98	03/30/2026 18:54
Resp Rate	19	03/31/2026 11:00	14	03/31/2026 08:59	**C** 52	03/31/2026 03:45
SBP	**H** 159	03/31/2026 11:00	**H** 148	03/31/2026 10:14	**C** 198	03/31/2026 07:00
DBP	67	03/31/2026 11:00	65	03/31/2026 10:14	**H** 90	03/30/2026 21:30
MAP	97	03/31/2026 11:00	94	03/31/2026 10:14	129	03/30/2026 18:00
SpO2	99	03/31/2026 11:00	**C** 86	03/31/2026 04:15	100	03/30/2026 21:29
O2 Flow Rate					on 4	l/min
O2 Therapy					Nasal cannula	

Ventilator Settings (Last Within 24hrs)

Ventilator Mode: SPN-BiPAP (02:05)
FIO2: 30 % (00:00)
Inspiratory Pressure: 5 cm H2O (02:05)
Expiratory Pressure: 8 cm H2O (02:05)

Intake and Output (current encounter)

	03/30 07:06	**03/31 07:00**	**03/30 07:00**	**03/29 07:00**	**03/28 07:00**
	03/31 12:24	**03/31 12:24**	**03/31 06:59**	**03/30 06:59**	**03/29 06:59**
Intake	452.6	152.6	300	0	0
Output	1800	0	1800	0	0
Net Total	-1347.4	152.6	-1500	0	0

Physical Exam

Inpatient Medications
Medications (22) Active
Scheduled: (11)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 10 mg Tab (amLODIPine) 10 mg 1 Tabs, Oral, Daily
cefTRIAXone/D5W (Rocephin) 50 mL, IV Piggyback, q24H Interval
doxycycline/D5W (doxycycline) 100 mg 100 mL, IV Piggyback, q12H
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
heparin 5000 units/mL injection 1 mL (heparin) 5,000 units 1 mL, SubCutaneous, q12H
hydrALAZINE 50 mg Tab (hydrALAZINE) 50 mg 1 Tabs, Oral, TID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
losartan 100 mg Tab (losartan) 100 mg 1 Tabs, Oral, Daily

Progress Note - Physician

metoprolol succinate 50 mg XL Tab (Metoprolol Succinate ER) 50 mg 1 Tabs, Oral, qHS
sevelamer carbonate 800 mg Tab (sevelamer) 800 mg 1 Tabs, Oral, TID w/Meals
Continuous: (0)
PRN: (11)
acetaminophen 325 mg Tab (Tylenol) 650 mg 2 Tabs, Oral, q4H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q3H
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID
Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 25 gm 50 mL, IV Push, q1H Interval
docusate sodium 100 mg Cap (Colace) 100 mg 1 Caps, Oral, BID
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H
glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q2H Interval
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q2H Interval
melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H

72 hour Antibiotic History

Active Antibiotics	Day of Therapy:	Last Administered	First Administered
cefTRIAXone 2 gm, 50 mL, 100 mL/hr, IV Piggyback, q24H Interval	1	03/31/2026 10:13	03/31/2026 10:13
doxycycline 100 mg, 100 mL, 100 mL/hr, IV Piggyback, q12H	2	03/31/2026 08:50	03/30/2026 21:13

Completed/Discontinued Antibiotics	Stop Date/Time	Last Administered	First Administered
azithromycin 500 mg, 250 mL, 250 mL/hr, IV Piggyback, Once	03/30/2026 10:10	03/30/2026 10:09	03/30/2026 10:09
cefTRIAXone 1 gm, 50 mL, 100 mL/hr, IV Piggyback, Once	03/30/2026 09:18	03/30/2026 09:10	03/30/2026 09:10

Results
Recent Labs
Blood Gases

Art Na	137 mmol/L (Normal)	03/30/2026 08:43
Art K	6.3 mmol/L (Critical)	03/30/2026 08:43
Art iCa	0.96 mmol/L (Low)	03/30/2026 08:43
pH Arterial	7.54 (High)	03/30/2026 08:43
Art pCO2	39.4 mmHg (Normal)	03/30/2026 08:43
Art pO2	153.0 mmHg (High)	03/30/2026 08:43
HCO3 Art	34.4 mmol/L (High)	03/30/2026 08:43
Arterial pCO2(T)	39.4 mmHg (N/A)	03/30/2026 08:43

Progress Note - Physician

Arterial pH(T)	7.54 (N/A)	03/30/2026 08:43
Arterial pO2(T)	153.0 mmHg (N/A)	03/30/2026 08:43
Art Base Excess	10.6 mmol/L (High)	03/30/2026 08:43
Art sO2	99 % (High)	03/30/2026 08:43
Art tHB	10.3 gm/dL (Low)	03/30/2026 08:43
Art Hct	32 % (Low)	03/30/2026 08:43
POC Pt Temp	37.0 DegC (N/A)	03/30/2026 08:43
POC FIO2	50 % (N/A)	03/30/2026 08:43
POC Sample	Arterial (N/A)	03/30/2026 08:43
POC Site	Radial Right (N/A)	03/30/2026 08:43
POC Allen Test	POS (N/A)	03/30/2026 08:43
POC Del Sys	BIPAP (N/A)	03/30/2026 08:43
POC Vent Mode	NO VENT (N/A)	03/30/2026 08:43
POC Set RR	14 br/min (N/A)	03/30/2026 08:43
POC IPAP	8 cm H2O (N/A)	03/30/2026 08:43
POC EPAP	4 cm H2O (N/A)	03/30/2026 08:43
Notified To and RBV	DR DAVIDSON (N/A)	03/30/2026 08:43
Notified By	vhsmurrat (N/A)	03/30/2026 08:43
Notified Note	ALL CV WITH RB (N/A)	03/30/2026 08:43

Cardiac

BNP	2004 pg/mL (High)	03/30/2026 07:31
Troponin TNIH	328.77 ng/L (Critical)	03/30/2026 22:34

General Chemistry

Glucose Level	72 mg/dL (Low)	03/31/2026 02:04
POC Glucose	75 mg/dL (Normal)	03/31/2026 07:11
Sodium	140 mmol/L (Normal)	03/31/2026 02:04
Potassium	4.2 mmol/L (Normal)	03/31/2026 02:04
Chloride	100 mmol/L (Normal)	03/31/2026 02:04
CO2	31 mmol/L (Normal)	03/31/2026 02:04
Anion Gap	9 mmol/L (Normal)	03/31/2026 02:04
BUN	17 mg/dL (Normal)	03/31/2026 02:04
Creatinine	2.19 mg/dL (High)	03/31/2026 02:04
BUN/Creat Ratio	8 (Normal)	03/31/2026 02:04
Calcium	9.0 mg/dL (Normal)	03/31/2026 02:04
Albumin. Level	3.4 gm/dL (Normal)	03/31/2026 02:04
TP	6.4 gm/dL (Normal)	03/30/2026 22:34
A/G Ratio	1.3 (Normal)	03/30/2026 22:34
T Bili	0.8 mg/dL (Normal)	03/30/2026 22:34

Progress Note - Physician

D Bili	0.2 mg/dL (Normal)	03/30/2026 22:34
Alk Phos	125 units/L (High)	03/30/2026 22:34
AST	97 units/L (High)	03/30/2026 22:34
ALT	49 units/L (Normal)	03/30/2026 22:34
Mg Lvl	1.9 mg/dL (Normal)	03/31/2026 02:04
Phosphorus	5.6 mg/dL (High)	03/31/2026 02:04
Estimated Creatinine Clearance	10.11 mL/min ()	03/31/2026 02:58
eGFR Cr	See Comment mL/min/1.73m2 (N/A)	03/31/2026 02:04
eGFR Pediatric	Not Reported (N/A)	03/31/2026 02:04
Lactic Acid Lvl	1.8 mmol/L (Normal)	03/30/2026 08:48
Hgb A1C	4.3 % (Normal)	03/30/2026 11:15
MAR Glucose Result	102 ()	03/30/2026 09:49

General Hematology

WBC	5.87 x10e3/mcL (Normal)	03/31/2026 02:04
RBC	3.65 x10e6/mcL (Normal)	03/31/2026 02:04
Hgb	8.4 gm/dL (Low)	03/31/2026 02:04
Hct	25.6 % (Low)	03/31/2026 02:04
MCV	70.1 Femtoliters (Low)	03/31/2026 02:04
MCH	23.0 pg (Low)	03/31/2026 02:04
MCHC	32.8 gm/dL (Normal)	03/31/2026 02:04
RDW-SD	45.2 Femtoliters (Normal)	03/31/2026 02:04
RDW-CV	18.6 % (High)	03/31/2026 02:04
Plt	141 x10e3/mcL (Normal)	03/31/2026 02:04
MPV	9.6 Femtoliters (Normal)	03/31/2026 02:04
Imm Plt Fract	3.7 % (Normal)	03/31/2026 02:04
NRBC %	0.2 /100 WBC (Normal)	03/30/2026 07:31
NRBC #	0.02 x10e3/mcL (High)	03/30/2026 07:31
Neut % Auto	80.7 % (N/A)	03/31/2026 02:04
Lymph % Auto	12.4 % (N/A)	03/31/2026 02:04
Mono % Auto	3.7 % (N/A)	03/31/2026 02:04
Eos % Auto	2.0 % (N/A)	03/31/2026 02:04
Baso % Auto	0.7 % (N/A)	03/31/2026 02:04
Immature Grans %	0.5 % (Normal)	03/31/2026 02:04

Progress Note - Physician

Neut # Auto	4.73 x10e3/mcL (Normal)	03/31/2026 02:04
Lymph # Auto	0.73 x10e3/mcL (Low)	03/31/2026 02:04
Mono # Auto	0.22 x10e3/mcL (Normal)	03/31/2026 02:04
Eos # Auto	0.12 x10e3/mcL (Normal)	03/31/2026 02:04
Baso # Auto	0.04 x10e3/mcL (Normal)	03/31/2026 02:04
Immature Grans # Auto	0.03 x10e3/mcL (Normal)	03/31/2026 02:04
RBC Morph	Abnormal (Abnormal)	03/30/2026 07:31
Plt Morphology	Normal (Normal)	03/30/2026 07:31
Polychrom	1+ (Abnormal)	03/30/2026 07:31
Micro	2+ (Abnormal)	03/30/2026 07:31
Plt Est	Adequate (Normal)	03/30/2026 07:31
Schistocytes	1+ (Abnormal)	03/30/2026 07:31
Target Cells (Codocytes)	3+ (Abnormal)	03/30/2026 07:31
Teardrop Cells (Dacrocytes)	1+ (Abnormal)	03/30/2026 07:31

Hepatitis

Hep A IgM	Reactive (Abnormal)	03/30/2026 10:54
Hep B Core IgM	Non-Reactive (Normal)	03/30/2026 11:04
Hepatitis B Surface Antigen	Non-Reactive (Normal)	03/30/2026 11:04
Hep Bs Ab Qn	31.9 milli intl units/mL (High)	03/30/2026 11:04
Hep Bs Ab Qn Interp	Reactive (Abnormal)	03/30/2026 11:04
Hep C Ab	Non-Reactive (Normal)	03/30/2026 10:54

Infectious Disease

Flu A RNA	Negative (Normal)	03/30/2026 11:15
Flu B RNA	Negative (Normal)	03/30/2026 11:15

Other Chemistry

Procalcitonin Lvl	1.56 ng/mL (High)	03/30/2026 11:15

Other Microbiology

MRSA by PCR	Not Detected (Normal)	03/31/2026 01:48

SARS-CoV-2/Covid-19

Progress Note - Physician

SARS-CoV2 RNA	Negative (Normal)	03/30/2026 11:15

Thyroid

T3 Free	2.87 pg/mL (Normal)	03/30/2026 11:15
T4 Free	1.74 ng/dL (High)	03/30/2026 11:15
TSH Ultrasensitive	3.046 mc Intl units/mL (Normal)	03/30/2026 11:15

BMP, Mg, and Phos (Last Within 24hrs)
Sodium: 140 mmol/L (02:04)
Potassium: 4.2 mmol/L (02:04)
Chloride: 100 mmol/L (02:04)
CO2: 31 mmol/L (02:04)
BUN: 17 mg/dL (02:04)
Creatinine: 2.19 mg/dL High (02:04)
Glucose Level: 72 mg/dL Low (02:04)
Calcium: 9 mg/dL (02:04)
Mg Lvl: 1.9 mg/dL (02:04)
Phosphorus: 5.6 mg/dL High (02:04)

LFT (Last Within 24hrs)
AST: 97 units/L High (22:34)
ALT: 49 units/L (22:34)
Alk Phos: 125 units/L High (22:34)
T Bili: 0.8 mg/dL (22:34)
D Bili: 0.2 mg/dL (22:34)
TP: 6.4 gm/dL (22:34)

Microbiology

Culture Blood ARD **In-Lab**

Source:	Blood	Body Site:	
Collected Dt/Tm:	03/31/2026 02:12	Last Updated Dt/Tm:	03/31/2026 00:41

Culture Blood ARD **In-Lab**

Source:	Blood	Body Site:	
Collected Dt/Tm:	03/31/2026 02:04	Last Updated Dt/Tm:	03/31/2026 00:41

Culture Sputum **Final**

Source:	Sputum Expectorated	Body Site:	
Collected Dt/Tm:	03/30/2026 22:34	Last Updated Dt/Tm:	03/31/2026 08:53

Gram stain contains >=10 squamous epithelial cells per low power field which suggests oropharyngeal and/or nasopharyngeal contamination with no predominant bacterial morphology. Culture will not be performed.

Progress Note - Physician

Culture Blood ARD **Preliminary**

Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	03/31/2026 09:02

No growth at 24 hours.

Culture Blood ARD **Preliminary**

Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	03/31/2026 10:02

Morganella morganii
Identification obtained by Sepsityper rapid identification method.
Culture confirmation to follow.
Susceptibility to follow.

Assessment/Plan
Assessment:
Physical Exam
General: Thin appearing elderly female
Mental Status: Baseline dementia
Head: Normocephalic, atraumatic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact. Anicteric.
Ear, Nose and Throat: Oropharynx clear, moist mucous membranes. Trachea midline.
Neck: No obvious neck masses
Respiratory: Clear to auscultation bilaterally. No wheezing, crackles or rhonchi. Normal respiratory effort.
Cardiovascular: Regular rate and rhythm, no murmurs, gallops or rubs.
Gastrointestinal: Non-tender, non-distended. Normal bowel sounds.
Genitourinary: No suprapubic tenderness.
Neurologic: Cranial nerves II-XII grossly intact. Moves all extremities.
Light touch sensation is grossly intact bilaterally.

End-stage renal disease on hemodialysis Monday Wednesday Friday via IJ permacath
Hyperkalemia
Hypertension
Shortness of breath
Pneumonia

Plan
Hemodialysis stat yesterday given hyperkalemia and shortness of breath
Monitor daily for dialysis needs
Potassium should improve postdialysis
Continue amlodipine hydralazine metoprolol

Progress Note - Physician

Continue sevelamer. Monitor phosphorus level
Antibiotics. Follow-up cultures

Discharge Planning:
» Discharge Planning:
» Discharge To, Anticipated: Home with family care

Electronically Signed By: Vornicu, Maria MD
On: 03.31.2026 12:24 PDT

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	3/31/2026 08:17 PDT
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Hirabayashi DO,Ellyn (3/31/2026 08:17 PDT)
SIGN INFORMATION:	Techathaveewat DO,Worawith (3/31/2026 14:40 PDT); Hirabayashi DO,Ellyn (3/31/2026 11:47 PDT)

Subjective

Chief Complaint/Reason for Consultation
Chief Complaint: Patient brought in by EMS from home with complaints of SOB. Per the family, patient started to complain of SOB this morning. Patient found with a RA sat of <50%. Patient provided with BVM ventilation and then placed on NRB. Patient A/Ox0. EMS unable to obtain demographics on scene. BGL 119

Past Medical History
Active Problems(3)
Hemodialysis
Hepatitis A virus
High cholesterol

Past Surgical History
No surgery history documented.

Social History
No social history documented.

Objective

Measurements (most recent)

Height (cm)	157.48 cm	Stasiewski RN, Sabryna	03/30 07:08
Height (inches)	62.00 inch	SYSTEM	03/30 07:08
Weight	37.50 kg	Parala RN, Marielle Vianne	03/30 23:48

Progress Note - Physician

Weight (lbs)	82.67 lb	SYSTEM	03/30 23:48
Body Mass Index	15.12 kg/m2	Parala RN, Marielle Vianne	03/30 23:48
Scale Type	Bed scale	Parala RN, Marielle Vianne	03/30 23:48

Vital Signs (24 hrs)	**Last Charted**		**Minimum**		**Maximum**	
Temp	37.1	03/31/2026 04:00	36.9	03/30/2026 22:45	37.1	03/30/2026 19:57
Apical Heart Rate	87	03/31/2026 07:12	87	03/31/2026 07:12	98	03/30/2026 18:54
Resp Rate	**H** 24	03/31/2026 07:41	15	03/31/2026 07:00	**C** 52	03/31/2026 03:45
SBP	**C** 182	03/31/2026 07:15	**H** 148	03/30/2026 11:30	**C** 198	03/31/2026 07:00
DBP	77	03/31/2026 07:15	70	03/30/2026 11:30	**H** 90	03/30/2026 21:30
MAP	110	03/31/2026 07:15	100	03/30/2026 11:30	129	03/30/2026 18:00
SpO2	100	03/31/2026 07:41	**C** 86	03/31/2026 04:15	100	03/30/2026 21:29
O2 Flow Rate	on 2 l/min				on 4 l/min	
O2 Therapy	Nasal cannula				Nasal cannula	

Ventilator Settings (Last Within 24hrs)

Ventilator Mode: SPN-BiPAP (02:05) FIO2: 3 % (00:00)

Inspiratory Pressure: 5 cm H2O (02:05)

Expiratory Pressure: 8 cm H2O (02:05)

Intake and Output (current encounter)

	03/30 07:06 03/31 08:17	03/31 07:00 03/31 08:17	03/30 07:00 03/31 06:59	03/29 07:00 03/30 06:59	03/28 07:00 03/29 06:59
Intake	300	0	300	0	0
Output	1800	0	1800	0	0
Net Total	-1500	0	-1500	0	0

Physical Exam

Home Medications

amLODIPine (amLODIPine 10 mg oral tablet)
hydrALAZINE (hydrALAZINE 50 mg oral tablet)
losartan (losartan 100 mg oral tablet)
metoprolol (metoprolol succinate 50 mg oral tablet, extended release)
minoxidil (minoxidil 2.5 mg oral tablet)
multivitamin (Rena-Vite oral tablet)
ondansetron (ondansetron 4 mg oral tablet)

Progress Note - Physician

sevelamer (sevelamer 800 mg oral tablet)
zolpidem (zolpidem 5 mg oral tablet)

Inpatient Medications
Medications (22) Active
Scheduled: (11)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 10 mg Tab (amLODIPine) 10 mg 1 Tabs, Oral, Daily
cefTRIAXone/D5W (Rocephin) 50 mL, IV Piggyback, q24H Interval
doxycycline/D5W (doxycycline) 100 mg 100 mL, IV Piggyback, q12H
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
heparin 5000 units/mL injection 1 mL (heparin) 5,000 units 1 mL, SubCutaneous, q12H
hydrALAZINE 50 mg Tab (hydrALAZINE) 50 mg 1 Tabs, Oral, TID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
losartan 100 mg Tab (losartan) 100 mg 1 Tabs, Oral, Daily
metoprolol succinate 50 mg XL Tab (Metoprolol Succinate ER) 50 mg 1 Tabs, Oral, qHS
sevelamer carbonate 800 mg Tab (sevelamer) 800 mg 1 Tabs, Oral, TID w/Meals
Continuous: (0)
PRN: (11)
acetaminophen 325 mg Tab (Tylenol) 650 mg 2 Tabs, Oral, q4H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q3H
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID
Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 25 gm 50 mL, IV Push, q1H Interval
docusate sodium 100 mg Cap (Colace) 100 mg 1 Caps, Oral, BID
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H
glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q2H Interval
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q2H Interval
melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H

72 hour Antibiotic History

Active Antibiotics	Day of Therapy:	Last Administered	First Administered
doxycycline 100 mg, 100 mL, 100 mL/hr, IV Piggyback, q12H	2	03/30/2026 21:13	03/30/2026 21:13

Completed/Discontinued Antibiotics	Stop Date/Time	Last Administered	First Administered
azithromycin 500 mg, 250 mL, 250 mL/hr, IV Piggyback, Once	03/30/2026 10:10	03/30/2026 10:09	03/30/2026 10:09
cefTRIAXone 1 gm, 50 mL, 100 mL/hr, IV Piggyback, Once	03/30/2026 09:18	03/30/2026 09:10	03/30/2026 09:10

Progress Note - Physician

Results
Recent Labs
Blood Gases

Art Na	137 mmol/L (Normal)	03/30/2026 08:43
Art K	6.3 mmol/L (Critical)	03/30/2026 08:43
Art iCa	0.96 mmol/L (Low)	03/30/2026 08:43
pH Arterial	7.54 (High)	03/30/2026 08:43
Art pCO2	39.4 mmHg (Normal)	03/30/2026 08:43
Art pO2	153.0 mmHg (High)	03/30/2026 08:43
HCO3 Art	34.4 mmol/L (High)	03/30/2026 08:43
Arterial pCO2(T)	39.4 mmHg (N/A)	03/30/2026 08:43
Arterial pH(T)	7.54 (N/A)	03/30/2026 08:43
Arterial pO2(T)	153.0 mmHg (N/A)	03/30/2026 08:43
Art Base Excess	10.6 mmol/L (High)	03/30/2026 08:43
Art sO2	99 % (High)	03/30/2026 08:43
Art tHB	10.3 gm/dL (Low)	03/30/2026 08:43
Art Hct	32 % (Low)	03/30/2026 08:43
POC Pt Temp	37.0 DegC (N/A)	03/30/2026 08:43
POC FIO2	50 % (N/A)	03/30/2026 08:43
POC Sample	Arterial (N/A)	03/30/2026 08:43
POC Site	Radial Right (N/A)	03/30/2026 08:43
POC Allen Test	POS (N/A)	03/30/2026 08:43
POC Del Sys	BIPAP (N/A)	03/30/2026 08:43
POC Vent Mode	NO VENT (N/A)	03/30/2026 08:43
POC Set RR	14 br/min (N/A)	03/30/2026 08:43
POC IPAP	8 cm H2O (N/A)	03/30/2026 08:43
POC EPAP	4 cm H2O (N/A)	03/30/2026 08:43
Notified To and RBV	DR DAVIDSON (N/A)	03/30/2026 08:43
Notified By	vhsmurrat (N/A)	03/30/2026 08:43
Notified Note	ALL CV WITH RB (N/A)	03/30/2026 08:43

Cardiac

BNP	2004 pg/mL (High)	03/30/2026 07:31
Troponin TNIH	328.77 ng/L (Critical)	03/30/2026 22:34

General Chemistry

Glucose Level	72 mg/dL (Low)	03/31/2026 02:04
POC Glucose	75 mg/dL (Normal)	03/31/2026 07:11
Sodium	140 mmol/L (Normal)	03/31/2026 02:04
Potassium	4.2 mmol/L (Normal)	03/31/2026 02:04
Chloride	100 mmol/L (Normal)	03/31/2026 02:04

Progress Note - Physician

CO2	31 mmol/L (Normal)	03/31/2026 02:04
Anion Gap	9 mmol/L (Normal)	03/31/2026 02:04
BUN	17 mg/dL (Normal)	03/31/2026 02:04
Creatinine	2.19 mg/dL (High)	03/31/2026 02:04
BUN/Creat Ratio	8 (Normal)	03/31/2026 02:04
Calcium	9.0 mg/dL (Normal)	03/31/2026 02:04
Albumin. Level	3.4 gm/dL (Normal)	03/31/2026 02:04
TP	6.4 gm/dL (Normal)	03/30/2026 22:34
A/G Ratio	1.3 (Normal)	03/30/2026 22:34
T Bili	0.8 mg/dL (Normal)	03/30/2026 22:34
D Bili	0.2 mg/dL (Normal)	03/30/2026 22:34
Alk Phos	125 units/L (High)	03/30/2026 22:34
AST	97 units/L (High)	03/30/2026 22:34
ALT	49 units/L (Normal)	03/30/2026 22:34
Mg Lvl	1.9 mg/dL (Normal)	03/31/2026 02:04
Phosphorus	5.6 mg/dL (High)	03/31/2026 02:04
Estimated Creatinine Clearance	10.11 mL/min ()	03/31/2026 02:58
eGFR Cr	See Comment mL/min/1.73m2 (N/A)	03/31/2026 02:04
eGFR Pediatric	Not Reported (N/A)	03/31/2026 02:04
Lactic Acid Lvl	1.8 mmol/L (Normal)	03/30/2026 08:48
Hgb A1C	4.3 % (Normal)	03/30/2026 11:15
MAR Glucose Result	102 ()	03/30/2026 09:49

General Hematology

WBC	5.87 x10e3/mcL (Normal)	03/31/2026 02:04
RBC	3.65 x10e6/mcL (Normal)	03/31/2026 02:04
Hgb	8.4 gm/dL (Low)	03/31/2026 02:04
Hct	25.6 % (Low)	03/31/2026 02:04
MCV	70.1 Femtoliters (Low)	03/31/2026 02:04
MCH	23.0 pg (Low)	03/31/2026 02:04
MCHC	32.8 gm/dL (Normal)	03/31/2026 02:04
RDW-SD	45.2 Femtoliters (Normal)	03/31/2026 02:04
RDW-CV	18.6 % (High)	03/31/2026 02:04
Plt	141 x10e3/mcL (Normal)	03/31/2026 02:04
MPV	9.6 Femtoliters (Normal)	03/31/2026 02:04

Progress Note - Physician

Imm Plt Fract	3.7 % (Normal)	03/31/2026 02:04
NRBC %	0.2 /100 WBC (Normal)	03/30/2026 07:31
NRBC #	0.02 x10e3/mcL (High)	03/30/2026 07:31
Neut % Auto	80.7 % (N/A)	03/31/2026 02:04
Lymph % Auto	12.4 % (N/A)	03/31/2026 02:04
Mono % Auto	3.7 % (N/A)	03/31/2026 02:04
Eos % Auto	2.0 % (N/A)	03/31/2026 02:04
Baso % Auto	0.7 % (N/A)	03/31/2026 02:04
Immature Grans %	0.5 % (Normal)	03/31/2026 02:04
Neut # Auto	4.73 x10e3/mcL (Normal)	03/31/2026 02:04
Lymph # Auto	0.73 x10e3/mcL (Low)	03/31/2026 02:04
Mono # Auto	0.22 x10e3/mcL (Normal)	03/31/2026 02:04
Eos # Auto	0.12 x10e3/mcL (Normal)	03/31/2026 02:04
Baso # Auto	0.04 x10e3/mcL (Normal)	03/31/2026 02:04
Immature Grans # Auto	0.03 x10e3/mcL (Normal)	03/31/2026 02:04
RBC Morph	Abnormal (Abnormal)	03/30/2026 07:31
Plt Morphology	Normal (Normal)	03/30/2026 07:31
Polychrom	1+ (Abnormal)	03/30/2026 07:31
Micro	2+ (Abnormal)	03/30/2026 07:31
Plt Est	Adequate (Normal)	03/30/2026 07:31
Schistocytes	1+ (Abnormal)	03/30/2026 07:31
Target Cells (Codocytes)	3+ (Abnormal)	03/30/2026 07:31
Teardrop Cells (Dacrocytes)	1+ (Abnormal)	03/30/2026 07:31

Hepatitis

Hep A IgM	Reactive (Abnormal)	03/30/2026 10:54
Hep B Core IgM	Non-Reactive (Normal)	03/30/2026 11:04
Hepatitis B Surface Antigen	Non-Reactive (Normal)	03/30/2026 11:04
Hep Bs Ab Qn	31.9 milli intl units/mL (High)	03/30/2026 11:04
Hep Bs Ab Qn Interp	Reactive (Abnormal)	03/30/2026 11:04
Hep C Ab	Non-Reactive (Normal)	03/30/2026 10:54

Progress Note - Physician

Other Chemistry

Procalcitonin Lvl	1.56 ng/mL (High)	03/30/2026 11:15

Thyroid

T3 Free	2.87 pg/mL (Normal)	03/30/2026 11:15
T4 Free	1.74 ng/dL (High)	03/30/2026 11:15
TSH Ultrasensitive	3.046 mc Intl units/mL (Normal)	03/30/2026 11:15

Abnormal Labs

Blood Gases

Art K	6.3 mmol/L (Critical)	03/30/2026 08:43
Art iCa	0.96 mmol/L (Low)	03/30/2026 08:43
pH Arterial	7.54 (High)	03/30/2026 08:43
Art pO2	153.0 mmHg (High)	03/30/2026 08:43
HCO3 Art	34.4 mmol/L (High)	03/30/2026 08:43
Art Base Excess	10.6 mmol/L (High)	03/30/2026 08:43
Art sO2	99 % (High)	03/30/2026 08:43
Art tHB	10.3 gm/dL (Low)	03/30/2026 08:43
Art Hct	32 % (Low)	03/30/2026 08:43

Cardiac

BNP	2004 pg/mL (High)	03/30/2026 07:31
Troponin TNIH	328.77 ng/L (Critical)	03/30/2026 22:34

General Chemistry

Glucose Level	72 mg/dL (Low)	03/31/2026 02:04
Creatinine	2.19 mg/dL (High)	03/31/2026 02:04
Alk Phos	125 units/L (High)	03/30/2026 22:34
AST	97 units/L (High)	03/30/2026 22:34
Phosphorus	5.6 mg/dL (High)	03/31/2026 02:04

General Hematology

Hgb	8.4 gm/dL (Low)	03/31/2026 02:04
Hct	25.6 % (Low)	03/31/2026 02:04
MCV	70.1 Femtoliters (Low)	03/31/2026 02:04
MCH	23.0 pg (Low)	03/31/2026 02:04
RDW-CV	18.6 % (High)	03/31/2026 02:04
NRBC #	0.02 x10e3/mcL (High)	03/30/2026 07:31
Lymph # Auto	0.73 x10e3/mcL (Low)	03/31/2026 02:04
RBC Morph	Abnormal (Abnormal)	03/30/2026 07:31
Polychrom	1+ (Abnormal)	03/30/2026 07:31

Progress Note - Physician

Micro	2+ (Abnormal)	03/30/2026 07:31
Schistocytes	1+ (Abnormal)	03/30/2026 07:31
Target Cells (Codocytes)	3+ (Abnormal)	03/30/2026 07:31
Teardrop Cells (Dacrocytes)	1+ (Abnormal)	03/30/2026 07:31

Hepatitis

Hep A IgM	Reactive (Abnormal)	03/30/2026 10:54
Hep Bs Ab Qn	31.9 milli intl units/mL (High)	03/30/2026 11:04
Hep Bs Ab Qn Interp	Reactive (Abnormal)	03/30/2026 11:04

Other Chemistry

Procalcitonin Lvl	1.56 ng/mL (High)	03/30/2026 11:15

Thyroid

T4 Free	1.74 ng/dL (High)	03/30/2026 11:15

CBC, BMP, Coagulation Trend (last 4 resulted)

WBC	**5.87** 03/31/2026 02:04	**12.25 H** 03/30/2026 07:31		
Hgb	**8.4 L** 03/31/2026 02:04	**11.2** 03/30/2026 07:31		
Hct	**25.6 L** 03/31/2026 02:04	**33.7** 03/30/2026 07:31		
Baso # Auto	**0.04** 03/31/2026 02:04	**0.03** 03/30/2026 07:31		
Baso % Auto	**0.7** 03/31/2026 02:04	**0.2** 03/30/2026 07:31		
Eos # Auto	**0.12** 03/31/2026 02:04	**0.02** 03/30/2026 07:31		
Eos % Auto	**2.0** 03/31/2026 02:04	**0.2** 03/30/2026 07:31		
Immature Grans %	**0.5** 03/31/2026 02:04	**0.6** 03/30/2026 07:31		
Immature Grans # Auto	**0.03** 03/31/2026 02:04	**0.07** 03/30/2026 07:31		
Lymph # Auto	**0.73 L** 03/31/2026 02:04	**1.33** 03/30/2026 07:31		
Lymph % Auto	**12.4** 03/31/2026 02:04	**10.9** 03/30/2026 07:31		
Mono # Auto	**0.22** 03/31/2026 02:04	**0.47** 03/30/2026 07:31		

Progress Note - Physician

Mono % Auto	**3.7** 03/31/2026 02:04	**3.8** 03/30/2026 07:31		
Neut # Auto	**4.73** 03/31/2026 02:04	**10.33 H** 03/30/2026 07:31		
Neut % Auto	**80.7** 03/31/2026 02:04	**84.3** 03/30/2026 07:31		
Plt	**141** 03/31/2026 02:04	**230** 03/30/2026 07:31		
Glucose Level	**72 L** 03/31/2026 02:04	**189 H** 03/30/2026 07:31		
Sodium	**140** 03/31/2026 02:04	**139** 03/30/2026 07:31		
Potassium	**4.2** 03/31/2026 02:04	**5.9 H** 03/30/2026 07:31		
Chloride	**100** 03/31/2026 02:04	**94 L** 03/30/2026 07:31		
CO2	**31** 03/31/2026 02:04	**27** 03/30/2026 07:31		
Creatinine	**2.19 H** 03/31/2026 02:04	**4.80 H** 03/30/2026 07:31		
BUN	**17** 03/31/2026 02:04	**44 H** 03/30/2026 07:31		
BUN/Creat Ratio	**8** 03/31/2026 02:04	**9** 03/30/2026 07:31		
Mg Lvl	**1.9** 03/31/2026 02:04			
Phosphorus	**5.6 H** 03/31/2026 02:04	**10.5 H** 03/30/2026 13:08		
Calcium	**9.0** 03/31/2026 02:04	**8.6 L** 03/30/2026 07:31		
Albumin. Level	**3.4** 03/31/2026 02:04	**3.6** 03/30/2026 22:34	**3.4** 03/30/2026 11:15	
T Bili	**0.8** 03/30/2026 22:34	**0.4** 03/30/2026 11:15		
D Bili	**0.2** 03/30/2026 22:34	**0.2** 03/30/2026 11:15		
Alk Phos	**125 H** 03/30/2026 22:34	**150 H** 03/30/2026 11:15		
AST	**97 H** 03/30/2026 22:34	**57 H** 03/30/2026 11:15		
ALT	**49** 03/30/2026 22:34	**51** 03/30/2026 11:15		

Progress Note - Physician

Assessment/Plan
Assessment:
Subjective:
Patient is a 90-year-old female with a PMHx of ESRD on HD, chronic hypoxic respiratory failure requiring 2–5 L home O2 who presented to Valley Hospital with reports of increasing shortness of breath. Patient was initially admitted to the ICU for stat dialysis and continuous BiPAP.

Seen and examined at bedside. Today she is back to her baseline O2. Now s/p stat HD. Overall doing well.

Physical exam:
General: Thin appearing Vietnamese speaking woman sitting up in bed.
Mental Status: Alert and oriented to person, place, time. Normal affect.
Head: Normocephalic, atraumatic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact. Anicteric.
Ear, Nose and Throat: Oropharynx clear, moist mucous membranes. Trachea midline.
Neck: No obvious neck masses
Respiratory: Clear to auscultation bilaterally. No wheezing, crackles or rhonchi. Normal respiratory effort. On LFNC
Cardiovascular: Regular rate and rhythm, no murmurs, gallops or rubs. Right sided subclavian HD catheter noted
Gastrointestinal: Non-tender, non-distended. Normal bowel sounds.
Genitourinary: No suprapubic tenderness.
Neurologic: Cranial nerves II-XII grossly intact. Moves all extremities.
Light touch sensation is grossly intact bilaterally.

Assessment:
Acute on chronic hypoxic respiratory failure 2/2 pneumonia versus fluid overload, improved
Dyspnea
Sepsis with RBC, HR 2/2 respiratory source
Troponinemia 2/2 demand ischemia
ESRD on HD MWF
Hyperkalemia 2/2 above
Hepatitis A IgM positive
Oxygen dependent reportedly on 2 to 5 L at home
Hypertension
Dementia

Plan:
Downgraded from ICU level of care
CT chest/abdomen/pelvis with interlobular septal thickening and adjacent ground-glass opacities in bilateral lungs possibly pulmonary edema versus infectious/inflammatory versus less likely hemorrhagic. Small to moderate right and small left-sided pleural effusions.
Acute on chronic hypoxic respite failure secondary to pneumonia in addition to fluid overload
– Blood cultures 1/2 positive for gram-negative rods
– Continue antibiotics with Rocephin, doxycycline. Will adjust as indicated by cultures and sensitivities
– Breathing treatments
– Goal SpO2 92%, titrated down to LFNC 2L
Nephrology consulted appreciate recommendation
– S/p stat HD
Troponinemia secondary to demand ischemia, patient denies chest pain
– TTE pending
PRN antihypertensives and antiemetics
Speech therapy eval pending
Replete electrolytes as needed, routine labs, and supportive care.

Patient has 5 children but main caretaker is brother who lives with her. ICU team discussed with extensively regarding CODE STATUS with patient's daughter and brother at the bedside who had different opinion regarding CODE STATUS. They wish to proceed with full

Progress Note - Physician

code no shock for now but has plan to discuss among family members.

Consults: ICU, nephrology Dr. Vornicu
Diet: N.p.o., pending speech therapy evaluation
Activity: Early and progressive mobilization
GI prophylaxis: None
DVT prophylaxis: Subcu heparin, SCD
CODE STATUS: Full code, no shock per family
Dispo: IMC

Will discuss plan with attending physician. Awaiting further recommendations.

Ellyn Hirabayashi, DO
Internal Medicine - PGY3
Valley Hospital Medical Center

Discharge Planning:
» Discharge Planning:
» Discharge To, Anticipated: Home with family care

Electronically Signed By: Hirabayashi, Ellyn DO
On: 03.31.2026 11:47 PDT

Electronically Signed On: 03.31.2026 14:40 PDT
Techathaveewat, Worawith DO

Patient Information Form

Hospital Name: VALLEY HOSPITAL MEDICAL CENTER **Phone:** (702) 388-4000

Hospital Address: 620 Shadow Ln LAS VEGAS, NV 89106

Patient Demographics

Patient Name: VU, NANCY
Medical Rec #: 63892637 **SSN:**
Marital Status: W **Gender:** F
Date of Birth: 07-12-1935 **Age:** 90
Religion: NON
Episode ID: 1101330304
Height:
Weight:

Admit and Length of Stay Information

Unit: MICU
Admit Type: EMERGENCY **Room:** 016
Admission Date: 03-30-2026 **Bed:** 01
Est. Discharge Date: 04-07-2026 **ALC Date:**
Pt. Functional Status Prior to Admission:
PCP: Wu, Julie
Attending Physician: Techathaveewat, Worawith
Phone Number:

Diagnosis Information:

Rugs: **Primary:** Hypoxemia **Secondary:** End stage renal disease

Discharge Notes:

Notes:

Mode of Transportation:
Will patient receive radiation or dialysis off-site?
☐ Yes ☐ No

Payer Source:
Schedule of Treatments:

Patient Address

Living Arrangement: Select One
Facility Name:
Street: 6750 DEL REY AVE UNIT 12
City: LAS VEGAS
State/Zip: NV 89146
Home Phone: (510)449-6994
Work Phone:

Next of Kin

First Name/MI: KHANH
Last Name: DANG
Street:
City:
State/Zip:
Home Phone: (669)312-8595
Work Phone:
Relation: N
☐ Emerg. Contact ☐ POA

Emergency Contact:

First Name/MI:
Last Name:
Street:
City:
State/Zip:
Home Phone:
Work Phone:
Relation:
☐ Emerg. Contact ☐ POA

Payer Information:

Ins. Group ID#:
Primary Payer: MEDICARE PART A **Member ID#:** 9U55KH1VF40
Contact person at Ins. Co. (First/MI/Last): **Phone:**
Patient has met 3 consecutive, acute level of care days during this admission & may be eligible for the Medicare Extended Care Benefit. ☐ Yes ☐ No ☐ N/A ☐ Unknown

Secondary Payer: MEDICAID 2ND MCARE **Member ID#:** 00002923815 **Phone #:**
Other Payer: **Member ID#:** **Phone #:**

Income if known: ☐ Private Funds ☐ SSA ☐ SSI ☐ Pension ☐ VA ☐ Other

Patient Medicaid Eligible? ☐ Yes ☐ No **If Yes, submitted by our financial office?** ☐ Yes ☐ No
Contact person in financial office (First/MI/Last):

No Fault: Must have claim number, name of insurance company, name of insured, telephone number, and policy number. This information is needed even if no fault is exhausted. No faults must have secondary insurance information.

Case Contact

First/MI/Last: Esther Larks **Phone** (702) 388-4000 ext. 46505 **Date:** 04-01-2026

VHM- Valley Hospital Medical Center
620 Shadow Lane
Las Vegas, NV 89106-4194
(702) 388-4000

Patient Information

Patient Name: VU, NANCY
Home Address: 6750 DEL REY AVE UNIT 124
LAS VEGAS, NV 891461058
Mobile: (510) 449-6994
Employer Name: Retired
Employer Phone:
Retire Date: 07/01/2000

Sex: Female **Language:** Vietnamese
DOB: 07/12/1935 **Race:** Asian
Age: 90 Years **SSN:**
Religion: None
Marital Status: Widowed
Contact: Phone Calls and Visitor
EMail: No Email Address

Guarantor Information

Guarantor Name: VU, NANCY
Patient's Reltn: Self
Billing Address: 6750 DEL REY AVE UNIT 124
LAS VEGAS, NV 891461058
Billing Phone: (510) 449-6994

Sex: Female **DOB:** 07/12/1935
Employer Name: Retired
Employer Phone:

Contact Information

Emergency Contact
Contact Name: KHANH DANG
Patient's Reltn: Mother
Sex:
Preferred Phone: (669) 312-8595

Next of Kin
Contact Name:
Patient's Reltn:
Sex:
Preferred Phone:

Primary Insurance

Subscriber Name: VU, NANCY
Patient's Reltn: Self
Sex: Female
DOB: 07/11/1935

Employer Name: Retired
Employer Phone:
Financial Class: Medicare
Group Name:

Insurance Name: Medicare Part A Only - VHS
Claim Address: MEDICARE PART A
MECHANICSBURG, PA 17055
Insurance Phone: (877) 235-8073
Policy Number: 9U55KH1VF40
Group Number:
Authorization Number:
Authorization Phone:
Auth Req:
Authorization Contact:

Secondary Insurance

Subscriber Name: VU, NANCY
Patient's Reltn: Self
Sex: Female
DOB: 07/11/1935

Employer Name: Retired
Employer Phone:
Financial Class: Medicaid
Group Name:

Insurance Name: Medicaid Secondary to Medicare -
Claim Address: MEDICAID 2ND MCARE
Reno, NV 89520
Insurance Phone: (877) 638-3472
Policy Number: 00002923815
Group Number:
Authorization Number:
Authorization Phone:
Auth Req:
Authorization Contact:

Encounter Information

Reg Dt/Tm: 03/30/2026 07:06 PDT
Est Dt of Arrival:
Inpt Adm Dt/Tm: 03/30/2026 09:37 PDT
Disch Dt/Tm:
Observ Dt/Tm:
VIP Indicator:
Admit Reason: Hypoxemia
Accident:
Accident Dt/Tm:

Patient Type: Inpatient
Medical Service: Intermediate
Location: VHM MICU
Room/Bed: 016 / 01
Est Responsibility: 0.00

Admit Type: Emergency
Admit Source: Emergency Department
Reg Clerk: Stasiewski RN, Sabryna
Admitting: Delmundo DO, Lloyd
Attending Techathaveewat DO, Worawith
PCP: Wu MD, Julie U
Referring: No Referring Physician
Last Visit Date: 03/30/2026

VU, NANCY
Female / 90 Years

MEDICAL RECORD

EL0012

Patient Identification

1101330304-63892637

VU, NANCY
07/12/1935 90 Ye F
ADM/REG DT: 03/30/2026 09:37

CCPS_REG_XR_FACESHEET Printed By: USER, XR DEFAULT on 04/01/2026 16:04to printer cr_facesheet_12002
Registration last updated by: Contreras, Theresa on 03/31/2026 15:10

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female

Admit: 3/30/2026
Disch: Disch Time:
FIN: VHM1101330304
Readmit Risk: 39 - LOW 03/31/26 05:45
Attending: Techathaveewat DO,Worawith

Facesheets

DOCUMENT NAME: Facesheets
SERVICE DATE/TIME: 3/30/2026 00:00 PDT
RESULT STATUS: Unauth
PERFORM INFORMATION:
SIGN INFORMATION:

FACE SHEET REGISTRATION FORM_20260330.pdf
Please click on link to display result.

Attachment(s): 3/30/2026 00:00 PDT FACE SHEET REGISTRATION FORM_20260330.pdf

VHMC-Vu, Nancy-Enc #1101330304-IPT-IMC- FACE SHEET REGISTRATION FORM - 3/30/2026 - 1 pg

MED REC# **63892637**

PT#	PATIENT NAME	CELL PHONE
1101330304	**VU, NANCY**	(510)449-6994

ADDRESS	COUNTY
6750 DEL REY AVE UNIT 124	

CITY	STATE	ZIP	PHONE	PATIENT LANGUAGE
LAS VEGAS	NV	891461058	(510)449-6994	VIETNAMESE

AGE	D.O.B.	SEX	RACE	MS	ETHNICITY	PREVIOUS NAME	RELIGION	INFANT AGE
90Y	07/12/1935	F	5	W	N	VHMCCHEPEN, FEMALE	NON	

PT EMPLOYER	TELEPHONE	EXT	OCCUPATION
Retired,			

EMPLOYER ADDRESS				
CITY	STATE	ZIP	RETIREMENT DATE	DISASTER TAG ID
			//	

Contact Information

NEAREST RELATIVE NAME				
RLTN	ADDRESS			
CITY	STATE	ZIP	PHONE	EXT.

EMERGENCY CONTACT NAME				
DANG, KHANH				
RLTN	ADDRESS			
N				
CITY	STATE	ZIP	PHONE	EXT.
			(669)312-8595	

Guarantor Information

GUARANTOR NAME	RLTN
VU, NANCY	S

ADDRESS	PHONE
6750 DEL REY AVE UNIT 124	(510)449-6994

CITY	STATE	ZIP	OCCUPATION
LAS VEGAS	NV	891461058	

GUARANTOR EMPLOYER	PHONE
Retired	

ADDRESS		
CITY	STATE	ZIP

Insurance Information

PC: M

INSURANCE NAME 1	PLAN	PHONE	POLICY#	GROUP#	AUTH#	AUTHORIZED BY
MEDICARE PART A	A00	(877)235-8073	9U55KH1VF40			

MAILING ADDRESS	CITY	STATE	ZIP
PO BOX 3385	MECHANICSBURG	PA	17055

SUBSCRIBER NAME	MAIL TO NAME	INS SEX	D.O.B.	GNM
VU, NANCY	MEDICARE PART A,	F	19350712	

COB	PAYOR ID	UR PHONE	EXT	INS VERIFIED	SUBSCRIBER RLTN	NOC REF NUMBER	NOC REF BY
1	A00			V	S		

INSURANCE NAME 2	PLAN	PHONE	POLICY#	GROUP#	AUTH#	AUTHORIZED BY
MEDICAID 2ND MCARE	D74	(877)638-3472	00002923815			

MAILING ADDRESS	CITY	STATE	ZIP
PO BOX 30042	Reno	NV	89520

SUBSCRIBER NAME	MAIL TO NAME	INS SEX	D.O.B.	GNM
VU, NANCY	MEDICAID 2ND MCARE	F	19350712	

COB	PAYOR ID	UR PHONE	EXT	INS VERIFIED	SUBSCRIBER RLTN	NOC REF NUMBER	NOC REF BY
2	D74			V	S		

INSURANCE NAME 3	PLAN	PHONE	POLICY#	GROUP#	AUTH#	AUTHORIZED BY

MAILING ADDRESS	CITY	STATE	ZIP

SUBSCRIBER NAME	MAIL TO NAME	INS SEX	D.O.B.	GNM

COB	PAYOR ID	UR PHONE	EXT	INS VERIFIED	SUBSCRIBER RLTN	NOC REF NUMBER	NOC REF BY

Admitting Information

ADM DATE	TIME	SRC	PT TYPE	PRI	ACCIDENT INFO.	VOLUNTARY ADM	ORGAN DNR	ISOL	PT STS
03/30/2026	0937	EO	H	X	//				X

ATTENDING PHYSICIAN	ATTENDING #	REF SRC	ADM BY	PREV ADM DATE	PREVIOUS FACILITY
Techathaveewat Worawith	939298		VHSCONTRT		

ADM DIAGNOSIS	HOSP SVC	DIAGNOSTIC CODE	SURGERY DATE	BLDLESS	N/S	RM/BED	Ethnicity
^Hypoxemia	IMC		//		MICU	016 01	N

ADMITTING PHYSICIAN	ADMITTING #	DISCHARGE DATE	TIME	DISCHARGE DISP	PREVIOUS DISCHARGE DATE
Delmundo Lloyd	916387				

PROCEDURE	CLINICAL COMMENT	MODE OF ARRIVAL / ACCOMPANIED BY
		AMB^Ambulance^VHM_INVISION

REFERRING PHYSICIAN	REFERRING #	EMAIL ADDRESS

REL OF INFO	ADV DIR	LIVING WILL	LOCATION OF WILL / DIRECTIVE	FAMILY DOCTOR	FAMILY DOCTOR #	DIV
				Wu	115444	VHM

Valley Hospital Medical Center
620 Shadow Lane
Las Vegas, NV 89106

MEDICAL RECORD

EL0012

UHS-9002
Rev. 04/2025

Patient Identification

1101330304-63892637

VU, NANCY
DOB: 07/12/1935 90Y SX: F IMC
MRN: 63892637 ADM/REG DT:03/30/2026
Valley Hospital Medical Center

Page 1 of 1

Progress Note - Physician

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	4/1/2026 13:55 PDT
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Vornicu MD,Maria D (4/1/2026 13:55 PDT)
SIGN INFORMATION:	Vornicu MD,Maria D (4/1/2026 13:55 PDT)

Subjective
End-stage renal disease on dialysis
Patient seen and examined during dialysis
Tolerating well
Discussed with dialysis staff
Dialysis parameters reviewed
Blood pressure elevated
Just received medications
Blood culture positive for morganella 1 out of 2

Chief Complaint/Reason for Consultation
Chief Complaint: Patient brought in by EMS from home with complaints of SOB. Per the family, patient started to complain of SOB this morning. Patient found with a RA sat of <50%. Patient provided with BVM ventilation and then placed on NRB. Patient A/Ox0. EMS unable to obtain demographics on scene. BGL 119

Allergies
Allergies (Active and Proposed Allergies Only)
No Known Allergies (Severity: Unknown severity, Onset: Unknown)

Objective

Measurements (most recent)

Height (cm)	157.48 cm	Erickson RD, LD, Lauren	03/31 11:07
Height (inches)	62.00 inch	SYSTEM	03/31 11:16
Weight	41.40 kg	Parala RN, Marielle Vianne	04/01 06:13
Weight (lbs)	91.27 lb	SYSTEM	04/01 06:13
Body Mass Index	15.12 kg/m2	Parala RN, Marielle Vianne	04/01 06:13
Scale Type	Bed scale	Parala RN, Marielle Vianne	04/01 06:13

Vital Signs (24 hrs)	**Last Charted**		**Minimum**		**Maximum**	
Temp	36.7	04/01/2026 08:00	36.7	04/01/2026 08:00	37	03/31/2026 16:00
Apical Heart Rate	84	04/01/2026 05:36	84	04/01/2026 05:36	**H** 102	03/31/2026 21:41
Resp Rate	20	04/01/2026 12:20	15	04/01/2026 05:00	**C** 37	03/31/2026 18:00
SBP	**H** 174	04/01/2026 06:01	131	03/31/2026 14:06	**C** 187	04/01/2026 02:37
DBP	77	04/01/2026 06:01	60	03/31/2026 14:06	**H** 82	04/01/2026 00:01
MAP	110	04/01/2026 06:01	86	03/31/2026 14:06	118	04/01/2026 00:01
SpO2	100	04/01/2026 12:20	**C** 84	03/31/2026 16:00	100	03/31/2026 18:29

Progress Note - Physician

O2 Flow Rate	on 2 l/min	on 2 l/min	on 2 l/min
O2 Therapy	Nasal cannula	Nasal cannula	Nasal cannula

Ventilator Settings (Last Within 24hrs)
Ventilator Mode: SPN-BiPAP (02:02)
Inspiratory Pressure: 5 cm H2O (02:02)
Expiratory Pressure: 8 cm H2O (02:02)

Intake and Output (current encounter)

	03/30 07:06 04/01 13:55	04/01 07:00 04/01 13:55	03/31 07:00 04/01 06:59	03/30 07:00 03/31 06:59	03/29 07:00 03/30 06:59
Intake	1713.3	300	1113.3	300	0
Output	3300	1500	0	1800	0
Net Total	-1586.7	-1200	1113.3	-1500	0
Urine Count	1	1	0	0	0
Stool Count	2	1	0	1	0

Physical Exam

Inpatient Medications
Medications (25) Active
Scheduled: (12)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 10 mg Tab (amLODIPine) 10 mg 1 Tabs, Oral, Daily
cefTRIAXone/D5W (Rocephin) 50 mL, IV Piggyback, q24H Interval
doxycycline/D5W (doxycycline) 100 mg 100 mL, IV Piggyback, q12H
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
heparin 5000 units/mL injection 1 mL (heparin) 5,000 units 1 mL, SubCutaneous, q12H
hydrALAZINE 50 mg Tab (hydrALAZINE) 50 mg 1 Tabs, Oral, TID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
losartan 100 mg Tab (losartan) 100 mg 1 Tabs, Oral, Daily
metoprolol succinate 50 mg XL Tab (Metoprolol Succinate ER) 50 mg 1 Tabs, Oral, qHS
sevelamer carbonate 800 mg Tab (sevelamer) 800 mg 1 Tabs, Oral, TID w/Meals
spironolactone 25 mg Tab (spironolactone) 25 mg 1 Tabs, Oral, BID
Continuous: (0)
PRN: (13)
acetaminophen 325 mg Tab (Tylenol) 650 mg 2 Tabs, Oral, q4H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q3H
benzonatate 100 mg Cap (Tessalon Perles) 100 mg 1 Caps, Oral, TID
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID
Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 25 gm 50 mL, IV Push, q1H Interval
docusate sodium 100 mg Cap (Colace) 100 mg 1 Caps, Oral, BID
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H

Progress Note - Physician

glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q2H Interval
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q2H Interval
melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H
sodium citrate 4% soln 5 mL (sodium citrate) 200 mg 5 mL, IntraCatheter, As Directed

72 hour Antibiotic History

Active Antibiotics	Day of Therapy:	Last Administered	First Administered
cefTRIAXone 2 gm, 50 mL, 100 mL/hr, IV Piggyback, q24H Interval	2	04/01/2026 13:14	03/31/2026 10:13
doxycycline 100 mg, 100 mL, 100 mL/hr, IV Piggyback, q12H	3	04/01/2026 07:58	03/30/2026 21:13

Completed/Discontinued Antibiotics	Stop Date/Time	Last Administered	First Administered
azithromycin 500 mg, 250 mL, 250 mL/hr, IV Piggyback, Once	03/30/2026 10:10	03/30/2026 10:09	03/30/2026 10:09
cefTRIAXone 1 gm, 50 mL, 100 mL/hr, IV Piggyback, Once	03/30/2026 09:18	03/30/2026 09:10	03/30/2026 09:10

Results
Recent Labs
General Chemistry

Glucose Level	98 mg/dL (Normal)	04/01/2026 10:27
POC Glucose	75 mg/dL (Normal)	03/31/2026 07:11
Sodium	141 mmol/L (Normal)	04/01/2026 10:27
Potassium	3.8 mmol/L (Normal)	04/01/2026 10:27
Chloride	101 mmol/L (Normal)	04/01/2026 10:27
CO2	30 mmol/L (Normal)	04/01/2026 10:27
Anion Gap	10 mmol/L (Normal)	04/01/2026 10:27
BUN	20 mg/dL (High)	04/01/2026 10:27
Creatinine	2.26 mg/dL (High)	04/01/2026 10:27
BUN/Creat Ratio	9 (Normal)	04/01/2026 10:27
Calcium	8.9 mg/dL (Normal)	04/01/2026 10:27
Albumin. Level	3.3 gm/dL (Normal)	04/01/2026 10:27
TP	6.1 gm/dL (Normal)	04/01/2026 10:27
A/G Ratio	1.1 (Normal)	04/01/2026 10:27
T Bili	0.3 mg/dL (Normal)	04/01/2026 10:27

D Bili	0.1 mg/dL (Normal)	04/01/2026 10:27
Alk Phos	118 units/L (High)	04/01/2026 10:27
AST	35 units/L (Normal)	04/01/2026 10:27
ALT	36 units/L (Normal)	04/01/2026 10:27
Mg Lvl	1.9 mg/dL (Normal)	04/01/2026 10:27
Phosphorus	4.4 mg/dL (Normal)	04/01/2026 10:27
Estimated Creatinine Clearance	9.79 mL/min ()	04/01/2026 11:02
eGFR Cr	20 mL/min/1.73m2 (N/A)	04/01/2026 10:27
eGFR Pediatric	Not Reported (N/A)	04/01/2026 10:27
Iron	77 mcg/dL (Normal)	03/31/2026 13:21
Total Iron Binding Capacity	148 mcg/dL (Low)	03/31/2026 13:21
Transferrin Saturation	52 % (High)	03/31/2026 13:21
Folate Lvl	21.85 ng/mL (High)	03/31/2026 13:21
Vitamin B12 Lvl	479 pg/mL (Normal)	03/31/2026 13:21
Ferritin Lvl	1244.0 ng/mL (High)	03/31/2026 13:21

General Hematology

WBC	5.23 x10e3/mcL (Normal)	04/01/2026 06:12
RBC	3.57 x10e6/mcL (Low)	04/01/2026 06:12
Hgb	8.3 gm/dL (Low)	04/01/2026 06:12
Hct	25.2 % (Low)	04/01/2026 06:12
MCV	70.6 Femtoliters (Low)	04/01/2026 06:12
MCH	23.2 pg (Low)	04/01/2026 06:12
MCHC	32.9 gm/dL (Normal)	04/01/2026 06:12
RDW-SD	45.5 Femtoliters (Normal)	04/01/2026 06:12
RDW-CV	18.2 % (High)	04/01/2026 06:12
Plt	164 x10e3/mcL (Normal)	04/01/2026 06:12
MPV	see comment Femtoliters (N/A)	04/01/2026 06:12
Imm Plt Fract	3.8 % (Normal)	04/01/2026 06:12
Neut % Auto	74.4 % (N/A)	04/01/2026 06:12
Lymph % Auto	14.9 % (N/A)	04/01/2026 06:12
Mono % Auto	5.9 % (N/A)	04/01/2026 06:12
Eos % Auto	3.6 % (N/A)	04/01/2026 06:12
Baso % Auto	0.8 % (N/A)	04/01/2026 06:12
Immature Grans %	0.4 % (Normal)	04/01/2026 06:12

Progress Note - Physician

Neut # Auto	3.89 x10e3/mcL (Normal)	04/01/2026 06:12
Lymph # Auto	0.78 x10e3/mcL (Low)	04/01/2026 06:12
Mono # Auto	0.31 x10e3/mcL (Normal)	04/01/2026 06:12
Eos # Auto	0.19 x10e3/mcL (Normal)	04/01/2026 06:12
Baso # Auto	0.04 x10e3/mcL (Normal)	04/01/2026 06:12
Immature Grans # Auto	0.02 x10e3/mcL (Normal)	04/01/2026 06:12
Retic %	0.68 % (Normal)	03/31/2026 13:21
Retic Abs	2.41 x10e4/mcL (Normal)	03/31/2026 13:21
Imm Retic Frac%	12.6 % (High)	03/31/2026 13:21

Hepatitis

Hep A Total Ab	Reactive (Abnormal)	03/31/2026 13:21

Infectious Disease

Flu A RNA	Negative (Normal)	03/30/2026 11:15
Flu B RNA	Negative (Normal)	03/30/2026 11:15
Influenza B	Not Detected (Normal)	03/31/2026 01:48
Influenza A	Not Detected (Normal)	03/31/2026 01:48
RSV	Not Detected (Normal)	03/31/2026 01:48

Other Microbiology

MRSA by PCR	Not Detected (Normal)	03/31/2026 01:48
Adenovirus	Not Detected (Normal)	03/31/2026 01:48
Coronavirus 229E	Not Detected (Normal)	03/31/2026 01:48
Coronavirus HKU1	Not Detected (Normal)	03/31/2026 01:48
Coronavirus NL63	Not Detected (Normal)	03/31/2026 01:48
Coronavirus OC43	Not Detected (Normal)	03/31/2026 01:48
Human Metapneumovirus	Not Detected (Normal)	03/31/2026 01:48
Human Rhinovirus/Enterovirus	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 1	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 2	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 3	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 4	Not Detected (Normal)	03/31/2026 01:48
Bordetella pertussis	Not Detected (Normal)	03/31/2026 01:48

Bordetella parapertussis	Not Detected (Normal)	03/31/2026 01:48
Chlamydia pneumoniae	Not Detected (Normal)	03/31/2026 01:48
Mycoplasma pneumoniae	Not Detected (Normal)	03/31/2026 01:48

SARS-CoV-2/Covid-19

SARS-CoV2 RNA	Negative (Normal)	03/30/2026 11:15
SARS-CoV-2	Not Detected (Normal)	03/31/2026 01:48

BMP, Mg, and Phos (Last Within 24hrs)
Sodium: 141 mmol/L (10:27)
Potassium: 3.8 mmol/L (10:27)
Chloride: 101 mmol/L (10:27)
CO2: 30 mmol/L (10:27)
BUN: 20 mg/dL High (10:27)
Creatinine: 2.26 mg/dL High (10:27)
Glucose Level: 98 mg/dL (10:27)
Calcium: 8.9 mg/dL (10:27)
Mg Lvl: 1.9 mg/dL (10:27)
Phosphorus: 4.4 mg/dL (10:27)

LFT (Last Within 24hrs)
AST: 35 units/L (10:27)
ALT: 36 units/L (10:27)
Alk Phos: 118 units/L High (10:27)
T Bili: 0.3 mg/dL (10:27)
D Bili: 0.1 mg/dL (10:27)
TP: 6.1 gm/dL (10:27)

Microbiology

Culture Blood ARD **Preliminary**

Source: Blood Body Site:
Collected Dt/Tm: 03/31/2026 02:12 Last Updated Dt/Tm: 04/01/2026 08:01

No growth at 24 hours.

Culture Blood ARD **Preliminary**

Source: Blood Body Site:
Collected Dt/Tm: 03/31/2026 02:04 Last Updated Dt/Tm: 04/01/2026 08:01

No growth at 24 hours.

Progress Note - Physician

Culture Sputum **Completed**

Source:	Sputum Expectorated	Body Site:	
Collected Dt/Tm:	03/30/2026 22:34	Last Updated Dt/Tm:	03/31/2026 12:42

>10 epithelial cells /lpf
>25 White Blood Cells /lpf
Many gram positive cocci in pairs
Many Gram Negative Rods
Few Gram Negative Coccobacilli
Few Gram Positive Rods
Gram stain contains >=10 squamous epithelial cells per low power field which suggests oropharyngeal and/or nasopharyngeal contamination with no predominant bacterial morphology. Culture will not be performed. Notified: ALAJA RUIZ,RN 3/31/2026 12:27:14 PP

Culture Blood ARD **Preliminary**

Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	04/01/2026 09:01

No growth at 48 hours.

Culture Blood ARD **Preliminary**

Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	04/01/2026 00:22

Morganella morganii
Identification obtained by Sepsityper rapid identification method.
Susceptibility to follow.

Assessment/Plan
Assessment:
Physical Exam
General: Thin appearing elderly female
Mental Status: Baseline dementia
Head: Normocephalic, atraumatic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact. Anicteric.
Ear, Nose and Throat: Oropharynx clear, moist mucous membranes. Trachea midline.
Neck: No obvious neck masses
Respiratory: Clear to auscultation bilaterally. No wheezing, crackles or rhonchi. Normal respiratory effort.
Cardiovascular: Regular rate and rhythm, no murmurs, gallops or rubs.
Gastrointestinal: Non-tender, non-distended. Normal bowel sounds.
Genitourinary: No suprapubic tenderness.
Neurologic: Cranial nerves II-XII grossly intact. Moves all extremities.

Progress Note - Physician

Light touch sensation is grossly intact bilaterally.

End-stage renal disease on hemodialysis Monday Wednesday Friday via IJ permacath
Hyperkalemia
Hypertension
Shortness of breath
Pneumonia
Bacteremia

Plan
Seen and examined during dialysis today
Tolerating well
Monitor daily for dialysis needs
Potassium controlled with dialysis
Continue amlodipine hydralazine metoprolol
Continue sevelamer. Monitor phosphorus level
Antibiotics. Follow-up cultures
Discussed with ID. Will hold off permacath exchange for now

Discharge Planning:
» Discharge Planning:
» Discharge To, Anticipated: Home with family care

Electronically Signed By: Vornicu, Maria MD
On: 04.01.2026 13:55 PDT

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	4/1/2026 13:27 PDT
RESULT STATUS:	Modified
PERFORM INFORMATION:	Delmundo DO,Lloyd (4/1/2026 15:43 PDT); Kim DO,Andrew H (4/1/2026 13:28 PDT)
SIGN INFORMATION:	Delmundo DO,Lloyd (4/1/2026 15:43 PDT); Kim DO,Andrew H (4/1/2026 13:28 PDT)

Addendum by Delmundo DO, Lloyd on April 01, 2026 15:42:43 PDT
I discussed with resident/fellow at length. I was present for critical and key services and am participating in patient management, in combination with the resident or fellow.
Agree with assessment and plan

on 2 liters nc
hd
echo normal efu
abx
IS
PTOT
control bp

L. Del Mundo, DO

Progress Note - Physician

Pulmonary/Critical Care

Electronically Signed By: Delmundo, Lloyd DO
On: 04.01.2026 15:43 PDT

Subjective
no acute events overnight
on 2L NC sat well
vitals otherwise stable
appears comfortable
sitting in bed

Chief Complaint/Reason for Consultation
Chief Complaint: Patient brought in by EMS from home with complaints of SOB. Per the family, patient started to complain of SOB this morning. Patient found with a RA sat of <50%. Patient provided with BVM ventilation and then placed on NRB. Patient A/Ox0. EMS unable to obtain demographics on scene. BGL 119

Past Medical History
Active Problems(3)
Hemodialysis
Hepatitis A virus
High cholesterol

Social History
No social history documented.

Family History
No Family History documented.

Objective

Measurements (most recent)

Height (cm)	157.48 cm	Erickson RD, LD, Lauren	03/31 11:07
Height (inches)	62.00 inch	SYSTEM	03/31 11:16
Weight	41.40 kg	Parala RN, Marielle Vianne	04/01 06:13
Weight (lbs)	91.27 lb	SYSTEM	04/01 06:13
Body Mass Index	15.12 kg/m2	Parala RN, Marielle Vianne	04/01 06:13
Scale Type	Bed scale	Parala RN, Marielle Vianne	04/01 06:13

Vital Signs (24 hrs)	**Last Charted**		**Minimum**		**Maximum**	
Temp	36.7	04/01/2026 08:00	36.7	04/01/2026 08:00	37	03/31/2026 16:00
Apical Heart Rate	84	04/01/2026 05:36	84	04/01/2026 05:36	**H** 102	03/31/2026 21:41
Resp Rate	20	04/01/2026 12:20	15	04/01/2026 05:00	**C** 39	03/31/2026 13:53

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Progress Note - Physician

SBP	**H** 174	04/01/2026 06:01	131	03/31/2026 14:06	**C** 187	04/01/2026 02:37
DBP	77	04/01/2026 06:01	60	03/31/2026 14:06	**H** 82	04/01/2026 00:01
MAP	110	04/01/2026 06:01	86	03/31/2026 14:06	118	04/01/2026 00:01
SpO2	100	04/01/2026 12:20	**C** 84	03/31/2026 16:00	100	03/31/2026 13:53
O2 Flow Rate	on 2 l/min		on 2 l/min			
O2 Therapy	Nasal cannula		Nasal cannula			

Perfusion Assessment

Capillary Refill Left Foot: Less than 2 seconds (04/01/26 08:00:00)

Capillary Refill Left Hand: Less than 2 seconds (04/01/26 08:00:00)

Capillary Refill Right Foot: Less than 2 seconds (04/01/26 08:00:00)

Capillary Refill Right Hand: Less than 2 seconds (04/01/26 08:00:00)

Dorsalis Pedis Pulse, Left: 1+ Barely detectable/thready (04/01/26 08:00:00)

Dorsalis Pedis Pulse, Right: 1+ Barely detectable/thready (04/01/26 08:00:00)

Posttibial Pulse, Left: 1+ Barely detectable/thready (03/31/26 20:00:00)

Posttibial Pulse, Right: 1+ Barely detectable/thready (03/31/26 20:00:00)

Radial Pulse, Left: 2+ Slightly diminished but greater than 1+ (04/01/26 08:00:00)

Radial Pulse, Right: 2+ Slightly diminished but greater than 1+ (04/01/26 08:00:00)

Pain Scores (Last Within 24hrs)
Numeric Pain Scale: Unable to obtain score; sleeping (eyes closed), no distress (10:00)

Ventilator Settings (Last Within 24hrs)

Ventilator Mode: SPN-BiPAP (02:02)

Inspiratory Pressure: 5 cm H2O (02:02)

Expiratory Pressure: 8 cm H2O (02:02)

Intake and Output (current encounter)

	03/30 07:06	04/01 07:00	03/31 07:00	03/30 07:00	03/29 07:00
	04/01 13:27	04/01 13:27	04/01 06:59	03/31 06:59	03/30 06:59
Intake	1413.3	0	1113.3	300	0

Progress Note - Physician

Output	1800	0	0	1800	0
Net Total	-386.7	0	1113.3	-1500	0
Urine Count	1	1	0	0	0
Stool Count	2	1	0	1	0

Stroke Assessments
Glasgow Coma Scale
Eye Opening Response Glasgow: Spontaneously (04/01/26 08:00:00)
Best Verbal Response Glasgow: Disoriented and converses (04/01/26 08:00:00)
Best Motor Response Glasgow: Obeys commands (04/01/26 08:00:00)
Glasgow Coma Score: 14 (04/01/26 08:00:00)

Physical Exam

Home Medications
amLODIPine (amLODIPine 10 mg oral tablet)
hydrALAZINE (hydrALAZINE 50 mg oral tablet)
losartan (losartan 100 mg oral tablet)
metoprolol (metoprolol succinate 50 mg oral tablet, extended release)
minoxidil (minoxidil 2.5 mg oral tablet)
multivitamin (Rena-Vite oral tablet)
ondansetron (ondansetron 4 mg oral tablet)
sevelamer (sevelamer 800 mg oral tablet)
zolpidem (zolpidem 5 mg oral tablet)

Inpatient Medications
Medications (25) Active
Scheduled: (12)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 10 mg Tab (amLODIPine) 10 mg 1 Tabs, Oral, Daily
cefTRIAXone/D5W (Rocephin) 50 mL, IV Piggyback, q24H Interval
doxycycline/D5W (doxycycline) 100 mg 100 mL, IV Piggyback, q12H
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
heparin 5000 units/mL injection 1 mL (heparin) 5,000 units 1 mL, SubCutaneous, q12H
hydrALAZINE 50 mg Tab (hydrALAZINE) 50 mg 1 Tabs, Oral, TID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
losartan 100 mg Tab (losartan) 100 mg 1 Tabs, Oral, Daily
metoprolol succinate 50 mg XL Tab (Metoprolol Succinate ER) 50 mg 1 Tabs, Oral, qHS
sevelamer carbonate 800 mg Tab (sevelamer) 800 mg 1 Tabs, Oral, TID w/Meals
spironolactone 25 mg Tab (spironolactone) 25 mg 1 Tabs, Oral, BID
Continuous: (0)
PRN: (13)
acetaminophen 325 mg Tab (Tylenol) 650 mg 2 Tabs, Oral, q4H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q3H
benzonatate 100 mg Cap (Tessalon Perles) 100 mg 1 Caps, Oral, TID
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID

Progress Note - Physician

Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 25 gm 50 mL, IV Push, q1H Interval
docusate sodium 100 mg Cap (Colace) 100 mg 1 Caps, Oral, BID
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H
glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q2H Interval
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q2H Interval
melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H
sodium citrate 4% soln 5 mL (sodium citrate) 200 mg 5 mL, IntraCatheter, As Directed

72 hour Antibiotic History

Active Antibiotics	Day of Therapy:	Last Administered	First Administered
cefTRIAXone 2 gm, 50 mL, 100 mL/hr, IV Piggyback, q24H Interval	2	04/01/2026 13:14	03/31/2026 10:13
doxycycline 100 mg, 100 mL, 100 mL/hr, IV Piggyback, q12H	3	04/01/2026 07:58	03/30/2026 21:13

Completed/Discontinued Antibiotics	Stop Date/Time	Last Administered	First Administered
azithromycin 500 mg, 250 mL, 250 mL/hr, IV Piggyback, Once	03/30/2026 10:10	03/30/2026 10:09	03/30/2026 10:09
cefTRIAXone 1 gm, 50 mL, 100 mL/hr, IV Piggyback, Once	03/30/2026 09:18	03/30/2026 09:10	03/30/2026 09:10

Results
Recent Labs
General Chemistry

Glucose Level	98 mg/dL (Normal)	04/01/2026 10:27
POC Glucose	75 mg/dL (Normal)	03/31/2026 07:11
Sodium	141 mmol/L (Normal)	04/01/2026 10:27
Potassium	3.8 mmol/L (Normal)	04/01/2026 10:27
Chloride	101 mmol/L (Normal)	04/01/2026 10:27
CO2	30 mmol/L (Normal)	04/01/2026 10:27
Anion Gap	10 mmol/L (Normal)	04/01/2026 10:27
BUN	20 mg/dL (High)	04/01/2026 10:27
Creatinine	2.26 mg/dL (High)	04/01/2026 10:27
BUN/Creat Ratio	9 (Normal)	04/01/2026 10:27

Progress Note - Physician

Calcium	8.9 mg/dL (Normal)	04/01/2026 10:27
Albumin. Level	3.3 gm/dL (Normal)	04/01/2026 10:27
TP	6.1 gm/dL (Normal)	04/01/2026 10:27
A/G Ratio	1.1 (Normal)	04/01/2026 10:27
T Bili	0.3 mg/dL (Normal)	04/01/2026 10:27
D Bili	0.1 mg/dL (Normal)	04/01/2026 10:27
Alk Phos	118 units/L (High)	04/01/2026 10:27
AST	35 units/L (Normal)	04/01/2026 10:27
ALT	36 units/L (Normal)	04/01/2026 10:27
Mg Lvl	1.9 mg/dL (Normal)	04/01/2026 10:27
Phosphorus	4.4 mg/dL (Normal)	04/01/2026 10:27
Estimated Creatinine Clearance	9.79 mL/min ()	04/01/2026 11:02
eGFR Cr	20 mL/min/1.73m2 (N/A)	04/01/2026 10:27
eGFR Pediatric	Not Reported (N/A)	04/01/2026 10:27
Iron	77 mcg/dL (Normal)	03/31/2026 13:21
Total Iron Binding Capacity	148 mcg/dL (Low)	03/31/2026 13:21
Transferrin Saturation	52 % (High)	03/31/2026 13:21
Folate Lvl	21.85 ng/mL (High)	03/31/2026 13:21
Vitamin B12 Lvl	479 pg/mL (Normal)	03/31/2026 13:21
Ferritin Lvl	1244.0 ng/mL (High)	03/31/2026 13:21

General Hematology

WBC	5.23 x10e3/mcL (Normal)	04/01/2026 06:12
RBC	3.57 x10e6/mcL (Low)	04/01/2026 06:12
Hgb	8.3 gm/dL (Low)	04/01/2026 06:12
Hct	25.2 % (Low)	04/01/2026 06:12
MCV	70.6 Femtoliters (Low)	04/01/2026 06:12
MCH	23.2 pg (Low)	04/01/2026 06:12
MCHC	32.9 gm/dL (Normal)	04/01/2026 06:12
RDW-SD	45.5 Femtoliters (Normal)	04/01/2026 06:12
RDW-CV	18.2 % (High)	04/01/2026 06:12
Plt	164 x10e3/mcL (Normal)	04/01/2026 06:12
MPV	see comment Femtoliters (N/A)	04/01/2026 06:12
Imm Plt Fract	3.8 % (Normal)	04/01/2026 06:12
Neut % Auto	74.4 % (N/A)	04/01/2026 06:12

Progress Note - Physician

Lymph % Auto	14.9 % (N/A)	04/01/2026 06:12
Mono % Auto	5.9 % (N/A)	04/01/2026 06:12
Eos % Auto	3.6 % (N/A)	04/01/2026 06:12
Baso % Auto	0.8 % (N/A)	04/01/2026 06:12
Immature Grans %	0.4 % (Normal)	04/01/2026 06:12
Neut # Auto	3.89 x10e3/mcL (Normal)	04/01/2026 06:12
Lymph # Auto	0.78 x10e3/mcL (Low)	04/01/2026 06:12
Mono # Auto	0.31 x10e3/mcL (Normal)	04/01/2026 06:12
Eos # Auto	0.19 x10e3/mcL (Normal)	04/01/2026 06:12
Baso # Auto	0.04 x10e3/mcL (Normal)	04/01/2026 06:12
Immature Grans # Auto	0.02 x10e3/mcL (Normal)	04/01/2026 06:12
Retic %	0.68 % (Normal)	03/31/2026 13:21
Retic Abs	2.41 x10e4/mcL (Normal)	03/31/2026 13:21
Imm Retic Frac%	12.6 % (High)	03/31/2026 13:21

Hepatitis

Hep A Total Ab	Reactive (Abnormal)	03/31/2026 13:21

Infectious Disease

Flu A RNA	Negative (Normal)	03/30/2026 11:15
Flu B RNA	Negative (Normal)	03/30/2026 11:15
Influenza B	Not Detected (Normal)	03/31/2026 01:48
Influenza A	Not Detected (Normal)	03/31/2026 01:48
RSV	Not Detected (Normal)	03/31/2026 01:48

Other Microbiology

MRSA by PCR	Not Detected (Normal)	03/31/2026 01:48
Adenovirus	Not Detected (Normal)	03/31/2026 01:48
Coronavirus 229E	Not Detected (Normal)	03/31/2026 01:48
Coronavirus HKU1	Not Detected (Normal)	03/31/2026 01:48
Coronavirus NL63	Not Detected (Normal)	03/31/2026 01:48
Coronavirus OC43	Not Detected (Normal)	03/31/2026 01:48
Human Metapneumovirus	Not Detected (Normal)	03/31/2026 01:48
Human Rhinovirus/Enterovirus	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 1	Not Detected (Normal)	03/31/2026 01:48

Parainfluenza Virus 2	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 3	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 4	Not Detected (Normal)	03/31/2026 01:48
Bordetella pertussis	Not Detected (Normal)	03/31/2026 01:48
Bordetella parapertussis	Not Detected (Normal)	03/31/2026 01:48
Chlamydia pneumoniae	Not Detected (Normal)	03/31/2026 01:48
Mycoplasma pneumoniae	Not Detected (Normal)	03/31/2026 01:48

SARS-CoV-2/Covid-19

SARS-CoV2 RNA	Negative (Normal)	03/30/2026 11:15
SARS-CoV-2	Not Detected (Normal)	03/31/2026 01:48

Abnormal Labs

General Chemistry

BUN	20 mg/dL (High)	04/01/2026 10:27
Creatinine	2.26 mg/dL (High)	04/01/2026 10:27
Alk Phos	118 units/L (High)	04/01/2026 10:27
Total Iron Binding Capacity	148 mcg/dL (Low)	03/31/2026 13:21
Transferrin Saturation	52 % (High)	03/31/2026 13:21
Folate Lvl	21.85 ng/mL (High)	03/31/2026 13:21
Ferritin Lvl	1244.0 ng/mL (High)	03/31/2026 13:21

General Hematology

RBC	3.57 x10e6/mcL (Low)	04/01/2026 06:12
Hgb	8.3 gm/dL (Low)	04/01/2026 06:12
Hct	25.2 % (Low)	04/01/2026 06:12
MCV	70.6 Femtoliters (Low)	04/01/2026 06:12
MCH	23.2 pg (Low)	04/01/2026 06:12
RDW-CV	18.2 % (High)	04/01/2026 06:12
MPV	see comment Femtoliters (N/A)	04/01/2026 06:12
Lymph # Auto	0.78 x10e3/mcL (Low)	04/01/2026 06:12
Imm Retic Frac%	12.6 % (High)	03/31/2026 13:21

Hepatitis

Hep A Total Ab	Reactive (Abnormal)	03/31/2026 13:21

Progress Note - Physician

CBC, BMP, Coagulation Trend (last 4 resulted)

WBC	**5.23** 04/01/2026 06:12	**5.13** 03/31/2026 13:21	**5.87** 03/31/2026 02:04	**12.25 H** 03/30/2026 07:31
Hgb	**8.3 L** 04/01/2026 06:12	**8.3 L** 03/31/2026 13:21	**8.4 L** 03/31/2026 02:04	**11.2** 03/30/2026 07:31
Hct	**25.2 L** 04/01/2026 06:12	**26.0 L** 03/31/2026 13:21	**25.6 L** 03/31/2026 02:04	**33.7** 03/30/2026 07:31
Baso # Auto	**0.04** 04/01/2026 06:12	**0.04** 03/31/2026 02:04	**0.03** 03/30/2026 07:31	
Baso % Auto	**0.8** 04/01/2026 06:12	**0.7** 03/31/2026 02:04	**0.2** 03/30/2026 07:31	
Eos # Auto	**0.19** 04/01/2026 06:12	**0.12** 03/31/2026 02:04	**0.02** 03/30/2026 07:31	
Eos % Auto	**3.6** 04/01/2026 06:12	**2.0** 03/31/2026 02:04	**0.2** 03/30/2026 07:31	
Immature Grans %	**0.4** 04/01/2026 06:12	**0.5** 03/31/2026 02:04	**0.6** 03/30/2026 07:31	
Immature Grans # Auto	**0.02** 04/01/2026 06:12	**0.03** 03/31/2026 02:04	**0.07** 03/30/2026 07:31	
Lymph # Auto	**0.78 L** 04/01/2026 06:12	**0.73 L** 03/31/2026 02:04	**1.33** 03/30/2026 07:31	
Lymph % Auto	**14.9** 04/01/2026 06:12	**12.4** 03/31/2026 02:04	**10.9** 03/30/2026 07:31	
Mono # Auto	**0.31** 04/01/2026 06:12	**0.22** 03/31/2026 02:04	**0.47** 03/30/2026 07:31	
Mono % Auto	**5.9** 04/01/2026 06:12	**3.7** 03/31/2026 02:04	**3.8** 03/30/2026 07:31	
Neut # Auto	**3.89** 04/01/2026 06:12	**4.73** 03/31/2026 02:04	**10.33 H** 03/30/2026 07:31	
Neut % Auto	**74.4** 04/01/2026 06:12	**80.7** 03/31/2026 02:04	**84.3** 03/30/2026 07:31	
Retic Abs	**2.41** 03/31/2026 13:21			
Retic %	**0.68** 03/31/2026 13:21			
Plt	**164** 04/01/2026 06:12	**158** 03/31/2026 13:21	**141** 03/31/2026 02:04	**230** 03/30/2026 07:31
Glucose Level	**98** 04/01/2026 10:27	**72 L** 03/31/2026 02:04	**189 H** 03/30/2026 07:31	
Sodium	**141** 04/01/2026 10:27	**140** 03/31/2026 02:04	**139** 03/30/2026 07:31	
Potassium	**3.8** 04/01/2026 10:27	**4.2** 03/31/2026 02:04	**5.9 H** 03/30/2026 07:31	
Chloride	**101** 04/01/2026 10:27	**100** 03/31/2026 02:04	**94 L** 03/30/2026 07:31	

Progress Note - Physician

CO2	**30** 04/01/2026 10:27	**31** 03/31/2026 02:04	**27** 03/30/2026 07:31	
Creatinine	**2.26 H** 04/01/2026 10:27	**2.19 H** 03/31/2026 02:04	**4.80 H** 03/30/2026 07:31	
BUN	**20 H** 04/01/2026 10:27	**17** 03/31/2026 02:04	**44 H** 03/30/2026 07:31	
BUN/Creat Ratio	**9** 04/01/2026 10:27	**8** 03/31/2026 02:04	**9** 03/30/2026 07:31	
Mg Lvl	**1.9** 04/01/2026 10:27	**1.9** 03/31/2026 02:04		
Phosphorus	**4.4** 04/01/2026 10:27	**5.6 H** 03/31/2026 02:04	**10.5 H** 03/30/2026 13:08	
Calcium	**8.9** 04/01/2026 10:27	**9.0** 03/31/2026 02:04	**8.6 L** 03/30/2026 07:31	
Albumin. Level	**3.2** 04/01/2026 10:27	**3.3** 04/01/2026 10:27	**3.4** 03/31/2026 02:04	**3.6** 03/30/2026 22:34
T Bili	**0.3** 04/01/2026 10:27	**0.8** 03/30/2026 22:34	**0.4** 03/30/2026 11:15	
D Bili	**0.1** 04/01/2026 10:27	**0.2** 03/30/2026 22:34	**0.2** 03/30/2026 11:15	
Alk Phos	**118 H** 04/01/2026 10:27	**125 H** 03/30/2026 22:34	**150 H** 03/30/2026 11:15	
AST	**35** 04/01/2026 10:27	**97 H** 03/30/2026 22:34	**57 H** 03/30/2026 11:15	
ALT	**36** 04/01/2026 10:27	**49** 03/30/2026 22:34	**51** 03/30/2026 11:15	

Blood Glucose Trend
Glucose Level: 98 mg/dL (04/01/26 10:27:00)

CBC (Last Within 24hrs)
WBC: 5.23 x10e3/mcL (06:12)
Hgb: 8.3 gm/dL Low (06:12)
Hct: 25.2 % Low (06:12)
Plt: 164 x10e3/mcL (06:12)

Differential (Last Within 24hrs)
- Automated -
Neut % Auto: 74.4 % (06:12)
Lymph % Auto: 14.9 % (06:12)
Mono % Auto: 5.9 % (06:12)
Eos % Auto: 3.6 % (06:12)
Baso % Auto: 0.8 % (06:12)
Immature Grans %: 0.4 % (06:12)

Progress Note - Physician

Neut # Auto: 3.89 x10e3/mcL (06:12)
Lymph # Auto: 0.78 x10e3/mcL Low (06:12)
Mono # Auto: 0.31 x10e3/mcL (06:12)
Baso # Auto: 0.04 x10e3/mcL (06:12)
Immature Grans # Auto: 0.02 x10e3/mcL (06:12)

BMP, Mg, and Phos (Last Within 24hrs)
Sodium: 141 mmol/L (10:27)
Potassium: 3.8 mmol/L (10:27)
Chloride: 101 mmol/L (10:27)
CO2: 30 mmol/L (10:27)
BUN: 20 mg/dL High (10:27)
Creatinine: 2.26 mg/dL High (10:27)
Glucose Level: 98 mg/dL (10:27)
Calcium: 8.9 mg/dL (10:27)
Mg Lvl: 1.9 mg/dL (10:27)
Phosphorus: 4.4 mg/dL (10:27)

LFT (Last Within 24hrs)
AST: 35 units/L (10:27)
ALT: 36 units/L (10:27)
Alk Phos: 118 units/L High (10:27)
T Bili: 0.3 mg/dL (10:27)
D Bili: 0.1 mg/dL (10:27)
TP: 6.1 gm/dL (10:27)

Microbiology

Culture Blood ARD **Preliminary**

Source:	Blood	Body Site:	
Collected Dt/Tm:	03/31/2026 02:12	Last Updated Dt/Tm:	04/01/2026 08:01

No growth at 24 hours.

Culture Blood ARD **Preliminary**

Source:	Blood	Body Site:	
Collected Dt/Tm:	03/31/2026 02:04	Last Updated Dt/Tm:	04/01/2026 08:01

No growth at 24 hours.

Culture Sputum **Completed**

Source:	Sputum Expectorated	Body Site:	
Collected Dt/Tm:	03/30/2026 22:34	Last Updated Dt/Tm:	03/31/2026 12:42

Progress Note - Physician

>10 epithelial cells /lpf
>25 White Blood Cells /lpf
Many gram positive cocci in pairs
Many Gram Negative Rods
Few Gram Negative Coccobacilli
Few Gram Positive Rods
Gram stain contains >=10 squamous epithelial cells per low power field which suggests oropharyngeal and/or nasopharyngeal contamination with no predominant bacterial morphology. Culture will not be performed. Notified: ALAJA RUIZ,RN 3/31/2026 12:27:14 PP

Culture Blood ARD		**Preliminary**	
Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	04/01/2026 09:01

No growth at 48 hours.

Culture Blood ARD		**Preliminary**	
Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	04/01/2026 00:22

Morganella morganii
Identification obtained by Sepsityper rapid identification method.
Susceptibility to follow.

Cardiology Labs
BNP: 2004 pg/mL High (03/30/26 07:31:00)
Troponin TNIH: 328.77 ng/L Critical (03/30/26 22:34:00)
Troponin TNIH: 379.71 ng/L Critical (03/30/26 17:36:00)
Troponin TNIH: 292.67 ng/L Critical (03/30/26 13:08:00)

Assessment/Plan
Assessment:
Physical Exam
General: Thin appearing minimi speaking elderly female
Mental Status: Alert and oriented to person, place, time. Normal affect.
Head: Normocephalic, atraumatic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact. Anicteric.
Ear, Nose and Throat: Oropharynx clear, moist mucous membranes. Trachea midline.
Neck: No obvious neck masses
Respiratory: Clear to auscultation bilaterally. No wheezing, crackles or rhonchi. Normal respiratory effort.
Cardiovascular: Regular rate and rhythm, no murmurs, gallops or rubs.
Gastrointestinal: Non-tender, non-distended. Normal bowel sounds.
Genitourinary: No suprapubic tenderness.

Progress Note - Physician

Neurologic: Cranial nerves II-XII grossly intact. Moves all extremities.
Light touch sensation is grossly intact bilaterally.

Assessment
Acute hypoxic respiratory failure 2/2 pneumonia versus fluid overload
Dyspnea
Sepsis with RBC, HR 2/2 respiratory source
Troponinemia 2/2 demand ischemia
ESRD on HD MWF
Hyperkalemia 2/2 above
Oxygen dependent reportedly on 2 to 5 L at home
Hypertension
Dementia

Recommendations
Currently on 2 L nasal cannula, Wean as tolerated
Goal SpO2 > 92%
Uses O2 at home at 2-3L, so she is at baseline
Hypoxia has improved after HD
Nephrology consulted, HD per nephrology
Monitor I's and O strictly
Currently on ceftriaxone and doxycycline
Echo done, EF 65-70% LV normal
Breathing treatments prn
Encourage IS use
PT/OT as tolerated
Rest of care per primary team

Discharge Planning:
» Discharge Planning:
» Discharge To, Anticipated: Home with family care

Electronically Signed By: Kim, Andrew DO
On: 04.01.2026 13:28 PDT

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	4/1/2026 07:11 PDT
RESULT STATUS:	Unauth
PERFORM INFORMATION:	Hirabayashi DO,Ellyn (4/1/2026 07:11 PDT)
SIGN INFORMATION:	Hirabayashi DO,Ellyn (4/1/2026 12:15 PDT)

Subjective

Chief Complaint/Reason for Consultation
Chief Complaint: Patient brought in by EMS from home with complaints of SOB. Per the family, patient started to complain of SOB this morning. Patient found with a RA sat of <50%. Patient provided with BVM ventilation and then placed on NRB. Patient A/Ox0. EMS unable to obtain demographics on scene. BGL 119

Past Medical History

Progress Note - Physician

Active Problems(3)
Hemodialysis
Hepatitis A virus
High cholesterol

Past Surgical History
No surgery history documented.

Social History
No social history documented.

Objective

Measurements (most recent)

Height (cm)	157.48 cm	Erickson RD, LD, Lauren	03/31 11:07
Height (inches)	62.00 inch	SYSTEM	03/31 11:16
Weight	41.40 kg	Parala RN, Marielle Vianne	04/01 06:13
Weight (lbs)	91.27 lb	SYSTEM	04/01 06:13
Body Mass Index	15.12 kg/m2	Parala RN, Marielle Vianne	04/01 06:13
Scale Type	Bed scale	Parala RN, Marielle Vianne	04/01 06:13

Vital Signs (24 hrs)	**Last Charted**		**Minimum**		**Maximum**	
Temp	37.1	04/01/2026 00:00	**L** 36.3	03/31/2026 08:00	37.2	03/31/2026 12:00
Apical Heart Rate	84	04/01/2026 05:36	84	04/01/2026 05:36	**H** 102	03/31/2026 21:41
Resp Rate	18	04/01/2026 06:58	14	03/31/2026 08:59	**C** 39	03/31/2026 13:53
SBP	**H** 174	04/01/2026 06:01	126	03/31/2026 12:06	**C** 187	04/01/2026 02:37
DBP	77	04/01/2026 06:01	60	03/31/2026 14:06	**C** 99	03/31/2026 12:06
MAP	110	04/01/2026 06:01	86	03/31/2026 14:06	118	04/01/2026 00:01
SpO2	99	04/01/2026 06:58	**C** 84	03/31/2026 16:00	100	03/31/2026 07:41
O2 Flow Rate	on 2 l/min		on 2 l/min		on 2 l/min	
O2 Therapy	Nasal cannula		Nasal cannula		Nasal cannula	

Ventilator Settings (Last Within 24hrs)
Ventilator Mode: SPN-BiPAP (02:02)
Inspiratory Pressure: 5 cm H2O (02:02)
Expiratory Pressure: 8 cm H2O (02:02)

Progress Note - Physician

Intake and Output (current encounter)

	03/30 07:06 04/01 07:11	04/01 07:00 04/01 07:11	03/31 07:00 04/01 06:59	03/30 07:00 03/31 06:59	03/29 07:00 03/30 06:59
Intake	1213.3	0	913.3	300	0
Output	1800	0	0	1800	0
Net Total	-586.7	0	913.3	-1500	0
Stool Count	1	0	0	1	0

Physical Exam

Home Medications
amLODIPine (amLODIPine 10 mg oral tablet)
hydrALAZINE (hydrALAZINE 50 mg oral tablet)
losartan (losartan 100 mg oral tablet)
metoprolol (metoprolol succinate 50 mg oral tablet, extended release)
minoxidil (minoxidil 2.5 mg oral tablet)
multivitamin (Rena-Vite oral tablet)
ondansetron (ondansetron 4 mg oral tablet)
sevelamer (sevelamer 800 mg oral tablet)
zolpidem (zolpidem 5 mg oral tablet)

Inpatient Medications
Medications (24) Active
Scheduled: (11)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 10 mg Tab (amLODIPine) 10 mg 1 Tabs, Oral, Daily
cefTRIAXone/D5W (Rocephin) 50 mL, IV Piggyback, q24H Interval
doxycycline/D5W (doxycycline) 100 mg 100 mL, IV Piggyback, q12H
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
heparin 5000 units/mL injection 1 mL (heparin) 5,000 units 1 mL, SubCutaneous, q12H
hydrALAZINE 50 mg Tab (hydrALAZINE) 50 mg 1 Tabs, Oral, TID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
losartan 100 mg Tab (losartan) 100 mg 1 Tabs, Oral, Daily
metoprolol succinate 50 mg XL Tab (Metoprolol Succinate ER) 50 mg 1 Tabs, Oral, qHS
sevelamer carbonate 800 mg Tab (sevelamer) 800 mg 1 Tabs, Oral, TID w/Meals
Continuous: (0)
PRN: (13)
acetaminophen 325 mg Tab (Tylenol) 650 mg 2 Tabs, Oral, q4H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q3H
benzonatate 100 mg Cap (Tessalon Perles) 100 mg 1 Caps, Oral, TID
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID
Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 25 gm 50 mL, IV Push, q1H Interval
docusate sodium 100 mg Cap (Colace) 100 mg 1 Caps, Oral, BID
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H
glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q2H Interval
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q2H Interval

Progress Note - Physician

melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H
sodium citrate 4% soln 5 mL (sodium citrate) 200 mg 5 mL, IntraCatheter, As Directed

72 hour Antibiotic History

Active Antibiotics	Day of Therapy:	Last Administered	First Administered
doxycycline 100 mg, 100 mL, 100 mL/hr, IV Piggyback, q12H	2	03/31/2026 21:45	03/30/2026 21:13
cefTRIAXone 2 gm, 50 mL, 100 mL/hr, IV Piggyback, q24H Interval	2	03/31/2026 10:13	03/31/2026 10:13

Completed/Discontinued Antibiotics	Stop Date/Time	Last Administered	First Administered
azithromycin 500 mg, 250 mL, 250 mL/hr, IV Piggyback, Once	03/30/2026 10:10	03/30/2026 10:09	03/30/2026 10:09
cefTRIAXone 1 gm, 50 mL, 100 mL/hr, IV Piggyback, Once	03/30/2026 09:18	03/30/2026 09:10	03/30/2026 09:10

Results
Recent Labs
General Chemistry

Glucose Level	72 mg/dL (Low)	03/31/2026 02:04
POC Glucose	75 mg/dL (Normal)	03/31/2026 07:11
Sodium	140 mmol/L (Normal)	03/31/2026 02:04
Potassium	4.2 mmol/L (Normal)	03/31/2026 02:04
Chloride	100 mmol/L (Normal)	03/31/2026 02:04
CO2	31 mmol/L (Normal)	03/31/2026 02:04
Anion Gap	9 mmol/L (Normal)	03/31/2026 02:04
BUN	17 mg/dL (Normal)	03/31/2026 02:04
Creatinine	2.19 mg/dL (High)	03/31/2026 02:04
BUN/Creat Ratio	8 (Normal)	03/31/2026 02:04
Calcium	9.0 mg/dL (Normal)	03/31/2026 02:04
Albumin. Level	3.4 gm/dL (Normal)	03/31/2026 02:04
Mg Lvl	1.9 mg/dL (Normal)	03/31/2026 02:04
Phosphorus	5.6 mg/dL (High)	03/31/2026 02:04
Estimated Creatinine Clearance	10.11 mL/min ()	03/31/2026 02:58
eGFR Cr	See Comment mL/min/1.73m2 (N/A)	03/31/2026 02:04

Progress Note - Physician

eGFR Pediatric	Not Reported (N/A)	03/31/2026 02:04
Iron	77 mcg/dL (Normal)	03/31/2026 13:21
Total Iron Binding Capacity	148 mcg/dL (Low)	03/31/2026 13:21
Transferrin Saturation	52 % (High)	03/31/2026 13:21
Folate Lvl	21.85 ng/mL (High)	03/31/2026 13:21
Vitamin B12 Lvl	479 pg/mL (Normal)	03/31/2026 13:21
Ferritin Lvl	1244.0 ng/mL (High)	03/31/2026 13:21

General Hematology

WBC	5.13 x10e3/mcL (Normal)	03/31/2026 13:21
RBC	3.64 x10e6/mcL (Normal)	03/31/2026 13:21
Hgb	8.3 gm/dL (Low)	03/31/2026 13:21
Hct	26.0 % (Low)	03/31/2026 13:21
MCV	71.4 Femtoliters (Low)	03/31/2026 13:21
MCH	22.8 pg (Low)	03/31/2026 13:21
MCHC	31.9 gm/dL (Normal)	03/31/2026 13:21
RDW-SD	46.7 Femtoliters (Normal)	03/31/2026 13:21
RDW-CV	18.3 % (High)	03/31/2026 13:21
Plt	158 x10e3/mcL (Normal)	03/31/2026 13:21
MPV	See comment Femtoliters (N/A)	03/31/2026 13:21
Imm Plt Fract	4.6 % (Normal)	03/31/2026 13:21
Neut % Auto	80.7 % (N/A)	03/31/2026 02:04
Lymph % Auto	12.4 % (N/A)	03/31/2026 02:04
Mono % Auto	3.7 % (N/A)	03/31/2026 02:04
Eos % Auto	2.0 % (N/A)	03/31/2026 02:04
Baso % Auto	0.7 % (N/A)	03/31/2026 02:04
Immature Grans %	0.5 % (Normal)	03/31/2026 02:04
Neut # Auto	4.73 x10e3/mcL (Normal)	03/31/2026 02:04
Lymph # Auto	0.73 x10e3/mcL (Low)	03/31/2026 02:04
Mono # Auto	0.22 x10e3/mcL (Normal)	03/31/2026 02:04
Eos # Auto	0.12 x10e3/mcL (Normal)	03/31/2026 02:04
Baso # Auto	0.04 x10e3/mcL (Normal)	03/31/2026 02:04

Immature Grans # Auto	0.03 x10e3/mcL (Normal)	03/31/2026 02:04
Retic %	0.68 % (Normal)	03/31/2026 13:21
Retic Abs	2.41 x10e4/mcL (Normal)	03/31/2026 13:21
Imm Retic Frac%	12.6 % (High)	03/31/2026 13:21

Hepatitis

Hep A Total Ab	Reactive (Abnormal)	03/31/2026 13:21

Infectious Disease

Flu A RNA	Negative (Normal)	03/30/2026 11:15
Flu B RNA	Negative (Normal)	03/30/2026 11:15
Influenza B	Not Detected (Normal)	03/31/2026 01:48
Influenza A	Not Detected (Normal)	03/31/2026 01:48
RSV	Not Detected (Normal)	03/31/2026 01:48

Other Microbiology

MRSA by PCR	Not Detected (Normal)	03/31/2026 01:48
Adenovirus	Not Detected (Normal)	03/31/2026 01:48
Coronavirus 229E	Not Detected (Normal)	03/31/2026 01:48
Coronavirus HKU1	Not Detected (Normal)	03/31/2026 01:48
Coronavirus NL63	Not Detected (Normal)	03/31/2026 01:48
Coronavirus OC43	Not Detected (Normal)	03/31/2026 01:48
Human Metapneumovirus	Not Detected (Normal)	03/31/2026 01:48
Human Rhinovirus/Enterovirus	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 1	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 2	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 3	Not Detected (Normal)	03/31/2026 01:48
Parainfluenza Virus 4	Not Detected (Normal)	03/31/2026 01:48
Bordetella pertussis	Not Detected (Normal)	03/31/2026 01:48
Bordetella parapertussis	Not Detected (Normal)	03/31/2026 01:48
Chlamydia pneumoniae	Not Detected (Normal)	03/31/2026 01:48
Mycoplasma pneumoniae	Not Detected (Normal)	03/31/2026 01:48

SARS-CoV-2/Covid-19

SARS-CoV2 RNA	Negative (Normal)	03/30/2026 11:15
SARS-CoV-2	Not Detected (Normal)	03/31/2026 01:48

Progress Note - Physician

Abnormal Labs

General Chemistry

Glucose Level	72 mg/dL (Low)	03/31/2026 02:04
Creatinine	2.19 mg/dL (High)	03/31/2026 02:04
Phosphorus	5.6 mg/dL (High)	03/31/2026 02:04
Total Iron Binding Capacity	148 mcg/dL (Low)	03/31/2026 13:21
Transferrin Saturation	52 % (High)	03/31/2026 13:21
Folate Lvl	21.85 ng/mL (High)	03/31/2026 13:21
Ferritin Lvl	1244.0 ng/mL (High)	03/31/2026 13:21

General Hematology

Hgb	8.3 gm/dL (Low)	03/31/2026 13:21
Hct	26.0 % (Low)	03/31/2026 13:21
MCV	71.4 Femtoliters (Low)	03/31/2026 13:21
MCH	22.8 pg (Low)	03/31/2026 13:21
RDW-CV	18.3 % (High)	03/31/2026 13:21
MPV	See comment Femtoliters (N/A)	03/31/2026 13:21
Lymph # Auto	0.73 x10e3/mcL (Low)	03/31/2026 02:04
Imm Retic Frac%	12.6 % (High)	03/31/2026 13:21

Hepatitis

Hep A Total Ab	Reactive (Abnormal)	03/31/2026 13:21

CBC, BMP, Coagulation Trend (last 4 resulted)

WBC	**5.13** 03/31/2026 13:21	**5.87** 03/31/2026 02:04	**12.25 H** 03/30/2026 07:31	
Hgb	**8.3 L** 03/31/2026 13:21	**8.4 L** 03/31/2026 02:04	**11.2** 03/30/2026 07:31	
Hct	**26.0 L** 03/31/2026 13:21	**25.6 L** 03/31/2026 02:04	**33.7** 03/30/2026 07:31	
Baso # Auto	**0.04** 03/31/2026 02:04	**0.03** 03/30/2026 07:31		
Baso % Auto	**0.7** 03/31/2026 02:04	**0.2** 03/30/2026 07:31		
Eos # Auto	**0.12** 03/31/2026 02:04	**0.02** 03/30/2026 07:31		
Eos % Auto	**2.0** 03/31/2026 02:04	**0.2** 03/30/2026 07:31		

Progress Note - Physician

Immature Grans %	**0.5** 03/31/2026 02:04	**0.6** 03/30/2026 07:31		
Immature Grans # Auto	**0.03** 03/31/2026 02:04	**0.07** 03/30/2026 07:31		
Lymph # Auto	**0.73 L** 03/31/2026 02:04	**1.33** 03/30/2026 07:31		
Lymph % Auto	**12.4** 03/31/2026 02:04	**10.9** 03/30/2026 07:31		
Mono # Auto	**0.22** 03/31/2026 02:04	**0.47** 03/30/2026 07:31		
Mono % Auto	**3.7** 03/31/2026 02:04	**3.8** 03/30/2026 07:31		
Neut # Auto	**4.73** 03/31/2026 02:04	**10.33 H** 03/30/2026 07:31		
Neut % Auto	**80.7** 03/31/2026 02:04	**84.3** 03/30/2026 07:31		
Retic Abs	**2.41** 03/31/2026 13:21			
Retic %	**0.68** 03/31/2026 13:21			
Plt	**158** 03/31/2026 13:21	**141** 03/31/2026 02:04	**230** 03/30/2026 07:31	
Glucose Level	**72 L** 03/31/2026 02:04	**189 H** 03/30/2026 07:31		
Sodium	**140** 03/31/2026 02:04	**139** 03/30/2026 07:31		
Potassium	**4.2** 03/31/2026 02:04	**5.9 H** 03/30/2026 07:31		
Chloride	**100** 03/31/2026 02:04	**94 L** 03/30/2026 07:31		
CO2	**31** 03/31/2026 02:04	**27** 03/30/2026 07:31		
Creatinine	**2.19 H** 03/31/2026 02:04	**4.80 H** 03/30/2026 07:31		
BUN	**17** 03/31/2026 02:04	**44 H** 03/30/2026 07:31		
BUN/Creat Ratio	**8** 03/31/2026 02:04	**9** 03/30/2026 07:31		
Mg Lvl	**1.9** 03/31/2026 02:04			
Phosphorus	**5.6 H** 03/31/2026 02:04	**10.5 H** 03/30/2026 13:08		
Calcium	**9.0** 03/31/2026 02:04	**8.6 L** 03/30/2026 07:31		
Albumin. Level	**3.4** 03/31/2026 02:04	**3.6** 03/30/2026 22:34	**3.4** 03/30/2026 11:15	

Progress Note - Physician

T Bili	**0.8** 03/30/2026 22:34	**0.4** 03/30/2026 11:15		
D Bili	**0.2** 03/30/2026 22:34	**0.2** 03/30/2026 11:15		
Alk Phos	**125 H** 03/30/2026 22:34	**150 H** 03/30/2026 11:15		
AST	**97 H** 03/30/2026 22:34	**57 H** 03/30/2026 11:15		
ALT	**49** 03/30/2026 22:34	**51** 03/30/2026 11:15		

Assessment/Plan
Assessment:
Subjective:
Patient is a 90-year-old female with a PMHx of ESRD on HD, chronic hypoxic respiratory failure requiring 2–5 L home O2 who presented to Valley Hospital with reports of increasing shortness of breath. Patient was initially admitted to the ICU for stat dialysis and continuous BiPAP.

Patient seen examined at bedside. Today she reports that she is feeling much better. Discussed current condition
and possible discharge pending sensitivities of bacteria and whether or not she can be transition to oral antibiotic therapy. She does already have home health set up, placed case management consult to verify.
Pending sensitivities and ID recommendations.

Physical exam:
General: Thin appearing Asian woman sitting up in bed.
Mental Status: Alert and oriented to person, place, time. Normal affect.
Head: Normocephalic, atraumatic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact. Anicteric.
Ear, Nose and Throat: Oropharynx clear, moist mucous membranes. Trachea midline.
Neck: No obvious neck masses
Respiratory: Clear to auscultation bilaterally. No wheezing, crackles or rhonchi. Normal respiratory effort. On LFNC
Cardiovascular: Regular rate and rhythm, no murmurs, gallops or rubs. Right sided subclavian HD catheter noted
Gastrointestinal: Non-tender, non-distended. Normal bowel sounds.
Genitourinary: No suprapubic tenderness.
Neurologic: Cranial nerves II-XII grossly intact. Moves all extremities.
Light touch sensation is grossly intact bilaterally.

Assessment:
Acute on chronic hypoxic respiratory failure 2/2 pneumonia versus fluid overload, improved
Dyspnea
Morganella morganii bacteremia
Sepsis with RBC, HR 2/2 respiratory source
Troponinemia 2/2 demand ischemia
ESRD on HD MWF
Hyperkalemia 2/2 above
Hepatitis A IgM positive
Oxygen dependent reportedly on 2 to 5 L at home
Hypertension
Dementia

Progress Note - Physician

Plan:
Downgraded from ICU level of care
CT chest/abdomen/pelvis with interlobular septal thickening and adjacent ground-glass opacities in bilateral lungs possibly pulmonary edema versus infectious/inflammatory versus less likely hemorrhagic. Small to moderate right and small left-sided pleural effusions.
Acute on chronic hypoxic respite failure secondary to pneumonia in addition to fluid overload
– Blood cultures 1/2 positive for gram-negative rods
– Continue antibiotics with Rocephin, doxycycline. Will adjust as indicated by cultures and sensitivities
– Breathing treatments
– Goal SpO2 92%, titrated down to LFNC 2L
ID is on consult. Appreciate recommendations.
– Continue ceftriaxone, doxycycline
– Blood culture 1/2 positive for Morganella morganii. Pending susceptibilities
Nephrology consulted appreciate recommendation
– S/p stat HD
Troponinemia secondary to demand ischemia, patient denies chest pain
– TTE pending
PRN antihypertensives and antiemetics
Replete electrolytes as needed, routine labs, and supportive care.

Patient has 5 children but main caretaker is brother who lives with her. ICU team discussed with extensively regarding CODE STATUS with patient's daughter and brother at the bedside who had different opinion regarding CODE STATUS. They wish to proceed with full code no shock for now but has plan to discuss among family members.

Consults: ICU, nephrology Dr. Vornicu
Diet: N.p.o., pending speech therapy evaluation
Activity: Early and progressive mobilization
GI prophylaxis: None
DVT prophylaxis: Subcu heparin, SCD
CODE STATUS: Full code, no shock per family
Dispo: IMC

Will discuss plan with attending physician. Awaiting further recommendations.

Ellyn Hirabayashi, DO
Internal Medicine - PGY3
Valley Hospital Medical Center

Discharge Planning:
» Discharge Planning:
» Discharge To, Anticipated: Home with family care

Progress Note - Physician

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	3/31/2026 13:04 PDT
RESULT STATUS:	Modified
PERFORM INFORMATION:	Delmundo DO,Lloyd (3/31/2026 17:01 PDT); Kim DO, Andrew H (3/31/2026 13:04 PDT)
SIGN INFORMATION:	Delmundo DO,Lloyd (3/31/2026 17:01 PDT); Kim DO, Andrew H (3/31/2026 13:04 PDT)

Addendum by Delmundo DO, Lloyd on March 31, 2026 17:01:08 PDT
I discussed with resident/fellow at length. I was present for critical and key services and am participating in patient management, in combination with the resident or fellow.
Agree with assessment and plan

wob on 2 liters via NC
hd per nephrology abx on doxy
echo pending read
much better than yesterday
IS
PTOT
Will continue to follow from a pulmonary standpoint

L. Del Mundo, DO
Pulmonary/Critical Care

Electronically Signed By: Delmundo, Lloyd DO
On: 03.31.2026 17:01 PDT

Subjective
downgraded from ICU yesterday
now on 2L NC, was previously on bipap
vitals stable
doing better after HD

Chief Complaint/Reason for Consultation
Chief Complaint: Patient brought in by EMS from home with complaints of SOB. Per the family, patient started to complain of SOB this morning. Patient found with a RA sat of <50%. Patient provided with BVM ventilation and then placed on NRB. Patient A/Ox0. EMS unable to obtain demographics on scene. BGL 119

Past Medical History
Active Problems(3)
Hemodialysis
Hepatitis A virus
High cholesterol

Social History
No social history documented.

Family History
No Family History documented.

Progress Note - Physician

Objective

Measurements (most recent)

Height (cm)	157.48 cm	Erickson RD, LD, Lauren	03/31 11:07
Height (inches)	62.00 inch	SYSTEM	03/31 11:16
Weight	37.50 kg	Parala RN, Marielle Vianne	03/30 23:48
Weight (lbs)	82.67 lb	SYSTEM	03/30 23:48
Body Mass Index	15.12 kg/m2	Parala RN, Marielle Vianne	03/30 23:48
Scale Type	Bed scale	Parala RN, Marielle Vianne	03/30 23:48

Vital Signs (24 hrs)	**Last Charted**		**Minimum**		**Maximum**	
Temp	37.2	03/31/2026 12:00	36.9	03/30/2026 22:45	37.1	03/30/2026 19:57
Apical Heart Rate	87	03/31/2026 07:12	87	03/31/2026 07:12	98	03/30/2026 18:54
Resp Rate	19	03/31/2026 11:00	14	03/31/2026 08:59	**C** 52	03/31/2026 03:45
SBP	**H** 159	03/31/2026 11:00	**H** 148	03/31/2026 10:14	**C** 198	03/31/2026 07:00
DBP	67	03/31/2026 11:00	65	03/31/2026 10:14	**H** 90	03/30/2026 21:30
MAP	97	03/31/2026 11:00	94	03/31/2026 10:14	129	03/30/2026 18:00
SpO2	99	03/31/2026 11:00	**C** 86	03/31/2026 04:15	100	03/30/2026 21:29
O2 Flow Rate					on 4 l/min	
O2 Therapy					Nasal cannula	

Perfusion Assessment

Capillary Refill Left Foot: Less than 2 seconds (03/31/26 08:00:00)

Capillary Refill Left Hand: Less than 2 seconds (03/31/26 08:00:00)

Capillary Refill Right Foot: Less than 2 seconds (03/31/26 08:00:00)

Capillary Refill Right Hand: Less than 2 seconds (03/31/26 08:00:00)

Dorsalis Pedis Pulse, Left: 1+ Barely detectable/thready (03/31/26 08:00:00)

Dorsalis Pedis Pulse, Right: 1+ Barely detectable/thready (03/31/26 08:00:00)

Posttibial Pulse, Left: 1+ Barely detectable/thready (03/31/26 00:00:00)

Posttibial Pulse, Right: 1+ Barely detectable/thready (03/31/26 00:00:00)

Progress Note - Physician

Pain Scores (Last Within 24hrs)
FACES Pain Scale Score: 0 = No hurt (17:00)
Numeric Pain Scale: 0 = No pain (12:00)

Ventilator Settings (Last Within 24hrs)
Ventilator Mode: SPN-BiPAP (02:05) FIO2: 30 % (00:00)
Inspiratory Pressure: 5 cm H2O (02:05)
Expiratory Pressure: 8 cm H2O (02:05)

Intake and Output (current encounter)

	03/30 07:06 03/31 13:04	03/31 07:00 03/31 13:04	03/30 07:00 03/31 06:59	03/29 07:00 03/30 06:59	03/28 07:00 03/29 06:59
Intake	452.6	152.6	300	0	0
Output	1800	0	1800	0	0
Net Total	-1347.4	152.6	-1500	0	0

Stroke Assessments
Glasgow Coma Scale
Eye Opening Response Glasgow: Spontaneously (03/31/26 08:00:00)
Best Verbal Response Glasgow: Disoriented and converses (03/31/26 08:00:00)
Best Motor Response Glasgow: Obeys commands (03/31/26 08:00:00)
Glasgow Coma Score: 14 (03/31/26 08:00:00)

Physical Exam

Home Medications
amLODIPine (amLODIPine 10 mg oral tablet)
hydrALAZINE (hydrALAZINE 50 mg oral tablet)
losartan (losartan 100 mg oral tablet)
metoprolol (metoprolol succinate 50 mg oral tablet, extended release)
minoxidil (minoxidil 2.5 mg oral tablet)
multivitamin (Rena-Vite oral tablet)
ondansetron (ondansetron 4 mg oral tablet)
sevelamer (sevelamer 800 mg oral tablet)
zolpidem (zolpidem 5 mg oral tablet)

Inpatient Medications
Medications (22) Active
Scheduled: (11)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 10 mg Tab (amLODIPine) 10 mg 1 Tabs, Oral, Daily

Progress Note - Physician

cefTRIAXone/D5W (Rocephin) 50 mL, IV Piggyback, q24H Interval
doxycycline/D5W (doxycycline) 100 mg 100 mL, IV Piggyback, q12H
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
heparin 5000 units/mL injection 1 mL (heparin) 5,000 units 1 mL, SubCutaneous, q12H
hydrALAZINE 50 mg Tab (hydrALAZINE) 50 mg 1 Tabs, Oral, TID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
losartan 100 mg Tab (losartan) 100 mg 1 Tabs, Oral, Daily
metoprolol succinate 50 mg XL Tab (Metoprolol Succinate ER) 50 mg 1 Tabs, Oral, qHS
sevelamer carbonate 800 mg Tab (sevelamer) 800 mg 1 Tabs, Oral, TID w/Meals
Continuous: (0)
PRN: (11)
acetaminophen 325 mg Tab (Tylenol) 650 mg 2 Tabs, Oral, q4H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q3H
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID
Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 25 gm 50 mL, IV Push, q1H Interval
docusate sodium 100 mg Cap (Colace) 100 mg 1 Caps, Oral, BID
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H
glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q2H Interval
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q2H Interval
melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H

72 hour Antibiotic History

Active Antibiotics	Day of Therapy:	Last Administered	First Administered
cefTRIAXone 2 gm, 50 mL, 100 mL/hr, IV Piggyback, q24H Interval	1	03/31/2026 10:13	03/31/2026 10:13
doxycycline 100 mg, 100 mL, 100 mL/hr, IV Piggyback, q12H	2	03/31/2026 08:50	03/30/2026 21:13

Completed/Discontinued Antibiotics	Stop Date/Time	Last Administered	First Administered
azithromycin 500 mg, 250 mL, 250 mL/hr, IV Piggyback, Once	03/30/2026 10:10	03/30/2026 10:09	03/30/2026 10:09
cefTRIAXone 1 gm, 50 mL, 100 mL/hr, IV Piggyback, Once	03/30/2026 09:18	03/30/2026 09:10	03/30/2026 09:10

Results
Recent Labs
Blood Gases

Progress Note - Physician

Art Na	137 mmol/L (Normal)	03/30/2026 08:43
Art K	6.3 mmol/L (Critical)	03/30/2026 08:43
Art iCa	0.96 mmol/L (Low)	03/30/2026 08:43
pH Arterial	7.54 (High)	03/30/2026 08:43
Art pCO2	39.4 mmHg (Normal)	03/30/2026 08:43
Art pO2	153.0 mmHg (High)	03/30/2026 08:43
HCO3 Art	34.4 mmol/L (High)	03/30/2026 08:43
Arterial pCO2(T)	39.4 mmHg (N/A)	03/30/2026 08:43
Arterial pH(T)	7.54 (N/A)	03/30/2026 08:43
Arterial pO2(T)	153.0 mmHg (N/A)	03/30/2026 08:43
Art Base Excess	10.6 mmol/L (High)	03/30/2026 08:43
Art sO2	99 % (High)	03/30/2026 08:43
Art tHB	10.3 gm/dL (Low)	03/30/2026 08:43
Art Hct	32 % (Low)	03/30/2026 08:43
POC Pt Temp	37.0 DegC (N/A)	03/30/2026 08:43
POC FIO2	50 % (N/A)	03/30/2026 08:43
POC Sample	Arterial (N/A)	03/30/2026 08:43
POC Site	Radial Right (N/A)	03/30/2026 08:43
POC Allen Test	POS (N/A)	03/30/2026 08:43
POC Del Sys	BIPAP (N/A)	03/30/2026 08:43
POC Vent Mode	NO VENT (N/A)	03/30/2026 08:43
POC Set RR	14 br/min (N/A)	03/30/2026 08:43
POC IPAP	8 cm H2O (N/A)	03/30/2026 08:43
POC EPAP	4 cm H2O (N/A)	03/30/2026 08:43
Notified To and RBV	DR DAVIDSON (N/A)	03/30/2026 08:43
Notified By	vhsmurrat (N/A)	03/30/2026 08:43
Notified Note	ALL CV WITH RB (N/A)	03/30/2026 08:43

Cardiac

BNP	2004 pg/mL (High)	03/30/2026 07:31
Troponin TNIH	328.77 ng/L (Critical)	03/30/2026 22:34

General Chemistry

Glucose Level	72 mg/dL (Low)	03/31/2026 02:04
POC Glucose	75 mg/dL (Normal)	03/31/2026 07:11
Sodium	140 mmol/L (Normal)	03/31/2026 02:04
Potassium	4.2 mmol/L (Normal)	03/31/2026 02:04
Chloride	100 mmol/L (Normal)	03/31/2026 02:04
CO2	31 mmol/L (Normal)	03/31/2026 02:04
Anion Gap	9 mmol/L (Normal)	03/31/2026 02:04

Progress Note - Physician

BUN	17 mg/dL (Normal)	03/31/2026 02:04
Creatinine	2.19 mg/dL (High)	03/31/2026 02:04
BUN/Creat Ratio	8 (Normal)	03/31/2026 02:04
Calcium	9.0 mg/dL (Normal)	03/31/2026 02:04
Albumin. Level	3.4 gm/dL (Normal)	03/31/2026 02:04
TP	6.4 gm/dL (Normal)	03/30/2026 22:34
A/G Ratio	1.3 (Normal)	03/30/2026 22:34
T Bili	0.8 mg/dL (Normal)	03/30/2026 22:34
D Bili	0.2 mg/dL (Normal)	03/30/2026 22:34
Alk Phos	125 units/L (High)	03/30/2026 22:34
AST	97 units/L (High)	03/30/2026 22:34
ALT	49 units/L (Normal)	03/30/2026 22:34
Mg Lvl	1.9 mg/dL (Normal)	03/31/2026 02:04
Phosphorus	5.6 mg/dL (High)	03/31/2026 02:04
Estimated Creatinine Clearance	10.11 mL/min ()	03/31/2026 02:58
eGFR Cr	See Comment mL/min/1.73m2 (N/A)	03/31/2026 02:04
eGFR Pediatric	Not Reported (N/A)	03/31/2026 02:04
Lactic Acid Lvl	1.8 mmol/L (Normal)	03/30/2026 08:48
Hgb A1C	4.3 % (Normal)	03/30/2026 11:15
MAR Glucose Result	102 ()	03/30/2026 09:49

General Hematology

WBC	5.87 x10e3/mcL (Normal)	03/31/2026 02:04
RBC	3.65 x10e6/mcL (Normal)	03/31/2026 02:04
Hgb	8.4 gm/dL (Low)	03/31/2026 02:04
Hct	25.6 % (Low)	03/31/2026 02:04
MCV	70.1 Femtoliters (Low)	03/31/2026 02:04
MCH	23.0 pg (Low)	03/31/2026 02:04
MCHC	32.8 gm/dL (Normal)	03/31/2026 02:04
RDW-SD	45.2 Femtoliters (Normal)	03/31/2026 02:04
RDW-CV	18.6 % (High)	03/31/2026 02:04
Plt	141 x10e3/mcL (Normal)	03/31/2026 02:04
MPV	9.6 Femtoliters (Normal)	03/31/2026 02:04
Imm Plt Fract	3.7 % (Normal)	03/31/2026 02:04

Progress Note - Physician

NRBC %	0.2 /100 WBC (Normal)	03/30/2026 07:31
NRBC #	0.02 x10e3/mcL (High)	03/30/2026 07:31
Neut % Auto	80.7 % (N/A)	03/31/2026 02:04
Lymph % Auto	12.4 % (N/A)	03/31/2026 02:04
Mono % Auto	3.7 % (N/A)	03/31/2026 02:04
Eos % Auto	2.0 % (N/A)	03/31/2026 02:04
Baso % Auto	0.7 % (N/A)	03/31/2026 02:04
Immature Grans %	0.5 % (Normal)	03/31/2026 02:04
Neut # Auto	4.73 x10e3/mcL (Normal)	03/31/2026 02:04
Lymph # Auto	0.73 x10e3/mcL (Low)	03/31/2026 02:04
Mono # Auto	0.22 x10e3/mcL (Normal)	03/31/2026 02:04
Eos # Auto	0.12 x10e3/mcL (Normal)	03/31/2026 02:04
Baso # Auto	0.04 x10e3/mcL (Normal)	03/31/2026 02:04
Immature Grans # Auto	0.03 x10e3/mcL (Normal)	03/31/2026 02:04
RBC Morph	Abnormal (Abnormal)	03/30/2026 07:31
Plt Morphology	Normal (Normal)	03/30/2026 07:31
Polychrom	1+ (Abnormal)	03/30/2026 07:31
Micro	2+ (Abnormal)	03/30/2026 07:31
Plt Est	Adequate (Normal)	03/30/2026 07:31
Schistocytes	1+ (Abnormal)	03/30/2026 07:31
Target Cells (Codocytes)	3+ (Abnormal)	03/30/2026 07:31
Teardrop Cells (Dacrocytes)	1+ (Abnormal)	03/30/2026 07:31

Hepatitis

Hep A IgM	Reactive (Abnormal)	03/30/2026 10:54
Hep B Core IgM	Non-Reactive (Normal)	03/30/2026 11:04
Hepatitis B Surface Antigen	Non-Reactive (Normal)	03/30/2026 11:04
Hep Bs Ab Qn	31.9 milli intl units/mL (High)	03/30/2026 11:04
Hep Bs Ab Qn Interp	Reactive (Abnormal)	03/30/2026 11:04
Hep C Ab	Non-Reactive (Normal)	03/30/2026 10:54

Infectious Disease

Flu A RNA	Negative (Normal)	03/30/2026 11:15
Flu B RNA	Negative (Normal)	03/30/2026 11:15

Other Chemistry

Procalcitonin Lvl	1.56 ng/mL (High)	03/30/2026 11:15

Other Microbiology

MRSA by PCR	Not Detected (Normal)	03/31/2026 01:48

SARS-CoV-2/Covid-19

SARS-CoV2 RNA	Negative (Normal)	03/30/2026 11:15

Thyroid

T3 Free	2.87 pg/mL (Normal)	03/30/2026 11:15
T4 Free	1.74 ng/dL (High)	03/30/2026 11:15
TSH Ultrasensitive	3.046 mc Intl units/mL (Normal)	03/30/2026 11:15

Abnormal Labs

Blood Gases

Art K	6.3 mmol/L (Critical)	03/30/2026 08:43
Art iCa	0.96 mmol/L (Low)	03/30/2026 08:43
pH Arterial	7.54 (High)	03/30/2026 08:43
Art pO2	153.0 mmHg (High)	03/30/2026 08:43
HCO3 Art	34.4 mmol/L (High)	03/30/2026 08:43
Art Base Excess	10.6 mmol/L (High)	03/30/2026 08:43
Art sO2	99 % (High)	03/30/2026 08:43
Art tHB	10.3 gm/dL (Low)	03/30/2026 08:43
Art Hct	32 % (Low)	03/30/2026 08:43

Cardiac

BNP	2004 pg/mL (High)	03/30/2026 07:31
Troponin TNIH	328.77 ng/L (Critical)	03/30/2026 22:34

General Chemistry

Glucose Level	72 mg/dL (Low)	03/31/2026 02:04
Creatinine	2.19 mg/dL (High)	03/31/2026 02:04
Alk Phos	125 units/L (High)	03/30/2026 22:34
AST	97 units/L (High)	03/30/2026 22:34
Phosphorus	5.6 mg/dL (High)	03/31/2026 02:04

General Hematology

Hgb	8.4 gm/dL (Low)	03/31/2026 02:04

Progress Note - Physician

Hct	25.6 % (Low)	03/31/2026 02:04
MCV	70.1 Femtoliters (Low)	03/31/2026 02:04
MCH	23.0 pg (Low)	03/31/2026 02:04
RDW-CV	18.6 % (High)	03/31/2026 02:04
NRBC #	0.02 x10e3/mcL (High)	03/30/2026 07:31
Lymph # Auto	0.73 x10e3/mcL (Low)	03/31/2026 02:04
RBC Morph	Abnormal (Abnormal)	03/30/2026 07:31
Polychrom	1+ (Abnormal)	03/30/2026 07:31
Micro	2+ (Abnormal)	03/30/2026 07:31
Schistocytes	1+ (Abnormal)	03/30/2026 07:31
Target Cells (Codocytes)	3+ (Abnormal)	03/30/2026 07:31
Teardrop Cells (Dacrocytes)	1+ (Abnormal)	03/30/2026 07:31

Hepatitis

Hep A IgM	Reactive (Abnormal)	03/30/2026 10:54
Hep Bs Ab Qn	31.9 milli intl units/mL (High)	03/30/2026 11:04
Hep Bs Ab Qn Interp	Reactive (Abnormal)	03/30/2026 11:04

Other Chemistry

Procalcitonin Lvl	1.56 ng/mL (High)	03/30/2026 11:15

Thyroid

T4 Free	1.74 ng/dL (High)	03/30/2026 11:15

CBC, BMP, Coagulation Trend (last 4 resulted)

WBC	**5.87** 03/31/2026 02:04	**12.25 H** 03/30/2026 07:31		
Hgb	**8.4 L** 03/31/2026 02:04	**11.2** 03/30/2026 07:31		
Hct	**25.6 L** 03/31/2026 02:04	**33.7** 03/30/2026 07:31		
Baso # Auto	**0.04** 03/31/2026 02:04	**0.03** 03/30/2026 07:31		
Baso % Auto	**0.7** 03/31/2026 02:04	**0.2** 03/30/2026 07:31		
Eos # Auto	**0.12** 03/31/2026 02:04	**0.02** 03/30/2026 07:31		
Eos % Auto	**2.0** 03/31/2026 02:04	**0.2** 03/30/2026 07:31		

Progress Note - Physician

Immature Grans %	**0.5** 03/31/2026 02:04	**0.6** 03/30/2026 07:31		
Immature Grans # Auto	**0.03** 03/31/2026 02:04	**0.07** 03/30/2026 07:31		
Lymph # Auto	**0.73 L** 03/31/2026 02:04	**1.33** 03/30/2026 07:31		
Lymph % Auto	**12.4** 03/31/2026 02:04	**10.9** 03/30/2026 07:31		
Mono # Auto	**0.22** 03/31/2026 02:04	**0.47** 03/30/2026 07:31		
Mono % Auto	**3.7** 03/31/2026 02:04	**3.8** 03/30/2026 07:31		
Neut # Auto	**4.73** 03/31/2026 02:04	**10.33 H** 03/30/2026 07:31		
Neut % Auto	**80.7** 03/31/2026 02:04	**84.3** 03/30/2026 07:31		
Plt	**141** 03/31/2026 02:04	**230** 03/30/2026 07:31		
Glucose Level	**72 L** 03/31/2026 02:04	**189 H** 03/30/2026 07:31		
Sodium	**140** 03/31/2026 02:04	**139** 03/30/2026 07:31		
Potassium	**4.2** 03/31/2026 02:04	**5.9 H** 03/30/2026 07:31		
Chloride	**100** 03/31/2026 02:04	**94 L** 03/30/2026 07:31		
CO2	**31** 03/31/2026 02:04	**27** 03/30/2026 07:31		
Creatinine	**2.19 H** 03/31/2026 02:04	**4.80 H** 03/30/2026 07:31		
BUN	**17** 03/31/2026 02:04	**44 H** 03/30/2026 07:31		
BUN/Creat Ratio	**8** 03/31/2026 02:04	**9** 03/30/2026 07:31		
Mg Lvl	**1.9** 03/31/2026 02:04			
Phosphorus	**5.6 H** 03/31/2026 02:04	**10.5 H** 03/30/2026 13:08		
Calcium	**9.0** 03/31/2026 02:04	**8.6 L** 03/30/2026 07:31		
Albumin. Level	**3.4** 03/31/2026 02:04	**3.6** 03/30/2026 22:34	**3.4** 03/30/2026 11:15	
T Bili	**0.8** 03/30/2026 22:34	**0.4** 03/30/2026 11:15		
D Bili	**0.2** 03/30/2026 22:34	**0.2** 03/30/2026 11:15		

Progress Note - Physician

Alk Phos	**125 H** 03/30/2026 22:34	**150 H** 03/30/2026 11:15		
AST	**97 H** 03/30/2026 22:34	**57 H** 03/30/2026 11:15		
ALT	**49** 03/30/2026 22:34	**51** 03/30/2026 11:15		

Blood Glucose Trend

Glucose Level: 72 mg/dL Low (03/31/26 02:04:00)

POC Glucose: 75 mg/dL (03/31/26 07:11:00)

POC Glucose: 70 mg/dL Low (03/31/26 02:14:00)

CBC (Last Within 24hrs)
WBC: 5.87 x10e3/mcL (02:04)
Hgb: 8.4 gm/dL Low (02:04)
Hct: 25.6 % Low (02:04)
Plt: 141 x10e3/mcL (02:04)

Differential (Last Within 24hrs)

- Automated -

Neut % Auto: 80.7 % (02:04)

Lymph % Auto: 12.4 % (02:04)

Mono % Auto: 3.7 % (02:04)

Eos % Auto: 2 % (02:04)

Baso % Auto: 0.7 % (02:04)

Immature Grans %: 0.5 % (02:04)

Neut # Auto: 4.73 x10e3/mcL (02:04)

Lymph # Auto: 0.73 x10e3/mcL Low (02:04)

Mono # Auto: 0.22 x10e3/mcL (02:04)

Baso # Auto: 0.04 x10e3/mcL (02:04)

Immature Grans # Auto: 0.03 x10e3/mcL (02:04)

BMP, Mg, and Phos (Last Within 24hrs)
Sodium: 140 mmol/L (02:04)
Potassium: 4.2 mmol/L (02:04)
Chloride: 100 mmol/L (02:04)
CO2: 31 mmol/L (02:04)
BUN: 17 mg/dL (02:04)
Creatinine: 2.19 mg/dL High (02:04)
Glucose Level: 72 mg/dL Low (02:04)
Calcium: 9 mg/dL (02:04)
Mg Lvl: 1.9 mg/dL (02:04)
Phosphorus: 5.6 mg/dL High (02:04)

Progress Note - Physician

LFT (Last Within 24hrs)
AST: 97 units/L High (22:34)
ALT: 49 units/L (22:34)
Alk Phos: 125 units/L High (22:34)
T Bili: 0.8 mg/dL (22:34)
D Bili: 0.2 mg/dL (22:34)
TP: 6.4 gm/dL (22:34)

Microbiology

Culture Blood ARD **In-Lab**

Source:	Blood	Body Site:	
Collected Dt/Tm:	03/31/2026 02:12	Last Updated Dt/Tm:	03/31/2026 00:41

Culture Blood ARD **In-Lab**

Source:	Blood	Body Site:	
Collected Dt/Tm:	03/31/2026 02:04	Last Updated Dt/Tm:	03/31/2026 00:41

Culture Sputum **Completed**

Source:	Sputum Expectorated	Body Site:	
Collected Dt/Tm:	03/30/2026 22:34	Last Updated Dt/Tm:	03/31/2026 12:42

>10 epithelial cells /lpf
>25 White Blood Cells /lpf
Many gram positive cocci in pairs
Many Gram Negative Rods
Few Gram Negative Coccobacilli
Few Gram Positive Rods
Gram stain contains >=10 squamous epithelial cells per low power field which suggests oropharyngeal and/or nasopharyngeal contamination with no predominant bacterial morphology. Culture will not be performed. Notified: ALAJA RUIZ,RN 3/31/2026 12:27:14 PP

Culture Blood ARD **Preliminary**

Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	03/31/2026 09:02

No growth at 24 hours.

Culture Blood ARD **Preliminary**

Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	03/31/2026 10:02

Progress Note - Physician

Morganella morganii
Identification obtained by Sepsityper rapid identification method.
Culture confirmation to follow.
Susceptibility to follow.

Cardiology Labs
BNP: 2004 pg/mL High (03/30/26 07:31:00)
Troponin TNIH: 328.77 ng/L Critical (03/30/26 22:34:00)
Troponin TNIH: 379.71 ng/L Critical (03/30/26 17:36:00)
Troponin TNIH: 292.67 ng/L Critical (03/30/26 13:08:00)

Assessment/Plan
Assessment:
Physical Exam
General: Thin appearing minimi speaking elderly female
Mental Status: Alert and oriented to person, place, time. Normal affect.
Head: Normocephalic, atraumatic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact. Anicteric.
Ear, Nose and Throat: Oropharynx clear, moist mucous membranes. Trachea midline.
Neck: No obvious neck masses
Respiratory: Clear to auscultation bilaterally. No wheezing, crackles or rhonchi. Normal respiratory effort.
Cardiovascular: Regular rate and rhythm, no murmurs, gallops or rubs.
Gastrointestinal: Non-tender, non-distended. Normal bowel sounds.
Genitourinary: No suprapubic tenderness.
Neurologic: Cranial nerves II-XII grossly intact. Moves all extremities.
Light touch sensation is grossly intact bilaterally.

Assessment
Acute hypoxic respiratory failure 2/2 pneumonia versus fluid overload
Dyspnea
Sepsis with RBC, HR 2/2 respiratory source
Troponinemia 2/2 demand ischemia
ESRD on HD MWF
Hyperkalemia 2/2 above
Oxygen dependent reportedly on 2 to 5 L at home
Hypertension
Dementia

Recommendations
Currently on 2 L nasal cannula,Wean as tolerated
Goal SpO2 greater than 92%
O2 level appears to be at baseline
Hypoxia likely secondary to pulmonary edema/fluid overload
Nephrology consulted, HD per nephrology
Monitor I's and O strictly

Progress Note - Physician

Currently on doxycycline
Echo done, read pending
Breathing treatments as needed
Encourage IS use
PT/OT as tolerated
Rest of care per primary team

Discharge Planning:
» Discharge Planning:
» Discharge To, Anticipated: Home with family care

Electronically Signed By: Kim, Andrew DO
On: 03.31.2026 13:04 PDT

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	3/31/2026 12:23 PDT
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Vornicu MD,Maria D (3/31/2026 12:24 PDT)
SIGN INFORMATION:	Vornicu MD,Maria D (3/31/2026 12:24 PDT)

Subjective
End-stage renal disease on dialysis
Hemodialysis yesterday with 1.5 L ultrafiltration
Currently on nasal cannula
Baseline dementia

Chief Complaint/Reason for Consultation
Chief Complaint: Patient brought in by EMS from home with complaints of SOB. Per the family, patient started to complain of SOB this morning. Patient found with a RA sat of <50%. Patient provided with BVM ventilation and then placed on NRB. Patient A/Ox0. EMS unable to obtain demographics on scene. BGL 119

Allergies
Allergies (Active and Proposed Allergies Only)
No Known Allergies (Severity: Unknown severity, Onset: Unknown)

Objective

Measurements (most recent)

Height (cm)	157.48 cm	Erickson RD, LD, Lauren	03/31 11:07
Height (inches)	62.00 inch	SYSTEM	03/31 11:16
Weight	37.50 kg	Parala RN, Marielle Vianne	03/30 23:48
Weight (lbs)	82.67 lb	SYSTEM	03/30 23:48
Body Mass Index	15.12 kg/m2	Parala RN, Marielle Vianne	03/30 23:48

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Progress Note - Physician

Scale Type Bed scale Parala RN, Marielle Vianne 03/30 23:48

Vital Signs (24 hrs)	**Last Charted**		**Minimum**		**Maximum**	
Temp	37.2	03/31/2026 12:00	36.9	03/30/2026 22:45	37.1	03/30/2026 19:57
Apical Heart Rate	87	03/31/2026 07:12	87	03/31/2026 07:12	98	03/30/2026 18:54
Resp Rate	19	03/31/2026 11:00	14	03/31/2026 08:59	**C** 52	03/31/2026 03:45
SBP	**H** 159	03/31/2026 11:00	**H** 148	03/31/2026 10:14	**C** 198	03/31/2026 07:00
DBP	67	03/31/2026 11:00	65	03/31/2026 10:14	**H** 90	03/30/2026 21:30
MAP	97	03/31/2026 11:00	94	03/31/2026 10:14	129	03/30/2026 18:00
SpO2	99	03/31/2026 11:00	**C** 86	03/31/2026 04:15	100	03/30/2026 21:29
O2 Flow Rate					on 4 l/min	
O2 Therapy					Nasal cannula	

Ventilator Settings (Last Within 24hrs)
Ventilator Mode: SPN-BiPAP (02:05) FIO2: 30 % (00:00)
Inspiratory Pressure: 5 cm H2O (02:05)
Expiratory Pressure: 8 cm H2O (02:05)

Intake and Output (current encounter)

	03/30 07:06 03/31 12:24	**03/31 07:00 03/31 12:24**	**03/30 07:00 03/31 06:59**	**03/29 07:00 03/30 06:59**	**03/28 07:00 03/29 06:59**
Intake	452.6	152.6	300	0	0
Output	1800	0	1800	0	0
Net Total	-1347.4	152.6	-1500	0	0

Physical Exam

Inpatient Medications
Medications (22) Active
Scheduled: (11)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 10 mg Tab (amLODIPine) 10 mg 1 Tabs, Oral, Daily
cefTRIAXone/D5W (Rocephin) 50 mL, IV Piggyback, q24H Interval
doxycycline/D5W (doxycycline) 100 mg 100 mL, IV Piggyback, q12H
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
heparin 5000 units/mL injection 1 mL (heparin) 5,000 units 1 mL, SubCutaneous, q12H
hydrALAZINE 50 mg Tab (hydrALAZINE) 50 mg 1 Tabs, Oral, TID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
losartan 100 mg Tab (losartan) 100 mg 1 Tabs, Oral, Daily

Progress Note - Physician

metoprolol succinate 50 mg XL Tab (Metoprolol Succinate ER) 50 mg 1 Tabs, Oral, qHS
sevelamer carbonate 800 mg Tab (sevelamer) 800 mg 1 Tabs, Oral, TID w/Meals
Continuous: (0)
PRN: (11)
acetaminophen 325 mg Tab (Tylenol) 650 mg 2 Tabs, Oral, q4H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q3H
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID
Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 25 gm 50 mL, IV Push, q1H Interval
docusate sodium 100 mg Cap (Colace) 100 mg 1 Caps, Oral, BID
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H
glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q2H Interval
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q2H Interval
melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H

72 hour Antibiotic History

Active Antibiotics	Day of Therapy:	Last Administered	First Administered
cefTRIAXone 2 gm, 50 mL, 100 mL/hr, IV Piggyback, q24H Interval	1	03/31/2026 10:13	03/31/2026 10:13
doxycycline 100 mg, 100 mL, 100 mL/hr, IV Piggyback, q12H	2	03/31/2026 08:50	03/30/2026 21:13

Completed/Discontinued Antibiotics	Stop Date/Time	Last Administered	First Administered
azithromycin 500 mg, 250 mL, 250 mL/hr, IV Piggyback, Once	03/30/2026 10:10	03/30/2026 10:09	03/30/2026 10:09
cefTRIAXone 1 gm, 50 mL, 100 mL/hr, IV Piggyback, Once	03/30/2026 09:18	03/30/2026 09:10	03/30/2026 09:10

Results
Recent Labs
Blood Gases

Art Na	137 mmol/L (Normal)	03/30/2026 08:43
Art K	6.3 mmol/L (Critical)	03/30/2026 08:43
Art iCa	0.96 mmol/L (Low)	03/30/2026 08:43
pH Arterial	7.54 (High)	03/30/2026 08:43
Art pCO2	39.4 mmHg (Normal)	03/30/2026 08:43
Art pO2	153.0 mmHg (High)	03/30/2026 08:43
HCO3 Art	34.4 mmol/L (High)	03/30/2026 08:43
Arterial pCO2(T)	39.4 mmHg (N/A)	03/30/2026 08:43

Progress Note - Physician

Arterial pH(T)	7.54 (N/A)	03/30/2026 08:43
Arterial pO2(T)	153.0 mmHg (N/A)	03/30/2026 08:43
Art Base Excess	10.6 mmol/L (High)	03/30/2026 08:43
Art sO2	99 % (High)	03/30/2026 08:43
Art tHB	10.3 gm/dL (Low)	03/30/2026 08:43
Art Hct	32 % (Low)	03/30/2026 08:43
POC Pt Temp	37.0 DegC (N/A)	03/30/2026 08:43
POC FIO2	50 % (N/A)	03/30/2026 08:43
POC Sample	Arterial (N/A)	03/30/2026 08:43
POC Site	Radial Right (N/A)	03/30/2026 08:43
POC Allen Test	POS (N/A)	03/30/2026 08:43
POC Del Sys	BIPAP (N/A)	03/30/2026 08:43
POC Vent Mode	NO VENT (N/A)	03/30/2026 08:43
POC Set RR	14 br/min (N/A)	03/30/2026 08:43
POC IPAP	8 cm H2O (N/A)	03/30/2026 08:43
POC EPAP	4 cm H2O (N/A)	03/30/2026 08:43
Notified To and RBV	DR DAVIDSON (N/A)	03/30/2026 08:43
Notified By	vhsmurrat (N/A)	03/30/2026 08:43
Notified Note	ALL CV WITH RB (N/A)	03/30/2026 08:43

Cardiac

BNP	2004 pg/mL (High)	03/30/2026 07:31
Troponin TNIH	328.77 ng/L (Critical)	03/30/2026 22:34

General Chemistry

Glucose Level	72 mg/dL (Low)	03/31/2026 02:04
POC Glucose	75 mg/dL (Normal)	03/31/2026 07:11
Sodium	140 mmol/L (Normal)	03/31/2026 02:04
Potassium	4.2 mmol/L (Normal)	03/31/2026 02:04
Chloride	100 mmol/L (Normal)	03/31/2026 02:04
CO2	31 mmol/L (Normal)	03/31/2026 02:04
Anion Gap	9 mmol/L (Normal)	03/31/2026 02:04
BUN	17 mg/dL (Normal)	03/31/2026 02:04
Creatinine	2.19 mg/dL (High)	03/31/2026 02:04
BUN/Creat Ratio	8 (Normal)	03/31/2026 02:04
Calcium	9.0 mg/dL (Normal)	03/31/2026 02:04
Albumin. Level	3.4 gm/dL (Normal)	03/31/2026 02:04
TP	6.4 gm/dL (Normal)	03/30/2026 22:34
A/G Ratio	1.3 (Normal)	03/30/2026 22:34
T Bili	0.8 mg/dL (Normal)	03/30/2026 22:34

Progress Note - Physician

D Bili	0.2 mg/dL (Normal)	03/30/2026 22:34
Alk Phos	125 units/L (High)	03/30/2026 22:34
AST	97 units/L (High)	03/30/2026 22:34
ALT	49 units/L (Normal)	03/30/2026 22:34
Mg Lvl	1.9 mg/dL (Normal)	03/31/2026 02:04
Phosphorus	5.6 mg/dL (High)	03/31/2026 02:04
Estimated Creatinine Clearance	10.11 mL/min ()	03/31/2026 02:58
eGFR Cr	See Comment mL/min/1.73m2 (N/A)	03/31/2026 02:04
eGFR Pediatric	Not Reported (N/A)	03/31/2026 02:04
Lactic Acid Lvl	1.8 mmol/L (Normal)	03/30/2026 08:48
Hgb A1C	4.3 % (Normal)	03/30/2026 11:15
MAR Glucose Result	102 ()	03/30/2026 09:49

General Hematology

WBC	5.87 x10e3/mcL (Normal)	03/31/2026 02:04
RBC	3.65 x10e6/mcL (Normal)	03/31/2026 02:04
Hgb	8.4 gm/dL (Low)	03/31/2026 02:04
Hct	25.6 % (Low)	03/31/2026 02:04
MCV	70.1 Femtoliters (Low)	03/31/2026 02:04
MCH	23.0 pg (Low)	03/31/2026 02:04
MCHC	32.8 gm/dL (Normal)	03/31/2026 02:04
RDW-SD	45.2 Femtoliters (Normal)	03/31/2026 02:04
RDW-CV	18.6 % (High)	03/31/2026 02:04
Plt	141 x10e3/mcL (Normal)	03/31/2026 02:04
MPV	9.6 Femtoliters (Normal)	03/31/2026 02:04
Imm Plt Fract	3.7 % (Normal)	03/31/2026 02:04
NRBC %	0.2 /100 WBC (Normal)	03/30/2026 07:31
NRBC #	0.02 x10e3/mcL (High)	03/30/2026 07:31
Neut % Auto	80.7 % (N/A)	03/31/2026 02:04
Lymph % Auto	12.4 % (N/A)	03/31/2026 02:04
Mono % Auto	3.7 % (N/A)	03/31/2026 02:04
Eos % Auto	2.0 % (N/A)	03/31/2026 02:04
Baso % Auto	0.7 % (N/A)	03/31/2026 02:04
Immature Grans %	0.5 % (Normal)	03/31/2026 02:04

Progress Note - Physician

Neut # Auto	4.73 x10e3/mcL (Normal)	03/31/2026 02:04
Lymph # Auto	0.73 x10e3/mcL (Low)	03/31/2026 02:04
Mono # Auto	0.22 x10e3/mcL (Normal)	03/31/2026 02:04
Eos # Auto	0.12 x10e3/mcL (Normal)	03/31/2026 02:04
Baso # Auto	0.04 x10e3/mcL (Normal)	03/31/2026 02:04
Immature Grans # Auto	0.03 x10e3/mcL (Normal)	03/31/2026 02:04
RBC Morph	Abnormal (Abnormal)	03/30/2026 07:31
Plt Morphology	Normal (Normal)	03/30/2026 07:31
Polychrom	1+ (Abnormal)	03/30/2026 07:31
Micro	2+ (Abnormal)	03/30/2026 07:31
Plt Est	Adequate (Normal)	03/30/2026 07:31
Schistocytes	1+ (Abnormal)	03/30/2026 07:31
Target Cells (Codocytes)	3+ (Abnormal)	03/30/2026 07:31
Teardrop Cells (Dacrocytes)	1+ (Abnormal)	03/30/2026 07:31

Hepatitis

Hep A IgM	Reactive (Abnormal)	03/30/2026 10:54
Hep B Core IgM	Non-Reactive (Normal)	03/30/2026 11:04
Hepatitis B Surface Antigen	Non-Reactive (Normal)	03/30/2026 11:04
Hep Bs Ab Qn	31.9 milli intl units/mL (High)	03/30/2026 11:04
Hep Bs Ab Qn Interp	Reactive (Abnormal)	03/30/2026 11:04
Hep C Ab	Non-Reactive (Normal)	03/30/2026 10:54

Infectious Disease

Flu A RNA	Negative (Normal)	03/30/2026 11:15
Flu B RNA	Negative (Normal)	03/30/2026 11:15

Other Chemistry

Procalcitonin Lvl	1.56 ng/mL (High)	03/30/2026 11:15

Other Microbiology

MRSA by PCR	Not Detected (Normal)	03/31/2026 01:48

SARS-CoV-2/Covid-19

Progress Note - Physician

SARS-CoV2 RNA	Negative (Normal)	03/30/2026 11:15

Thyroid

T3 Free	2.87 pg/mL (Normal)	03/30/2026 11:15
T4 Free	1.74 ng/dL (High)	03/30/2026 11:15
TSH Ultrasensitive	3.046 mc Intl units/mL (Normal)	03/30/2026 11:15

BMP, Mg, and Phos (Last Within 24hrs)
Sodium: 140 mmol/L (02:04)
Potassium: 4.2 mmol/L (02:04)
Chloride: 100 mmol/L (02:04)
CO2: 31 mmol/L (02:04)
BUN: 17 mg/dL (02:04)
Creatinine: 2.19 mg/dL High (02:04)
Glucose Level: 72 mg/dL Low (02:04)
Calcium: 9 mg/dL (02:04)
Mg Lvl: 1.9 mg/dL (02:04)
Phosphorus: 5.6 mg/dL High (02:04)

LFT (Last Within 24hrs)
AST: 97 units/L High (22:34)
ALT: 49 units/L (22:34)
Alk Phos: 125 units/L High (22:34)
T Bili: 0.8 mg/dL (22:34)
D Bili: 0.2 mg/dL (22:34)
TP: 6.4 gm/dL (22:34)

Microbiology

Culture Blood ARD **In-Lab**

Source:	Blood	Body Site:	
Collected Dt/Tm:	03/31/2026 02:12	Last Updated Dt/Tm:	03/31/2026 00:41

Culture Blood ARD **In-Lab**

Source:	Blood	Body Site:	
Collected Dt/Tm:	03/31/2026 02:04	Last Updated Dt/Tm:	03/31/2026 00:41

Culture Sputum **Final**

Source:	Sputum Expectorated	Body Site:	
Collected Dt/Tm:	03/30/2026 22:34	Last Updated Dt/Tm:	03/31/2026 08:53

Gram stain contains >=10 squamous epithelial cells per low power field which suggests oropharyngeal and/or nasopharyngeal contamination with no predominant bacterial morphology. Culture will not be performed.

Progress Note - Physician

Culture Blood ARD **Preliminary**

Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	03/31/2026 09:02

No growth at 24 hours.

Culture Blood ARD **Preliminary**

Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	03/31/2026 10:02

Morganella morganii
Identification obtained by Sepsityper rapid identification method.
Culture confirmation to follow.
Susceptibility to follow.

Assessment/Plan
Assessment:
Physical Exam
General: Thin appearing elderly female
Mental Status: Baseline dementia
Head: Normocephalic, atraumatic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact. Anicteric.
Ear, Nose and Throat: Oropharynx clear, moist mucous membranes. Trachea midline.
Neck: No obvious neck masses
Respiratory: Clear to auscultation bilaterally. No wheezing, crackles or rhonchi. Normal respiratory effort.
Cardiovascular: Regular rate and rhythm, no murmurs, gallops or rubs.
Gastrointestinal: Non-tender, non-distended. Normal bowel sounds.
Genitourinary: No suprapubic tenderness.
Neurologic: Cranial nerves II-XII grossly intact. Moves all extremities.
Light touch sensation is grossly intact bilaterally.

End-stage renal disease on hemodialysis Monday Wednesday Friday via IJ permacath
Hyperkalemia
Hypertension
Shortness of breath
Pneumonia

Plan
Hemodialysis stat yesterday given hyperkalemia and shortness of breath
Monitor daily for dialysis needs
Potassium should improve postdialysis
Continue amlodipine hydralazine metoprolol

Progress Note - Physician

Continue sevelamer. Monitor phosphorus level
Antibiotics. Follow-up cultures

Discharge Planning:
» Discharge Planning:
» Discharge To, Anticipated: Home with family care

Electronically Signed By: Vornicu, Maria MD
On: 03.31.2026 12:24 PDT

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	3/31/2026 08:17 PDT
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Hirabayashi DO,Ellyn (3/31/2026 08:17 PDT)
SIGN INFORMATION:	Techathaveewat DO,Worawith (3/31/2026 14:40 PDT); Hirabayashi DO,Ellyn (3/31/2026 11:47 PDT)

Subjective

Chief Complaint/Reason for Consultation
Chief Complaint: Patient brought in by EMS from home with complaints of SOB. Per the family, patient started to complain of SOB this morning. Patient found with a RA sat of <50%. Patient provided with BVM ventilation and then placed on NRB. Patient A/Ox0. EMS unable to obtain demographics on scene. BGL 119

Past Medical History
Active Problems(3)
Hemodialysis
Hepatitis A virus
High cholesterol

Past Surgical History
No surgery history documented.

Social History
No social history documented.

Objective

Measurements (most recent)

Height (cm)	157.48 cm	Stasiewski RN, Sabryna	03/30 07:08
Height (inches)	62.00 inch	SYSTEM	03/30 07:08
Weight	37.50 kg	Parala RN, Marielle Vianne	03/30 23:48

Progress Note - Physician

Weight (lbs)	82.67 lb	SYSTEM	03/30 23:48
Body Mass Index	15.12 kg/m2	Parala RN, Marielle Vianne	03/30 23:48
Scale Type	Bed scale	Parala RN, Marielle Vianne	03/30 23:48

Vital Signs (24 hrs)	**Last Charted**		**Minimum**		**Maximum**	
Temp	37.1	03/31/2026 04:00	36.9	03/30/2026 22:45	37.1	03/30/2026 19:57
Apical Heart Rate	87	03/31/2026 07:12	87	03/31/2026 07:12	98	03/30/2026 18:54
Resp Rate	**H** 24	03/31/2026 07:41	15	03/31/2026 07:00	**C** 52	03/31/2026 03:45
SBP	**C** 182	03/31/2026 07:15	**H** 148	03/30/2026 11:30	**C** 198	03/31/2026 07:00
DBP	77	03/31/2026 07:15	70	03/30/2026 11:30	**H** 90	03/30/2026 21:30
MAP	110	03/31/2026 07:15	100	03/30/2026 11:30	129	03/30/2026 18:00
SpO2	100	03/31/2026 07:41	**C** 86	03/31/2026 04:15	100	03/30/2026 21:29
O2 Flow Rate	on 2 l/min				on 4 l/min	
O2 Therapy	Nasal cannula				Nasal cannula	

Ventilator Settings (Last Within 24hrs)

Ventilator Mode: SPN-BiPAP (02:05) FIO2: 3 % (00:00)

Inspiratory Pressure: 5 cm H2O (02:05)

Expiratory Pressure: 8 cm H2O (02:05)

Intake and Output (current encounter)

	03/30 07:06 03/31 08:17	03/31 07:00 03/31 08:17	03/30 07:00 03/31 06:59	03/29 07:00 03/30 06:59	03/28 07:00 03/29 06:59
Intake	300	0	300	0	0
Output	1800	0	1800	0	0
Net Total	-1500	0	-1500	0	0

Physical Exam

Home Medications

amLODIPine (amLODIPine 10 mg oral tablet)
hydrALAZINE (hydrALAZINE 50 mg oral tablet)
losartan (losartan 100 mg oral tablet)
metoprolol (metoprolol succinate 50 mg oral tablet, extended release)
minoxidil (minoxidil 2.5 mg oral tablet)
multivitamin (Rena-Vite oral tablet)
ondansetron (ondansetron 4 mg oral tablet)

Progress Note - Physician

sevelamer (sevelamer 800 mg oral tablet)
zolpidem (zolpidem 5 mg oral tablet)

Inpatient Medications
Medications (22) Active
Scheduled: (11)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 10 mg Tab (amLODIPine) 10 mg 1 Tabs, Oral, Daily
cefTRIAXone/D5W (Rocephin) 50 mL, IV Piggyback, q24H Interval
doxycycline/D5W (doxycycline) 100 mg 100 mL, IV Piggyback, q12H
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
heparin 5000 units/mL injection 1 mL (heparin) 5,000 units 1 mL, SubCutaneous, q12H
hydrALAZINE 50 mg Tab (hydrALAZINE) 50 mg 1 Tabs, Oral, TID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
losartan 100 mg Tab (losartan) 100 mg 1 Tabs, Oral, Daily
metoprolol succinate 50 mg XL Tab (Metoprolol Succinate ER) 50 mg 1 Tabs, Oral, qHS
sevelamer carbonate 800 mg Tab (sevelamer) 800 mg 1 Tabs, Oral, TID w/Meals
Continuous: (0)
PRN: (11)
acetaminophen 325 mg Tab (Tylenol) 650 mg 2 Tabs, Oral, q4H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q3H
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID
Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 25 gm 50 mL, IV Push, q1H Interval
docusate sodium 100 mg Cap (Colace) 100 mg 1 Caps, Oral, BID
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H
glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q2H Interval
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q2H Interval
melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H

72 hour Antibiotic History

Active Antibiotics	Day of Therapy:	Last Administered	First Administered
doxycycline 100 mg, 100 mL, 100 mL/hr, IV Piggyback, q12H	2	03/30/2026 21:13	03/30/2026 21:13

Completed/Discontinued Antibiotics	Stop Date/Time	Last Administered	First Administered
azithromycin 500 mg, 250 mL, 250 mL/hr, IV Piggyback, Once	03/30/2026 10:10	03/30/2026 10:09	03/30/2026 10:09
cefTRIAXone 1 gm, 50 mL, 100 mL/hr, IV Piggyback, Once	03/30/2026 09:18	03/30/2026 09:10	03/30/2026 09:10

Progress Note - Physician

Results
Recent Labs
Blood Gases

Art Na	137 mmol/L (Normal)	03/30/2026 08:43
Art K	6.3 mmol/L (Critical)	03/30/2026 08:43
Art iCa	0.96 mmol/L (Low)	03/30/2026 08:43
pH Arterial	7.54 (High)	03/30/2026 08:43
Art pCO2	39.4 mmHg (Normal)	03/30/2026 08:43
Art pO2	153.0 mmHg (High)	03/30/2026 08:43
HCO3 Art	34.4 mmol/L (High)	03/30/2026 08:43
Arterial pCO2(T)	39.4 mmHg (N/A)	03/30/2026 08:43
Arterial pH(T)	7.54 (N/A)	03/30/2026 08:43
Arterial pO2(T)	153.0 mmHg (N/A)	03/30/2026 08:43
Art Base Excess	10.6 mmol/L (High)	03/30/2026 08:43
Art sO2	99 % (High)	03/30/2026 08:43
Art tHB	10.3 gm/dL (Low)	03/30/2026 08:43
Art Hct	32 % (Low)	03/30/2026 08:43
POC Pt Temp	37.0 DegC (N/A)	03/30/2026 08:43
POC FIO2	50 % (N/A)	03/30/2026 08:43
POC Sample	Arterial (N/A)	03/30/2026 08:43
POC Site	Radial Right (N/A)	03/30/2026 08:43
POC Allen Test	POS (N/A)	03/30/2026 08:43
POC Del Sys	BIPAP (N/A)	03/30/2026 08:43
POC Vent Mode	NO VENT (N/A)	03/30/2026 08:43
POC Set RR	14 br/min (N/A)	03/30/2026 08:43
POC IPAP	8 cm H2O (N/A)	03/30/2026 08:43
POC EPAP	4 cm H2O (N/A)	03/30/2026 08:43
Notified To and RBV	DR DAVIDSON (N/A)	03/30/2026 08:43
Notified By	vhsmurrat (N/A)	03/30/2026 08:43
Notified Note	ALL CV WITH RB (N/A)	03/30/2026 08:43

Cardiac

BNP	2004 pg/mL (High)	03/30/2026 07:31
Troponin TNIH	328.77 ng/L (Critical)	03/30/2026 22:34

General Chemistry

Glucose Level	72 mg/dL (Low)	03/31/2026 02:04
POC Glucose	75 mg/dL (Normal)	03/31/2026 07:11
Sodium	140 mmol/L (Normal)	03/31/2026 02:04
Potassium	4.2 mmol/L (Normal)	03/31/2026 02:04
Chloride	100 mmol/L (Normal)	03/31/2026 02:04

Progress Note - Physician

CO2	31 mmol/L (Normal)	03/31/2026 02:04
Anion Gap	9 mmol/L (Normal)	03/31/2026 02:04
BUN	17 mg/dL (Normal)	03/31/2026 02:04
Creatinine	2.19 mg/dL (High)	03/31/2026 02:04
BUN/Creat Ratio	8 (Normal)	03/31/2026 02:04
Calcium	9.0 mg/dL (Normal)	03/31/2026 02:04
Albumin. Level	3.4 gm/dL (Normal)	03/31/2026 02:04
TP	6.4 gm/dL (Normal)	03/30/2026 22:34
A/G Ratio	1.3 (Normal)	03/30/2026 22:34
T Bili	0.8 mg/dL (Normal)	03/30/2026 22:34
D Bili	0.2 mg/dL (Normal)	03/30/2026 22:34
Alk Phos	125 units/L (High)	03/30/2026 22:34
AST	97 units/L (High)	03/30/2026 22:34
ALT	49 units/L (Normal)	03/30/2026 22:34
Mg Lvl	1.9 mg/dL (Normal)	03/31/2026 02:04
Phosphorus	5.6 mg/dL (High)	03/31/2026 02:04
Estimated Creatinine Clearance	10.11 mL/min ()	03/31/2026 02:58
eGFR Cr	See Comment mL/min/1.73m2 (N/A)	03/31/2026 02:04
eGFR Pediatric	Not Reported (N/A)	03/31/2026 02:04
Lactic Acid Lvl	1.8 mmol/L (Normal)	03/30/2026 08:48
Hgb A1C	4.3 % (Normal)	03/30/2026 11:15
MAR Glucose Result	102 ()	03/30/2026 09:49

General Hematology

WBC	5.87 x10e3/mcL (Normal)	03/31/2026 02:04
RBC	3.65 x10e6/mcL (Normal)	03/31/2026 02:04
Hgb	8.4 gm/dL (Low)	03/31/2026 02:04
Hct	25.6 % (Low)	03/31/2026 02:04
MCV	70.1 Femtoliters (Low)	03/31/2026 02:04
MCH	23.0 pg (Low)	03/31/2026 02:04
MCHC	32.8 gm/dL (Normal)	03/31/2026 02:04
RDW-SD	45.2 Femtoliters (Normal)	03/31/2026 02:04
RDW-CV	18.6 % (High)	03/31/2026 02:04
Plt	141 x10e3/mcL (Normal)	03/31/2026 02:04
MPV	9.6 Femtoliters (Normal)	03/31/2026 02:04

Progress Note - Physician

Imm Plt Fract	3.7 % (Normal)	03/31/2026 02:04
NRBC %	0.2 /100 WBC (Normal)	03/30/2026 07:31
NRBC #	0.02 x10e3/mcL (High)	03/30/2026 07:31
Neut % Auto	80.7 % (N/A)	03/31/2026 02:04
Lymph % Auto	12.4 % (N/A)	03/31/2026 02:04
Mono % Auto	3.7 % (N/A)	03/31/2026 02:04
Eos % Auto	2.0 % (N/A)	03/31/2026 02:04
Baso % Auto	0.7 % (N/A)	03/31/2026 02:04
Immature Grans %	0.5 % (Normal)	03/31/2026 02:04
Neut # Auto	4.73 x10e3/mcL (Normal)	03/31/2026 02:04
Lymph # Auto	0.73 x10e3/mcL (Low)	03/31/2026 02:04
Mono # Auto	0.22 x10e3/mcL (Normal)	03/31/2026 02:04
Eos # Auto	0.12 x10e3/mcL (Normal)	03/31/2026 02:04
Baso # Auto	0.04 x10e3/mcL (Normal)	03/31/2026 02:04
Immature Grans # Auto	0.03 x10e3/mcL (Normal)	03/31/2026 02:04
RBC Morph	Abnormal (Abnormal)	03/30/2026 07:31
Plt Morphology	Normal (Normal)	03/30/2026 07:31
Polychrom	1+ (Abnormal)	03/30/2026 07:31
Micro	2+ (Abnormal)	03/30/2026 07:31
Plt Est	Adequate (Normal)	03/30/2026 07:31
Schistocytes	1+ (Abnormal)	03/30/2026 07:31
Target Cells (Codocytes)	3+ (Abnormal)	03/30/2026 07:31
Teardrop Cells (Dacrocytes)	1+ (Abnormal)	03/30/2026 07:31

Hepatitis

Hep A IgM	Reactive (Abnormal)	03/30/2026 10:54
Hep B Core IgM	Non-Reactive (Normal)	03/30/2026 11:04
Hepatitis B Surface Antigen	Non-Reactive (Normal)	03/30/2026 11:04
Hep Bs Ab Qn	31.9 milli intl units/mL (High)	03/30/2026 11:04
Hep Bs Ab Qn Interp	Reactive (Abnormal)	03/30/2026 11:04
Hep C Ab	Non-Reactive (Normal)	03/30/2026 10:54

Progress Note - Physician

Other Chemistry

Procalcitonin Lvl	1.56 ng/mL (High)	03/30/2026 11:15

Thyroid

T3 Free	2.87 pg/mL (Normal)	03/30/2026 11:15
T4 Free	1.74 ng/dL (High)	03/30/2026 11:15
TSH Ultrasensitive	3.046 mc Intl units/mL (Normal)	03/30/2026 11:15

Abnormal Labs

Blood Gases

Art K	6.3 mmol/L (Critical)	03/30/2026 08:43
Art iCa	0.96 mmol/L (Low)	03/30/2026 08:43
pH Arterial	7.54 (High)	03/30/2026 08:43
Art pO2	153.0 mmHg (High)	03/30/2026 08:43
HCO3 Art	34.4 mmol/L (High)	03/30/2026 08:43
Art Base Excess	10.6 mmol/L (High)	03/30/2026 08:43
Art sO2	99 % (High)	03/30/2026 08:43
Art tHB	10.3 gm/dL (Low)	03/30/2026 08:43
Art Hct	32 % (Low)	03/30/2026 08:43

Cardiac

BNP	2004 pg/mL (High)	03/30/2026 07:31
Troponin TNIH	328.77 ng/L (Critical)	03/30/2026 22:34

General Chemistry

Glucose Level	72 mg/dL (Low)	03/31/2026 02:04
Creatinine	2.19 mg/dL (High)	03/31/2026 02:04
Alk Phos	125 units/L (High)	03/30/2026 22:34
AST	97 units/L (High)	03/30/2026 22:34
Phosphorus	5.6 mg/dL (High)	03/31/2026 02:04

General Hematology

Hgb	8.4 gm/dL (Low)	03/31/2026 02:04
Hct	25.6 % (Low)	03/31/2026 02:04
MCV	70.1 Femtoliters (Low)	03/31/2026 02:04
MCH	23.0 pg (Low)	03/31/2026 02:04
RDW-CV	18.6 % (High)	03/31/2026 02:04
NRBC #	0.02 x10e3/mcL (High)	03/30/2026 07:31
Lymph # Auto	0.73 x10e3/mcL (Low)	03/31/2026 02:04
RBC Morph	Abnormal (Abnormal)	03/30/2026 07:31
Polychrom	1+ (Abnormal)	03/30/2026 07:31

Progress Note - Physician

Micro	2+ (Abnormal)	03/30/2026 07:31
Schistocytes	1+ (Abnormal)	03/30/2026 07:31
Target Cells (Codocytes)	3+ (Abnormal)	03/30/2026 07:31
Teardrop Cells (Dacrocytes)	1+ (Abnormal)	03/30/2026 07:31

Hepatitis

Hep A IgM	Reactive (Abnormal)	03/30/2026 10:54
Hep Bs Ab Qn	31.9 milli intl units/mL (High)	03/30/2026 11:04
Hep Bs Ab Qn Interp	Reactive (Abnormal)	03/30/2026 11:04

Other Chemistry

Procalcitonin Lvl	1.56 ng/mL (High)	03/30/2026 11:15

Thyroid

T4 Free	1.74 ng/dL (High)	03/30/2026 11:15

CBC, BMP, Coagulation Trend (last 4 resulted)

WBC	**5.87** 03/31/2026 02:04	**12.25 H** 03/30/2026 07:31		
Hgb	**8.4 L** 03/31/2026 02:04	**11.2** 03/30/2026 07:31		
Hct	**25.6 L** 03/31/2026 02:04	**33.7** 03/30/2026 07:31		
Baso # Auto	**0.04** 03/31/2026 02:04	**0.03** 03/30/2026 07:31		
Baso % Auto	**0.7** 03/31/2026 02:04	**0.2** 03/30/2026 07:31		
Eos # Auto	**0.12** 03/31/2026 02:04	**0.02** 03/30/2026 07:31		
Eos % Auto	**2.0** 03/31/2026 02:04	**0.2** 03/30/2026 07:31		
Immature Grans %	**0.5** 03/31/2026 02:04	**0.6** 03/30/2026 07:31		
Immature Grans # Auto	**0.03** 03/31/2026 02:04	**0.07** 03/30/2026 07:31		
Lymph # Auto	**0.73 L** 03/31/2026 02:04	**1.33** 03/30/2026 07:31		
Lymph % Auto	**12.4** 03/31/2026 02:04	**10.9** 03/30/2026 07:31		
Mono # Auto	**0.22** 03/31/2026 02:04	**0.47** 03/30/2026 07:31		

Progress Note - Physician

Mono % Auto	**3.7** 03/31/2026 02:04	**3.8** 03/30/2026 07:31		
Neut # Auto	**4.73** 03/31/2026 02:04	**10.33 H** 03/30/2026 07:31		
Neut % Auto	**80.7** 03/31/2026 02:04	**84.3** 03/30/2026 07:31		
Plt	**141** 03/31/2026 02:04	**230** 03/30/2026 07:31		
Glucose Level	**72 L** 03/31/2026 02:04	**189 H** 03/30/2026 07:31		
Sodium	**140** 03/31/2026 02:04	**139** 03/30/2026 07:31		
Potassium	**4.2** 03/31/2026 02:04	**5.9 H** 03/30/2026 07:31		
Chloride	**100** 03/31/2026 02:04	**94 L** 03/30/2026 07:31		
CO2	**31** 03/31/2026 02:04	**27** 03/30/2026 07:31		
Creatinine	**2.19 H** 03/31/2026 02:04	**4.80 H** 03/30/2026 07:31		
BUN	**17** 03/31/2026 02:04	**44 H** 03/30/2026 07:31		
BUN/Creat Ratio	**8** 03/31/2026 02:04	**9** 03/30/2026 07:31		
Mg Lvl	**1.9** 03/31/2026 02:04			
Phosphorus	**5.6 H** 03/31/2026 02:04	**10.5 H** 03/30/2026 13:08		
Calcium	**9.0** 03/31/2026 02:04	**8.6 L** 03/30/2026 07:31		
Albumin. Level	**3.4** 03/31/2026 02:04	**3.6** 03/30/2026 22:34	**3.4** 03/30/2026 11:15	
T Bili	**0.8** 03/30/2026 22:34	**0.4** 03/30/2026 11:15		
D Bili	**0.2** 03/30/2026 22:34	**0.2** 03/30/2026 11:15		
Alk Phos	**125 H** 03/30/2026 22:34	**150 H** 03/30/2026 11:15		
AST	**97 H** 03/30/2026 22:34	**57 H** 03/30/2026 11:15		
ALT	**49** 03/30/2026 22:34	**51** 03/30/2026 11:15		

Progress Note - Physician

Assessment/Plan
Assessment:
Subjective:
Patient is a 90-year-old female with a PMHx of ESRD on HD, chronic hypoxic respiratory failure requiring 2–5 L home O2 who presented to Valley Hospital with reports of increasing shortness of breath. Patient was initially admitted to the ICU for stat dialysis and continuous BiPAP.

Seen and examined at bedside. Today she is back to her baseline O2. Now s/p stat HD. Overall doing well.

Physical exam:
General: Thin appearing Vietnamese speaking woman sitting up in bed.
Mental Status: Alert and oriented to person, place, time. Normal affect.
Head: Normocephalic, atraumatic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact. Anicteric.
Ear, Nose and Throat: Oropharynx clear, moist mucous membranes. Trachea midline.
Neck: No obvious neck masses
Respiratory: Clear to auscultation bilaterally. No wheezing, crackles or rhonchi. Normal respiratory effort. On LFNC
Cardiovascular: Regular rate and rhythm, no murmurs, gallops or rubs. Right sided subclavian HD catheter noted
Gastrointestinal: Non-tender, non-distended. Normal bowel sounds.
Genitourinary: No suprapubic tenderness.
Neurologic: Cranial nerves II-XII grossly intact. Moves all extremities.
Light touch sensation is grossly intact bilaterally.

Assessment:
Acute on chronic hypoxic respiratory failure 2/2 pneumonia versus fluid overload, improved
Dyspnea
Sepsis with RBC, HR 2/2 respiratory source
Troponinemia 2/2 demand ischemia
ESRD on HD MWF
Hyperkalemia 2/2 above
Hepatitis A IgM positive
Oxygen dependent reportedly on 2 to 5 L at home
Hypertension
Dementia

Plan:
Downgraded from ICU level of care
CT chest/abdomen/pelvis with interlobular septal thickening and adjacent ground-glass opacities in bilateral lungs possibly pulmonary edema versus infectious/inflammatory versus less likely hemorrhagic. Small to moderate right and small left-sided pleural effusions.
Acute on chronic hypoxic respite failure secondary to pneumonia in addition to fluid overload
– Blood cultures 1/2 positive for gram-negative rods
– Continue antibiotics with Rocephin, doxycycline. Will adjust as indicated by cultures and sensitivities
– Breathing treatments
– Goal SpO2 92%, titrated down to LFNC 2L
Nephrology consulted appreciate recommendation
– S/p stat HD
Troponinemia secondary to demand ischemia, patient denies chest pain
– TTE pending
PRN antihypertensives and antiemetics
Speech therapy eval pending
Replete electrolytes as needed, routine labs, and supportive care.

Patient has 5 children but main caretaker is brother who lives with her. ICU team discussed with extensively regarding CODE STATUS with patient's daughter and brother at the bedside who had different opinion regarding CODE STATUS. They wish to proceed with full

Progress Note - Physician

code no shock for now but has plan to discuss among family members.

Consults: ICU, nephrology Dr. Vornicu
Diet: N.p.o., pending speech therapy evaluation
Activity: Early and progressive mobilization
GI prophylaxis: None
DVT prophylaxis: Subcu heparin, SCD
CODE STATUS: Full code, no shock per family
Dispo: IMC

Will discuss plan with attending physician. Awaiting further recommendations.

Ellyn Hirabayashi, DO
Internal Medicine - PGY3
Valley Hospital Medical Center

Discharge Planning:
» Discharge Planning:
» Discharge To, Anticipated: Home with family care

Electronically Signed By: Hirabayashi, Ellyn DO
On: 03.31.2026 11:47 PDT

Electronically Signed On: 03.31.2026 14:40 PDT
Techathaveewat, Worawith DO

Orders

Order: **Diet**			
Order Date/Time: 3/30/2026 10:38 PDT			
Signed Date/Time: 3/30/2026 10:38 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Nutrition Services	Activity Type: Diets TransMedRec
Ordering Physician: Cho DO,Claire Sungkyung		Consulting Physician:	
Entered By: Cho DO,Claire Sungkyung on 3/30/2026 10:38 PDT			
Order Details: 3/30/26 10:38:00 AM PDT, Type: Renal Dialysis, Consistency: 6 - Soft & Bite Sized (Dysphagia), Consistency 0 - Thin, One on One, Special Instructions: medication 1 at a time			
Order Comment:			

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Orders

Order: **Consult to Case Management (Case Management Consult)**			
Order Date/Time: 4/1/2026 10:27 PDT			
Signed Date/Time: 4/1/2026 10:27 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Consults	Activity Type: Case Management Consults
End-state Date/Time: 4/1/2026 10:27 PDT		End-state Reason:	
Ordering Physician: Hirabayashi DO,Ellyn		Consulting Physician:	
Entered By: Hirabayashi DO,Ellyn on 4/1/2026 10:27 PDT			
Order Details: 4/1/26 10:27:00 AM PDT, Routine, Reason for Consult: D/C to Home w Home Health, No Isolation, anticipate HHC PT/OT for strength/conditioning CNA for ADLs and medication management Already use A+, please verify			
Order Comment:			

Order: **Activity**			
Order Date/Time: 3/30/2026 10:38 PDT			
Signed Date/Time: 3/30/2026 10:38 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Patient Care	Activity Type: Patient Activity TransMedRec
Ordering Physician: Cho DO,Claire Sungkyung		Consulting Physician:	
Entered By: Cho DO,Claire Sungkyung on 3/30/2026 10:38 PDT			
Order Details: 3/30/26 10:38:00 AM PDT, Type: Early and Progressive Mobilization, Routine, Constant order			
Order Comment:			

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Orders - Admission/Discharge (Curaspan)

Order: **Level of Care Change**			
Order Date/Time: 3/31/2026 06:54 PDT			
Signed Date/Time: 3/31/2026 06:54 PDT			
Order Status: Completed	Department Status: Completed	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/31/2026 07:42 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/31/2026 06:54 PDT			
Order Details: 3/31/26 6:54:50 AM PDT, Admitting: Delmundo DO, Lloyd, Attending: Techathaveewat DO, Worawith, Admit as Inpatient, Acute Intermediate Care			
Order Comment: Ordered secondary to modification of Level Of Care on the Admission/Observation/Transfer Order.			

Action Type: Complete	Action Date/Time: 3/31/2026 07:42 PDT	Action Personnel: Phelps,Shanae
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type:
Order Details: 03/31/26 6:54:50 PDT, Admitting: Delmundo DO, Lloyd, Attending: Techathaveewat DO, Worawith, Admit as Inpatient, Acute Intermediate Care		
Review Information: Doctor Cosign: Not Required		
Order Comment:		

Action Type: Order	Action Date/Time: 3/31/2026 06:54 PDT	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 03/31/26 6:54:50 PDT, Admitting: Delmundo DO, Lloyd, Attending: Techathaveewat DO, Worawith, Admit as Inpatient, Acute Intermediate Care		
Review Information: Nurse Review: Electronically Signed, Parala RN,Marielle Vianne on 3/31/2026 07:17 PDT Doctor Cosign: Not Required		
Order Comment: Ordered secondary to modification of Level Of Care on the Admission/Observation/Transfer Order.		

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Orders - Admission/Discharge (Curaspan)

Order: **Change Attending Physician**			
Order Date/Time: 3/31/2026 06:54 PDT			
Signed Date/Time: 3/31/2026 06:54 PDT			
Order Status: Completed	Department Status: Completed	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/31/2026 07:42 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/31/2026 06:54 PDT			
Order Details: 3/31/26 6:54:50 AM PDT, Admitting: Delmundo DO, Lloyd, Attending: Techathaveewat DO, Worawith			
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00			

Action Type: Complete	Action Date/Time: 3/31/2026 07:42 PDT	Action Personnel: Phelps,Shanae
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type:
Order Details: 03/31/26 6:54:50 PDT, Admitting: Delmundo DO, Lloyd, Attending: Techathaveewat DO, Worawith		
Review Information: Doctor Cosign: Not Required		
Order Comment:		

Action Type: Order	Action Date/Time: 3/31/2026 06:54 PDT	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 03/31/26 6:54:50 PDT, Admitting: Delmundo DO, Lloyd, Attending: Techathaveewat DO, Worawith		
Review Information: Nurse Review: Electronically Signed, Parala RN,Marielle Vianne on 3/31/2026 07:17 PDT Doctor Cosign: Not Required		
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00		

Orders - Admission/Discharge (Curaspan)

Order: **Admission/Observation/Transfer**			
Order Date/Time: 3/30/2026 09:37 PDT			
Signed Date/Time: 3/30/2026 09:37 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Admit/Transfer/Observation	Activity Type: Admit/Transfer/Observation
End-state Date/Time: 3/30/2026 09:37 PDT		End-state Reason:	
Ordering Physician: Techathaveewat DO,Worawith		Consulting Physician:	
Entered By: Davidson MD,Jeffrey A on 3/30/2026 09:37 PDT			
Order Details: 3/30/26 9:37:00 AM PDT, Estimated LOS 2 to 3 Nights, Admit as Inpatient, Level of Care Acute Intermediate Care, Admitting: Delmundo DO, Lloyd Attending: Techathaveewat DO, Worawith, Hypoxemia			
Order Comment:			

Action Type: Modify	Action Date/Time: 3/31/2026 06:54 PDT	Action Personnel: Cho DO,Claire Sungkyung
Responsible Provider/Electronically Signed by: Techathaveewat DO, Worawith	Supervising Provider:	Communication Type: Phone w/ Readback (Cosign)
Order Details: 03/30/26 9:37:00 PDT, Estimated LOS 2 to 3 Nights, Admit as Inpatient, Level of Care Acute Intermediate Care, Admitting: Delmundo DO, Lloyd Attending: Techathaveewat DO, Worawith, Hypoxemia		
Review Information: Nurse Review: Electronically Signed, Parala RN,Marielle Vianne on 3/31/2026 07:17 PDT Doctor Cosign: Electronically Signed, Techathaveewat DO,Worawith on 3/31/2026 11:19 PDT		
Order Comment:		

Action Type: Order	Action Date/Time: 3/30/2026 09:37 PDT	Action Personnel: Davidson MD,Jeffrey A
Responsible Provider/Electronically Signed by: Milne,C Dean DO	Supervising Provider:	Communication Type: Phone w/ Readback (Cosign)
Order Details: 03/30/26 9:37:00 PDT, Estimated LOS 2 to 3 Nights, Admit as Inpatient, Level of Care Critical Care, Admitting: Delmundo DO, Lloyd Attending: Milne, C Dean DO, Hypoxemia		
Review Information: Nurse Review: Electronically Signed, Alex RN,Christina on 3/30/2026 09:53 PDT Doctor Cosign: Electronically Signed, Techathaveewat DO,Worawith on 3/30/2026 10:22 PDT		
Order Comment:		

Physical Therapy Forms

DOCUMENT TYPE:	PT Examination - Text
SERVICE DATE/TIME:	4/1/2026 09:32 PDT
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Dampog PT,Lynne Rose (4/1/2026 09:27 PDT)
SIGNED INFORMATION:	Dampog PT,Lynne Rose (4/1/2026 09:27 PDT)

PT Evaluation/Reevaluation Entered On: 4/1/2026 9:37 PDT
Performed On: 4/1/2026 9:32 PDT by Dampog PT, Lynne Rose

Physical Therapy Forms

General Info
PT Assessment Type : Initial Evaluation
Physical Therapy Orders : PT Evaluation and Treatment - 03/31/26 12:22:00 PDT, Routine, Stop date 03/31/26 12:22:00 PDT
Precautions RTF : Active Orders:
» Precautions, 03/30/26 10:38:00 PDT, Routine, Precautions: Aspiration | Fall, Ordered
» Oxygen Therapy, 03/30/26 9:37:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 between 90 - 95%, Ordered

Discontinued Orders:
» Oxygen Therapy, 03/30/26 7:14:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 92%, Discontinued

Affect/Behavior Rehab : Appropriate, Cooperative
Basic Command Following : Intact
Pain Symptoms : No
Suspected Stroke Patient : No

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

Home Environment
Living Environment : Home Environment
Living Situation: Home with family care Performed By: Caravella SLP, Crystal M 03/31/2026
Sensory Deficits: Visual impairment, left eye, Visual impairment, right eye Performed By: Parala RN, Marielle Vianne 03/30/2026
Living Situation : Home with family care
Lives With : Other: brothers, sister

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

Location Bed
Primary Bedroom : 1st floor
Primary Bathroom : 1st floor
Kitchen : 1st floor
Laundry : 1st floor

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

Home Environment II
Living Environment : Home Environment
Living Situation: Home with family care Performed By: Caravella SLP, Crystal M 03/31/2026
Sensory Deficits: Visual impairment, left eye, Visual impairment, right eye Performed By: Parala RN, Marielle Vianne 03/30/2026
Home Equipment Rehab : Hospital bed, Manual wheelchair, Mechanical lift

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

Prior Functional Status Grid
ADL : Dependent
Mobility : Dependent
Instrumental ADL : Dependent
Cognitive-Communication Skills : Independent

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

LE Range/Strength
LE Overall Range of Motion Grid

Physical Therapy Forms

Left Lower Extremity Passive Range : Within functional limits
Left Lower Extremity Active Range : Within functional limits
Right Lower Extremity Passive Range : Within functional limits
Right Lower Extremity Active Range : Within functional limits

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

Lt Lower Extremity Strength : Other: grossly 3 to 3+/5
Rt Lower Extremity Strength : Other: grossly 3 to 3+/5

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

Balance
Kansas University Balance Scale : Yes
Sitting Balance Score : 1+

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

Mobility
Mobility Grid
Roll Left : Rehab Total assistance
Roll Right : Rehab Total assistance
Supine to Sit : Rehab Total assistance
Sit to Supine : Rehab Total assistance

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

Long Term Goals
PT Patient,Caregiver Goal : -

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

Plan
PT Anticipated Treatments, Needs : Discharge/Discontinue PT Treatment
Reason Plan not Established w/ Family : eval only
PT Plan/Goals Established w Pt/Caregiver : No

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

Additional Information
Additional Information PT : THERAPY SESSION TIME: 0854-0932
RN CLEARED FOR THERAPY: Aleja
PAIN LOCATION AND LEVEL: 0/10
ORIENTATION: x 3
PLOF: Lives with brothers and sister, uses hoyer lift from transfers when going to dialysis. DEpendent with ADL's. Per brother pt had HHPT previously and pt was able to take a few steps.

PT orders received, chart reviewed, pt is a90 yo female admitted with c/o SOB. PMHx: ESRD on HD, home 02 2-5L, htn, dementia. Upon arrival, pt was semi-supine in bed. Pt's brother at bedside and assisted with translation. Pt agreeable to PT eval and eval completed. Performed B/L LE AAROM-AROM. Total assist with supine to sit EOB.
At end of session, pt was in semi-fowlers position in bed. All needs met. All safety prec. in place. Pt is at her baseline. No further skilled PT services warranted in acute setting. Pt's brother reports that HHPT stopped seeing pt approx. 6 wks ago and requesting for HHPT to resume again, states he called them but has not heard back.

Environmental Safety Implemented : Bed in low position, Call device within reach, Familiarize patient with environment, Lock wheeled equipment, Personal items within reach, Encourage personal mobility support item use/within reach

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

PT Non Timed Modalities 2026

Physical Therapy Forms

PT Eval High Complexity Total Number Min : 15
PT Total Non Timed : 15

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

PT Timed Modalities 2026
PT Timed Min Total : 23
PT Timed Unit Total : 2
PT THERAPEUTIC ACT 15MIN : 15
Activity Therapeutic Num of Units : 1
PT THERAPEUTIC EXER 15MIN : 8
Exercise Therapeutic Num of Units : 1

Dampog PT, Lynne Rose - 4/1/2026 9:27 PDT

Occupational Therapy Forms

DOCUMENT TYPE:	OT Evaluation - Text
SERVICE DATE/TIME:	4/1/2026 09:28 PDT
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Bolor OT,Shelley A (4/1/2026 09:28 PDT)
SIGNED INFORMATION:	Bolor OT,Shelley A (4/1/2026 09:28 PDT)

OT Evaluation/Reevaluation Entered On: 4/1/2026 9:34 PDT
Performed On: 4/1/2026 9:28 PDT by Bolor OT, Shelley A

General Information
OT Assessment Type : Initial Evaluation
Occupational Therapy Orders : OT Evaluation and Treatment - 03/31/26 12:22:00 PDT, Routine, Stop date 03/31/26 12:22:00 PDT
Precautions RTF : Active Orders:
» Precautions, 03/30/26 10:38:00 PDT, Routine, Precautions: Aspiration | Fall, Ordered
» Oxygen Therapy, 03/30/26 9:37:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 between 90 - 95%, Ordered

Discontinued Orders:
» Oxygen Therapy, 03/30/26 7:14:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 92%, Discontinued

Orientation Assessment Rehab : Identifies self
Affect/Behavior Rehab : Appropriate, Calm, Cooperative
Basic Command Following : Intact
Safety/Judgment : Impaired
Pain Symptoms : No
Suspected Stroke Patient : No
Culture/Spiritual Beliefs to Incorporate : Yes

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Occupational Therapy Forms

Home Environment
Living Environment : Home Environment
Living Situation: Home with family care Performed By: Caravella SLP, Crystal M 03/31/2026
Sensory Deficits: Visual impairment, left eye, Visual impairment, right eye Performed By: Parala RN, Marielle Vianne 03/30/2026
Living Situation : Home with family care
Lives With : Other: siblings/family
Lives In : Apartment

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Rehabilitation Stairs Grid
Primary Bedroom : 1st floor
Primary Bathroom : 1st floor
Kitchen : 1st floor
Laundry : 1st floor

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Home Environment II
Living Environment : Home Environment
Living Situation: Home with family care Performed By: Parala RN, Marielle Vianne 03/30/2026
Sensory Deficits: Visual impairment, left eye, Visual impairment, right eye Performed By: Parala RN, Marielle Vianne 03/30/2026
Home Equipment Rehab : Manual wheelchair, Other: hoyer lift

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

ADL : Dependent
Mobility : Dependent
Instrumental ADL : Dependent
Cognitive-Communication Skills : Independent

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Basic ADL
Activities of Daily Living Grid
Grooming : Min A
UE Dressing : Maximal Assistance
LE Dressing : Total Assistance
Toileting : Total Assistance

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Limiting Factors : Weakness

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Balance
General Balance : Sitting
General Balance Assessment : Poor

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

UE Strength/ROM
Upper Extremity Overall ROM Grid
Left Upper Extremity Passive Range : Within functional limits
Left Upper Extremity Active Range : Within functional limits
Right Upper Extremity Passive Range : Within functional limits
Right Upper Extremity Active Range : Within functional limits

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Left Upper Extremity Strength Grid
Scapular Elevation : 3

Occupational Therapy Forms

Shoulder Flexion : 3
Shoulder Extension : 3
Shoulder Abduction : 3
Shoulder Adduction : 3
Shoulder External Rotation : 3
Shoulder Internal Rotation : 3
Elbow Flexion : 3
Elbow Extension : 3
Forearm Pronation : 3
Forearm Supination : 3
Wrist Flexion : 3
Wrist Extension : 3
Hand Grip : 3
Finger Flexion : 3
Finger Extension : 3

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Right Upper Extremity Strength Grid
Scapular Elevation : 3
Shoulder Flexion : 3
Shoulder Extension : 3
Shoulder Abduction : 3
Shoulder Adduction : 3
Shoulder External Rotation : 3
Shoulder Internal Rotation : 3
Elbow Flexion : 3
Elbow Extension : 3
Forearm Pronation : 3
Forearm Supination : 3
Wrist Flexion : 3
Wrist Extension : 3
Hand Grip : 3
Finger Flexion : 3
Finger Extension : 3

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Mobility
Mobility Grid
Roll Left : Rehab Total assistance
Roll Right : Rehab Total assistance
Supine to Sit : Rehab Total assistance
Sit to Supine : Rehab Total assistance

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

OT Assessment
Impairments/Limitations : Basic activity of daily living deficits, Mobility deficits, Strength deficits

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Education
Teaching Method : Demonstration, Explanation
Learner(s) Present : Self

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Occupational Therapy Education Grid

Occupational Therapy Forms

Activity of Daily Living Training : Verbalizes understanding, Demonstrates
Bed Positioning : Verbalizes understanding, Demonstrates

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Long Term Goals
OT Patient/Caregiver Goal : n/a

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Long Term Goal 5

	Long Term Goal 1
Goal :	OT IE only
	Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Plan
OT Anticipated Treatments, Needs : Discharge/Discontinue OT treatment
Post-Acute Discharge Therapy Recommendation OT : No further skilled therapy services warranted
IDR OT Recommend/Goal : no further OT services rec post d/c
OT Evaluation Charge : Yes
OT Tx Plan/Goals Established w Patient : Yes

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Additional Information
Additional Information OT : RN CLEARED FOR THERAPY: Aleja
PAIN LOCATION AND LEVEL: none
ORIENTATION: AxOx3
PLOF: Dependent

OT orders received, chart reviewed, pt is admitted for hypoxemia. Upon arrival, pt was in semi supine. Pt. brother, David, translates for pt (Vietnamese).Pt agreeable to OT eval and eval completed. See ADL/mobility grid for details.
At end of session, pt was returned to supine c all needs within reach and RN aware.

BARRIERS TO SAFE DC/IMPAIRMENTS AND LIMITATIONS: n/a
THERAPY RECOMMENDATION: no further OT services rec-- family care at home to resume

Patient Position : Semi-Fowler's
Environmental Safety Implemented : Adequate room lighting, Bed in low position, Call device within reach, Familiarize patient with environment, Lock wheeled equipment, Personal items within reach, Encourage personal mobility support item use/within reach, Encourage sensory support item use/within reach

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

OT Non Timed Modalities 2026
OT Treatment Start Time : 4/1/2026 8:53 PDT
OT Treatment End Time : 4/1/2026 9:31 PDT
Eval High Complexity Total Number Min : 15
OT Total NonTimed Mod : 15

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

OT Timed Modalities 2026
OT Time Treatment Started : 4/1/2026 8:53 PDT
OT Time Treatment Ended : 4/1/2026 9:31 PDT
Rehab Patient : No

Occupational Therapy Forms

OT Timed Min Total : 23
OT Timed Units Total : 2
OT SELF CARE MGT TRAIN 15M : 23
OT SELF CARE MGT TRAIN Num of Units : 2

Bolor OT, Shelley A - 4/1/2026 9:28 PDT

Speech Therapy

DOCUMENT TYPE:	Clinical Swallowing - Text
SERVICE DATE/TIME:	3/31/2026 08:15 PDT
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Caravella SLP,Crystal M (3/31/2026 08:58 PDT)
SIGNED INFORMATION:	Caravella SLP,Crystal M (3/31/2026 08:58 PDT)

ST Bedside Swallow Eval Entered On: 3/31/2026 9:09 PDT
Performed On: 3/31/2026 8:15 PDT by Caravella SLP, Crystal M

General Information
Speech Orders : ST Bedside Swallow Exam - 03/31/26 7:11:00 PDT, Bedside Swallow Exam, Routine, Stop date 03/31/26 7:11:00 PDT, 20179621233.0
Speech Therapy Eval and Treat - 03/31/26 7:11:00 PDT, Bedside Swallow Exam, Routine, Stop date 03/31/26 7:11:00 PDT
ST Bedside Swallow Exam - 03/30/26 15:26:00 PDT, Bedside Swallow Exam, Routine, Stop date 03/30/26 15:26:00 PDT, 20176418327.0
Speech Therapy Eval and Treat - 03/30/26 15:26:00 PDT, Bedside Swallow Exam, Routine, Stop date 03/30/26 15:26:00 PDT
Precautions RTF : Active Orders:
» Precautions, 03/30/26 10:38:00 PDT, Routine, Precautions: Aspiration | Fall, Ordered
» Oxygen Therapy, 03/30/26 9:37:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 between 90 - 95%, Ordered

Discontinued Orders:
» Oxygen Therapy, 03/30/26 7:14:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 92%, Discontinued

History of Swallow Studies : No qualifying data available
Level of Consciousness Rehab : Alert
Affect/Behavior Rehab : Flat
Basic Command Following : Impaired
Orientation Assessment Rehab : Identifies self, Not oriented to situation, Not oriented to time
Pain Interfering With SLP Treatment : No

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Home Environment

Speech Therapy

Living Environment : Home Environment
Living Situation: Home with family care Performed By: Parala RN, Marielle Vianne 03/30/2026
Sensory Deficits: Visual impairment, left eye, Visual impairment, right eye Performed By: Parala RN, Marielle Vianne 03/30/2026
Living Situation : Home with family care

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Pre-Assessment
Oxygen Therapy : Nasal cannula
SpO2 : 97 %
Respiratory Comments : 3LNC
Tracheostomy Information : Oxygen Flow Rate: 2 L/min Performed by: Rapelje RRT, Glenn 03/31/26 07:41:00
FIO2: 30 (Modified) Performed by: Parala RN, Marielle Vianne 03/31/26 00:00:00

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Oral-Facial Exam
Facial Movement : Within functional limits
Dentition : Own teeth, Missing teeth

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Oral Structure Grid
Labial Structure : Within functional limits
Mandible : Within functional limits
Lingual Structure : Within functional limits

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Oral Function Grid
Labial : Within functional limits
Mandible : Within functional limits
Lingual : Within functional limits
Velopharyngeal : Within functional limits

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Secretion Management : Adequate

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Swallowing Exam
Patient Position : Upright 90 degrees
Reflexive Cough : Absent

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Liquids Swallowing Exam Adult Grid

	Ice Chips	Thin Liquid by Half Teaspoon	Thin Liquid by Bottle	Thin Liquid by Cup
Presentation :	Speech-Language Pathologist	Speech-Language Pathologist	Self, Speech-Language Pathologist	Speech-Language Pathologist
Oral Symptoms :	None observed	None observed	None observed	None observed
Pharyngeal Symptoms :	None observed	None observed	None observed	None observed
	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Speech Therapy

Consistency Swallowing Exam Adult Grid

	Puree Consistency	Soft Consistency	Regular Consistency
Presentation :	Speech-Language Pathologist	Speech-Language Pathologist	Self
Oral Symptoms :	None observed	None observed	Oral residue, Slowed oral transit time
Pharyngeal Symptoms :	None observed	None observed	None observed
	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Assessment/Recommendations
Clinical Swallowing Impression Grid
Within Functional Limits : Liquids, Solids

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Diet Consistency : IDDSI 6 Soft and Bite Sized
Liquid Viscosity : Thin
Medications : Whole with drink
Supervision : 1:1
Positioning : Upright 90 degrees
Safe Feeding/Swallow Precautions : Remain upright after meals 30 minutes, Small bites of food
Clinical Swallowing Recommendations : Dysphagia treatment

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Education
Learner(s) Present for Session : Explanation
Learner(s) Present : Self
Barriers to Learning : Language barrier, Acuity of Illness, Cognitive deficits, Emotional state, Vision impairment

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Document learning evaluation for Responsible Learner(s)
Diet Restrictions/Recommendations : Needs practice/supervision
Feeding Strategies : Needs practice/supervision
Speech Language/Pathology Tx Plan : Needs practice/supervision
Swallowing Strategies : Needs practice/supervision

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Long Term Goals
SLP Patient, Caregiver Goals : "I want more cookies"

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Cognitive-Communication LTG Grid

	SLP Long Term Goal #1
Goal :	Pt will consume thin liquids via cup/straw and

Speech Therapy

	hard solid foods without evidence of pulmonary compromise
Status :	Initial
	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Short Term Goals
Clinical Swallowing STG Grid

	Goal #1	Goal #2
Type :	Thin liquid by straw	Soft solid consistency
Presentation :	Therapist	Therapist
Total Amount :	Whole meal	Whole meal
Descriptors :	No clinical signs of aspiration	No clinical signs of aspiration
Status :	Initial	Initial
	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT	Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Plan
Planned Treatments : Dysphagia therapy
Frequency : 1-2 times per week
Duration : 1-2 weeks
SLP Intensity Per Day : 30-60 minutes
Post-Acute Discharge Therapy Recommendation ST : Continued intermittent therapy to address functional deficits
Rehabilitation Potential : Good
Plan/Goals Established With Patient/Caregiver : Yes

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

Additional Information
Additional Information SLP : Bedside swallow eval completed. Pt found in bed awake and alert. Pt asking to eat/drink and states that she is hungry. Pt admitted d/t SOB from home with family. Pt bedbound at home, hx of Dementia. Pt HOH and with visual deficits. Pt speaks both Vietnamese and English. Pt able to follow approx 50% simple directives during exam today. Pt found to be oriented x 2 and to year.

Pt presented with ice chips and thin liquid via tsp/cup/straw, puree, softened and hard solids. Pt assisted with feeding d/t visual deficits. Good anterior hyolaryngeal elevation appreicated during swallows with timely swallow initiations. No overt s/o aspiration observed with trials. Pt unable to follow directives for 3 oz water challenge. Pt with slow mastication of hard solid and trace to mild oral residue d/t missing dentition. No difficulty observed with softened solids.

Pt was able to take whole pills one at a time with RN after she had been cleared for a diet. No pocketing of pills noted post swallows.

Speech Therapy

Recommend:
Soft and bite sized diet with Thin liquids, 1:1 assist, pills whole 1 at a time
ST to f/u in tx

D/w nurse Aleja

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

ST Non Timed Modalities 2026
ST Treatment Start Time : 3/31/2026 8:15 PDT
ST Treatment End Time : 3/31/2026 9:09 PDT
ST Eval Oral/Swallow Func Minutes : 15
ST Total Non Timed : 15

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

ST Timed Modalities 2026
ST Timed Min Total : 39
ST Timed Units Total : 3
ST THERAP EXERCISE-15M : 39
ST THERAP EXERCISE Units : 3

Caravella SLP, Crystal M - 3/31/2026 8:58 PDT

History and Physical

DOCUMENT NAME:	History & Physical
SERVICE DATE/TIME:	3/30/2026 13:20 PDT
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Hirabayashi DO,Ellyn (3/30/2026 13:20 PDT)
SIGN INFORMATION:	Techathaveewat DO,Worawith (3/30/2026 13:28 PDT); Hirabayashi DO,Ellyn (3/30/2026 13:20 PDT)

Chief Complaint/Reason for Consultation
Patient brought in by EMS from home with complaints of SOB. Per the family, patient started to complain of SOB this morning. Patient found with a RA sat of <50%. Patient provided with BVM ventilation and then placed on NRB. Patient A/Ox0. EMS unable to obtain demographics on scene. BGL 119

History of Present Illness
90 years old minimally speaking female with PMH of ESRD on HD MWF, on home oxygen 2 to 5 L, hypertension, dementia, bedbound was brought by EMS from home with complaints of shortness of breath started overnight. History was limited patient was on continuous BiPAP and addition to language barrier with hard of hearing therefore history was gathered through family members including patient's daughter and brother who lives with patient. Patient complained of shortness of breath started overnight with mucus production, cough, generalized weakness and family called EMS due to worsening symptom. Reports compliance with dialysis session with last dialysis session on last Friday and that supposed to get dialysis today. Patient was placed on nonrebreather and transition to continuous BiPAP in ED. Labs shows hyper-K potassium 5.9 and 6.9 on ABG therefore receive hyper-K protocol including calcium, albuterol, insulin. ABG showed 7.5/39/153/34. Patient was able to follow simple commands with family and denies chest pain.

Histories
Allergies
Allergies (Active and Proposed Allergies Only)
No Known Allergies (Severity: Unknown severity, Onset: Unknown)

Past Medical History
Active Problems(3)
Hemodialysis
Hepatitis A virus
High cholesterol

Past Surgical History
No surgery history documented.

History and Physical

Past Medical History: ESRD on HD MWF, oxygen dependent home oxygen 2 to 5 L, hypertension, dementia
Past Surgical History: Breast implant
Social History: Denies tobacco, alcohol, and drug use. Lives with brother, mostly bedbound unable to ambulate by herself
Family History: Noncontributory
Allergies: NKDA
Medications: Hydralazine 50 mg 3 times daily, losartan 100 mg daily, metoprolol 50 mg ER, sevelamer 800 mg 3 times daily, amlodipine 10 mg, there is prednisone 10 mg last filled March 5 however her family denies taking prednisone

Objective

Measurements (most recent)

Height (cm)	157.48 cm	Stasiewski RN, Sabryna	03/30 07:08
Height (inches)	62.00 inch	SYSTEM	03/30 07:08
Weight	40.82 kg	Stasiewski RN, Sabryna	03/30 07:08
Weight (lbs)	89.99 lb	SYSTEM	03/30 07:08
Body Mass Index	16.46 kg/m2	Stasiewski RN, Sabryna	03/30 07:08

Vital Signs (24 hrs)	**Last Charted**		**Minimum**		**Maximum**	
Temp	36.9	03/30/2026 07:11	36.9	03/30/2026 07:11	36.9	03/30/2026 07:11
Heart Rate Monitored	91	03/30/2026 12:20	84	03/30/2026 09:00	**H** 104	03/30/2026 07:30
Resp Rate	18	03/30/2026 12:20	12	03/30/2026 07:08	**H** 26	03/30/2026 07:38
SBP	**H** 179	03/30/2026 12:20	130	03/30/2026 07:08	**H** 179	03/30/2026 10:00
DBP	**H** 83	03/30/2026 12:20	65	03/30/2026 08:00	**C** 91	03/30/2026 07:08
MAP	119	03/30/2026 12:20	94	03/30/2026 08:00	119	03/30/2026 09:30
SpO2	95	03/30/2026 12:20	95	03/30/2026 08:00	99	03/30/2026 08:15
O2 Flow Rate	on 6 l/min		on 50 %		on 50 %	
O2 Therapy	Nasal cannula		BiPAP		BiPAP	

Ventilator Settings (Last Within 24hrs)

Ventilator Mode: PC-SIMV (07:37) FIO2: 50 % (11:30)
Ventilator Frequency, Mandatory: 14 br/min (07:37)
Pressure Support: 4 cm H2O (07:37)
Positive End Expiratory Pressure: 4 cm H2O (07:37)

Intake and Output (current encounter)

No Data Available

Physical Exam

Social History

No social history documented.

Medications

Home Medications
No medications documented.

Inpatient Medications
Medications (21) Active
Scheduled: (10)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 10 mg Tab (amLODIPine) 10 mg 1 Tabs, Oral, Daily
cefTRIAXone/D5W (Rocephin) 50 mL, IV Piggyback, q24H Interval
doxycycline/D5W (doxycycline) 100 mg 100 mL, IV Piggyback, q12H
heparin 5000 units/mL injection 1 mL (heparin) 5,000 units 1 mL, SubCutaneous, q12H
hydrALAZINE 50 mg Tab (hydrALAZINE) 50 mg 1 Tabs, Oral, TID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
metoprolol succinate 50 mg XL Tab (Metoprolol Succinate ER) 50 mg 1 Tabs, Oral, qHS
sevelamer carbonate 800 mg Tab (sevelamer) 800 mg 1 Tabs, Oral, TID w/Meals
sodium citrate 4% soln 5 mL (sodium citrate) 200 mg 5 mL, IntraCatheter, On Call
Continuous: (0)
PRN: (11)
acetaminophen 325 mg Tab (Tylenol) 650 mg 2 Tabs, Oral, q4H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q3H
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID
Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 25 gm 50 mL, IV Push, q1H Interval
docusate sodium 100 mg Cap (Colace) 100 mg 1 Caps, Oral, BID
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H
glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q4H

History and Physical

General: Thin appearing minimi speaking elderly female on continuous BiPAP
Mental Status: Alert and oriented to person, place, time. Normal affect.
Head: Normocephalic, atraumatic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact. Anicteric.
Ear, Nose and Throat: Oropharynx clear, moist mucous membranes. Trachea midline.
Neck: No obvious neck masses
Respiratory: Clear to auscultation bilaterally. No wheezing, crackles or rhonchi. Normal respiratory effort.
Cardiovascular: Regular rate and rhythm, no murmurs, gallops or rubs.
Gastrointestinal: Non-tender, non-distended. Normal bowel sounds.
Genitourinary: No suprapubic tenderness.
Neurologic: Cranial nerves II-XII grossly intact. Moves all extremities.
Light touch sensation is grossly intact bilaterally.

Assessment/Plan
Assessment:
Assessment:
Acute hypoxic respiratory failure 2/2 pneumonia versus fluid overload
Dyspnea
Sepsis with RBC, HR 2/2 respiratory source
Troponinemia 2/2 demand ischemia
ESRD on HD MWF
Hyperkalemia 2/2 above
Oxygen dependent reportedly on 2 to 5 L at home
Hypertension
Dementia

Plan:
ICU for continuous BiPAP requirement
CT chest/abdomen/pelvis pending
Acute on chronic hypoxic respite failure secondary to pneumonia in addition to fluid overload
– Continue antibiotics with Rocephin
– Breathing treatment
– Goal SpO2 92%, will titrate down to high flow if tolerable
– Infectious workup pending
Nephrology consulted appreciate recommendation
– S/p hyper-K protocol, stat HD
Troponinemia secondary to demand ischemia, patient denies chest pain
– TTE pending
PRN antihypertensives and antiemetics
Replete electrolytes as needed, routine labs, and supportive care.

Patient has 5 children but main caretaker is brother who lives with her. ICU team discussed with extensively regarding CODE STATUS with patient's daughter and brother at the bedside who had different opinion regarding CODE STATUS. They wish to proceed with full code no shock for now but has plan to discuss among family members.

Consults: ICU, nephrology Dr. Vornicu
Diet: N.p.o., nursing swallow when able
Activity: Early and progressive mobilization
GI prophylaxis: None
DVT prophylaxis: Subcu heparin, SCD

labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q4H
melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H

72 hour Antibiotic History

Completed/Discontinued Antibiotics	Stop Date/Time	Last Administered	First Administered
azithromycin 500 mg, 250 mL, 250 mL/hr, IV Piggyback, Once	03/30/2026 10:10	03/30/2026 10:09	03/30/2026 10:09
cefTRIAXone 1 gm, 50 mL, 100 mL/hr, IV Piggyback, Once	03/30/2026 09:18	03/30/2026 09:10	03/30/2026 09:10

Results
Recent Labs
Blood Gases

Art Na	137 mmol/L (Normal)	03/30/2026 08:43
Art K	6.3 mmol/L (Critical)	03/30/2026 08:43
Art iCa	0.96 mmol/L (Low)	03/30/2026 08:43
pH Arterial	7.54 (High)	03/30/2026 08:43
Art pCO2	39.4 mmHg (Normal)	03/30/2026 08:43
Art pO2	153.0 mmHg (High)	03/30/2026 08:43
HCO3 Art	34.4 mmol/L (High)	03/30/2026 08:43
Arterial pCO2(T)	39.4 mmHg (N/A)	03/30/2026 08:43
Arterial pH(T)	7.54 (N/A)	03/30/2026 08:43
Arterial pO2(T)	153.0 mmHg (N/A)	03/30/2026 08:43
Art Base Excess	10.6 mmol/L (High)	03/30/2026 08:43
Art sO2	99 % (High)	03/30/2026 08:43
Art tHB	10.3 gm/dL (Low)	03/30/2026 08:43

History and Physical

CODE STATUS: Full code, no shock per family

Will discuss plan with attending physician. Awaiting further recommendations.

Ellyn Hirabayashi, DO
Internal Medicine - PGY3
Valley Hospital Medical Center

Discharge Planning:

Attending addendum: I was physically present and involved in the discussion and management of the patient by the resident, and during critical portions of assessment and services furnished.

Worawith Techathaveewat, DO
Internal Medicine

Art Hct	32 % (Low)	03/30/2026 08:43
POC Pt Temp	37.0 DegC (N/A)	03/30/2026 08:43
POC FIO2	50 % (N/A)	03/30/2026 08:43
POC Sample	Arterial (N/A)	03/30/2026 08:43
POC Site	Radial Right (N/A)	03/30/2026 08:43
POC Allen Test	POS (N/A)	03/30/2026 08:43
POC Del Sys	BIPAP (N/A)	03/30/2026 08:43
POC Vent Mode	NO VENT (N/A)	03/30/2026 08:43
POC Set RR	14 br/min (N/A)	03/30/2026 08:43
POC IPAP	8 cm H2O (N/A)	03/30/2026 08:43
POC EPAP	4 cm H2O (N/A)	03/30/2026 08:43
Notified To and RBV	DR DAVIDSON (N/A)	03/30/2026 08:43
Notified By	vhsmurrat (N/A)	03/30/2026 08:43
Notified Note	ALL CV WITH RB (N/A)	03/30/2026 08:43

Cardiac

BNP	2004 pg/mL (High)	03/30/2026 07:31
Troponin TNIH	238.44 ng/L (Critical)	03/30/2026 10:37

General Chemistry

Glucose Level	189 mg/dL (High)	03/30/2026 07:31
POC Glucose	102 mg/dL (Normal)	03/30/2026 09:34
Sodium	139 mmol/L (Normal)	03/30/2026 07:31
Potassium	5.9 mmol/L (High)	03/30/2026 07:31
Chloride	94 mmol/L (Low)	03/30/2026 07:31
CO2	27 mmol/L (Normal)	03/30/2026 07:31
Anion Gap	18 mmol/L (High)	03/30/2026 07:31
BUN	44 mg/dL (High)	03/30/2026 07:31
Creatinine	4.80 mg/dL (High)	03/30/2026 07:31
BUN/Creat Ratio	9 (Normal)	03/30/2026 07:31

History and Physical

Calcium	8.6 mg/dL (Low)	03/30/2026 07:31
Albumin. Level	3.4 gm/dL (Normal)	03/30/2026 11:15
TP	6.3 gm/dL (Normal)	03/30/2026 11:15
A/G Ratio	1.2 (Normal)	03/30/2026 11:15
T Bili	0.4 mg/dL (Normal)	03/30/2026 11:15
D Bili	0.2 mg/dL (Normal)	03/30/2026 11:15
Alk Phos	150 units/L (High)	03/30/2026 11:15
AST	57 units/L (High)	03/30/2026 11:15
ALT	51 units/L (Normal)	03/30/2026 11:15
Estimated Creatinine Clearance	5.02 mL/min ()	03/30/2026 08:46
eGFR Cr	8 mL/min/1.73m2 (N/A)	03/30/2026 07:31
eGFR Pediatric	Not Reported (N/A)	03/30/2026 07:31
Lactic Acid Lvl	1.8 mmol/L (Normal)	03/30/2026 08:48
MAR Glucose Result	102 ()	03/30/2026 09:49

General Hematology

WBC	12.25 x10e3/mcL (High)	03/30/2026 07:31
RBC	4.86 x10e6/mcL (Normal)	03/30/2026 07:31
Hgb	11.2 gm/dL (Normal)	03/30/2026 07:31
Hct	33.7 % (Normal)	03/30/2026 07:31
MCV	69.3 Femtoliters (Low)	03/30/2026 07:31
MCH	23.0 pg (Low)	03/30/2026 07:31
MCHC	33.2 gm/dL (Normal)	03/30/2026 07:31
RDW-SD	44.7 Femtoliters (Normal)	03/30/2026 07:31
RDW-CV	18.6 % (High)	03/30/2026 07:31
Plt	230 x10e3/mcL (Normal)	03/30/2026 07:31
MPV	See comment Femtoliters (N/A)	03/30/2026 07:31
Imm Plt Fract	4.0 % (Normal)	03/30/2026 07:31
NRBC %	0.2 /100 WBC (Normal)	03/30/2026 07:31

History and Physical

NRBC #	0.02 x10e3/mcL (High)	03/30/2026 07:31
Neut % Auto	84.3 % (N/A)	03/30/2026 07:31
Lymph % Auto	10.9 % (N/A)	03/30/2026 07:31
Mono % Auto	3.8 % (N/A)	03/30/2026 07:31
Eos % Auto	0.2 % (N/A)	03/30/2026 07:31
Baso % Auto	0.2 % (N/A)	03/30/2026 07:31
Immature Grans %	0.6 % (Normal)	03/30/2026 07:31
Neut # Auto	10.33 x10e3/mcL (High)	03/30/2026 07:31
Lymph # Auto	1.33 x10e3/mcL (Normal)	03/30/2026 07:31
Mono # Auto	0.47 x10e3/mcL (Normal)	03/30/2026 07:31
Eos # Auto	0.02 x10e3/mcL (Normal)	03/30/2026 07:31
Baso # Auto	0.03 x10e3/mcL (Normal)	03/30/2026 07:31
Immature Grans # Auto	0.07 x10e3/mcL (Normal)	03/30/2026 07:31
RBC Morph	Abnormal (Abnormal)	03/30/2026 07:31
Plt Morphology	Normal (Normal)	03/30/2026 07:31
Polychrom	1+ (Abnormal)	03/30/2026 07:31
Micro	2+ (Abnormal)	03/30/2026 07:31
Plt Est	Adequate (Normal)	03/30/2026 07:31
Schistocytes	1+ (Abnormal)	03/30/2026 07:31
Target Cells (Codocytes)	3+ (Abnormal)	03/30/2026 07:31
Teardrop Cells (Dacrocytes)	1+ (Abnormal)	03/30/2026 07:31

Hepatitis

Hep A IgM	Reactive (Abnormal)	03/30/2026 10:54
Hep B Core IgM	Non-Reactive (Normal)	03/30/2026 11:04
Hepatitis B Surface Antigen	Non-Reactive (Normal)	03/30/2026 11:04
Hep Bs Ab Qn	31.9 milli intl units/mL (High)	03/30/2026 11:04
Hep Bs Ab Qn Interp	Reactive (Abnormal)	03/30/2026 11:04
Hep C Ab	Non-Reactive (Normal)	03/30/2026 10:54

History and Physical

Other Chemistry

Procalcitonin Lvl	1.56 ng/mL (High)	03/30/2026 11:15

Thyroid

T3 Free	2.87 pg/mL (Normal)	03/30/2026 11:15
T4 Free	1.74 ng/dL (High)	03/30/2026 11:15
TSH Ultrasensitive	3.046 mc Intl units/mL (Normal)	03/30/2026 11:15

Abnormal Labs

Blood Gases

Art K	6.3 mmol/L (Critical)	03/30/2026 08:43
Art iCa	0.96 mmol/L (Low)	03/30/2026 08:43
pH Arterial	7.54 (High)	03/30/2026 08:43
Art pO2	153.0 mmHg (High)	03/30/2026 08:43
HCO3 Art	34.4 mmol/L (High)	03/30/2026 08:43
Art Base Excess	10.6 mmol/L (High)	03/30/2026 08:43
Art sO2	99 % (High)	03/30/2026 08:43
Art tHB	10.3 gm/dL (Low)	03/30/2026 08:43
Art Hct	32 % (Low)	03/30/2026 08:43

Cardiac

BNP	2004 pg/mL (High)	03/30/2026 07:31
Troponin TNIH	238.44 ng/L (Critical)	03/30/2026 10:37

General Chemistry

Glucose Level	189 mg/dL (High)	03/30/2026 07:31
Potassium	5.9 mmol/L (High)	03/30/2026 07:31
Chloride	94 mmol/L (Low)	03/30/2026 07:31
Anion Gap	18 mmol/L (High)	03/30/2026 07:31
BUN	44 mg/dL (High)	03/30/2026 07:31
Creatinine	4.80 mg/dL (High)	03/30/2026 07:31

History and Physical

Calcium	8.6 mg/dL (Low)	03/30/2026 07:31
Alk Phos	150 units/L (High)	03/30/2026 11:15
AST	57 units/L (High)	03/30/2026 11:15

General Hematology

WBC	12.25 x10e3/mcL (High)	03/30/2026 07:31
MCV	69.3 Femtoliters (Low)	03/30/2026 07:31
MCH	23.0 pg (Low)	03/30/2026 07:31
RDW-CV	18.6 % (High)	03/30/2026 07:31
MPV	See comment Femtoliters (N/A)	03/30/2026 07:31
NRBC #	0.02 x10e3/mcL (High)	03/30/2026 07:31
Neut # Auto	10.33 x10e3/mcL (High)	03/30/2026 07:31
RBC Morph	Abnormal (Abnormal)	03/30/2026 07:31
Polychrom	1+ (Abnormal)	03/30/2026 07:31
Micro	2+ (Abnormal)	03/30/2026 07:31
Schistocytes	1+ (Abnormal)	03/30/2026 07:31
Target Cells (Codocytes)	3+ (Abnormal)	03/30/2026 07:31
Teardrop Cells (Dacrocytes)	1+ (Abnormal)	03/30/2026 07:31

Hepatitis

Hep A IgM	Reactive (Abnormal)	03/30/2026 10:54
Hep Bs Ab Qn	31.9 milli intl units/mL (High)	03/30/2026 11:04
Hep Bs Ab Qn Interp	Reactive (Abnormal)	03/30/2026 11:04

Other Chemistry

Procalcitonin Lvl	1.56 ng/mL (High)	03/30/2026 11:15

Thyroid

T4 Free	1.74 ng/dL (High)	03/30/2026 11:15

History and Physical

Microbiology

Culture Blood ARD		**In-Lab**	
Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	03/30/2026 08:41

Culture Blood ARD		**In-Lab**	
Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	03/30/2026 08:41

Blood Gases (Last Within 24hrs)
pH Arterial: 7.54 High (08:43)
Art pO2: 153 mmHg High (08:43)
Art pCO2: 39.4 mmHg (08:43)
HCO3 Art: 34.4 mmol/L High (08:43)
Art sO2: 99 % High (08:43)

Electronically Signed By: Hirabayashi, Ellyn DO
On: 03.30.2026 13:20 PDT

Electronically Signed On: 03.30.2026 13:28 PDT
Techathaveewat, Worawith DO

DOCUMENT NAME:	History & Physical
SERVICE DATE/TIME:	3/30/2026 11:56 PDT
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Cho DO,Claire Sungkyung (3/30/2026 11:57 PDT)
SIGN INFORMATION:	Delmundo DO,Lloyd (3/30/2026 16:20 PDT); Cho DO,Claire Sungkyung (3/30/2026 11:57 PDT)

Chief Complaint/Reason for Consultation
Patient brought in by EMS from home with complaints of SOB. Per the family, patient started to complain of SOB this morning. Patient found with a RA sat of <50%. Patient provided with BVM ventilation and then placed on NRB. Patient A/Ox0. EMS unable to obtain demographics on scene. BGL 119

History of Present Illness
HPI: 90 years old minimally speaking female with PMH of ESRD on HD MWF, on home oxygen 2 to 5 L, hypertension, dementia, bedbound was brought by EMS from home with complaints of shortness of breath started overnight. History was limited patient was on continuous BiPAP and addition to language barrier with hard of hearing therefore history was gathered through family members including patient's daughter and

Medications
Home Medications
No medications documented.

Inpatient Medications
Medications (21) Active
Scheduled: (10)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 10 mg Tab (amLODIPine) 10 mg 1 Tabs, Oral, Daily
cefTRIAXone/D5W (Rocephin) 50 mL, IV

History and Physical

brother who lives with patient. Patient complained of shortness of breath started overnight with mucus production, cough, generalized weakness and family called EMS due to worsening symptom. Reports compliance with dialysis session with last dialysis session on last Friday and that supposed to get dialysis today. Patient was placed on nonrebreather and transition to continuous BiPAP in ED. Labs shows hyper-K potassium 5.9 and 6.9 on ABG therefore receive hyper-K protocol including calcium, albuterol, insulin. ABG showed 7.5/39/153/34. Patient was able to follow simple commands with family and denies chest pain.

Past Medical History: ESRD on HD MWF, oxygen dependent home oxygen 2 to 5 L, hypertension, dementia
Past Surgical History: Breast implant
Social History: Denies tobacco, alcohol, and drug use. Lives with brother, mostly bedbound unable to ambulate by herself
Family History: Noncontributory
Allergies: NKDA
Medications: Hydralazine 50 mg 3 times daily, losartan 100 mg daily, metoprolol 50 mg ER, sevelamer 800 mg 3 times daily, amlodipine 10 mg, there is prednisone 10 mg last filled March 5 however her family denies taking prednisone

Review of Systems
Review of systems negative except as otherwise described in HPI

Objective

Measurements (most recent)

Height (cm)	157.48 cm	Stasiewski RN, Sabryna	03/30 07:08
Height (inches)	62.00 inch	SYSTEM	03/30 07:08
Weight	40.82 kg	Stasiewski RN, Sabryna	03/30 07:08
Weight (lbs)	89.99 lb	SYSTEM	03/30 07:08
Body Mass Index	16.46 kg/m2	Stasiewski RN, Sabryna	03/30 07:08

Vital Signs (24 hrs)	Last Charted		Minimum		Maximum	
Temp	36.9	03/30/2026 07:11	36.9	03/30/2026 07:11	36.9	03/30/2026 07:11
Heart Rate Monitored	90	03/30/2026 11:30	84	03/30/2026 09:00	**H** 104	03/30/2026 07:30
Resp Rate	18	03/30/2026 11:30	12	03/30/2026 07:08	**H** 26	03/30/2026 07:38
SBP	**H** 148	03/30/2026 11:30	130	03/30/2026 07:08	**H** 179	03/30/2026 10:00
DBP	70	03/30/2026 11:30	65	03/30/2026 08:00	**C** 91	03/30/2026 07:08
MAP	100	03/30/2026 11:30	94	03/30/2026 08:00	119	03/30/2026 09:30
SpO2	99	03/30/2026 11:30	95	03/30/2026 08:00	99	03/30/2026 08:15
O2 Flow Rate	on 50 %		on 50 %		on 50 %	
O2 Therapy	BiPAP		BiPAP		BiPAP	

Intake and Output (current encounter)

Piggyback, q24H Interval
doxycycline/D5W (doxycycline) 100 mg 100 mL, IV Piggyback, q12H
heparin 5000 units/mL injection 1 mL (heparin) 5,000 units 1 mL, SubCutaneous, q12H
hydrALAZINE 50 mg Tab (hydrALAZINE) 50 mg 1 Tabs, Oral, TID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
metoprolol succinate 50 mg XL Tab (Metoprolol Succinate ER) 50 mg 1 Tabs, Oral, qHS
sevelamer carbonate 800 mg Tab (sevelamer) 800 mg 1 Tabs, Oral, TID w/Meals
sodium citrate 4% soln 5 mL (sodium citrate) 200 mg 5 mL, IntraCatheter, On Call
Continuous: (0)
PRN: (11)
acetaminophen 325 mg Tab (Tylenol) 650 mg 2 Tabs, Oral, q4H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q3H
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID
Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 25 gm 50 mL, IV Push, q1H Interval
docusate sodium 100 mg Cap (Colace) 100 mg 1 Caps, Oral, BID
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H
glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q4H
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q4H
melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H

72 hour Antibiotic History

Completed/Discontinued Antibiotics	Stop Date/Time	Last Administered	First Administered
azithromycin 500 mg, 250 mL, 250 mL/hr, IV Piggyback, Once	03/30/2026 10:10	03/30/2026 10:09	03/30/2026 10:09

History and Physical

No Data Available

cefTRIAXone 1 gm, 50 mL, 100 mL/hr, IV Piggyback, Once	03/30/2026 09:18	03/30/2026 09:10	03/30/2026 09:10

Physical Exam
General: Thin appearing minimi speaking elderly female on continuous BiPAP
Mental Status: Alert and oriented to person, place, time. Normal affect.
Head: Normocephalic, atraumatic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact. Anicteric.
Ear, Nose and Throat: Oropharynx clear, moist mucous membranes. Trachea midline.
Neck: No obvious neck masses
Respiratory: Clear to auscultation bilaterally. No wheezing, crackles or rhonchi. Normal respiratory effort.
Cardiovascular: Regular rate and rhythm, no murmurs, gallops or rubs.
Gastrointestinal: Non-tender, non-distended. Normal bowel sounds.
Genitourinary: No suprapubic tenderness.
Neurologic: Cranial nerves II-XII grossly intact. Moves all extremities.
Light touch sensation is grossly intact bilaterally.

Assessment/Plan

Assessment:
Acute hypoxic respiratory failure 2/2 pneumonia versus fluid overload
Dyspnea
Sepsis with RBC, HR 2/2 respiratory source
Troponinemia 2/2 demand ischemia
Hyper-K
ESRD on HD MWF
Oxygen dependent reportedly on 2 to 5 L at home
Hypertension
Dementia

Plan:
ICU for continuous BiPAP requirement
Acute on chronic hypoxic respite failure secondary to pneumonia in addition to fluid overload
– Continue antibiotics with Rocephin
–Breathing treatment
– Goal SpO2 92%, will titrate down to high flow if tolerable
–CT thorax/abdomen/pelvis for sepsis workup, infectious workup pending
Nephrology consulted appreciate recommendation
– S/p hyper-K protocol, stat HD
Troponinemia secondary to demand ischemia, patient denies chest pain
– Echocardiogram
PRN antihypertensives and antiemetics
Replete electrolytes as needed, routine labs, and supportive care.

Consults: Nephrology Dr. Vornicu
Diet: N.p.o., nursing swallow when able
Activity: Early and progressive mobilization
GI prophylaxis: None
DVT prophylaxis: Subcu heparin, SCD
CODE STATUS: Full code, no shock per family

Patient has 5 children but main caretaker is brother who lives with her. Discussed with

Results
Recent Labs
Blood Gases

Art Na	137 mmol/L (Normal)	03/30/2026 08:43
Art K	6.3 mmol/L (Critical)	03/30/2026 08:43
Art iCa	0.96 mmol/L (Low)	03/30/2026 08:43
pH Arterial	7.54 (High)	03/30/2026 08:43
Art pCO2	39.4 mmHg (Normal)	03/30/2026 08:43
Art pO2	153.0 mmHg (High)	03/30/2026 08:43
HCO3 Art	34.4 mmol/L (High)	03/30/2026 08:43
Arterial pCO2(T)	39.4 mmHg (N/A)	03/30/2026 08:43
Arterial pH(T)	7.54 (N/A)	03/30/2026 08:43
Arterial pO2(T)	153.0 mmHg (N/A)	03/30/2026 08:43
Art Base Excess	10.6 mmol/L (High)	03/30/2026 08:43
Art sO2	99 % (High)	03/30/2026 08:43
Art tHB	10.3 gm/dL (Low)	03/30/2026 08:43
Art Hct	32 % (Low)	03/30/2026 08:43
POC Pt Temp	37.0 DegC (N/A)	03/30/2026 08:43
POC FIO2	50 % (N/A)	03/30/2026 08:43
POC Sample	Arterial (N/A)	03/30/2026 08:43
POC Site	Radial Right (N/A)	03/30/2026 08:43
POC Allen Test	POS (N/A)	03/30/2026 08:43
POC Del Sys	BIPAP (N/A)	03/30/2026 08:43
POC Vent Mode	NO VENT (N/A)	03/30/2026 08:43

History and Physical

extensively regarding CODE STATUS with patient's daughter and brotherat the bedside who had different opinion regarding CODE STATUS. They wish to proceed with full code no shock for now but has plan to discuss among family members.

Will discuss case with attending physician. Further recommendations to follow.

Claire Cho, DO
Internal Medicine, PGY-3
Please contact "internal medicine, resident" on YouCallMD.

I discussed with resident/fellow at length. I was present for critical and key services and am participating in patient management, in combination with the resident or fellow. Agree with assessment and plan

on low flow o2
down to 6 liters
off bipap
control BP
is about to get HD
needs HD
empiric abx
If stable after HD, we will downgrade, as this looks like volume with effusions
Follow up imaging
code status is full
Please see above
ICU status for now while getting HD

DW staff

L. Del Mundo, DO
Pulmonary/Critical Care

POC Set RR	14 br/min (N/A)	03/30/2026 08:43
POC IPAP	8 cm H2O (N/A)	03/30/2026 08:43
POC EPAP	4 cm H2O (N/A)	03/30/2026 08:43
Notified To and RBV	DR DAVIDSON (N/A)	03/30/2026 08:43
Notified By	vhsmurrat (N/A)	03/30/2026 08:43
Notified Note	ALL CV WITH RB (N/A)	03/30/2026 08:43

Cardiac

BNP	2004 pg/mL (High)	03/30/2026 07:31
Troponin TNIH	238.44 ng/L (Critical)	03/30/2026 10:37

General Chemistry

Glucose Level	189 mg/dL (High)	03/30/2026 07:31
POC Glucose	102 mg/dL (Normal)	03/30/2026 09:34
Sodium	139 mmol/L (Normal)	03/30/2026 07:31
Potassium	5.9 mmol/L (High)	03/30/2026 07:31
Chloride	94 mmol/L (Low)	03/30/2026 07:31
CO2	27 mmol/L (Normal)	03/30/2026 07:31
Anion Gap	18 mmol/L (High)	03/30/2026 07:31
BUN	44 mg/dL (High)	03/30/2026 07:31
Creatinine	4.80 mg/dL (High)	03/30/2026 07:31
BUN/Creat Ratio	9 (Normal)	03/30/2026 07:31
Calcium	8.6 mg/dL (Low)	03/30/2026 07:31
Albumin. Level	3.4 gm/dL (Normal)	03/30/2026 11:15
TP	6.3 gm/dL (Normal)	03/30/2026 11:15
A/G Ratio	1.2 (Normal)	03/30/2026 11:15
T Bili	0.4 mg/dL (Normal)	03/30/2026 11:15
D Bili	0.2 mg/dL (Normal)	03/30/2026 11:15
Alk Phos	150 units/L (High)	03/30/2026 11:15
AST	57 units/L (High)	03/30/2026 11:15

History and Physical

ALT	51 units/L (Normal)	03/30/2026 11:15
Estimated Creatinine Clearance	5.02 mL/min ()	03/30/2026 08:46
eGFR Cr	8 mL/min/1.73m2 (N/A)	03/30/2026 07:31
eGFR Pediatric	Not Reported (N/A)	03/30/2026 07:31
Lactic Acid Lvl	1.8 mmol/L (Normal)	03/30/2026 08:48
MAR Glucose Result	102 ()	03/30/2026 09:49

General Hematology

WBC	12.25 x10e3/mcL (High)	03/30/2026 07:31
RBC	4.86 x10e6/mcL (Normal)	03/30/2026 07:31
Hgb	11.2 gm/dL (Normal)	03/30/2026 07:31
Hct	33.7 % (Normal)	03/30/2026 07:31
MCV	69.3 Femtoliters (Low)	03/30/2026 07:31
MCH	23.0 pg (Low)	03/30/2026 07:31
MCHC	33.2 gm/dL (Normal)	03/30/2026 07:31
RDW-SD	44.7 Femtoliters (Normal)	03/30/2026 07:31
RDW-CV	18.6 % (High)	03/30/2026 07:31
Plt	230 x10e3/mcL (Normal)	03/30/2026 07:31
MPV	See comment Femtoliters (N/A)	03/30/2026 07:31
Imm Plt Fract	4.0 % (Normal)	03/30/2026 07:31
NRBC %	0.2 /100 WBC (Normal)	03/30/2026 07:31
NRBC #	0.02 x10e3/mcL (High)	03/30/2026 07:31
Neut % Auto	84.3 % (N/A)	03/30/2026 07:31
Lymph % Auto	10.9 % (N/A)	03/30/2026 07:31
Mono % Auto	3.8 % (N/A)	03/30/2026 07:31
Eos % Auto	0.2 % (N/A)	03/30/2026 07:31
Baso % Auto	0.2 % (N/A)	03/30/2026 07:31
Immature Grans %	0.6 % (Normal)	03/30/2026 07:31
Neut # Auto	10.33 x10e3/mcL (High)	03/30/2026 07:31

History and Physical

Lymph # Auto	1.33 x10e3/mcL (Normal)	03/30/2026 07:31
Mono # Auto	0.47 x10e3/mcL (Normal)	03/30/2026 07:31
Eos # Auto	0.02 x10e3/mcL (Normal)	03/30/2026 07:31
Baso # Auto	0.03 x10e3/mcL (Normal)	03/30/2026 07:31
Immature Grans # Auto	0.07 x10e3/mcL (Normal)	03/30/2026 07:31

Thyroid

T3 Free	2.87 pg/mL (Normal)	03/30/2026 11:15

Abnormal Labs

Blood Gases

Art K	6.3 mmol/L (Critical)	03/30/2026 08:43
Art iCa	0.96 mmol/L (Low)	03/30/2026 08:43
pH Arterial	7.54 (High)	03/30/2026 08:43
Art pO2	153.0 mmHg (High)	03/30/2026 08:43
HCO3 Art	34.4 mmol/L (High)	03/30/2026 08:43
Art Base Excess	10.6 mmol/L (High)	03/30/2026 08:43
Art sO2	99 % (High)	03/30/2026 08:43
Art tHB	10.3 gm/dL (Low)	03/30/2026 08:43
Art Hct	32 % (Low)	03/30/2026 08:43

Cardiac

BNP	2004 pg/mL (High)	03/30/2026 07:31
Troponin TNIH	238.44 ng/L (Critical)	03/30/2026 10:37

General Chemistry

Glucose Level	189 mg/dL (High)	03/30/2026 07:31
Potassium	5.9 mmol/L (High)	03/30/2026 07:31
Chloride	94 mmol/L (Low)	03/30/2026 07:31
Anion Gap	18 mmol/L (High)	03/30/2026 07:31
BUN	44 mg/dL (High)	03/30/2026 07:31

History and Physical

Creatinine	4.80 mg/dL (High)	03/30/2026 07:31
Calcium	8.6 mg/dL (Low)	03/30/2026 07:31
Alk Phos	150 units/L (High)	03/30/2026 11:15
AST	57 units/L (High)	03/30/2026 11:15

General Hematology

WBC	12.25 x10e3/mcL (High)	03/30/2026 07:31
MCV	69.3 Femtoliters (Low)	03/30/2026 07:31
MCH	23.0 pg (Low)	03/30/2026 07:31
RDW-CV	18.6 % (High)	03/30/2026 07:31
MPV	See comment Femtoliters (N/A)	03/30/2026 07:31
NRBC #	0.02 x10e3/mcL (High)	03/30/2026 07:31
Neut # Auto	10.33 x10e3/mcL (High)	03/30/2026 07:31

CBC, BMP, Coagulation Trend (last 4 resulted)

WBC	**12.25 H** 03/30/2026 07:31			
Hgb	**11.2** 03/30/2026 07:31			
Hct	**33.7** 03/30/2026 07:31			
Baso # Auto	**0.03** 03/30/2026 07:31			
Baso % Auto	**0.2** 03/30/2026 07:31			
Eos # Auto	**0.02** 03/30/2026 07:31			
Eos % Auto	**0.2** 03/30/2026 07:31			

History and Physical

Immature Grans %	**0.6** 03/30/2026 07:31			
Immature Grans # Auto	**0.07** 03/30/2026 07:31			
Lymph # Auto	**1.33** 03/30/2026 07:31			
Lymph % Auto	**10.9** 03/30/2026 07:31			
Mono # Auto	**0.47** 03/30/2026 07:31			
Mono % Auto	**3.8** 03/30/2026 07:31			
Neut # Auto	**10.33 H** 03/30/2026 07:31			
Neut % Auto	**84.3** 03/30/2026 07:31			
Plt	**230** 03/30/2026 07:31			
Glucose Level	**189 H** 03/30/2026 07:31			
Sodium	**139** 03/30/2026 07:31			
Potassium	**5.9 H** 03/30/2026 07:31			
Chloride	**94 L** 03/30/2026 07:31			
CO2	**27** 03/30/2026 07:31			
Creatinine	**4.80 H** 03/30/2026 07:31			

History and Physical

BUN	**44 H** 03/30/2026 07:31			
BUN/Creat Ratio	**9** 03/30/2026 07:31			
Calcium	**8.6 L** 03/30/2026 07:31			
Albumin. Level	**3.4** 03/30/2026 11:15			
T Bili	**0.4** 03/30/2026 11:15			
D Bili	**0.2** 03/30/2026 11:15			
Alk Phos	**150 H** 03/30/2026 11:15			
AST	**57 H** 03/30/2026 11:15			
ALT	**51** 03/30/2026 11:15			

Blood Glucose Trend

Glucose Level: 189 mg/dL High (03/30/26 07:31:00)

POC Glucose: 102 mg/dL (03/30/26 09:34:00)

Electronically Signed By: Cho, Claire DO
On: 03.30.2026 11:57 PDT

Electronically Signed On: 03.30.2026 16:20 PDT
Delmundo, Lloyd DO

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Consultation

DOCUMENT NAME: Consult - Infectious Disease
SERVICE DATE/TIME: 3/31/2026 14:56 PDT
RESULT STATUS: Auth (Verified)
PERFORM INFORMATION: Joshi MD,Dhiresh R (3/31/2026 15:04 PDT)
SIGN INFORMATION: Joshi MD,Dhiresh R (3/31/2026 15:04 PDT)

Chief Complaint/Reason for Consultation

Patient brought in by EMS from home with complaints of SOB. Per the family, patient started to complain of SOB this morning. Patient found with a RA sat of <50%. Patient provided with BVM ventilation and then placed on NRB. Patient A/Ox0. EMS unable to obtain demographics on scene. BGL 119
Patient placed on BiPAP, no high fever chills, no headache chest pain nausea vomiting diarrhea reported, no urinary symptoms

History of Present Illness

90 years old minimally speaking female with PMH of ESRD on HD MWF, on home oxygen 2 to 5 L, hypertension, dementia, bedbound was brought by EMS from home with complaints of shortness of breath started overnight. History was limited patient was on continuous BiPAP and addition to language barrier with hard of hearing therefore history was gathered through family members including patient's daughter and brother who lives with patient. Patient complained of shortness of breath started overnight with mucus production, cough, generalized weakness and family called EMS due to worsening symptom. Reports compliance with dialysis session with last dialysis session on last Friday and that supposed to get dialysis today. Patient was placed on nonrebreather and transition to continuous BiPAP in ED. Labs shows hyper-K potassium 5.9 and 6.9 on ABG therefore receive hyper-K protocol including calcium, albuterol, insulin. ABG showed 7.5/39/153/34. Patient was able to follow simple commands with family and denies chest pain.

Objective

Vital Signs (24 hrs)	Last Charted		Minimum		Maximum	
Temp	37.2	03/31/2026 12:00	36.9	03/30/2026 22:45	37.1	03/30/2026 19:57
Apical Heart Rate	87	03/31/2026 07:12	87	03/31/2026 07:12	98	03/30/2026 18:54
Resp Rate	**C** 32	03/31/2026 14:06	14	03/31/2026 08:59	**C** 52	03/31/2026 03:45
SBP	131	03/31/2026 14:06	126	03/31/2026 12:06	**C** 198	03/31/2026 07:00
DBP	60	03/31/2026 14:06	60	03/31/2026 14:06	**C** 99	03/31/2026 12:06
MAP	86	03/31/2026 14:06	86	03/31/2026 14:06	129	03/30/2026 18:00
SpO2	98	03/31/2026 14:06	**C** 86	03/31/2026 04:15	100	03/30/2026 21:29
O2 Flow Rate					on 4 l/min	
O2 Therapy					Nasal cannula	

Ventilator Settings (Last Within 24hrs)
Ventilator Mode: SPN-BiPAP (02:05) FIO2: 30 % (00:00)

Histories

Allergies

Allergies (Active and Proposed Allergies Only)
No Known Allergies (Severity: Unknown severity, Onset: Unknown)

Past Medical History

Active Problems(3)
Hemodialysis
Hepatitis A virus
High cholesterol

Past Surgical History

No surgery history documented.

Social History

No social history documented.

Family History

No Family History documented.

Medications

Home Medications

amLODIPine (amLODIPine 10 mg oral tablet)
hydrALAZINE (hydrALAZINE 50 mg oral tablet)
losartan (losartan 100 mg oral tablet)
metoprolol (metoprolol succinate 50 mg oral tablet, extended release)
minoxidil (minoxidil 2.5 mg oral tablet)
multivitamin (Rena-Vite oral tablet)
ondansetron (ondansetron 4 mg oral tablet)
sevelamer (sevelamer 800 mg oral tablet)
zolpidem (zolpidem 5 mg oral tablet)

Consultation

Inspiratory Pressure: 5 cm H2O (02:05)
Expiratory Pressure: 8 cm H2O (02:05)

Physical Exam
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact. Anicteric.
Ear, Nose and Throat: Oropharynx clear, moist mucous membranes. Trachea midline.
Neck: No obvious neck masses
Respiratory: Clear to auscultation bilaterally. No wheezing, crackles or rhonchi. Normal respiratory effort.
Cardiovascular: Regular rate and rhythm, no murmurs, gallops or rubs.
Gastrointestinal: Non-tender, non-distended. Normal bowel sounds.
Genitourinary: No suprapubic tenderness.
Neurologic: Cranial nerves II-XII grossly intact. Moves all extremities.

Assessment/Plan
Diagnoses
Acute on chronic hypoxic respiratory failure (J96.21)
Acute pulmonary edema (J81.0)
Elevated troponin (R79.89)
End-stage renal disease (N18.6)
Hyperkalemia (E87.5)
Hyperkalemia (E87.5)
Pneumonia (J18.9)
Sepsis (A41.9)
1. Hypoxemia (R09.02)
Leukocytosis, resolving
Morganella bacteremia
Bilateral pleural effusion
Hepatitis A, IgM positive

Continue ceftriaxone pending sensitivity of Morganella
Adjust per creatinine clearance
Aspiration precautions
Aggressive pulmonary hygiene
Monitor LFTs
Repeat blood cultures
Echo
Patient has permacath
Panculture if SIRS 2/4
Notify ID if fevers > 100.4
Monitor labs
Aspiration precaution
Other management per IM/specialists
All current scans/imaging reviewed
Follow cultures
D/W RN

Thank you for the consult, will follow patient with you

Inpatient Medications
Medications (22) Active
Scheduled: (11)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 10 mg Tab (amLODIPine) 10 mg 1 Tabs, Oral, Daily
cefTRIAXone/D5W (Rocephin) 50 mL, IV Piggyback, q24H Interval
doxycycline/D5W (doxycycline) 100 mg 100 mL, IV Piggyback, q12H
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
heparin 5000 units/mL injection 1 mL (heparin) 5,000 units 1 mL, SubCutaneous, q12H
hydrALAZINE 50 mg Tab (hydrALAZINE) 50 mg 1 Tabs, Oral, TID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
losartan 100 mg Tab (losartan) 100 mg 1 Tabs, Oral, Daily
metoprolol succinate 50 mg XL Tab (Metoprolol Succinate ER) 50 mg 1 Tabs, Oral, qHS
sevelamer carbonate 800 mg Tab (sevelamer) 800 mg 1 Tabs, Oral, TID w/Meals
Continuous: (0)
PRN: (11)
acetaminophen 325 mg Tab (Tylenol) 650 mg 2 Tabs, Oral, q4H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q3H
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID
Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 25 gm 50 mL, IV Push, q1H Interval
docusate sodium 100 mg Cap (Colace) 100 mg 1 Caps, Oral, BID
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H
glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q2H Interval
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q2H Interval
melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H

Consultation

72 hour Antibiotic History

Active Antibiotics	Day of Therapy:	Last Administered	First Administered
cefTRIAXone 2 gm, 50 mL, 100 mL/hr, IV Piggyback, q24H Interval	1	03/31/2026 10:13	03/31/2026 10:13
doxycycline 100 mg, 100 mL, 100 mL/hr, IV Piggyback, q12H	2	03/31/2026 08:50	03/30/2026 21:13

Completed/Discontinued Antibiotics	Stop Date/Time	Last Administered	First Administered
azithromycin 500 mg, 250 mL, 250 mL/hr, IV Piggyback, Once	03/30/2026 10:10	03/30/2026 10:09	03/30/2026 10:09
cefTRIAXone 1 gm, 50 mL, 100 mL/hr, IV Piggyback, Once	03/30/2026 09:18	03/30/2026 09:10	03/30/2026 09:10

Results

Recent Labs

Blood Gases

Art Na	137 mmol/L (Normal)	03/30/2026 08:43
Art K	6.3 mmol/L (Critical)	03/30/2026 08:43
Art iCa	0.96 mmol/L (Low)	03/30/2026 08:43
pH Arterial	7.54 (High)	03/30/2026 08:43
Art pCO2	39.4 mmHg (Normal)	03/30/2026 08:43
Art pO2	153.0 mmHg (High)	03/30/2026 08:43
HCO3 Art	34.4 mmol/L (High)	03/30/2026 08:43
Arterial pCO2(T)	39.4 mmHg (N/A)	03/30/2026 08:43
Arterial pH(T)	7.54 (N/A)	03/30/2026 08:43
Arterial pO2(T)	153.0 mmHg (N/A)	03/30/2026 08:43
Art Base Excess	10.6 mmol/L (High)	03/30/2026 08:43
Art sO2	99 % (High)	03/30/2026 08:43

Consultation

Art tHB	10.3 gm/dL (Low)	03/30/2026 08:43
Art Hct	32 % (Low)	03/30/2026 08:43
POC Pt Temp	37.0 DegC (N/A)	03/30/2026 08:43
POC FIO2	50 % (N/A)	03/30/2026 08:43
POC Sample	Arterial (N/A)	03/30/2026 08:43
POC Site	Radial Right (N/A)	03/30/2026 08:43
POC Allen Test	POS (N/A)	03/30/2026 08:43
POC Del Sys	BIPAP (N/A)	03/30/2026 08:43
POC Vent Mode	NO VENT (N/A)	03/30/2026 08:43
POC Set RR	14 br/min (N/A)	03/30/2026 08:43
POC IPAP	8 cm H2O (N/A)	03/30/2026 08:43
POC EPAP	4 cm H2O (N/A)	03/30/2026 08:43
Notified To and RBV	DR DAVIDSON (N/A)	03/30/2026 08:43
Notified By	vhsmurrat (N/A)	03/30/2026 08:43
Notified Note	ALL CV WITH RB (N/A)	03/30/2026 08:43

Cardiac

BNP	2004 pg/mL (High)	03/30/2026 07:31
Troponin TNIH	328.77 ng/L (Critical)	03/30/2026 22:34

General Chemistry

Glucose Level	72 mg/dL (Low)	03/31/2026 02:04
POC Glucose	75 mg/dL (Normal)	03/31/2026 07:11
Sodium	140 mmol/L (Normal)	03/31/2026 02:04
Potassium	4.2 mmol/L (Normal)	03/31/2026 02:04
Chloride	100 mmol/L (Normal)	03/31/2026 02:04
CO2	31 mmol/L (Normal)	03/31/2026 02:04
Anion Gap	9 mmol/L (Normal)	03/31/2026 02:04
BUN	17 mg/dL (Normal)	03/31/2026 02:04
Creatinine	2.19 mg/dL (High)	03/31/2026 02:04

Consultation

BUN/Creat Ratio	8 (Normal)	03/31/2026 02:04
Calcium	9.0 mg/dL (Normal)	03/31/2026 02:04
Albumin. Level	3.4 gm/dL (Normal)	03/31/2026 02:04
TP	6.4 gm/dL (Normal)	03/30/2026 22:34
A/G Ratio	1.3 (Normal)	03/30/2026 22:34
T Bili	0.8 mg/dL (Normal)	03/30/2026 22:34
D Bili	0.2 mg/dL (Normal)	03/30/2026 22:34
Alk Phos	125 units/L (High)	03/30/2026 22:34
AST	97 units/L (High)	03/30/2026 22:34
ALT	49 units/L (Normal)	03/30/2026 22:34
Mg Lvl	1.9 mg/dL (Normal)	03/31/2026 02:04
Phosphorus	5.6 mg/dL (High)	03/31/2026 02:04
Estimated Creatinine Clearance	10.11 mL/min ()	03/31/2026 02:58
eGFR Cr	See Comment mL/min/1.73m2 (N/A)	03/31/2026 02:04
eGFR Pediatric	Not Reported (N/A)	03/31/2026 02:04
Lactic Acid Lvl	1.8 mmol/L (Normal)	03/30/2026 08:48
Hgb A1C	4.3 % (Normal)	03/30/2026 11:15
Ferritin Lvl	1244.0 ng/mL (High)	03/31/2026 13:21
MAR Glucose Result	102 ()	03/30/2026 09:49

General Hematology

WBC	5.87 x10e3/mcL (Normal)	03/31/2026 02:04
RBC	3.65 x10e6/mcL (Normal)	03/31/2026 02:04
Hgb	8.4 gm/dL (Low)	03/31/2026 02:04
Hct	25.6 % (Low)	03/31/2026 02:04
MCV	70.1 Femtoliters (Low)	03/31/2026 02:04
MCH	23.0 pg (Low)	03/31/2026 02:04
MCHC	32.8 gm/dL (Normal)	03/31/2026 02:04

Consultation

RDW-SD	45.2 Femtoliters (Normal)	03/31/2026 02:04
RDW-CV	18.6 % (High)	03/31/2026 02:04
Plt	141 x10e3/mcL (Normal)	03/31/2026 02:04
MPV	9.6 Femtoliters (Normal)	03/31/2026 02:04
Imm Plt Fract	3.7 % (Normal)	03/31/2026 02:04
NRBC %	0.2 /100 WBC (Normal)	03/30/2026 07:31
NRBC #	0.02 x10e3/mcL (High)	03/30/2026 07:31
Neut % Auto	80.7 % (N/A)	03/31/2026 02:04
Lymph % Auto	12.4 % (N/A)	03/31/2026 02:04
Mono % Auto	3.7 % (N/A)	03/31/2026 02:04
Eos % Auto	2.0 % (N/A)	03/31/2026 02:04
Baso % Auto	0.7 % (N/A)	03/31/2026 02:04
Immature Grans %	0.5 % (Normal)	03/31/2026 02:04
Neut # Auto	4.73 x10e3/mcL (Normal)	03/31/2026 02:04
Lymph # Auto	0.73 x10e3/mcL (Low)	03/31/2026 02:04
Mono # Auto	0.22 x10e3/mcL (Normal)	03/31/2026 02:04
Eos # Auto	0.12 x10e3/mcL (Normal)	03/31/2026 02:04
Baso # Auto	0.04 x10e3/mcL (Normal)	03/31/2026 02:04
Immature Grans # Auto	0.03 x10e3/mcL (Normal)	03/31/2026 02:04
RBC Morph	Abnormal (Abnormal)	03/30/2026 07:31
Plt Morphology	Normal (Normal)	03/30/2026 07:31
Polychrom	1+ (Abnormal)	03/30/2026 07:31
Micro	2+ (Abnormal)	03/30/2026 07:31
Plt Est	Adequate (Normal)	03/30/2026 07:31
Schistocytes	1+ (Abnormal)	03/30/2026 07:31
Target Cells (Codocytes)	3+ (Abnormal)	03/30/2026 07:31
Teardrop Cells (Dacrocytes)	1+ (Abnormal)	03/30/2026 07:31

Hepatitis

Consultation

Hep A IgM	Reactive (Abnormal)	03/30/2026 10:54
Hep B Core IgM	Non-Reactive (Normal)	03/30/2026 11:04
Hepatitis B Surface Antigen	Non-Reactive (Normal)	03/30/2026 11:04
Hep Bs Ab Qn	31.9 milli intl units/mL (High)	03/30/2026 11:04
Hep Bs Ab Qn Interp	Reactive (Abnormal)	03/30/2026 11:04
Hep C Ab	Non-Reactive (Normal)	03/30/2026 10:54

Infectious Disease

Flu A RNA	Negative (Normal)	03/30/2026 11:15
Flu B RNA	Negative (Normal)	03/30/2026 11:15

Other Chemistry

Procalcitonin Lvl	1.56 ng/mL (High)	03/30/2026 11:15

Other Microbiology

MRSA by PCR	Not Detected (Normal)	03/31/2026 01:48

SARS-CoV-2/Covid-19

SARS-CoV2 RNA	Negative (Normal)	03/30/2026 11:15

Thyroid

T3 Free	2.87 pg/mL (Normal)	03/30/2026 11:15
T4 Free	1.74 ng/dL (High)	03/30/2026 11:15
TSH Ultrasensitive	3.046 mc Intl units/mL (Normal)	03/30/2026 11:15

CBC, BMP, Coagulation Trend (last 4 resulted)

WBC	**5.87** 03/31/2026 02:04	**12.25 H** 03/30/2026 07:31		
Hgb	**8.4 L** 03/31/2026 02:04	**11.2** 03/30/2026 07:31		
Hct	**25.6 L** 03/31/2026 02:04	**33.7** 03/30/2026 07:31		

Consultation

Baso # Auto	**0.04** 03/31/2026 02:04	**0.03** 03/30/2026 07:31		
Baso % Auto	**0.7** 03/31/2026 02:04	**0.2** 03/30/2026 07:31		
Eos # Auto	**0.12** 03/31/2026 02:04	**0.02** 03/30/2026 07:31		
Eos % Auto	**2.0** 03/31/2026 02:04	**0.2** 03/30/2026 07:31		
Immature Grans %	**0.5** 03/31/2026 02:04	**0.6** 03/30/2026 07:31		
Immature Grans # Auto	**0.03** 03/31/2026 02:04	**0.07** 03/30/2026 07:31		
Lymph # Auto	**0.73 L** 03/31/2026 02:04	**1.33** 03/30/2026 07:31		
Lymph % Auto	**12.4** 03/31/2026 02:04	**10.9** 03/30/2026 07:31		
Mono # Auto	**0.22** 03/31/2026 02:04	**0.47** 03/30/2026 07:31		
Mono % Auto	**3.7** 03/31/2026 02:04	**3.8** 03/30/2026 07:31		
Neut # Auto	**4.73** 03/31/2026 02:04	**10.33 H** 03/30/2026 07:31		
Neut % Auto	**80.7** 03/31/2026 02:04	**84.3** 03/30/2026 07:31		
Plt	**141** 03/31/2026 02:04	**230** 03/30/2026 07:31		
Glucose Level	**72 L** 03/31/2026 02:04	**189 H** 03/30/2026 07:31		
Sodium	**140** 03/31/2026 02:04	**139** 03/30/2026 07:31		

Consultation

Potassium	**4.2** 03/31/2026 02:04	**5.9 H** 03/30/2026 07:31		
Chloride	**100** 03/31/2026 02:04	**94 L** 03/30/2026 07:31		
CO2	**31** 03/31/2026 02:04	**27** 03/30/2026 07:31		
Creatinine	**2.19 H** 03/31/2026 02:04	**4.80 H** 03/30/2026 07:31		
BUN	**17** 03/31/2026 02:04	**44 H** 03/30/2026 07:31		
BUN/Creat Ratio	**8** 03/31/2026 02:04	**9** 03/30/2026 07:31		
Mg Lvl	**1.9** 03/31/2026 02:04			
Phosphorus	**5.6 H** 03/31/2026 02:04	**10.5 H** 03/30/2026 13:08		
Calcium	**9.0** 03/31/2026 02:04	**8.6 L** 03/30/2026 07:31		
Albumin. Level	**3.4** 03/31/2026 02:04	**3.6** 03/30/2026 22:34	**3.4** 03/30/2026 11:15	
T Bili	**0.8** 03/30/2026 22:34	**0.4** 03/30/2026 11:15		
D Bili	**0.2** 03/30/2026 22:34	**0.2** 03/30/2026 11:15		
Alk Phos	**125 H** 03/30/2026 22:34	**150 H** 03/30/2026 11:15		
AST	**97 H** 03/30/2026 22:34	**57 H** 03/30/2026 11:15		
ALT	**49** 03/30/2026 22:34	**51** 03/30/2026 11:15		

Consultation

LFT (Last Within 24hrs)
AST: 97 units/L High (22:34)
ALT: 49 units/L (22:34)
Alk Phos: 125 units/L High (22:34)
T Bili: 0.8 mg/dL (22:34)
D Bili: 0.2 mg/dL (22:34)
TP: 6.4 gm/dL (22:34)

Microbiology

Culture Blood ARD		**In-Lab**	
Source:	Blood	Body Site:	
Collected Dt/Tm:	03/31/2026 02:12	Last Updated Dt/Tm:	03/31/2026 00:41

Culture Blood ARD		**In-Lab**	
Source:	Blood	Body Site:	
Collected Dt/Tm:	03/31/2026 02:04	Last Updated Dt/Tm:	03/31/2026 00:41

Culture Sputum		**Completed**	
Source:	Sputum Expectorated	Body Site:	
Collected Dt/Tm:	03/30/2026 22:34	Last Updated Dt/Tm:	03/31/2026 12:42

>10 epithelial cells /lpf
>25 White Blood Cells /lpf
Many gram positive cocci in pairs
Many Gram Negative Rods
Few Gram Negative Coccobacilli
Few Gram Positive Rods
Gram stain contains >=10 squamous epithelial cells per low power field which suggests oropharyngeal and/or nasopharyngeal contamination with no predominant bacterial morphology. Culture will not be performed. Notified: ALAJA RUIZ,RN 3/31/2026 12:27:14 PP

Culture Blood ARD		**Preliminary**	
Source:	Blood	Body Site:	Peripheral
Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	03/31/2026 09:02

No growth at 24 hours.

Culture Blood ARD		**Preliminary**	
Source:	Blood	Body Site:	Peripheral

Patient: VU, NANCY
MRN: VHM63892637
DOB/Sex: 7/12/1935 / Female
Attending: Techathaveewat DO,Worawith

Admit: 3/30/2026
Disch:
FIN: VHM1101330304

Consultation

Collected Dt/Tm:	03/30/2026 08:48	Last Updated Dt/Tm:	03/31/2026 10:02

Morganella morganii
Identification obtained by Sepsityper rapid identification method.
Culture confirmation to follow.
Susceptibility to follow.

Electronically Signed By: Joshi, Dhiresh MD
On: 03.31.2026 15:04 PDT

DOCUMENT NAME: Consult - Nephrology
SERVICE DATE/TIME: 3/30/2026 12:53 PDT
RESULT STATUS: Auth (Verified)
PERFORM INFORMATION: Vornicu MD,Maria D (3/30/2026 12:53 PDT)
SIGN INFORMATION: Vornicu MD,Maria D (3/30/2026 12:53 PDT)

Chief Complaint/Reason for Consultation

Patient brought in by EMS from home with complaints of SOB. Per the family, patient started to complain of SOB this morning. Patient found with a RA sat of <50%. Patient provided with BVM ventilation and then placed on NRB. Patient A/Ox0. EMS unable to obtain demographics on scene. BGL 119

History of Present Illness

End-stage renal disease on hemodialysis
90-year-old female with hypertension and dementia bedbound end-stage renal disease on hemodialysis Monday Wednesday Friday via IJ permacath admitted for shortness of breath
Patient is now on a nonrebreather mask
Labs in the emergency room show potassium high 5.9.
Chest x-ray shows Patchy airspace opacity in the right lower lobe suggestive of pneumonia.

Objective

Measurements (most recent)

Height (cm)	157.48 cm	Stasiewski RN, Sabryna	03/30 07:08
Height (inches)	62.00 inch	SYSTEM	03/30 07:08
Weight	40.82 kg	Stasiewski RN, Sabryna	03/30 07:08
Weight (lbs)	89.99 lb	SYSTEM	03/30 07:08
Body Mass Index	16.46 kg/m2	Stasiewski RN, Sabryna	03/30 07:08

Vital Signs (24 hrs)	**Last Charted**	**Minimum**	**Maximum**

Histories

Allergies

Allergies (Active and Proposed Allergies Only)
No Known Allergies (Severity: Unknown severity, Onset: Unknown)

Past Medical History

Active Problems(3)
Hemodialysis
Hepatitis A virus
High cholesterol

Past Surgical History

No surgery history documented.

Social History

No social history documented.

Past Psychiatric History

Past Psychiatric History
No psychiatric history documented.

Consultation

Temp	36.9	03/30/2026 07:11	36.9	03/30/2026 07:11	36.9	03/30/2026 07:11
Heart Rate Monitored	91	03/30/2026 12:20	84	03/30/2026 09:00	**H** 104	03/30/2026 07:30
Resp Rate	18	03/30/2026 12:20	12	03/30/2026 07:08	**H** 26	03/30/2026 07:38
SBP	**H** 179	03/30/2026 12:20	130	03/30/2026 07:08	**H** 179	03/30/2026 10:00
DBP	**H** 83	03/30/2026 12:20	65	03/30/2026 08:00	**C** 91	03/30/2026 07:08
MAP	119	03/30/2026 12:20	94	03/30/2026 08:00	119	03/30/2026 09:30
SpO2	95	03/30/2026 12:20	95	03/30/2026 08:00	99	03/30/2026 08:15
O2 Flow Rate	on 6 l/min		on 50 %		on 50 %	
O2 Therapy	Nasal cannula		BiPAP		BiPAP	

Ventilator Settings (Last Within 24hrs)

Ventilator Mode: PC-SIMV (07:37) FIO2: 50 % (11:30)
Ventilator Frequency, Mandatory: 14 br/min (07:37)
Pressure Support: 4 cm H2O (07:37)
Positive End Expiratory Pressure: 4 cm H2O (07:37)

Intake and Output (current encounter)

No Data Available

Physical Exam

Assessment/Plan
Assessment:
Physical Exam
General: Thin appearing elderly female
Mental Status: Baseline dementia
Head: Normocephalic, atraumatic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact. Anicteric.
Ear, Nose and Throat: Oropharynx clear, moist mucous membranes. Trachea midline.
Neck: No obvious neck masses
Respiratory: Clear to auscultation bilaterally. No wheezing, crackles or rhonchi. Normal respiratory effort.
Cardiovascular: Regular rate and rhythm, no murmurs, gallops or rubs.
Gastrointestinal: Non-tender, non-distended. Normal bowel sounds.
Genitourinary: No suprapubic tenderness.
Neurologic: Cranial nerves II-XII grossly intact. Moves all extremities.
Light touch sensation is grossly intact bilaterally.

End-stage renal disease on hemodialysis Monday Wednesday Friday via IJ permacath
Hyperkalemia
Hypertension

Family History
No Family History documented.

Medications
Home Medications
No medications documented.

Inpatient Medications
Medications (21) Active
Scheduled: (10)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 10 mg Tab (amLODIPine) 10 mg 1 Tabs, Oral, Daily
cefTRIAXone/D5W (Rocephin) 50 mL, IV Piggyback, q24H Interval
doxycycline/D5W (doxycycline) 100 mg 100 mL, IV Piggyback, q12H
heparin 5000 units/mL injection 1 mL (heparin) 5,000 units 1 mL, SubCutaneous, q12H
hydrALAZINE 50 mg Tab (hydrALAZINE) 50 mg 1 Tabs, Oral, TID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
metoprolol succinate 50 mg XL Tab (Metoprolol Succinate ER) 50 mg 1 Tabs, Oral, BID
sevelamer carbonate 800 mg Tab (sevelamer) 800 mg 1 Tabs, Oral, TID w/Meals
sodium citrate 4% soln 5 mL (sodium citrate) 200 mg 5 mL, IntraCatheter, On Call
Continuous: (0)
PRN: (11)
acetaminophen 325 mg Tab (Tylenol) 650 mg 2 Tabs, Oral, q4H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q3H
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID
Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 25 gm 50 mL, IV Push, q1H Interval
docusate sodium 100 mg Cap (Colace) 100 mg 1 Caps, Oral, BID
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H
glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE)

Consultation

Shortness of breath
Pneumonia

Plan
Hemodialysis stat given hyperkalemia and shortness of breath
Monitor daily for dialysis needs
Potassium should improve postdialysis
Continue amlodipine hydralazine metoprolol
Continue sevelamer. Monitor phosphorus level
Antibiotics. Follow-up cultures

Discharge Planning:

10 mg 0.5 mL, IV Push, q4H
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q4H
melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H

72 hour Antibiotic History

Completed/Discontinued Antibiotics	Stop Date/Time	Last Administered	First Administered
azithromycin 500 mg, 250 mL, 250 mL/hr, IV Piggyback, Once	03/30/2026 10:10	03/30/2026 10:09	03/30/2026 10:09
cefTRIAXone 1 gm, 50 mL, 100 mL/hr, IV Piggyback, Once	03/30/2026 09:18	03/30/2026 09:10	03/30/2026 09:10

Results
Recent Labs
Blood Gases

Art Na	137 mmol/L (Normal)	03/30/2026 08:43
Art K	6.3 mmol/L (Critical)	03/30/2026 08:43
Art iCa	0.96 mmol/L (Low)	03/30/2026 08:43
pH Arterial	7.54 (High)	03/30/2026 08:43
Art pCO2	39.4 mmHg (Normal)	03/30/2026 08:43
Art pO2	153.0 mmHg (High)	03/30/2026 08:43
HCO3 Art	34.4 mmol/L (High)	03/30/2026 08:43
Arterial pCO2(T)	39.4 mmHg (N/A)	03/30/2026 08:43
Arterial pH(T)	7.54 (N/A)	03/30/2026 08:43
Arterial pO2(T)	153.0 mmHg (N/A)	03/30/2026 08:43
Art Base Excess	10.6 mmol/L (High)	03/30/2026 08:43
Art sO2	99 % (High)	03/30/2026 08:43
Art tHB	10.3 gm/dL (Low)	03/30/2026 08:43

Consultation

Art Hct	32 % (Low)	03/30/2026 08:43
POC Pt Temp	37.0 DegC (N/A)	03/30/2026 08:43
POC FIO2	50 % (N/A)	03/30/2026 08:43
POC Sample	Arterial (N/A)	03/30/2026 08:43
POC Site	Radial Right (N/A)	03/30/2026 08:43
POC Allen Test	POS (N/A)	03/30/2026 08:43
POC Del Sys	BIPAP (N/A)	03/30/2026 08:43
POC Vent Mode	NO VENT (N/A)	03/30/2026 08:43
POC Set RR	14 br/min (N/A)	03/30/2026 08:43
POC IPAP	8 cm H2O (N/A)	03/30/2026 08:43
POC EPAP	4 cm H2O (N/A)	03/30/2026 08:43
Notified To and RBV	DR DAVIDSON (N/A)	03/30/2026 08:43
Notified By	vhsmurrat (N/A)	03/30/2026 08:43
Notified Note	ALL CV WITH RB (N/A)	03/30/2026 08:43

Cardiac

BNP	2004 pg/mL (High)	03/30/2026 07:31
Troponin TNIH	238.44 ng/L (Critical)	03/30/2026 10:37

General Chemistry

Glucose Level	189 mg/dL (High)	03/30/2026 07:31
POC Glucose	102 mg/dL (Normal)	03/30/2026 09:34
Sodium	139 mmol/L (Normal)	03/30/2026 07:31
Potassium	5.9 mmol/L (High)	03/30/2026 07:31
Chloride	94 mmol/L (Low)	03/30/2026 07:31
CO2	27 mmol/L (Normal)	03/30/2026 07:31
Anion Gap	18 mmol/L (High)	03/30/2026 07:31
BUN	44 mg/dL (High)	03/30/2026 07:31
Creatinine	4.80 mg/dL (High)	03/30/2026 07:31
BUN/Creat Ratio	9 (Normal)	03/30/2026 07:31

Consultation

Calcium	8.6 mg/dL (Low)	03/30/2026 07:31
Albumin. Level	3.4 gm/dL (Normal)	03/30/2026 11:15
TP	6.3 gm/dL (Normal)	03/30/2026 11:15
A/G Ratio	1.2 (Normal)	03/30/2026 11:15
T Bili	0.4 mg/dL (Normal)	03/30/2026 11:15
D Bili	0.2 mg/dL (Normal)	03/30/2026 11:15
Alk Phos	150 units/L (High)	03/30/2026 11:15
AST	57 units/L (High)	03/30/2026 11:15
ALT	51 units/L (Normal)	03/30/2026 11:15
Estimated Creatinine Clearance	5.02 mL/min ()	03/30/2026 08:46
eGFR Cr	8 mL/min/1.73m2 (N/A)	03/30/2026 07:31
eGFR Pediatric	Not Reported (N/A)	03/30/2026 07:31
Lactic Acid Lvl	1.8 mmol/L (Normal)	03/30/2026 08:48
MAR Glucose Result	102 ()	03/30/2026 09:49

General Hematology

WBC	12.25 x10e3/mcL (High)	03/30/2026 07:31
RBC	4.86 x10e6/mcL (Normal)	03/30/2026 07:31
Hgb	11.2 gm/dL (Normal)	03/30/2026 07:31
Hct	33.7 % (Normal)	03/30/2026 07:31
MCV	69.3 Femtoliters (Low)	03/30/2026 07:31
MCH	23.0 pg (Low)	03/30/2026 07:31
MCHC	33.2 gm/dL (Normal)	03/30/2026 07:31
RDW-SD	44.7 Femtoliters (Normal)	03/30/2026 07:31
RDW-CV	18.6 % (High)	03/30/2026 07:31
Plt	230 x10e3/mcL (Normal)	03/30/2026 07:31
MPV	See comment Femtoliters (N/A)	03/30/2026 07:31
Imm Plt Fract	4.0 % (Normal)	03/30/2026 07:31
NRBC %	0.2 /100 WBC (Normal)	03/30/2026 07:31

Consultation

NRBC #	0.02 x10e3/mcL (High)	03/30/2026 07:31
Neut % Auto	84.3 % (N/A)	03/30/2026 07:31
Lymph % Auto	10.9 % (N/A)	03/30/2026 07:31
Mono % Auto	3.8 % (N/A)	03/30/2026 07:31
Eos % Auto	0.2 % (N/A)	03/30/2026 07:31
Baso % Auto	0.2 % (N/A)	03/30/2026 07:31
Immature Grans %	0.6 % (Normal)	03/30/2026 07:31
Neut # Auto	10.33 x10e3/mcL (High)	03/30/2026 07:31
Lymph # Auto	1.33 x10e3/mcL (Normal)	03/30/2026 07:31
Mono # Auto	0.47 x10e3/mcL (Normal)	03/30/2026 07:31
Eos # Auto	0.02 x10e3/mcL (Normal)	03/30/2026 07:31
Baso # Auto	0.03 x10e3/mcL (Normal)	03/30/2026 07:31
Immature Grans # Auto	0.07 x10e3/mcL (Normal)	03/30/2026 07:31
RBC Morph	Abnormal (Abnormal)	03/30/2026 07:31
Plt Morphology	Normal (Normal)	03/30/2026 07:31
Polychrom	1+ (Abnormal)	03/30/2026 07:31
Micro	2+ (Abnormal)	03/30/2026 07:31
Plt Est	Adequate (Normal)	03/30/2026 07:31
Schistocytes	1+ (Abnormal)	03/30/2026 07:31
Target Cells (Codocytes)	3+ (Abnormal)	03/30/2026 07:31
Teardrop Cells (Dacrocytes)	1+ (Abnormal)	03/30/2026 07:31

Hepatitis

Hep A IgM	Reactive (Abnormal)	03/30/2026 10:54
Hep B Core IgM	Non-Reactive (Normal)	03/30/2026 11:04
Hepatitis B Surface Antigen	Non-Reactive (Normal)	03/30/2026 11:04
Hep Bs Ab Qn	31.9 milli intl units/mL (High)	03/30/2026 11:04
Hep Bs Ab Qn Interp	Reactive (Abnormal)	03/30/2026 11:04
Hep C Ab	Non-Reactive (Normal)	03/30/2026 10:54

Consultation

Other Chemistry

Procalcitonin Lvl	1.56 ng/mL (High)	03/30/2026 11:15

Thyroid

T3 Free	2.87 pg/mL (Normal)	03/30/2026 11:15
T4 Free	1.74 ng/dL (High)	03/30/2026 11:15
TSH Ultrasensitive	3.046 mc Intl units/mL (Normal)	03/30/2026 11:15

BMP, Mg, and Phos (Last Within 24hrs)
Sodium: 139 mmol/L (07:31)
Potassium: 5.9 mmol/L High (07:31)
Chloride: 94 mmol/L Low (07:31)
CO2: 27 mmol/L (07:31)
BUN: 44 mg/dL High (07:31)
Creatinine: 4.8 mg/dL High (07:31)
Glucose Level: 189 mg/dL High (07:31)
Calcium: 8.6 mg/dL Low (07:31)

Electronically Signed By: Vornicu, Maria MD
On: 03.30.2026 12:53 PDT